Krill oil supplementation with and without resistance training lowers the burden of the metabolic syndrome but does not affect glucose tolerance in sedentary adults: a randomized controlled trial

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1. Introduction

This document includes supplementary data and computational details related to the paper "Krill oil supplementation with and without resistance training lowers the burden of the metabolic syndrome but does not affect glucose tolerance in sedentary adults: a randomized controlled trial".

1.1 Project organisation

This notebook is built from source code available at the github repository trainome/alpha-omega-trial **[TODO: confirm repository URL; the local git origin is github.com/dhammarstrom/alpha-omega-trial]**. The data used in analyses is organized as an R data package available in a separate repository trainome/AOData.

Files used to build this supplementary document, the figures and the methods and results section of the paper are listed in Table 1.1. Stan models are stored in `models/`; when model code is executed locally, compiled model executables are also stored here. Model fits are stored in `fits/paper1/` but are not pushed to GitHub due to their large size. Model fits are recreated by running the model scripts in `models-scripts/`.

Table 1.1: Files used to build the supplementary document, figures and the methods and results section of the paper.

File	Content
Book chapters	
`_quarto.yml`	Book configuration: chapters, authors, execution and output settings.
`index.qmd`	Introduction, project organisation and computational environment.
`01-data.qmd`	Analytic approach and description of the data set and participants (figures 1 and 2).
`02-fa.qmd`	Effects of krill oil supplementation on fatty acid composition (models m2 to m6).
`03-metsyn-intro.qmd`	Introduction to the metabolic syndrome analysis, including the drop-out analysis (model m1).
`04-metsyn-models.qmd`	Development of the metabolic syndrome models on synthetic data (models s1 to s7).
`05-metsyn-postpred.qmd`	Posterior predictive checks, model expansions and model comparison with LOO (models s7 to s18).
`06-metsyn-simulations.qmd`	Simulation study comparing the multivariate (s13) and univariate (s18) models.
`07-metsyn-inference.qmd`	Inference on the metabolic syndrome criteria, the joint burden score, diagnosis and moderation of treatment effects (models s13, s18 to s22).
`08-metsyn-sensitivity.qmd`	Sensitivity of conclusions to the model structure and to the scale of the burden score.
`09-ogtt.qmd`	Analysis of the oral glucose-tolerance test (models mo1 to mo4).
`10-results.qmd`	Book version of the methods and results sections of the paper.
Paper	
`_methods-results.qmd`	Shared body of the methods and results sections, included by `99-methods-results.qmd` and `10-results.qmd`.
`99-methods-results.qmd`	Methods and results section of the paper (rendered to Word).
R scripts	
`R/packages.R`	Loads required packages.
`R/data-prep.R`	Prepares data for all analyses, including `data_load_s7s10()` that creates the Stan data for the metabolic syndrome models.
`R/helpers.R`	Helper functions for model fitting (`fit_wrap()`, `make_inits()`) and post-processing of model output.
`R/fig-opt-fun.R`	Color scale, theme and other settings for data visualisations.
`R/sim-fun.R`	Functions for simulating data sets used in model development and simulation studies.
`R/negligible.R`	Thresholds for negligible effects (from `01-data.qmd`) and probabilities of benefit, negligible effect and harm from posterior draws.
`R/pop-ref.R`	Population reference values for the metabolic syndrome criteria, burden score weights and conversion factors to the population-SD burden score.
`R/model-comp.R`	Simulation study comparing models s7 to s10 on data with a shared residual SD across treatment arms (output in `data-sim/`).
`R/model-comp2.R`	As `R/model-comp.R` with treatment-specific residual SDs in the simulated data (output in `data-sim2/`).
`R/model-comp3.R`	Simulation study comparing s13 and s18 under different levels of missingness and correlation (output in `data-sim3/`).
Figures	
`figures/figure-01.R`	Figure 1, participant flow (CONSORT) diagram.
`figures/figure-02.R`	Figure 2, study design and training data.
`figures/figure-03.R`	Figures 3 and 4, omega-3 index and fatty acid composition with group contrasts.
`figures/figure-05.R`	Figure 5, OGTT, fasting glucose and HOMA2-IR.
`figures/figure-06.R`	Figure 6, effects of krill oil supplementation on the metabolic syndrome.

File	Content
Stan models	
models/m1.stan	Drop-out analysis, log-odds of drop-out per treatment group predicted from baseline characteristics.
models/m2.stan	Fatty acid composition, complete-case ANCOVA-style model.
models/m3.stan	Fatty acid composition, all available data with imputation of missing values.
models/m4.stan	As m3 with treatment-specific residual SD.
models/m5.stan	As m3 with time-specific residual SD.
models/m6.stan	As m3 with time- and treatment-specific residual SD.
models/mo1.stan	OGTT, multilevel spline model at baseline.
models/mo2.stan	OGTT, multilevel spline model for three time-points.
models/mo3.stan	As mo2 with imputation of missing values.
models/mo4.stan	As mo3 with participant-level intercepts per time-point.
models/s1.stan	Metabolic syndrome criteria, separate models per criterion on complete cases.
models/s2.stan	Joint multivariate normal model of the criteria.
models/s3.stan	As s2 with imputation of missing values.
models/s4.stan	As s3 with sex as a predictor.
models/s5.stan	As s4 with both follow-up time-points, without correlations between time-points.
models/s6.stan	As s5 with a single correlation structure across types and time-points.
models/s7.stan	Univariate normal model with imputation of missing values.
models/s8.stan	Joint multivariate normal model with correlation across types and time-points.
models/s9.stan	As s7 with treatment-specific residual SD.
models/s10.stan	As s8 with treatment-specific residual SD.
models/s11.stan	As s9 with a Student-t likelihood.
models/s12.stan	As s10 with a multivariate Student-t likelihood.
models/s13.stan	As s12 with residual scales per time and type; the model used for inference.
models/s14.stan	As s10 with a latent-factor correlation structure.
models/s15.stan	As s12 with a latent-factor correlation structure.
models/s16.stan	As s14 with residual scales per time and type.
models/s17.stan	As s15 with residual scales per time and type.
models/s18.stan	Univariate Student-t counterpart of s13.
models/s19.stan	As s13 with a treatment × own-baseline interaction.
models/s20.stan	As s13 with a treatment × baseline metabolic burden interaction.
models/s21.stan	As s13 with a treatment × baseline relative fat mass interaction.
models/s22.stan	As s13 with a treatment × baseline omega-3 index interaction.
Model fitting	
models-scripts/model-<model>.R	Fits a model to the trial data and saves the fit (and LOO) in fits/paper1/, e.g. model-s13.R. -D6 scripts fit s14 to s17 with six latent factors.
models-scripts/refit-all.sh	Refits all models fitted to the trial data.
models-scripts/refit-s7-s20.sh	Refits models s7 to s20.
models-scripts/refit-s13-s22.sh	Refits models s13, s18, s21 and s22.
models-scripts/refit-s14-s17-d6.sh	Refits models s14 to s17 with six latent factors.
fits/paper1/	Saved model fits and LOO objects (not on GitHub).
Derived data	
data-derived/chain-diagnostics-s7-s10.RDS, data-derived/chain-diagnostics-s11-s12.RDS	Chain diagnostics of models s7 to s12 (chapter 5).
data-derived/ch-07-diag-crit-rubin-s13.RDS, data-derived/ch-07-diag-crit-rubin-s18.RDS	Analyses of metabolic syndrome diagnosis in 500 imputed data sets (posterior draws) from s13 and s18, combined with Rubin's rules (chapter 7, figure 6 and paper).
data-derived/ch-07-chain-diagnostics-s19-s22.RDS	Chain diagnostics of the moderation models (chapter 7).
data-derived/ch-07-moderation-*.RDS	Summaries of moderation parameters, predictions and burden score moderation (chapter 7).
data-derived/ms-s13-alpha-beta-draws.RDS, data-derived/ms-s19-s20-draws.RDS, data-derived/ogtt-mo4-pop-draws.RDS	Cached posterior draws from models s13, s19, s20 and mo4, used in the paper (re-created when older than the fits).
data-sim3/	Output from the simulation study in R/model-comp3.R (chapter 6).
Resources	
resources/bibliography.bib	References.
resources/typst-show.typ	Typst template partial adding authors and affiliations to the PDF.
renv.lock	R package versions.

1.2 Packages and computational environment

Stan (CmdStan) version 2.39.0 was used in compiling this notebook. R packages are listed below together with additional session info.

```
sessionInfo()
```

```
R version 4.6.0 (2026-04-24 ucrt)
Platform: x86_64-w64-mingw32/x64
Running under: Windows 11 x64 (build 26300)

Matrix products: default
```

```
  LAPACK version 3.12.1

locale:
[1] LC_COLLATE=Norwegian Bokmål_Norway.utf8
[2] LC_CTYPE=Norwegian Bokmål_Norway.utf8
[3] LC_MONETARY=Norwegian Bokmål_Norway.utf8
[4] LC_NUMERIC=C
[5] LC_TIME=Norwegian Bokmål_Norway.utf8

time zone: Europe/Oslo
tzcode source: internal

attached base packages:
[1] stats     graphics  grDevices utils     datasets  methods   base

other attached packages:
 [1] Gmisc_3.3.0       htmlTable_2.5.0   corrr_0.4.5       loo_2.9.0
 [5] posterior_1.7.0   cmdstanr_0.9.0    ggh4x_0.3.1       gt_1.3.0
 [9] patchwork_1.3.2   ggrepel_0.9.8     ggtext_0.1.2      lubridate_1.9.5
[13] forcats_1.0.1     stringr_1.6.0     dplyr_1.2.1       purrr_1.2.2
[17] readr_2.2.0       tidyr_1.3.2       tibble_3.3.1      ggplot2_4.0.3
[21] tidyverse_2.0.0   AOData_0.0.1.9000

loaded via a namespace (and not attached):
 [1] gtable_0.3.6         tensorA_0.36.2.1     xfun_0.57
 [4] htmlwidgets_1.6.4    processx_3.9.0       lattice_0.22-9
 [7] tzdb_0.5.0           vctrs_0.7.3          tools_4.6.0
[10] generics_0.1.4       parallel_4.6.0       cluster_2.1.8.2
[13] pkgconfig_2.0.3      data.table_1.18.4    checkmate_2.3.4
[16] RColorBrewer_1.1-3   S7_0.2.2             distributional_0.7.0
[19] lifecycle_1.0.5      compiler_4.6.0       farver_2.1.2
[22] textshaping_1.0.5    forestplot_3.2.0     htmltools_0.5.9
[25] yaml_2.3.12          Formula_1.2-5        pillar_1.11.1
[28] Hmisc_5.2-5          rpart_4.1.27         abind_1.4-8
[31] tidyselect_1.2.1     digest_0.6.39        stringi_1.8.7
[34] fastmap_1.2.0        grid_4.6.0           colorspace_2.1-2
[37] cli_3.6.6            magrittr_2.0.5       base64enc_0.1-6
[40] XML_3.99-0.23        foreign_0.8-91       withr_3.0.2
[43] scales_1.4.0         backports_1.5.1      timechange_0.4.0
[46] rmarkdown_2.31       matrixStats_1.5.0    nnet_7.3-20
[49] otel_0.2.0           gridExtra_2.3        ragg_1.5.2
[52] hms_1.1.4            evaluate_1.0.5       knitr_1.51
[55] rlang_1.2.0          gridtext_0.1.6       Rcpp_1.1.1-1.1
[58] glue_1.8.1           xml2_1.5.2           rstudioapi_0.18.0
[61] jsonlite_2.0.0       R6_2.6.1             systemfonts_1.3.2
[64] fs_2.1.0
```

2. Analytic approach, data description and study design

2.1 Data analysis

For inferential statistics, Bayesian models were constructed using the Stan probabilistic programming language (1) and fitted using the `cmdstanr` interface (2) in R (3). The choice of a Bayesian modeling framework allows us to naturally handle missing data in a wide class of models. This was important as it enabled us to model data from the project with the uncertainty associated with missing values incorporated into the posterior estimates.

A general workflow was adopted where models were incrementally built and tested for validity through simulations (4). First, implications of prior distributions were evaluated using prior-predictive simulations. Priors were selected to constrain model predictions and parameter values to *a priori* plausible ranges. This approach encourages the use of weakly informative priors on model parameters, which regularize models and shrink estimates and predictions without distorting the influence of the data on inference (5–7). In relevant cases, synthetic data were used to confirm that model inferences were not biased by model misspecification or coding errors (4). Samples from the joint posterior distribution were used to create posterior predictive checks, in which model implications are simulated and compared with characteristics of the observed data (4,7). Selected models were also assessed for inferential capabilities using synthetic data simulations where parameter values are known. The sensitivity of model specifications to inference was assessed by comparing predictions across different models.

All sampling from prior and posterior distributions were done using the Hamiltonian Monte Carlo (HMC) no-U-turn sampler (Stan Reference Manual: MCMC sampling). Performance of the HMC algorithm was assessed through chain convergence statistics (R-hat) and visually inspecting converging and stationary sampling across multiple HMC chains.

2.1.1 Presenting data and inference

Descriptive data are typically presented as mean and standard deviation (SD), unless otherwise noted. When presenting inferential statistics we aim to present results using the full posterior distributions or corresponding median point estimates with percentile intervals (50 and 95 % PI), and probability statements derived from full posterior distributions. Probability statements used in the text will be based on probability of benefit or harm ($|\text{effect}| > \delta$) and probability of a negligible effect ($|\text{effect}| \leq \delta$), where δ is the threshold for a negligible effect. We define δ as $0.1 \times$ SD where SD is the between individuals standard deviation at baseline or, when available, a corresponding population-based estimate (see below). Additionally we will report probability of *any* benefit as the probability mass > 0 in the direction of a favorable effect in the krill oil group.

The approach described above outlines how we describe the posterior distribution, but does not contain a decisions rule about how to interpret the estimates in light of a null-hypothesis of no effect of treatment. We will therefore also present estimates in relation to a region of practical equivalence (ROPE) (8), defined as $0 \pm \delta$. The ROPE procedure suggests that we *accept* the null hypothesis if the whole 95% highest density interval[1] (HDI) lies inside the ROPE, *reject* the null hypothesis if the whole 95% HDI lies outside the ROPE, and otherwise remain undecided (Figure 2.1).

[1] "The narrowest interval containing the specified probability mass" (6).

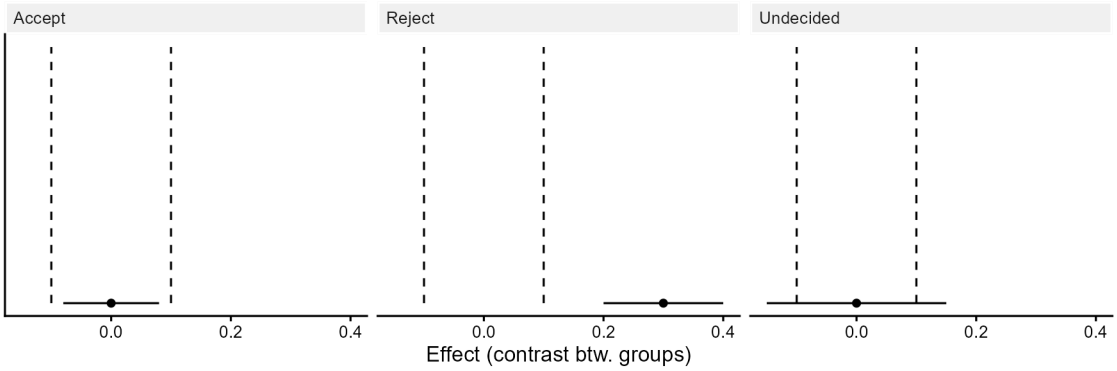

Figure 2.1: Example of ROPE decisions. Accepting the null-hypothesis is based on a 95% HDI inside the ROPE region (vertical lines), rejecting is done after observing the full 95% HDI outside the ROPE region, and an undecided result comes from a 95% HDI overlapping with any of the ROPE boundaries.

Given our sample size (148 participants), and a set of plausible baseline to follow-up correlations, accepting the null-hypothesis will in many cases be difficult because the sample size will likely be too small to achieve a precision high enough to show a full 95% HDI inside the pre-specified ROPE (Figure 2.2). We acknowledge this and accept that this decision rule, in many cases, will yield an undecided result.

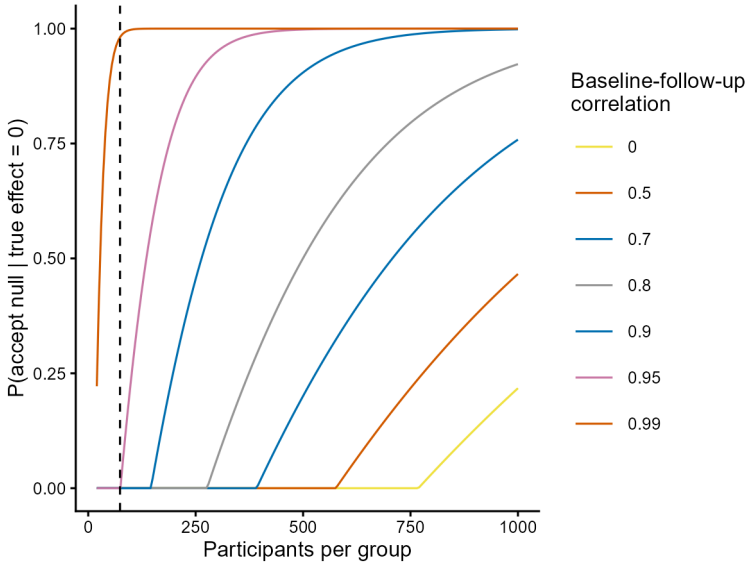

Figure 2.2: The probability of accepting the null-hypothesis using the ROPE procedure when the true effect is 0. Curves show the probability that the 95% interval of the baseline-adjusted difference between two groups lies entirely inside the ROPE (±δ, δ = 0.1 SD; effects in SD units) as a function of the number of participants per group, for different baseline to follow-up correlations. The dashed vertical line marks the number of participants randomized per group (n = 74).

2.2 Description of the data set and participants

The overall objective of the present analysis is to determine the effect of krill oil supplementation in combination with resistance training on glucose metabolism and indicators of metabolic health. Outcome variables in the analysis are:

- Whole blood fatty acid composition, including omega-3 and omega-6 fatty acids and the omega-3 index
- Blood glucose in response to an oral glucose tolerance-test (main outcome)
- Metabolic syndrome (derived from sub-scores)
  - Triglycerides

  - Waist circumference
  - Blood pressure
  - HDL Cholesterol
  - Fasting glucose
- Body mass index
- Relative fat mass
- HOMA2-IR
- Adiponectin
- LDL Cholesterol
- HbA1c

2.2.1 Determining thresholds for negligible effects

For the main outcome, the oral glucose tolerance test (OGTT; registered at ClinicalTrials.gov, NCT04279951), we will use our observed baseline SD, pooled within OGTT sampling time-point (0 to 120 min), for the full OGTT as the scale for δ (SD = 1.66 mmol/L, δ = 0.17 mmol/L). As population-based estimates of the full glucose curve are, to our knowledge, not available, we compared the variability in our sample to the Danish population-based cohort using the incremental area under the curve (iAUC) (9). The Danish study (Inter99) sampled glucose at 0, 30 and 120 min only. When our data were restricted to the same sampling scheme, the between-individual SD of the iAUC was comparable to Inter99 (114 vs. 135 mmol/L × min). Using all seven samples, the iAUC was larger and more variable (327 ± 150 mmol/L × min). This was because 68% of our participants reached peak glucose after 30 min, and a 0, 30 and 120 min scheme does not capture this peak (Table 2.1). Population-based estimates from sparsely sampled OGTTs would therefore underestimate the between-individual variability in the full glucose curve. This supports using our own baseline data as the scale for δ.

Table 2.1: Incremental area under the glucose curve (iAUC, mmol/L × min, mean and SD) at baseline in the present study, using all samples or the Inter99 sampling scheme, and in the Inter99 cohort (Table 2 in Holmkvist J, Banasik K, Andersen G, Unoki H, Jensen TS, Pisinger C, et al. (9), genotype groups combined).

	n	Mean	SD
Present study, 0-120 min (7 samples)	146	327	150
Present study, 0, 30, 120 min	146	298	114
Inter99, 0, 30, 120 min	5597	221	135

For the metabolic syndrome criteria, we used population-based estimates of the between-individual SD from three Nordic cohorts: the seventh survey of the Tromsø Study (2015–2016) (10), the FinHealth 2017 survey (11) and the SAMINOR 2 Clinical Survey (2012–2014) (12). When a study reported only sex- or ethnicity-specific estimates, we combined the groups into one overall SD that includes both within- and between-group variance. Triglycerides were modeled on the log scale, so the corresponding SDs were approximated on the log scale under a log-normal distribution, from the mean and SD (FinHealth) or the median and IQR (SAMINOR 2). Available population estimates were pooled across studies, weighted by sample size (Table 2.2).

Population-based and present-study SDs were similar for waist circumference and diastolic blood pressure. The population SDs were about 20% larger for systolic blood pressure and triglycerides, and about 50% larger for HDL cholesterol (Table 2.2). This is consistent with our participants being a more homogeneous sample than the general population, for example with fewer users of antihypertensive or lipid-lowering medication. Fasting glucose was the exception: the FinHealth SD (1.3 mmol/L) was more than twice ours (0.54 mmol/L). FinHealth included participants with diabetes and required only 4 h of fasting, and SAMINOR 2 used non-fasting samples. We therefore judged that no comparable population estimate was available for fasting glucose and used our own baseline SD. For all other criteria, δ was set to 0.1 × the pooled population SD.

Table 2.2: Between-individual SDs for the metabolic syndrome criteria at baseline in the present study and in population-based cohorts, and the resulting thresholds for negligible effects (δ = 0.1 × SD). Triglyceride SDs and δ are on the log scale. Population SDs were pooled across studies, weighted by sample size. For fasting glucose, δ is based on the present-study SD (see text).

	Present study SD	Population-based SD				δ
		Tromsø 7	FinHealth 2017	SAMINOR 2	Pooled	
Waist circumference (cm)	14	-	14	12	13	1.3
Systolic blood pressure (mm Hg)	16	18	19	18	19	1.9
Diastolic blood pressure (mm Hg)	11	10	11	9.7	10	1
HDL cholesterol (mmol/L)	0.31	0.5	0.4	0.43	0.47	0.047
Triglycerides (log mmol/L)	0.44	-	0.59	0.49	0.54	0.054
Fasting glucose (mmol/L)	0.54	-	1.3	-	1.3	0.054

For the related outcomes (body mass index, relative fat mass, HOMA2-IR, adiponectin, LDL cholesterol and HbA1c), the same cohorts provided population-based estimates for body mass index (FinHealth 2017 and Inter99) (9,11), LDL cholesterol and HbA1c (Tromsø 7) (10) (Table 2.3). HbA1c was reported in % (DCCT units) in the Tromsø Study and was converted to mmol/mol (IFCC units, mmol/mol = 10.929 × (% − 2.15)) before calculating a log-scale SD, as HbA1c was modelled on the log scale. The population SDs were similar to ours for body mass index and LDL cholesterol. For HbA1c the population SD was larger, likely because the Tromsø sample included participants with diabetes (3.6%) (10). Inter99 reported HOMA-IR based on the original HOMA model (9), which is not directly comparable to HOMA2-IR. No population-based estimates were available for relative fat mass or adiponectin. For these outcomes, δ was based on our baseline SD. For the remaining outcomes, δ was set to 0.1 × the pooled population SD.

Table 2.3: Between-individual SDs for related outcomes at baseline in the present study and in population-based cohorts, and the resulting thresholds for negligible effects (δ = 0.1 × SD). HOMA2-IR and HbA1c SDs and δ are on the log scale. Population SDs were pooled across studies, weighted by sample size. When no population estimate was available, δ is based on the present-study SD.

	Present study SD	Population-based SD				δ
		FinHealth 2017	Inter99	Tromsø 7	Pooled	
Body mass index (kg/m²)	5.4	5	4.6	-	4.8	0.48
Relative fat mass (%)	7.8	-	-	-	-	0.78
HOMA2-IR (log)	0.37	-	-	-	-	0.037
Adiponectin (mg/L)	6	-	-	-	-	0.6
LDL cholesterol (mmol/L)	0.88	-	-	1	1	0.1
HbA1c (log mmol/mol)	0.12	-	-	0.17	0.17	0.017

The metabolic syndrome burden score is the average of the standardized criteria (with HDL cholesterol reversed and systolic and diastolic blood pressure each given half weight). In the main analysis, each criterion was standardized using the sex-specific population-based references (fasting glucose: our own sex-specific baseline SD; see `R/pop-ref.R` and the sensitivity chapter), so that a treatment effect on the score is expressed in population SD units. The component thresholds do not carry over to the score directly. To keep the same definition as for the other outcomes, we set δ for the burden score to 0.1 × the between-individual SD of the score at baseline, calculated on the same scale (n = 141, SD = 0.55, δ = 0.055 population SD units). For the burden score standardized by the study SDs used in the model (sensitivity analysis), the corresponding values were SD = 0.66 and δ = 0.066 SD units. These thresholds are smaller than the effect on the score if every criterion changed by exactly its own δ in the same direction (0.104 population SD units and 0.118 model SD units, respectively), because the criteria are only partly correlated and their average varies less between individuals than each criterion alone.

2.3 Participants

At enrollment, participants were randomized into two groups, an active treatment-group (krill-oil, 4 g day^{-1}, providing 508 mg eicosapentaenoic acid [EPA] and 300 mg docosahexaenoic acid [DHA] per day) and a placebo control group (Placebo; Figure 2.3, Table 2.4).

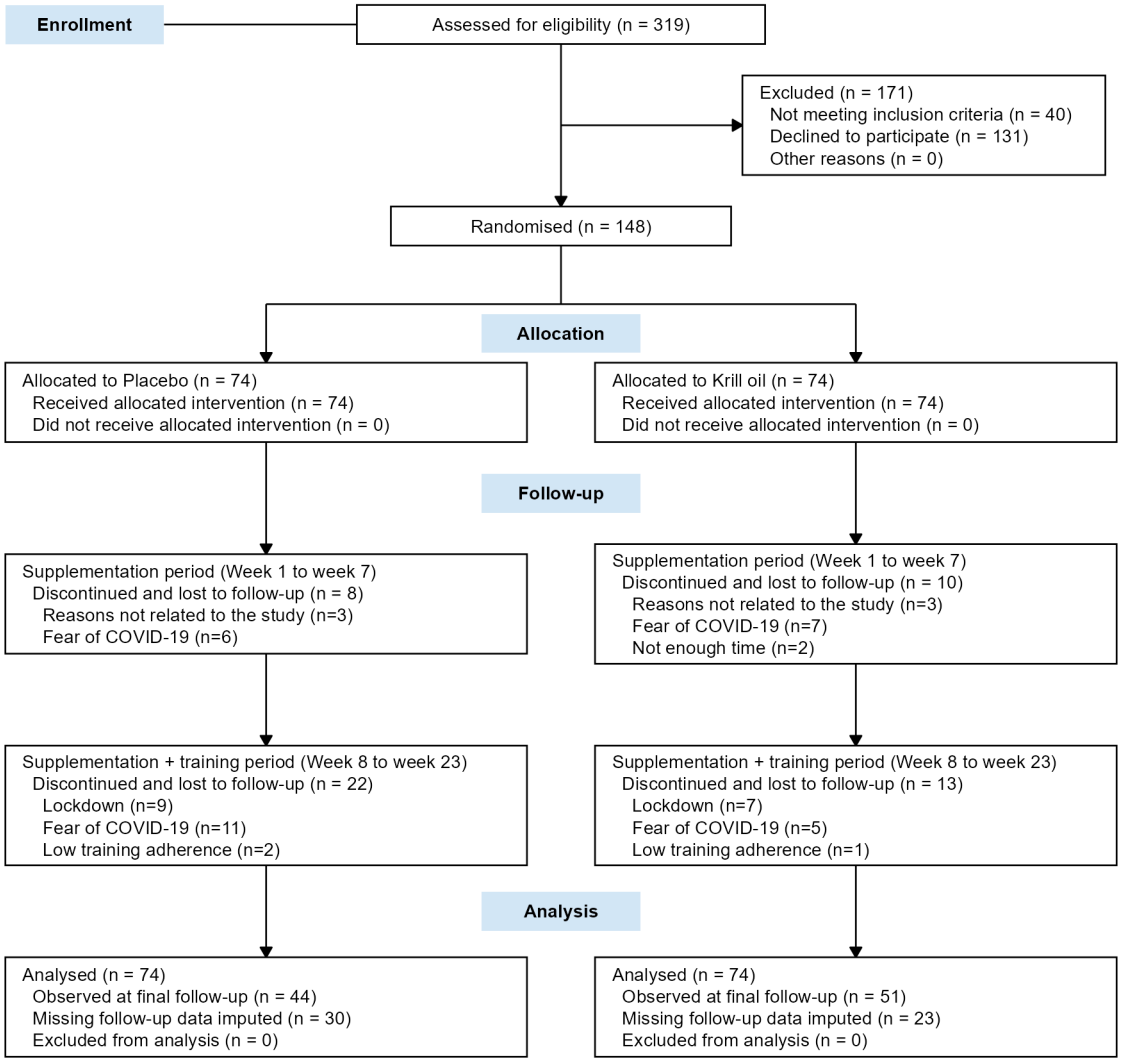

Figure 2.3: Flow of participants through the trial, presented according to the CONSORT 2025 statement. All randomized participants were included in the analysis of outcomes, with missing follow-up data imputed.

Table 2.4: Participant characteristics.

	Krill oil(♂ n = 32; ♀ n = 42)	Placebo(♂ n = 36; ♀ n = 38)
	Male	
Age (years)	47.3 (7.7) [29 - 60]	48.5 (5.8) [36 - 59]
Stature (cm)	180.5 (8.1) [153 - 193]	180.9 (6.3) [165 - 192]
Body mass (kg)	95.5 (17.1) [69 - 152]	97.8 (17.4) [71 - 142]
Body mass index (kg m^{-2})	29.4 (5.1) [20 - 45]	30.0 (6.0) [23 - 52]
	Female	
Age (years)	44.5 (8.1) [30 - 59]	46.6 (9.4) [23 - 60]
Stature (cm)	168.9 (6.0) [158 - 182]	167.6 (6.4) [152 - 180]
Body mass (kg)	83.6 (16.5) [58 - 136]	79.3 (14.2) [53 - 114]
Body mass index (kg m^{-2})	29.4 (5.8) [18 - 45]	28.2 (4.8) [20 - 40]

Data are presented as Mean (SD) [Min-Max]

Participants were given krill oil or placebo in a supplementation-only period followed by a lead-in and combined period (supplementation and resistance training; Figure 2.4).

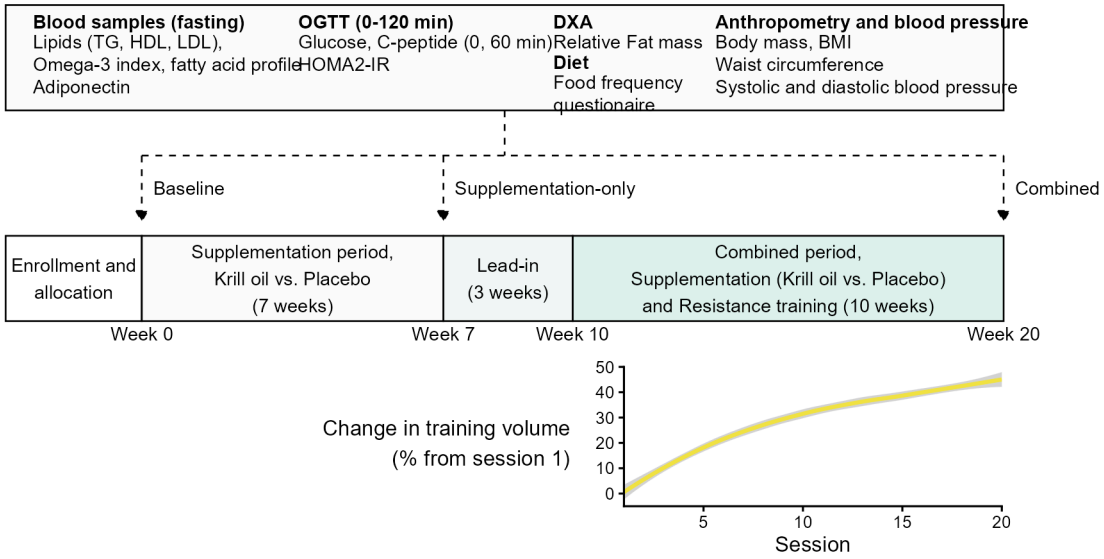

Figure 2.4: Study design. After baseline testing (week 0), participants received krill oil or placebo for 7 weeks, followed by 13 weeks of supervised resistance training, comprising a 3-week familiarization period (weeks 8–10) and 10 weeks of progressive resistance training (weeks 11–20), with supplementation continued throughout. The measurements listed were taken at each of the three time points: baseline (week 0), after supplementation alone (end of week 7) and after the combined period (end of week 20). The lower panel shows the change in training volume across the 20 training sessions, relative to session 1 and summed across all upper- and lower-body exercises. Line and shaded area show the smoothed trend with 95% confidence interval. BMI, body mass index; DXA, dual-energy X-ray absorptiometry; HDL, high-density lipoprotein cholesterol; HOMA2-IR, updated homeostasis model assessment of insulin resistance; LDL, low-density lipoprotein cholesterol; OGTT, oral glucose tolerance test; TG, triglycerides.

3. Effects of krill oil supplementation on whole blood fatty acids

Fatty acids, including EPA (C205n3) and DHA (C226n3), were measured as total fatty acids in frozen whole blood. We will assess the effect of krill oil supplementation on group level fatty acid profiles. As preliminary models indicated heteroscedasticity of residuals, log-transformed and standardized data was used in modelling. This allows for the same weakly regularizing priors for all fatty acids (FA). In addition to each fatty acid, the Omega 3 index (13) is included in these models as the EPA and DHA content relative to the total fatty acid content of whole blood. The omega 3 index is included in models from model `m3` (see below).

The models were designed to estimate differences between treatments (TX) at post-treatment time-points (T2 and T4) using pre-treatment ($\text{Pre}_{i[\text{FA}]}$) values as a covariate. The intercept in the model ($\alpha_{[\text{FA, TI, TX}]}$) gives us mean estimates for fatty-acid FA, time-point TI, and treatment TX.

$$
\begin{aligned}
y_{i[\text{FA}]} &\sim \text{Normal}\ (\mu_{i[\text{FA}]}, \sigma_{\text{FA}}) \\
\mu_{i[\text{FA}]} &= \alpha_{[\text{FA, TI, TX}]} + \beta_{\text{FA}} \times \text{Pre}_{i[\text{FA}]} \\
\alpha_{[\text{FA, TI, TX}]} &\sim \text{Normal}\ (0, 1) \\
\beta_{\text{FA}} &\sim \text{Normal}\ (1, 1) \\
\sigma_{\text{FA}} &\sim \text{Exponential}\ (1)
\end{aligned}
\tag{3.1}
$$

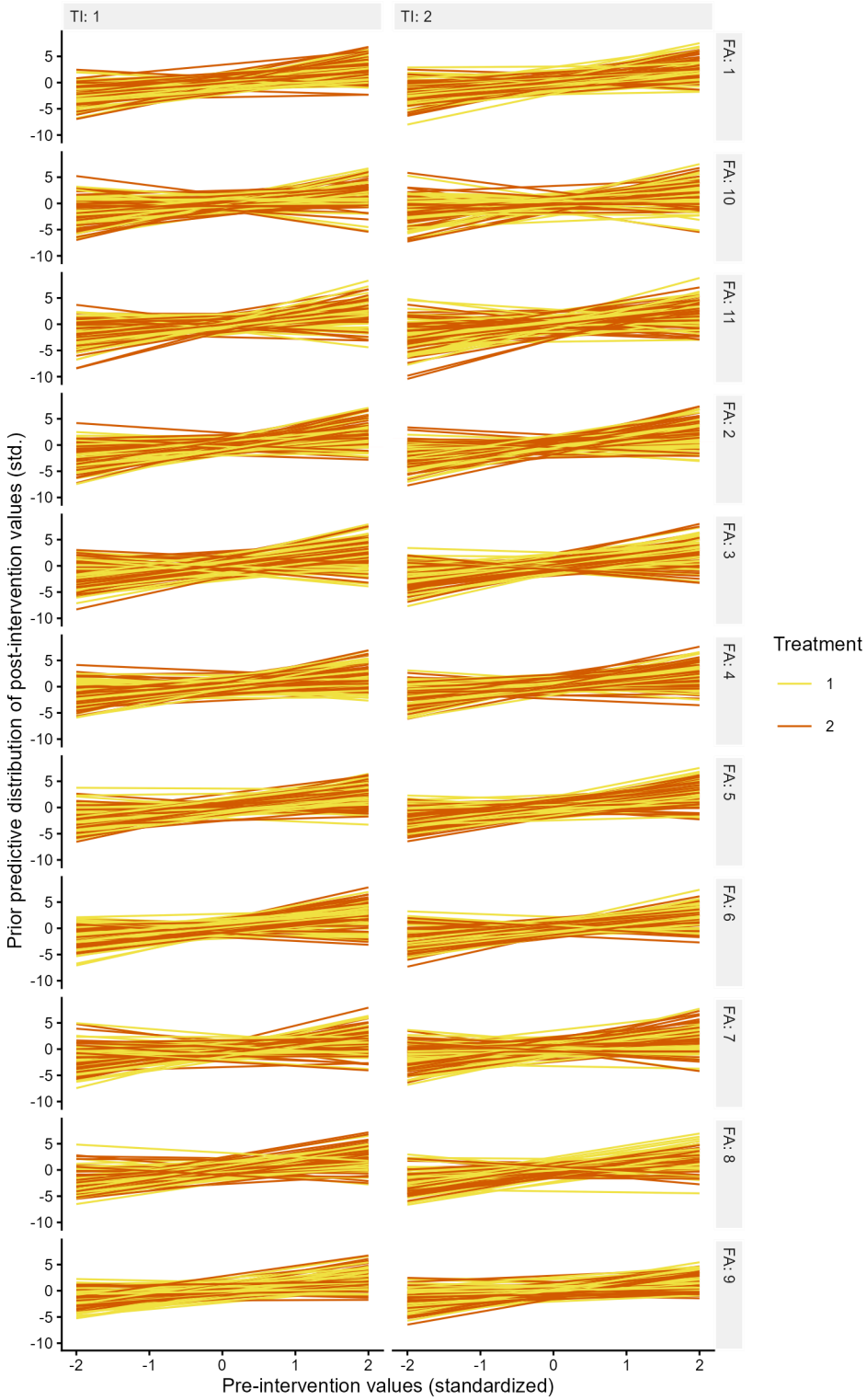

Figure 3.1: Prior predictive check for target-wise models of post-treatment fatty acid concentrations conditional on pre-treatment values. The model expects log-transformed standardized values and the priors suggest no difference between treatment groups and a proportional relationship between the pre-intervention values and post-intervention measurements.

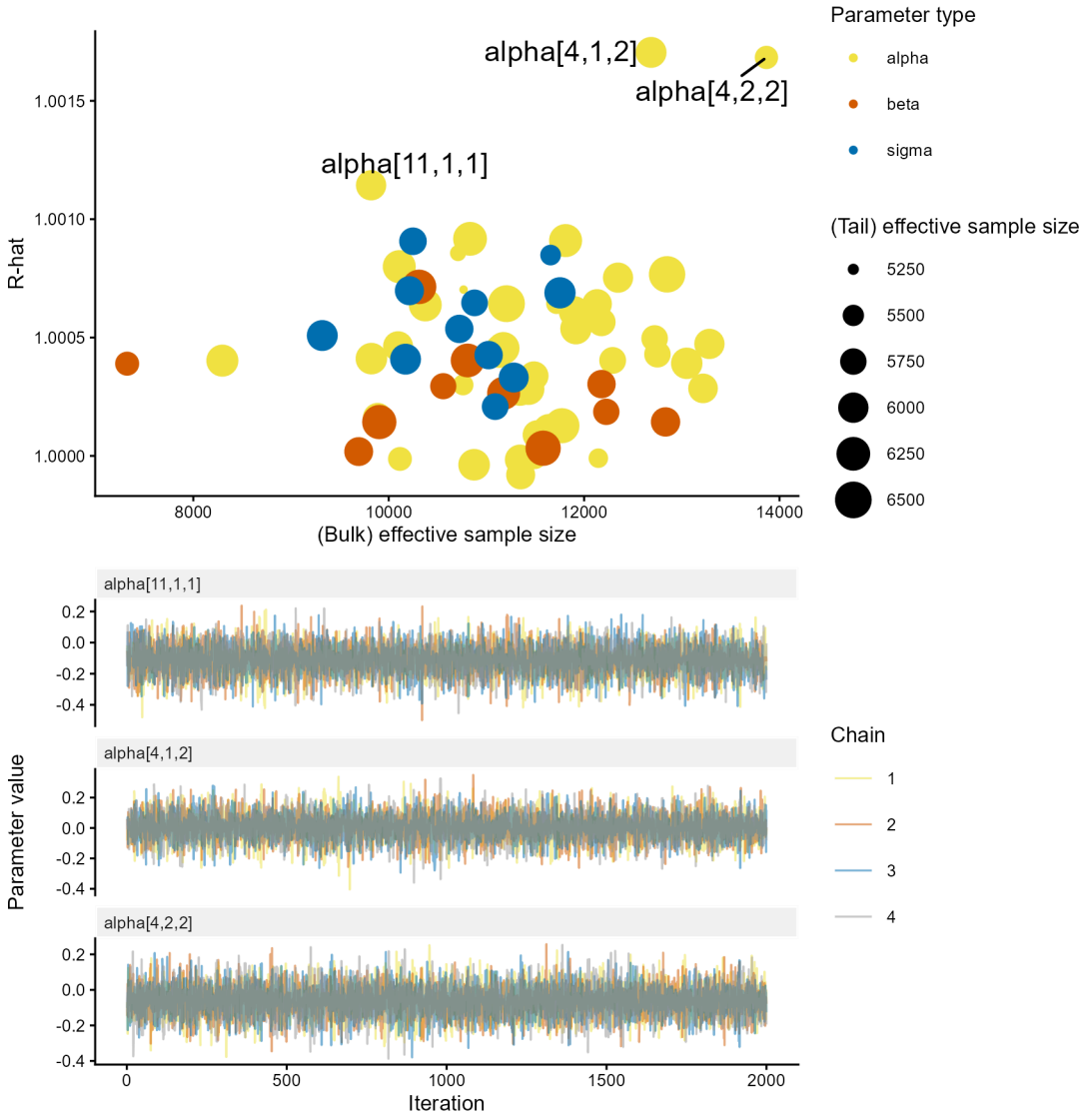

Figure 3.2: Model performance metrics from Model 2.

The first model uses only complete-cases. The prior predictive check confirms that the model expects eleven fatty acids, a positive relationship between pre-intervention values and post-treatment values, and no differences in treatments (Figure 3.1). Sampling from the posterior was non-problematic as R-hat values approached 1 and chains mixed and converged well (Figure 3.2).

Posterior predictive checks showed satisfactory patterns between replicated data (yrep; thin lines, one per draw in Figure 3.3) and observed data (thick lines in Figure 3.3).

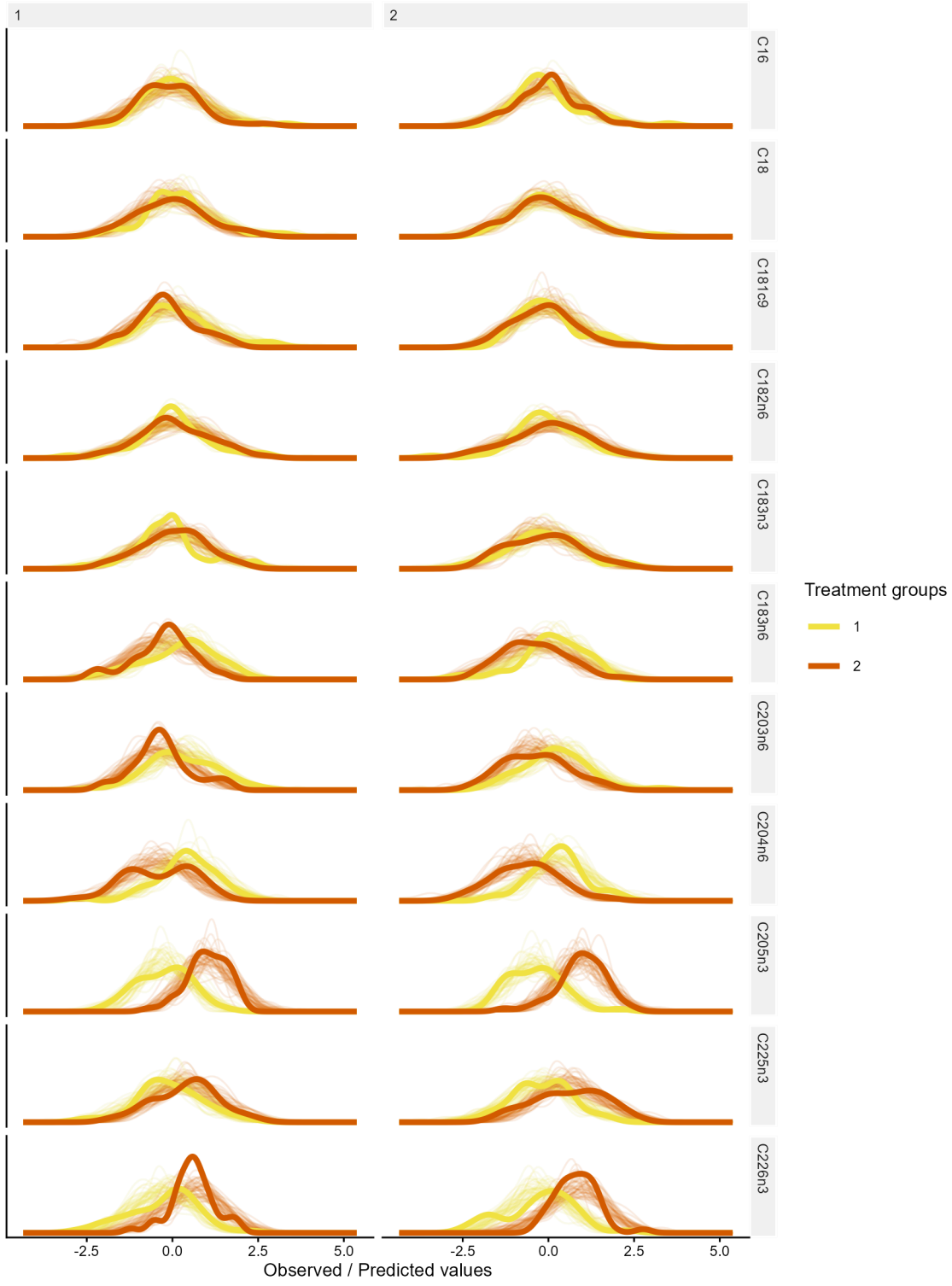

Figure 3.3: Posterior predictive check of Model 2.

Krill oil supplementation robustly increased whole blood fatty acid composition levels of C205n3, C225n3 and C226n3 fatty acids compared to placebo (Figure 3.4 and Figure 3.5). Post-treatment values were lower in the krill oil group for C181c9, C183n6, C203n6 and C204n6 with high probability at both T2 and T4 (Figure 3.5).

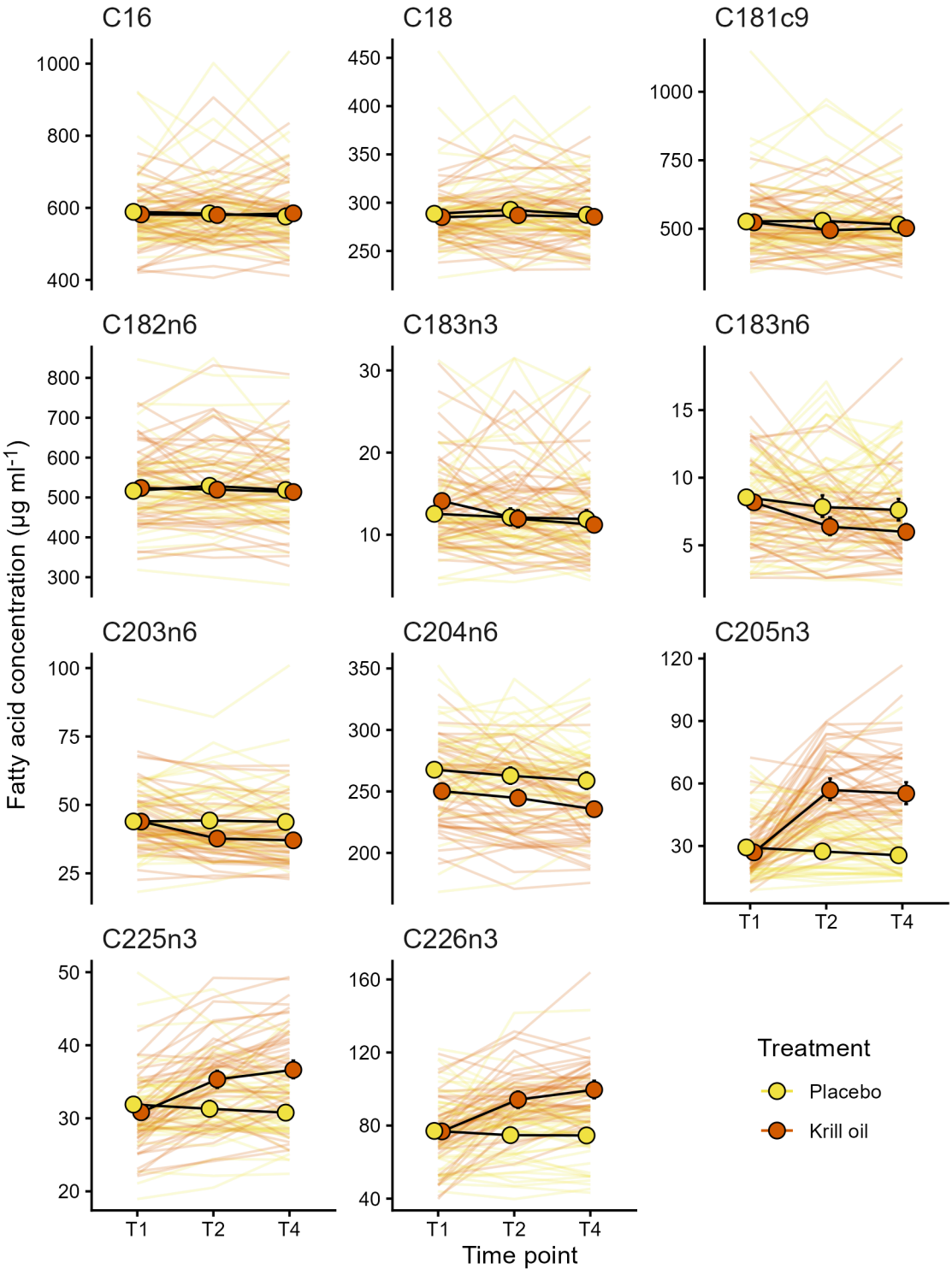

Figure 3.4: Predictions of fatty acid concentration in response to treatment. Pre-intervention values are means per treatment group, post-intervention values are posterior medians predicted from the model with 95% PI.

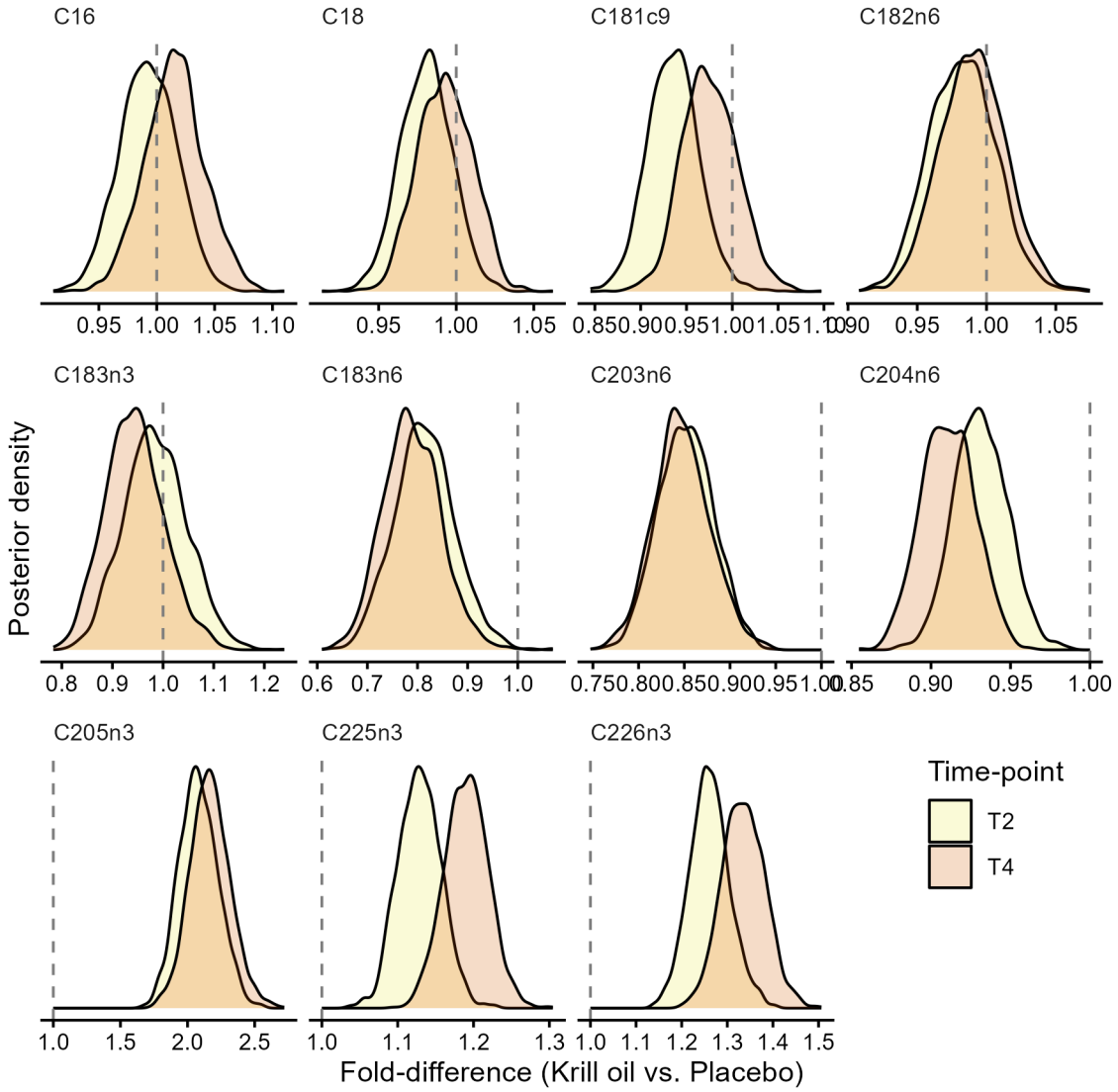

Figure 3.5: Posterior densities for differences between treatment groups in fatty acid concentrations.

Extending the first model with missing data imputations allows us to estimate the effect of treatment on fatty acid profiles with intention-to-treat (ITT) by averaging over the uncertainty associated with the missing values. Missing data imputation is done on post-treatment scores only with the total sample size being n = 144 with 14 and 36% of participants missing at time points T2 and T4, respectively.

The imputation mechanism replaces missing values with parameters estimated from observed data (pre-treatment values and treatment allocation) through the likelihood.

The imputation model is defined as:

$$
\begin{aligned}
y^{\text{all}}_{i[\text{FA}]} &\sim \text{Normal}\ (\mu_{i[\text{FA}]}, \sigma_{\text{FA}}) \\
\mu_{i[\text{FA}]} &= \alpha_{[\text{FA, TI, TX}]} + \beta_{\text{FA}} \times \text{Pre}_{i[\text{FA}]} \\
y^{\text{all}}_{i[\text{FA}]} &= \begin{cases} y^{\text{obs}}_{i[\text{FA}]} \text{ if observed} \\ y^{\text{mis}}_{i[\text{FA}]} \text{ if missing} \end{cases} \\
y^{\text{mis}}_{i[\text{FA}]} &\sim \text{Normal}\ (0, 2) \\
\alpha_{[\text{FA, TI, TX}]} &\sim \text{Normal}\ (0, 1) \\
\beta_{\text{FA}} &\sim \text{Normal}\ (1, 1) \\
\sigma_{\text{FA}} &\sim \text{Exponential}\ (1)
\end{aligned}
\tag{3.2}
$$

Prior predictive simulations confirm the general structure of the model (Figure 3.6). Model performance metrics from posterior sampling also indicates a functioning model (Figure 3.7).

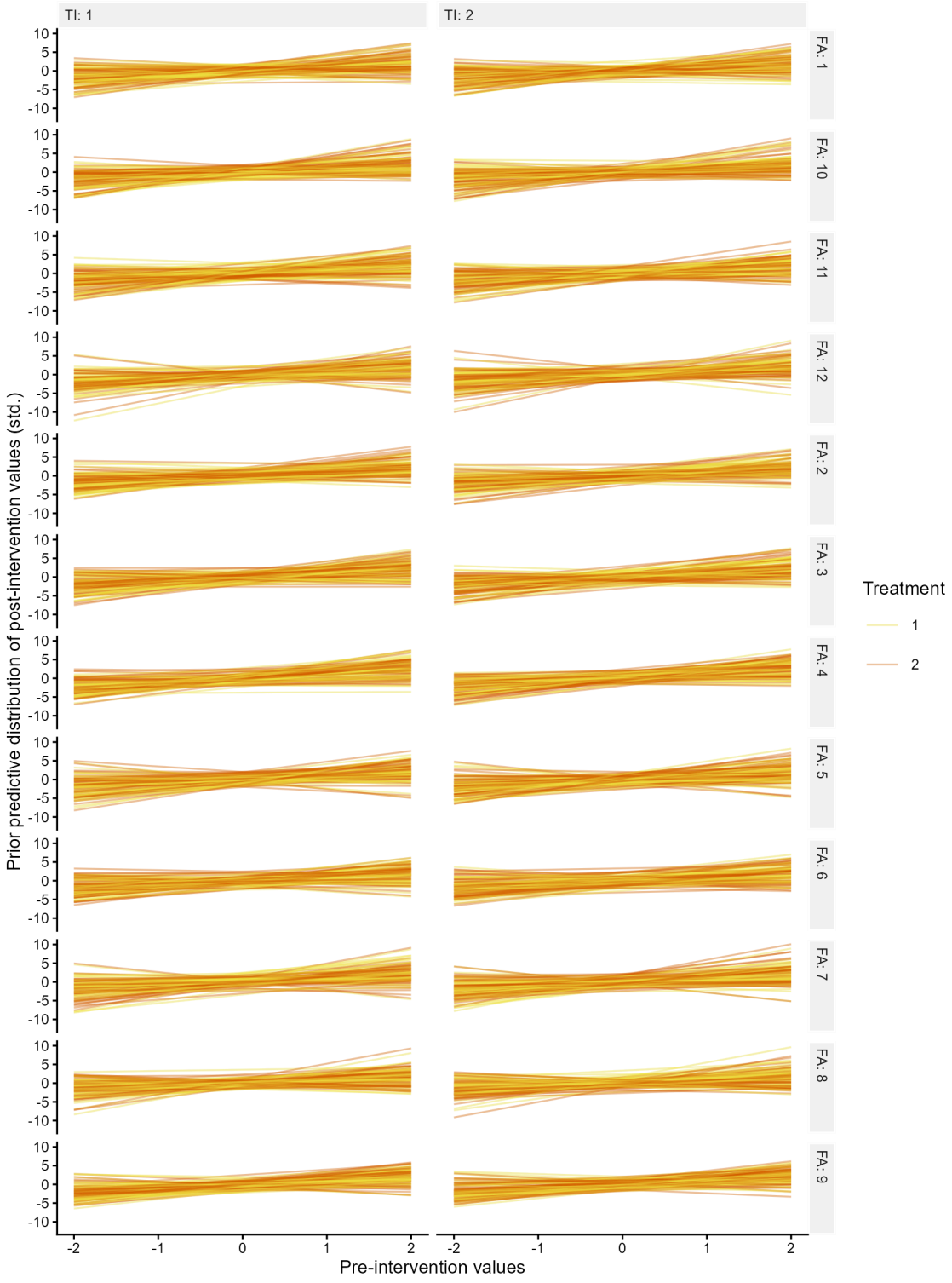

Figure 3.6: Prior predictive check model 3 (data imputation).

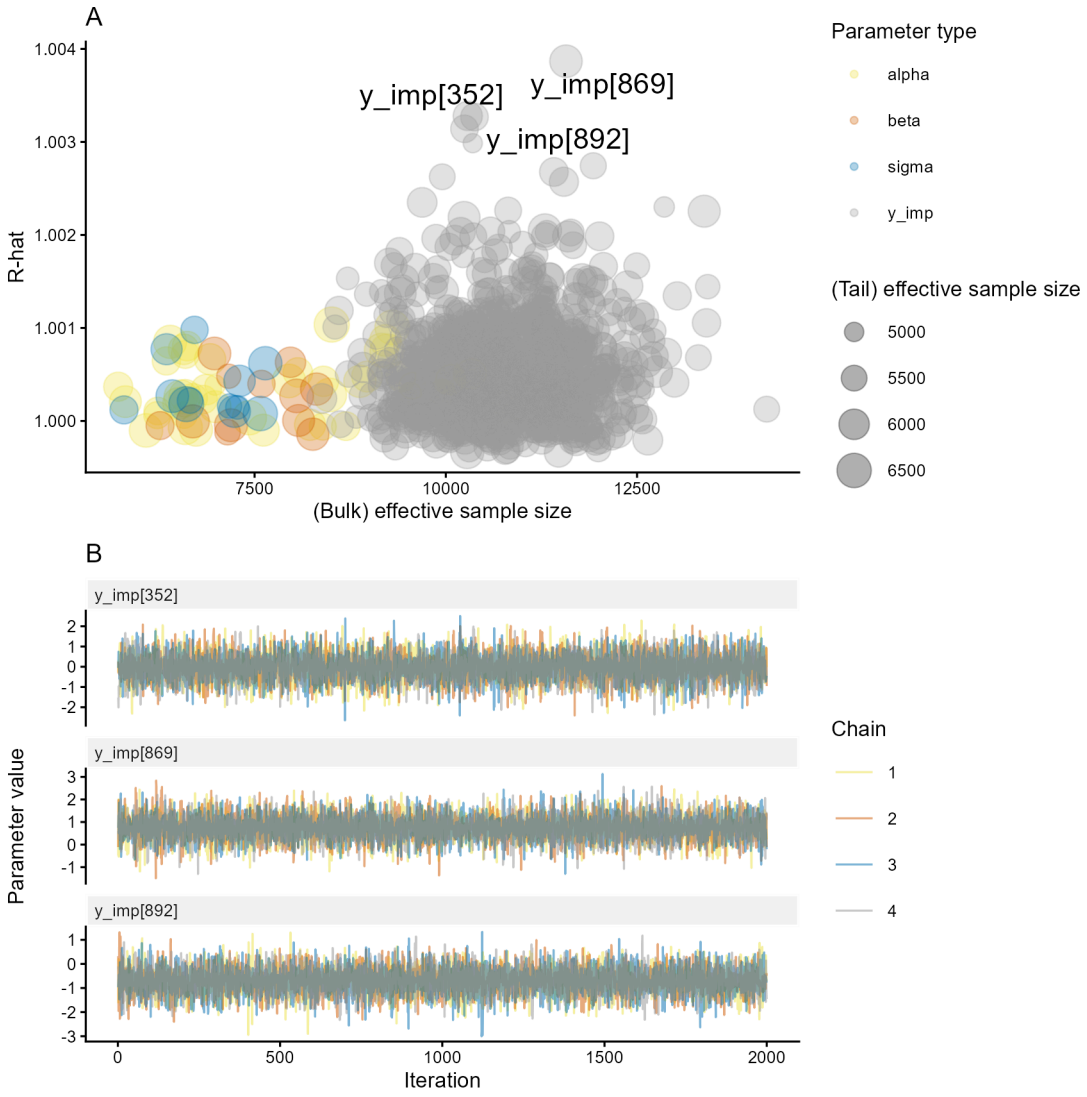

Figure 3.7: Model performance metrics model 3.

Posterior predictive checks indicated that the model capture general patterns in the data (Figure 3.8).

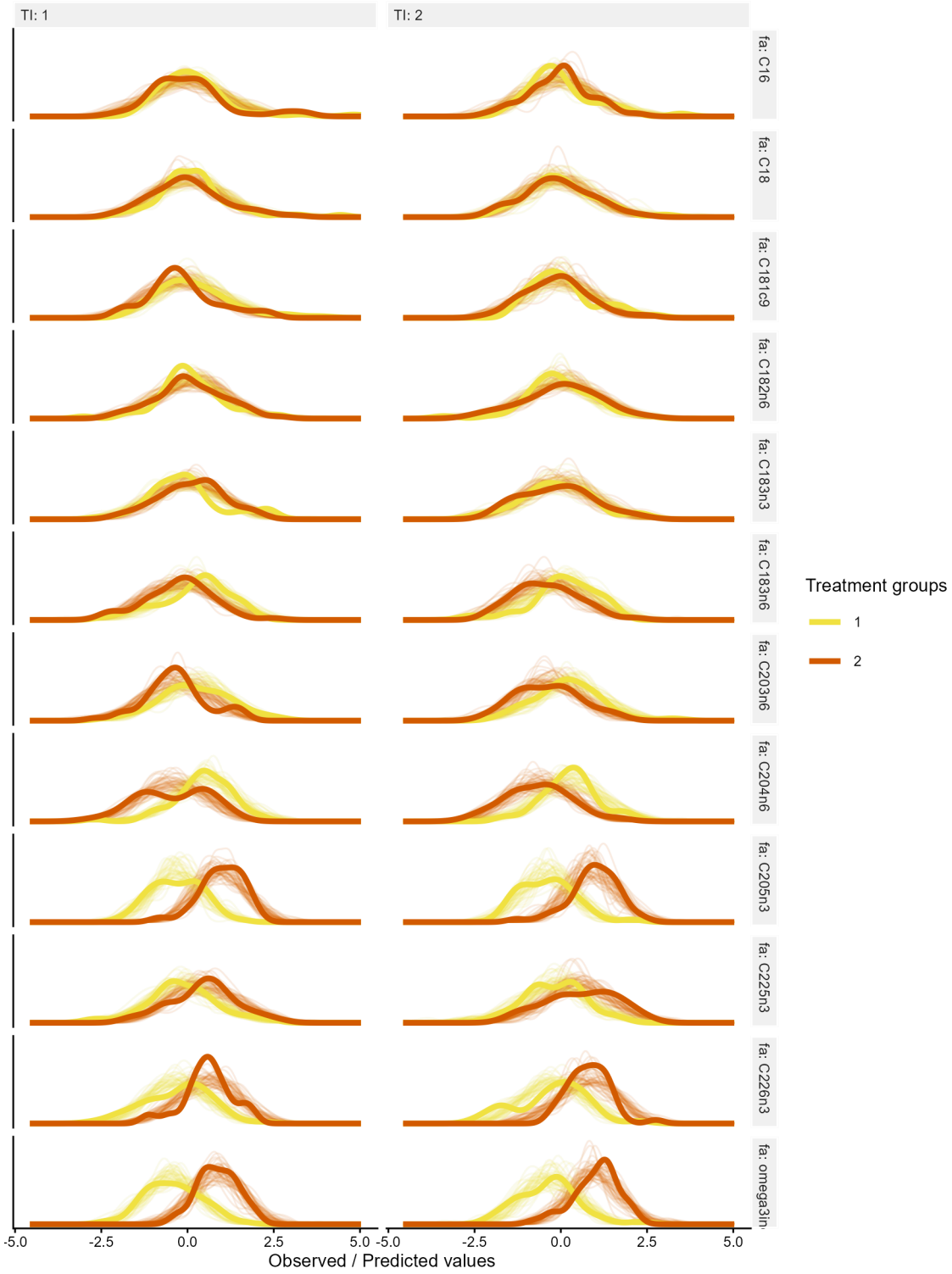

Figure 3.8: Posterior predictive check of Model 3. Thin lines represent model simulations (n = 50), thick lines represent observed data.

The imputation model generally agreed with the complete-case analysis on the effect of treatment on whole blood fatty acid profiles (Figure 3.9, Figure 3.10, Figure 3.11, and Figure 3.12).

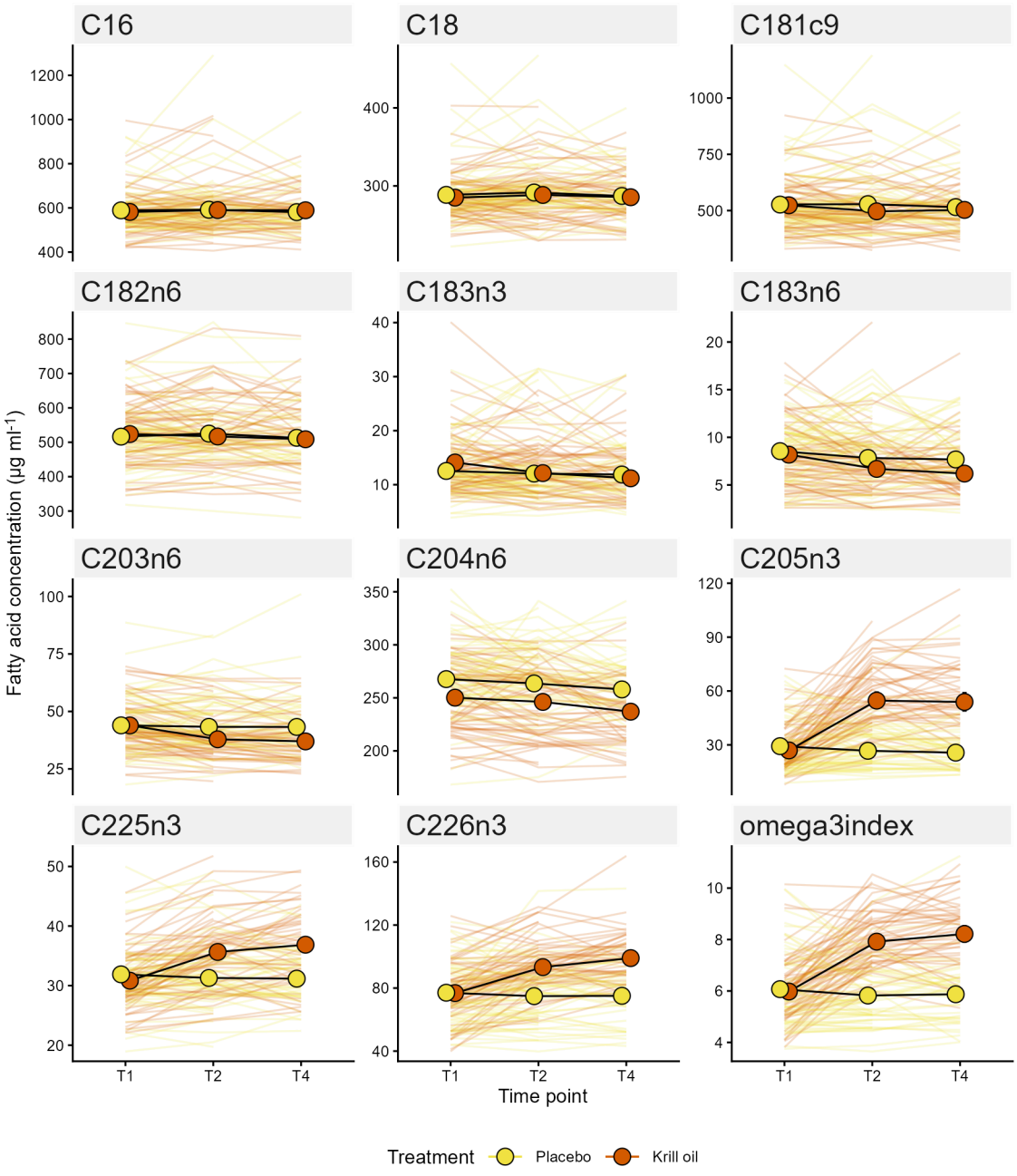

Figure 3.9: Model predictions. Pre-intervention values are observed means, post-treatment (T2, T4) values are model predictions (posterior medians) conditional on pre-intervention value with 95% percentile intervals. The omega-3 index is EPA and DHA content of whole blood expressed as a percent of total identified fatty acids.

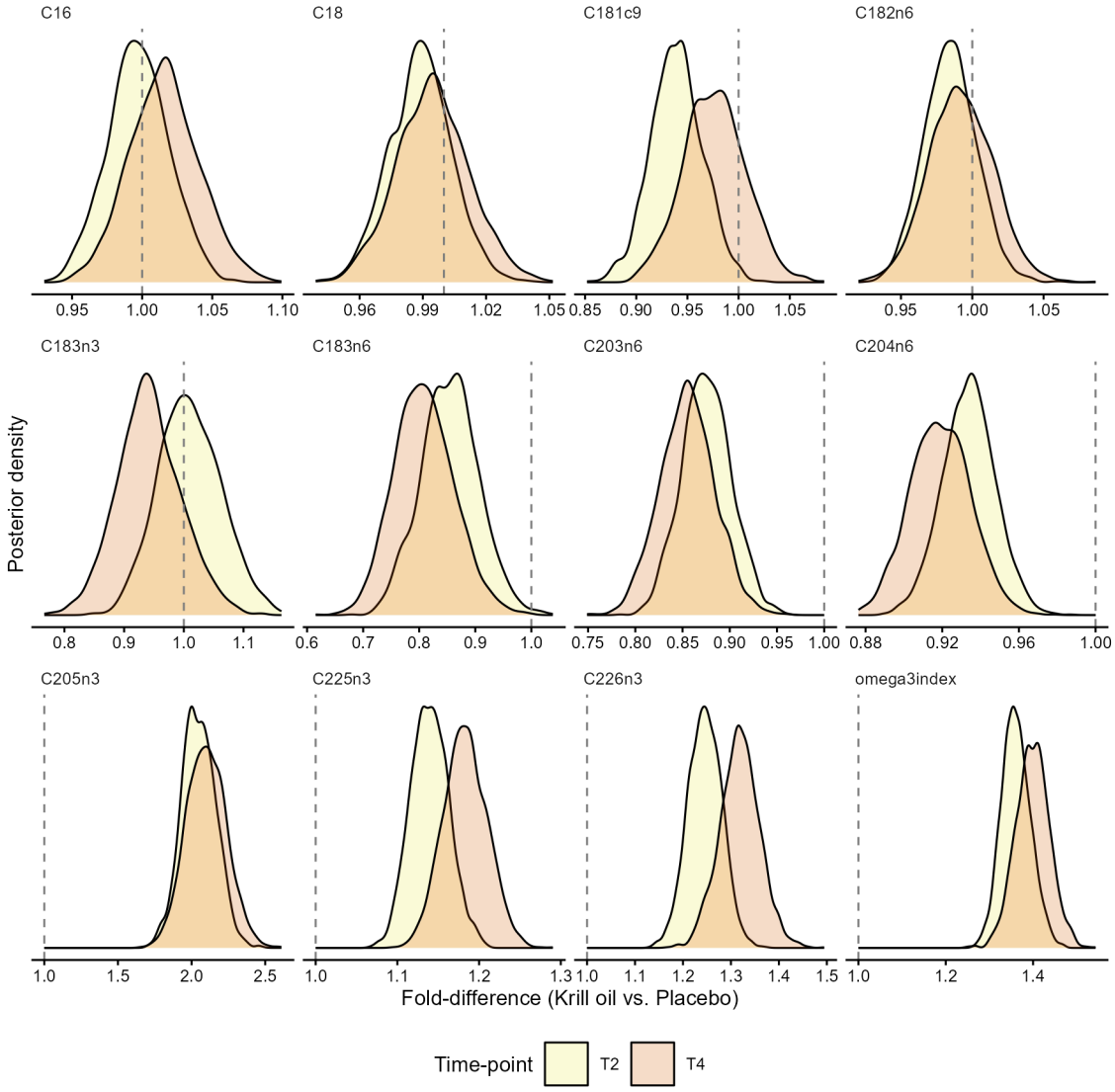

Figure 3.10: Posterior densities for differences between treatment groups in fatty acid concentrations (data imputation model).

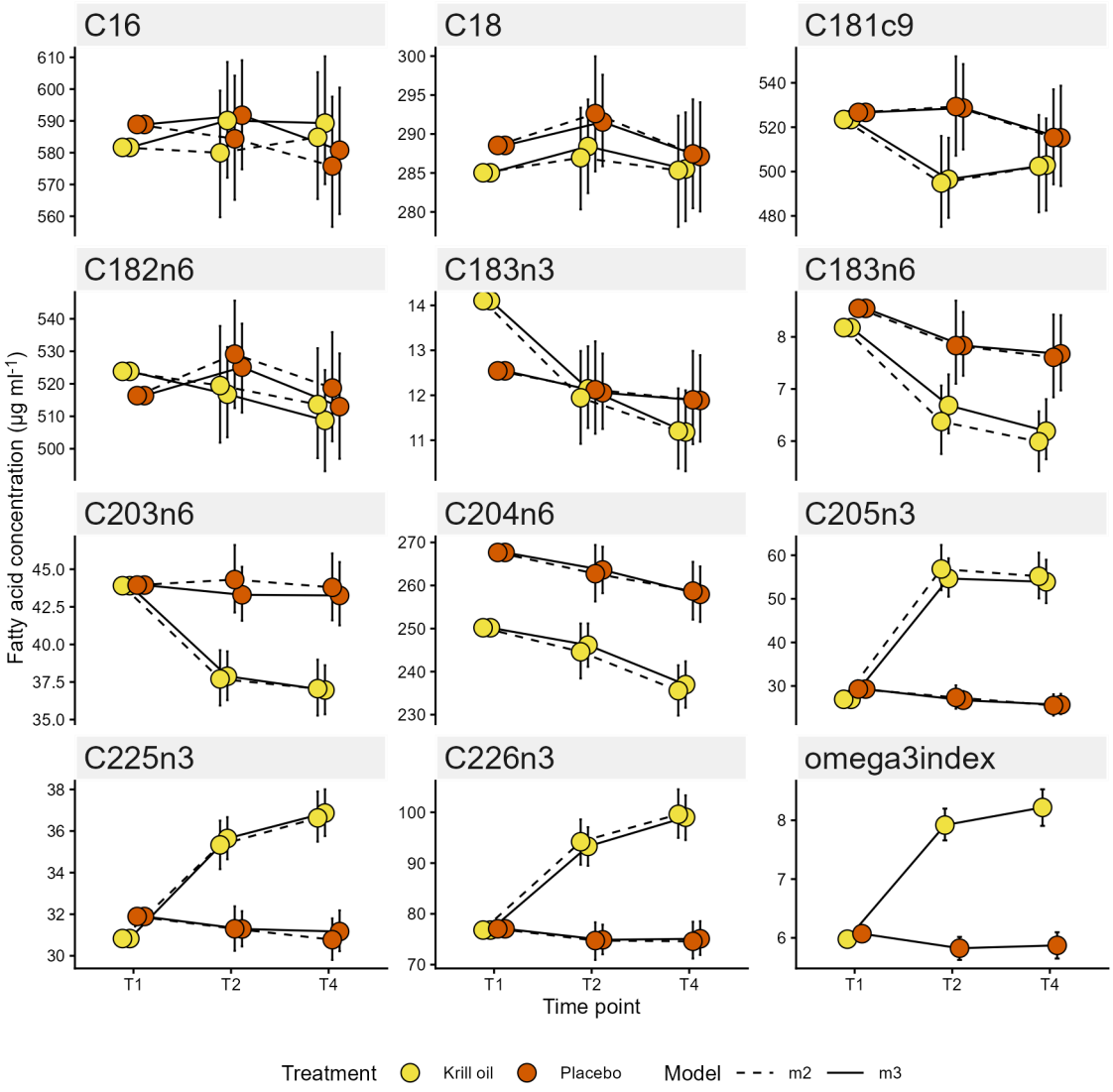

Figure 3.11: Posterior predictions for fatty acid concentrations in complete-case analysis (model m2) and data imputation model (model m3). Points are posterior medians with 95% percentile intervals.

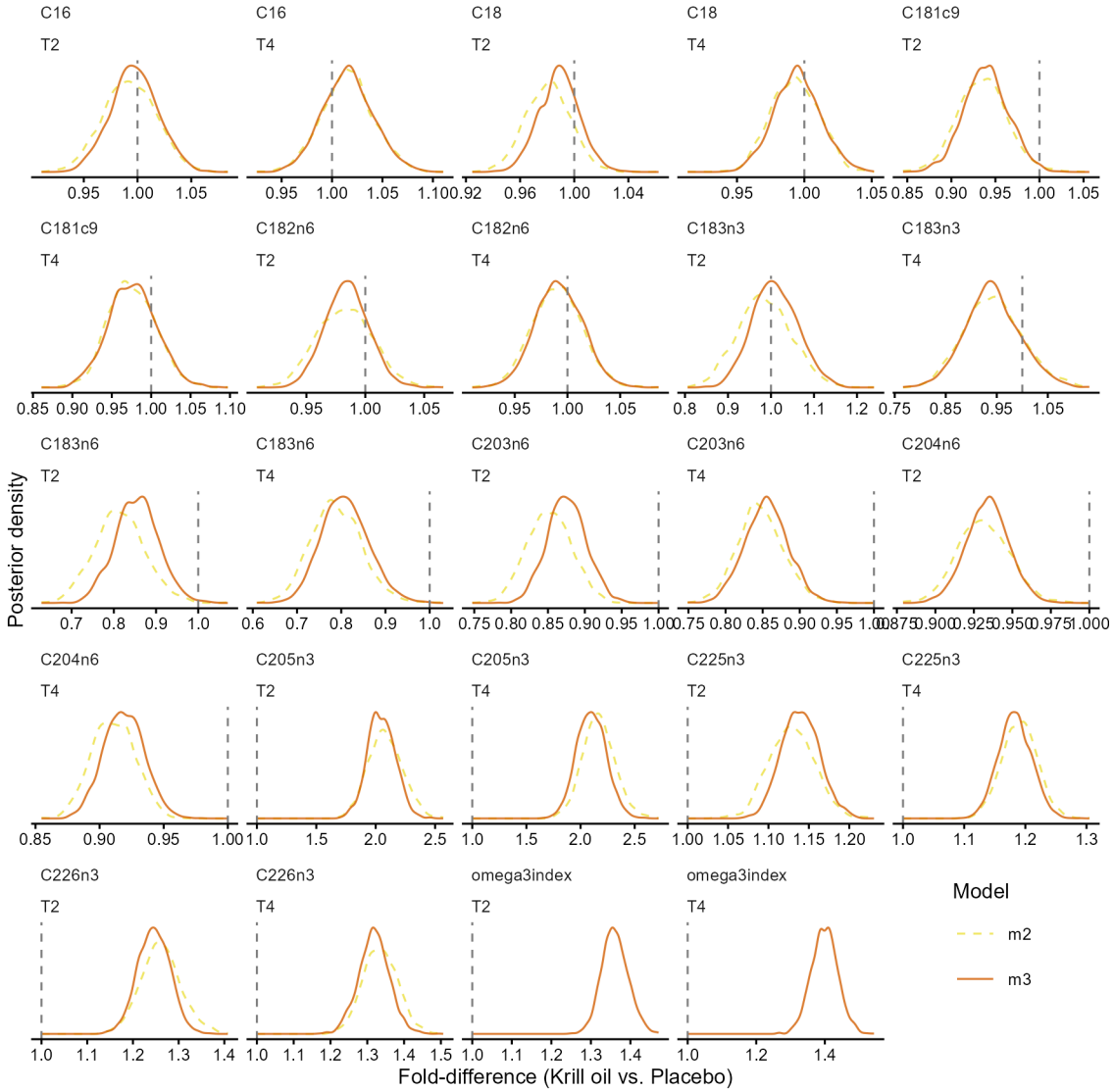

Figure 3.12: Comparing inference from complete-case and imputation models.

3.0.1 Model expansion: Heterogeneous variance

Model `m3` does not enable treatment- and time-wise heterogeneous variation. In model `m4` we therefore add $\sigma_{[\text{FA, TX}]} \sim \text{Exponential}\ (1)$, in model `m5` we add $\sigma_{[\text{FA, TI}]} \sim \text{Exponential}\ (1)$, and model `m6` adds $\sigma_{[\text{FA, TI, TX}]} \sim \text{Exponential}\ (1)$. Model performance metrics confirmed that all three extended models provided reliable chain convergence characteristics (Figure 3.13).

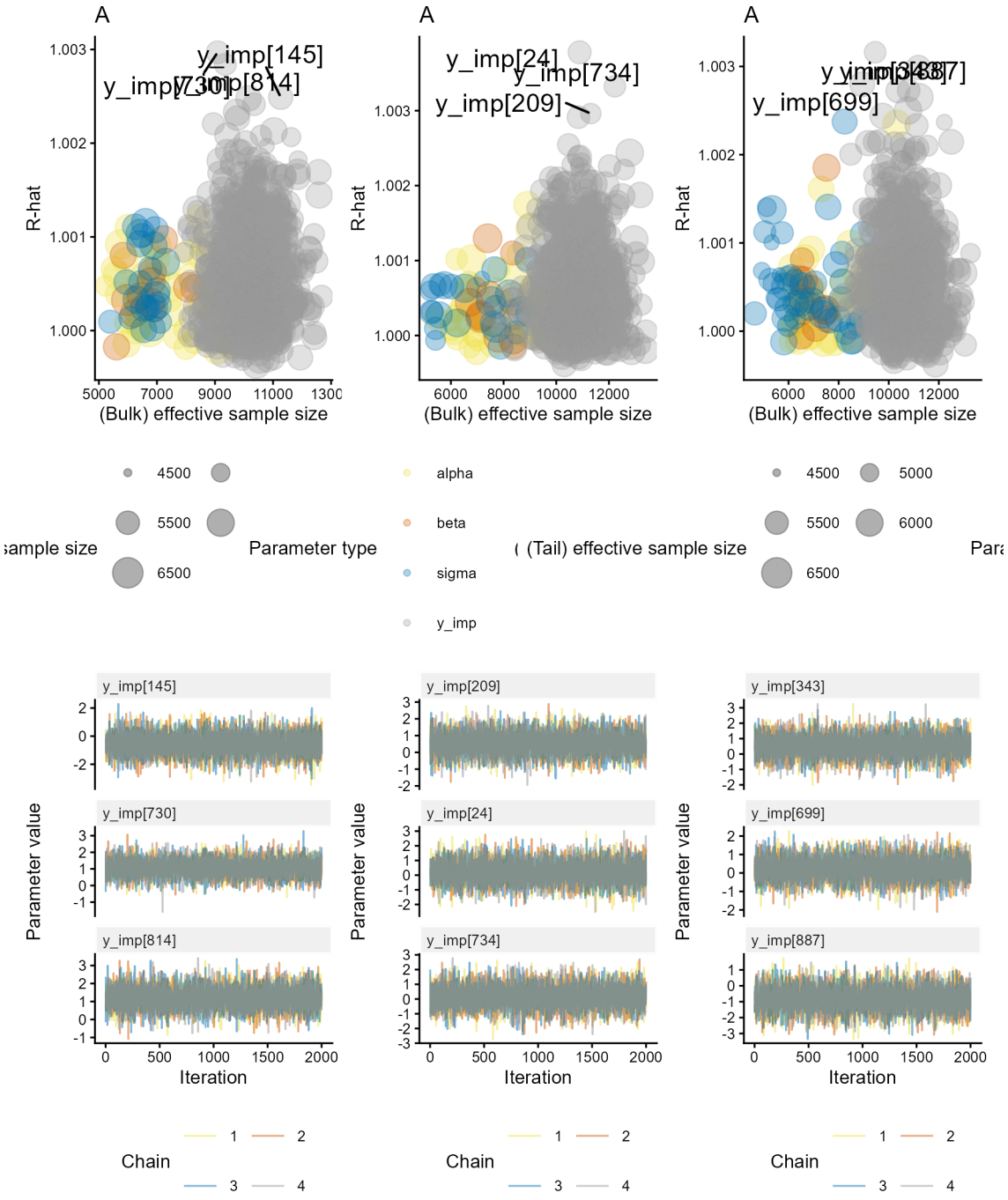

Figure 3.13: Model performance metrics models 4, 5, and 6.

Comparing the models in posterior predictive simulations of condition-specific (time, treatment and fatty-acid) SD across models indicated that no model displayed extreme deviations in replicated values compared to observed variation in the data (Table 3.1). This comparison agreed with the out of sample predictive performance where models `m3` to `m5` were comparable. Model `m6`, the most heavily parameterized model showed four unreliable Pareto k values but was also slightly worse in out of sample predictions (Table 3.2). Together this suggests that model `m4` is the preferred model with model `m6` being the worst model.

Table 3.1: Posterior predictive p-values comparing models m3 to m6.

Model	Proportion extreme p(< 0.05 | > 0.95)	Median p	Min p	Max p
m3	0.083	0.375	0.024	0.997
m4	0.021	0.363	0.078	0.954
m5	0.062	0.358	0.017	0.999
m6	0.021	0.429	0.051	0.963

Table 3.2: Expected pointwise predictive density across models.

Model	ΔELPD	SE(ΔELPD)	ELPD LOO
m4	0	0	−2,589.086
m3	−0.235	6.591	−2,589.321
m5	−3.156	9.228	−2,592.242
m6	−17.098	6.38	−2,606.184

3.1 Inference

We draw inference from model `m4` regarding the effect of treatment on whole blood fatty acid profiles.

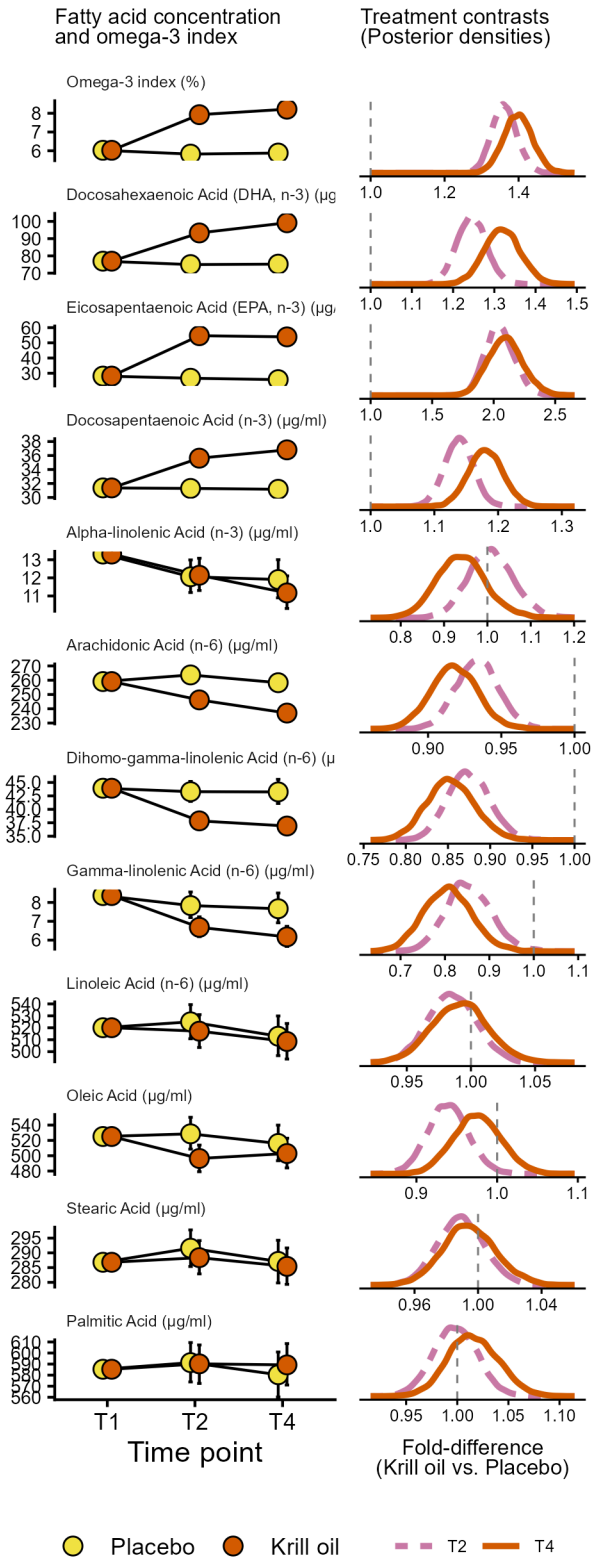

Figure 3.14: Adjusted group means and treatment contrasts of fatty acids concentrations and the omega-3 index in whole blood. Points in treatment-group means are posterior medians (observed means at T1) and error bars represent 95% percentile intervals, posterior contrasts are displayed as the full posterior distribution ratio between krill oil and placebo estimates at time-point T2 and T4 (1 indicating equivalent concentrations).

Krill oil supplementation led to robust increases in Docosahexaenoic (DHA; time-point T2 krill oil vs. placebo, % difference 24.5% [17.7, 31.7]; T4, 31.9% [23.5, 40.5]), Eicosapentaenoic (EPA; T2, 104.5% [83.0, 128.8]; T4, 109.4% [83.4, 138.4]), and Docosapentaenoic acid (T2, 13.9% [9.4, 18.4]; T4, 18.0% [12.8, 23.4]; Figure 3.14, Table 3.3). At the same time, omega-6 fatty acids were reduced in the krill oil group compared to placebo at both time-points (% difference; Gamma-linolenic acid T2, −14.8% [−24.3, −4.0]; Gamma-linolenic acid T4, −19.4% [−29.6, −8.1]; Arachidonic acid T2, −6.6% [−9.4, −3.7]; Arachidonic acid T4, −8.2% [−11.3, −5.0]; Dihomo-gamma-linolenic acid T2, −12.6% [−17.3, −7.5]; Dihomo-gamma-linolenic acid T4, −14.7% [−20.1, −8.9]; Figure 3.14, Table 3.3). While alpha-linolenic, linoleic, stearic and palmitic acids remained unchanged in krill oil compared to placebo (Figure 3.14, Table 3.3), Oleic acid was reduced in krill oil compared to placebo at T2 (-6.1% [−10.9, −1.0]) but not at T4 (-2.5% [−8.0, 3.4]).

Table 3.3: Descriptive data (observed mean and SD), adjusted mean estimates (posterior medians with 95% percentile intervals) and treatment contrasts (krill oil minus placebo; posterior medians with 95% percentile intervals) for fatty acids measured in whole blood before the intervention (time-point T1), after supplementation-only (T2), and after supplementation and resistance training (T4). All values are reported on the measurement scale (fatty acids, µg ml-1; omega-3 index, %).

	Time-point	Observed $\bar{x}$ (SD)		Estimate μ [95%]		Contrast [95%]
		Placebo	Krill oil	Placebo	Krill oil	Krill oil vs. placebo
Omega-3 index (%)	T1	6.1 (1.5)	6.0 (1.2)			
	T2	5.9 (1.2)	8.0 (1.1)	5.8 [5.6, 6.0]	7.9 [7.7, 8.2]	2.2 [1.8, 2.6]
	T4	6.0 (1.3)	8.3 (1.3)	5.9 [5.6, 6.1]	8.2 [7.9, 8.5]	2.4 [2.0, 2.8]
Docosa-hexaenoic Acid (DHA, n-3)	T1	77 (20)	77 (19)			
	T2	77 (19)	95 (17)	75 [72, 78]	93 [90, 96]	19.1 [14.3, 23.8]
	T4	78 (20)	100 (16)	75 [71, 79]	99 [95, 103]	25.0 [19.1, 30.5]
Eicosapen-taenoic Acid (EPA, n-3)	T1	30 (15)	27 (13)			
	T2	29 (12)	58 (18)	27 [25, 29]	55 [50, 59]	31.1 [25.7, 36.8]
	T4	29 (15)	58 (20)	26 [23, 28]	54 [49, 59]	31.3 [25.2, 37.7]
Docosapen-taenoic Acid (n-3)	T1	32 (5)	31 (5)			
	T2	32 (6)	36 (6)	31 [30, 32]	36 [35, 37]	4.5 [3.1, 5.8]
	T4	32 (5)	37 (6)	31 [30, 32]	37 [36, 38]	5.8 [4.2, 7.4]
Alpha-linolenic Acid (n-3)	T1	13 (5)	14 (6)			
	T2	13 (7)	14 (5)	12 [11, 13]	12 [11, 13]	0.1 [−1.1, 1.3]
	T4	13 (6)	13 (6)	12 [11, 13]	11 [10, 12]	−0.7 [−2.1, 0.6]
Arachidonic Acid (n-6)	T1	268 (33)	250 (34)			
	T2	272 (30)	242 (34)	264 [258, 269]	246 [241, 252]	−17.3 [−25.0, −9.6]
	T4	264 (29)	234 (31)	258 [252, 264]	237 [231, 243]	−21.1 [−29.4, −12.7]
Dihomo-gamma-linolenic Acid (n-6)	T1	44 (12)	43 (11)			
	T2	45 (13)	39 (10)	43 [41, 45]	38 [37, 39]	−5.4 [−7.6, −3.1]
	T4	45 (13)	38 (10)	43 [41, 46]	37 [35, 38]	−6.3 [−8.9, −3.6]
Gamma-linolenic Acid (n-6)	T1	8.5 (3.1)	8.0 (3.6)			
	T2	8.8 (3.6)	7.1 (3.3)	7.8 [7.2, 8.6]	6.7 [6.2, 7.2]	−1.1 [−2.0, −0.3]
	T4	8.2 (3.1)	6.6 (3.1)	7.7 [6.9, 8.5]	6.2 [5.7, 6.7]	−1.4 [−2.4, −0.6]
Linoleic Acid (n-6)	T1	522 (94)	531 (91)			
	T2	534 (96)	534 (98)	525 [511, 539]	517 [503, 531]	−7.9 [−27.5, 12.0]
	T4	521 (94)	529 (103)	513 [497, 530]	509 [494, 524]	−4.4 [−27.4, 17.8]
Oleic Acid	T1	534 (138)	521 (116)			
	T2	551 (151)	507 (121)	529 [509, 550]	496 [479, 514]	−32.3 [−59.6, −5.2]
	T4	530 (121)	510 (109)	516 [493, 540]	503 [484, 523]	−12.8 [−43.0, 16.7]
Stearic Acid	T1	289 (36)	285 (29)			
	T2	295 (38)	289 (35)	292 [285, 298]	288 [283, 294]	−3.2 [−11.7, 5.4]
	T4	290 (33)	286 (31)	287 [280, 294]	285 [279, 292]	−1.7 [−11.2, 8.1]
Palmitic Acid	T1	591 (96)	577 (105)			
	T2	606 (125)	595 (124)	591 [574, 609]	590 [572, 607]	−1.3 [−26.7, 23.6]
	T4	592 (109)	584 (87)	580 [560, 601]	589 [571, 609]	8.7 [−18.8, 37.1]

4. Effects of krill oil supplementation on indications of metabolic syndrome - Introduction

Here we aim to model the components of the metabolic syndrome definition (14), along with related outcomes (Table 4.2). The goal is to build a model that imputes missing data to enable an intention-to-treat (ITT) analysis of all randomized participants. Imputation will also allow for assessing the probability of meeting metabolic syndrome definition thresholds in each group while accounting for uncertainty associated with missing observations. The strategy will be to choose a model that will allow for sharing of information between variables to enable more efficient data imputation. We use the harmonised metabolic syndrome definition, where any three of the five criteria (waist circumference, triglycerides, HDL-cholesterol, blood pressure and fasting glucose) beyond their threshold values are sufficient for a diagnosis, and central obesity is not required (15).

Figure 4.1 shows distributions of the variables selected for joint modeling, including variables used to define metabolic syndrome (waist circumference, raised triglycerides, reduced HDL-cholesterol, raised blood pressure, raised fasting glucose).

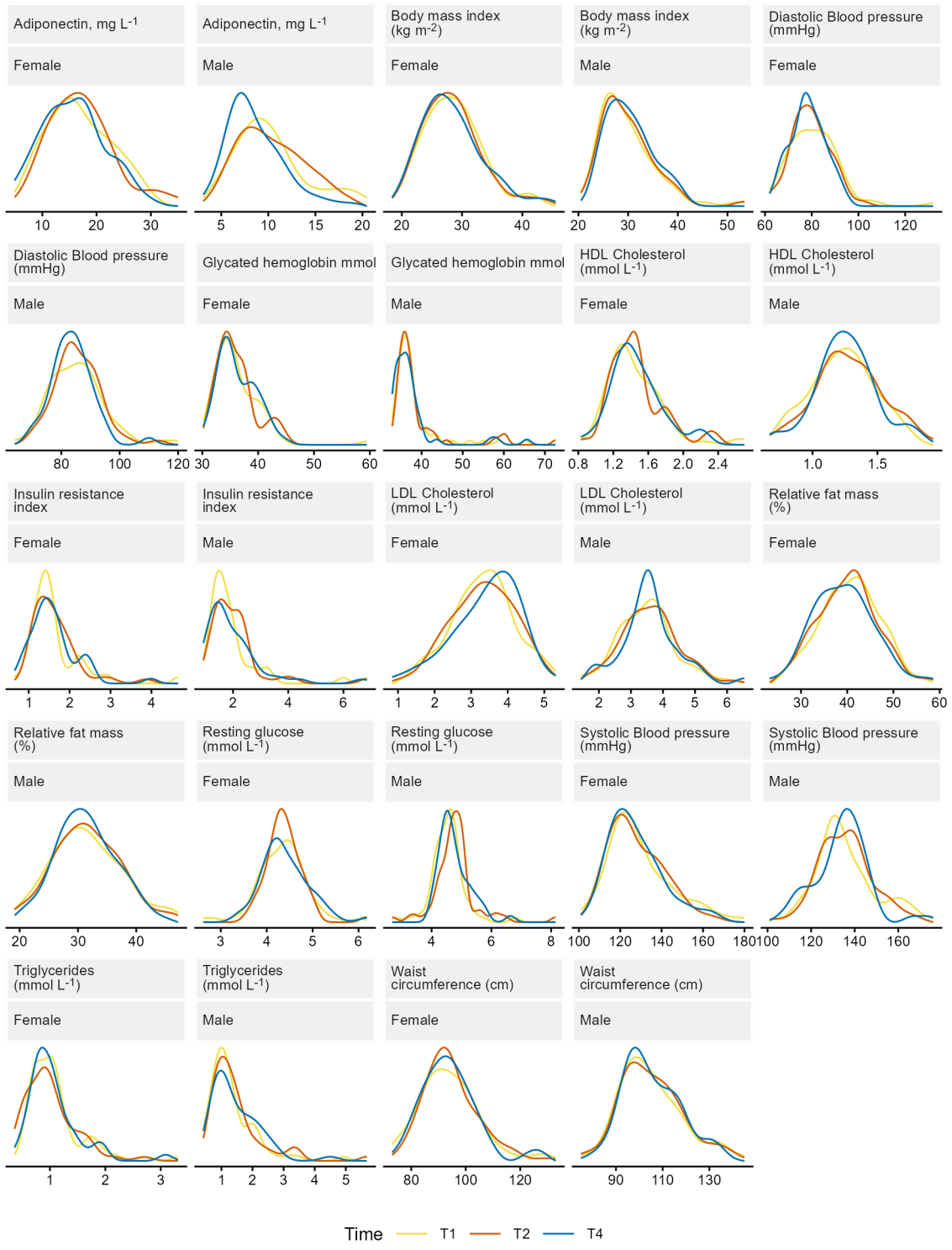

Figure 4.1: Observed data distributions of main outcomes.

Descriptive analysis of complete cases indicates variation in the proportion of participants meeting the criteria thresholds across treatment groups and post-intervention time points. Table 4.1 and Figure 4.2 show the proportions of participants who have three or more diagnostic criteria for metabolic syndrome among complete cases. We will expand these analyses later when incorporating the diagnosis in modelling of subscores in a joint model.

Table 4.1: Participants in the complete-case analysis of metabolic syndrome indications.

	Time		
	T1	T2	T4
	Male		
Placebo	21 (24%)	21 (33%)	21 (38%)
Krill oil	19 (26%)	19 (16%)	19 (32%)
	Female		
Placebo	16 (25%)	16 (12%)	16 (19%)
Krill oil	21 (24%)	21 (10%)	21 (24%)
	Total		
Placebo	37 (24%)	37 (24%)	37 (30%)
Krill oil	40 (25%)	40 (12%)	40 (28%)

Data are presented as n (Metabolic syndrome proportion).Time T1, Baseline;T2 Supplementation; T4, Combined.

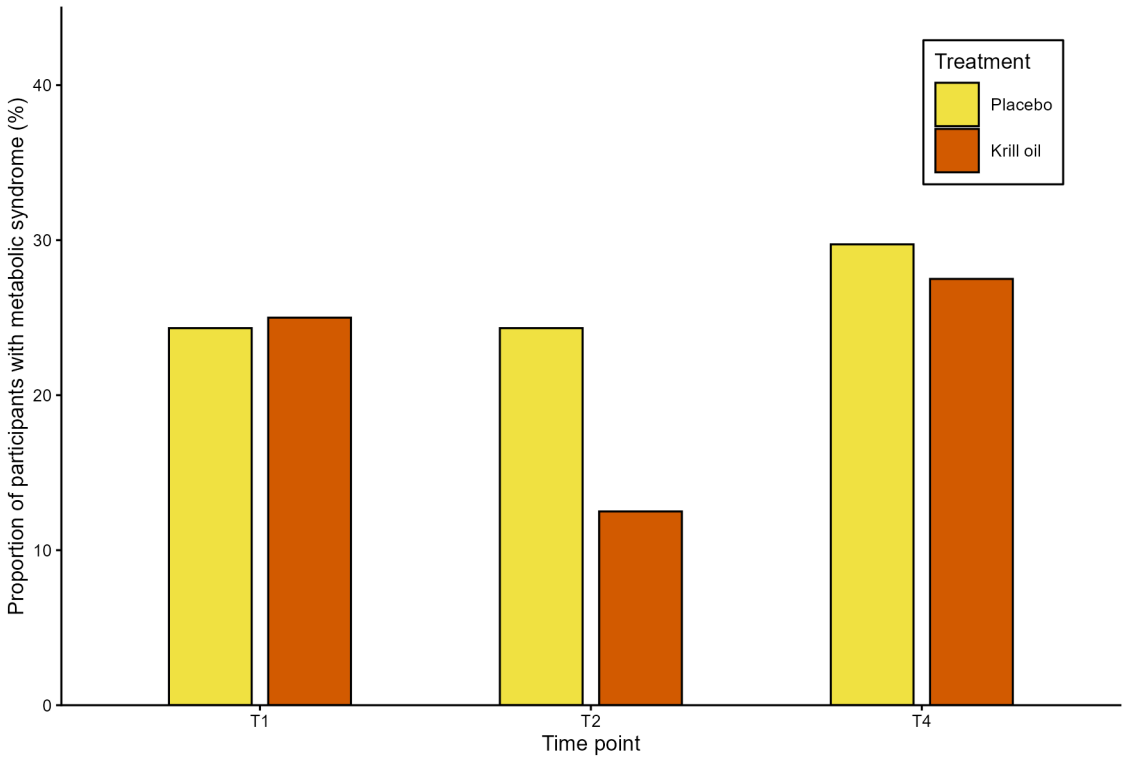

Figure 4.2: Proportion of participants with metabolic syndrome (at least three criteria thresholds).

Table 4.2: Components of the metabolic syndrome diagnostic criteria (harmonised definition, thresholds as used in this analysis) and related measurements used in modelling.

Subscore	Threshold
Waist circumference	≥ 94 cm for men, ≥ 80 cm for women
HDL Cholesterol	< 1.0 mmol/L for men, < 1.3 mmol/L for women
Fasting glucose	≥ 5.6 mmol/L
Triglycerides	≥ 1.7 mmol/L
Blood pressure	Systolic ≥ 130 and/or diastolic ≥ 85 mm Hg
Relative fat mass	NA
Body mass index	NA
Insulin resistance index	NA
Adiponectin	NA
LDL Cholesterol	NA
HbA1c	NA

Metabolic syndrome is defined as at least three of the five criteria with thresholds (harmonised definition, with the waist circumference thresholds for Europids). Drug treatment for elevated triglycerides, reduced HDL cholesterol, elevated blood pressure or elevated glucose was not counted as meeting a criterion.

For the joint model we also include measures related to metabolic health. Among these variables, HOMA2-IR is a derived variable based on C-peptide and fasting glucose calculated using the HOMA2 calculator. HOMA2-IR was calculated when C-peptide and glucose was available and any missing data was primarily seen in the C-peptide variable. Based on the strong correlation between C-peptide and HOMA2-IR vs. the more moderate correlation between glucose and HOMA2-IR (Figure 4.3) we chose to use the derived variable HOMA2-IR and drop C-peptide from the joint modelling to avoid duplicating information in the same model.

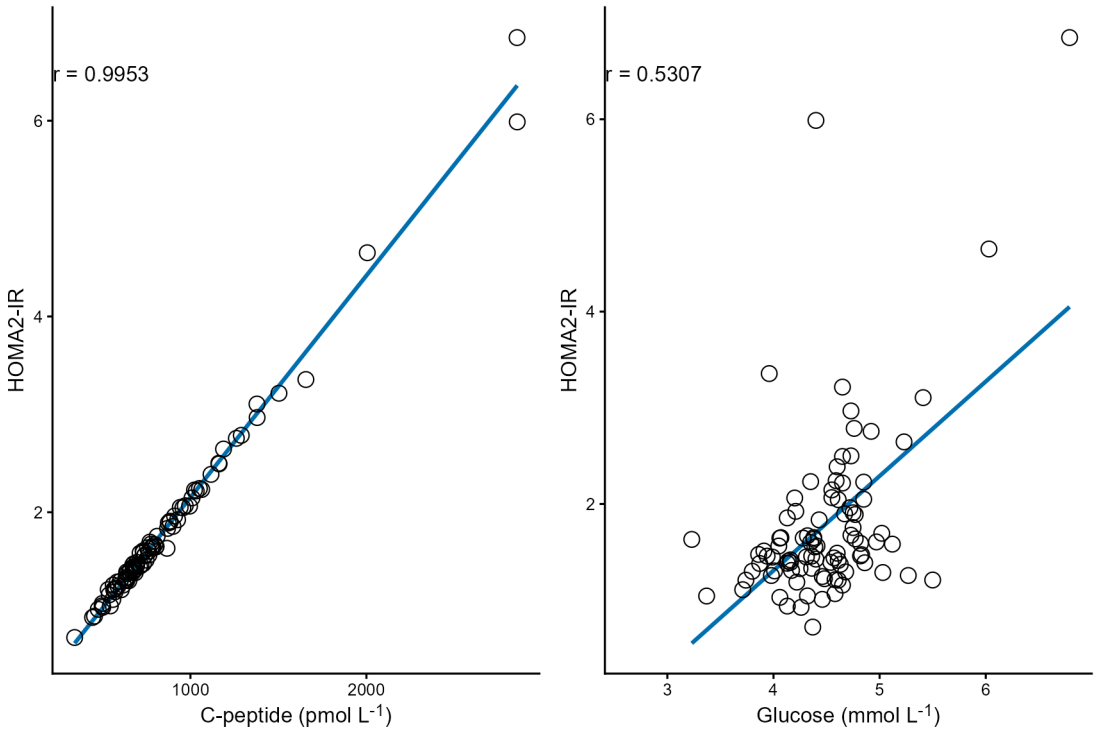

Figure 4.3: Associations between C-peptide or glucose and HOMA2-IR in samples collected at baseline. These associations indicate that HOMA2-IR more strongly captures variation in C-peptide.

Variables in the observed data are correlated within time points within variables and between variables across time points. To describe approximate variation and co-variation patterns, "naive" linear models were fitted with sex as a single predictor at time-point T1, sex, baseline values, and treatment as predictors for values at time-points T2 and T4. Residuals were extracted from the models, and estimates were plotted in Figure 4.4.

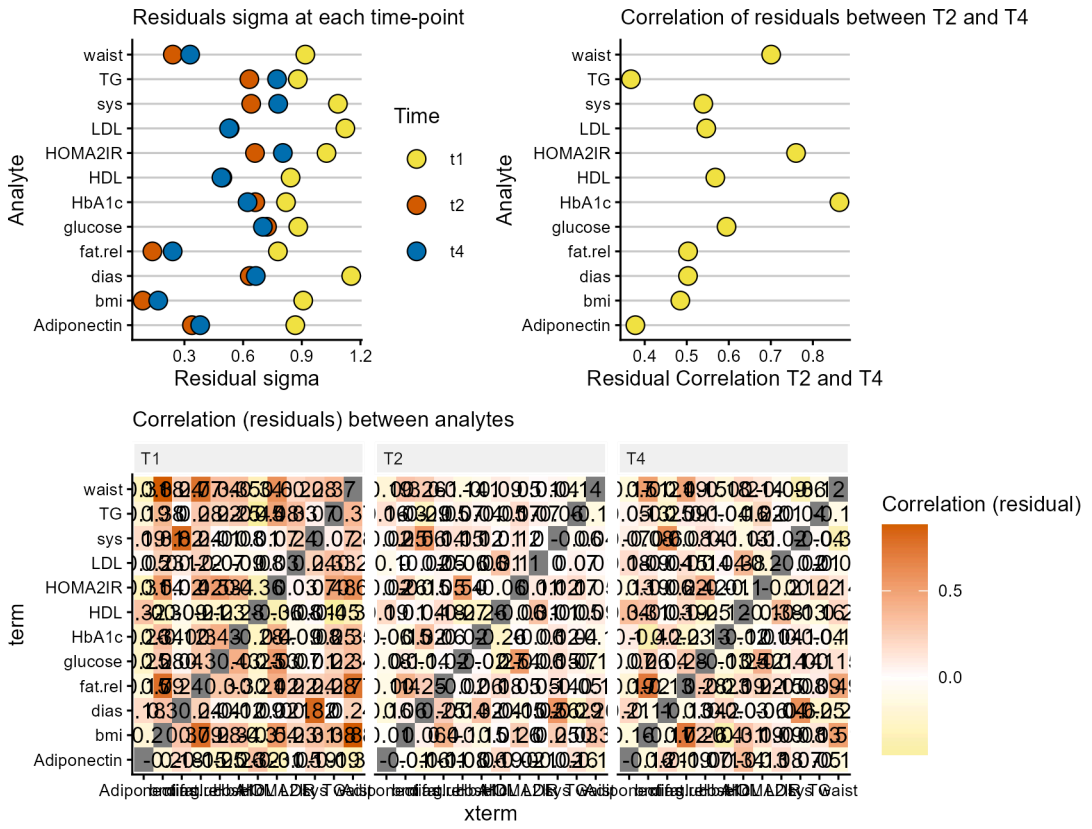

Figure 4.4: Residual variation and correlation pattern within and between variables in the observed data.

Correlation patterns among variables will be used to inform simulations (see Section 4.0.1). Correlations among variables also suggest that uncertainty associated with missing data can be reduced by exploiting the covariance among variables. The missing data patterns in the data set indicate that some missing data occurs in a single variable at a single time point whilst some participants have missing data in all variables at specific time points (Figure 4.5).

Figure 4.5: Observed and missing data across treatment groups and time-points. Rows are participants, columns are data points.

To assess if the outcome variables predict drop-out we fit a logistic regression model with drop-out (D_i) at time Time T2 or T4, coded as 0/1 as the outcome variable. The association between the standardized measurement type value ($X_{i[A]}$) and drop-out was determined while controlling for sex (S) within each treatment group (T).

$$
\begin{aligned}
D_{i[\text{Time}]} &\sim \text{Bernoulli } (p_i) \\
\text{Logit } (p_i) &= \alpha_{[A]} + \beta_{X[A]} X_{i[A]} + \beta_{S[A]} S_i + \beta_{T[A]} T_i + \beta_{TX[A]} T_i X_i \\
\alpha_{[A]} &\sim \text{Normal } (0, 0.5) \\
\beta_{X[A]} &\sim \text{Normal } (0, 0.5) \\
\beta_{S[A]} &\sim \text{Normal } (0, 0.5) \\
\beta_{T[A]} &\sim \text{Normal } (0, 0.5) \\
\beta_{TX[A]} &\sim \text{Normal } (0, 0.5)
\end{aligned}
\tag{4.1}
$$

In addition to measurements included in the metabolic syndrome diagnostic criteria (Fasting glucose, waist circumference, HDL, triglycerides, blood pressure), we also included relative fat mass, body mass index, age, insulin resistance index (HOMA2IR), LDL, adiponectin and HbA1c. Prior predictive checks (Figure 4.6) confirmed that the model expects thirteen predictors (age and twelve measurement types, with systolic and diastolic blood pressure as separate types) in two treatment groups and that the combined priors weakly regularize away from all/non drop-out.

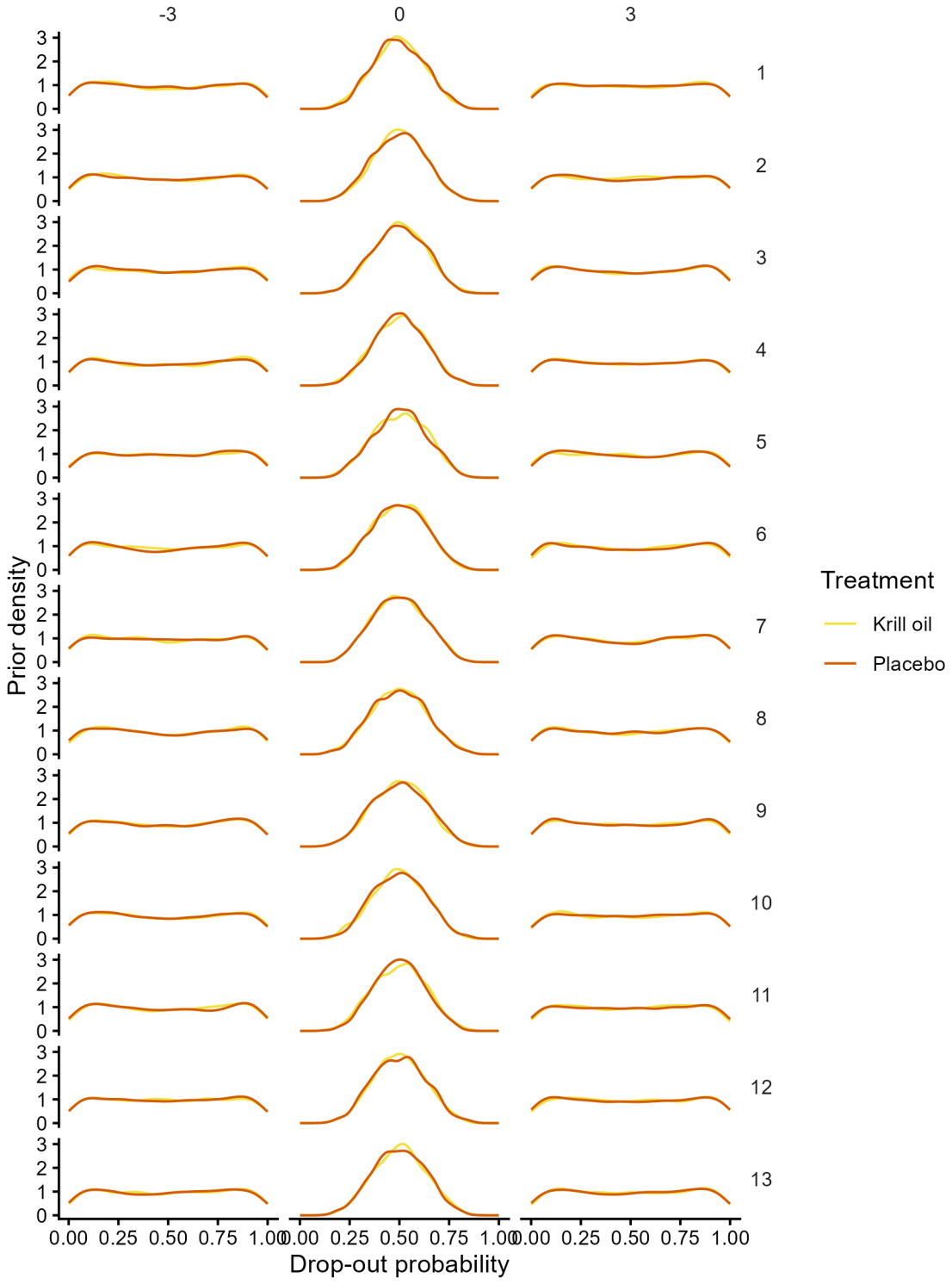

Figure 4.6: Prior predictive simulation for model 1. Columns are values of the predictor (SD-units) rows represents each predictor.

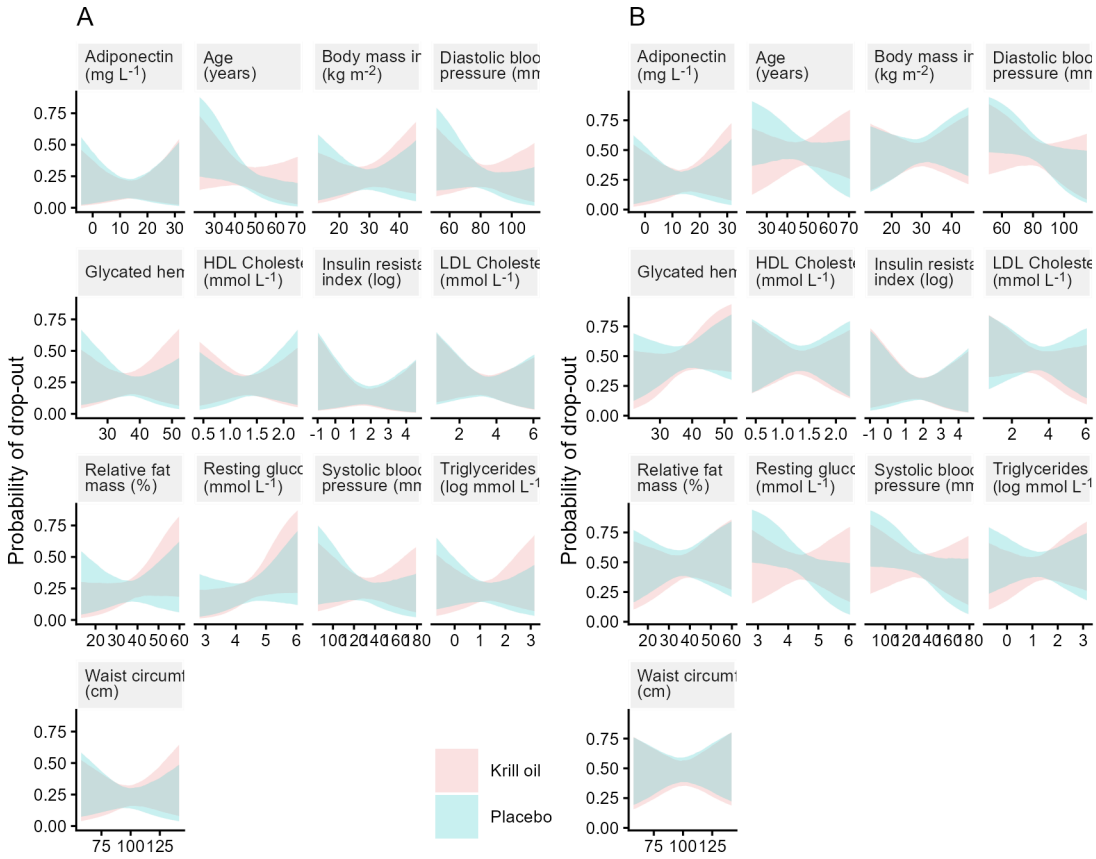

Figure 4.7: Probability of drop-out across values of baseline co-variates per treatment group to time-point 2 (A) and 4 (B). Estimates are adjusted for sex, intervals are 95% percentile intervals.

Table 4.3: Change in log-odds (Slope) for drop-out for 1 SD-unit change in baseline characteristic (Outcome) and differences in slope between treatment groups (Δ Treatment) at time T2 and time T4. Values are posterior medians with 95% percentile intervals.

Outcome	T2		T4	
	Slope	Δ Treatment	Slope	Δ Treatment
Adiponectin (mg L-1)	0.01 [−0.50, 0.53]	0.12 [−0.62, 0.84]	0.07 [−0.37, 0.51]	0.19 [−0.50, 0.88]
HDL Cholesterol (mmol L-1)	0.06 [−0.34, 0.43]	−0.15 [−0.79, 0.51]	−0.03 [−0.37, 0.30]	−0.03 [−0.62, 0.52]
Insulin resistance index (log)	−0.13 [−0.69, 0.38]	0.00 [−0.83, 0.78]	−0.10 [−0.59, 0.35]	−0.05 [−0.81, 0.70]
Glycated hemoglobin	0.01 [−0.41, 0.37]	0.21 [−0.47, 0.88]	0.27 [−0.10, 0.65]	0.21 [−0.35, 0.78]
LDL Cholesterol (mmol L-1)	−0.13 [−0.52, 0.22]	−0.03 [−0.69, 0.60]	−0.16 [−0.49, 0.15]	−0.13 [−0.68, 0.44]
Triglycerides (log mmol L-1)	−0.01 [−0.41, 0.37]	0.26 [−0.41, 0.92]	0.07 [−0.26, 0.38]	0.22 [−0.33, 0.77]
Age (years)	−0.41 [−0.78, −0.05]	0.30 [−0.31, 0.95]	−0.08 [−0.42, 0.26]	0.45 [−0.09, 0.98]
Body mass index (kg m-2)	0.08 [−0.29, 0.44]	0.20 [−0.39, 0.79]	0.11 [−0.21, 0.42]	−0.08 [−0.63, 0.47]
Diastolic blood pressure (mmHg)	−0.22 [−0.62, 0.14]	0.25 [−0.40, 0.92]	−0.34 [−0.70, −0.01]	0.23 [−0.38, 0.82]
Relative fat mass (%)	0.24 [−0.15, 0.62]	0.33 [−0.26, 0.98]	0.11 [−0.24, 0.48]	0.12 [−0.45, 0.70]
Fasting glucose (mmol L-1)	0.38 [0.00, 0.77]	0.26 [−0.38, 0.91]	−0.21 [−0.55, 0.13]	0.46 [−0.11, 1.08]
Systolic blood pressure (mmHg)	−0.15 [−0.53, 0.22]	0.26 [−0.37, 0.88]	−0.25 [−0.59, 0.09]	0.32 [−0.26, 0.91]
Waist circumference (cm)	0.01 [−0.39, 0.39]	0.19 [−0.38, 0.78]	0.03 [−0.30, 0.37]	−0.00 [−0.56, 0.55]

In conclusion, age was negatively associated with probability of drop-out at T2 (younger participants were more likely to drop out) without robust difference between treatment groups (Figure 4.7 A; Table 4.3). Higher resting glucose at baseline tended to predict drop-out to T2, but not to T4, with no strong difference between treatment groups (Figure 4.7 A and B; Table 4.3). Lower diastolic blood pressure at baseline tended to predict drop-out to T4 (Figure 4.7 B; Table 4.3). Together this suggests that baseline covariates did not robustly interact with treatment in predicting drop-out. However, baseline covariates were associated with drop-out: lower age and higher fasting glucose with drop-out to T2, and lower diastolic blood pressure with drop-out to T4.

4.0.1 Synthetic data

Synthetic data is generated with known parameter values to validate models and to enable exploration of the effects of model misspecification and missing data. The first data simulation function uses a multivariate normal distribution to simulate conditional means for K measurement types and N participants.

$$\begin{aligned} y_{i,j} &\sim \text{MVNormal}\ (\mu_{k[i,j]}, \Sigma) \\ \mu_{k[i,j]} &= \alpha + \beta_k X \end{aligned} \tag{4.2}$$

Where $y_{i,j}$ is a vector of outcome values for participant i in time-point j, drawn from a multivariate normal distribution with conditional means ($\mu_{k[i,j]}$ and a variance-covariance matrix (Σ). The conditional means are simulated using a linear predictor with parameter vector β and design matrix X.

The covariance matrix is constructed from two separate correlation matrices and a cross-time-point vector of correlations within each measurement type. Residual standard deviations for each type (and time) are used to combine standard deviations and correlations into the variance-covariance matrix[2].

[2]In the simulation function (`R/sim-fun.R`) separate variance-covariance matrices are constructed and then combined. But separate correlation matrices are the function's input.

$$
\begin{aligned}
\rho_{t2} &= \begin{bmatrix} 1 & \rho_{12} & \cdots \\ \rho_{21} & 1 & \cdots \\ \vdots & \vdots & \ddots \end{bmatrix} \\
\rho_{t4} &= \begin{bmatrix} 1 & \rho_{12} & \cdots \\ \rho_{21} & 1 & \cdots \\ \vdots & \vdots & \ddots \end{bmatrix} \\
\rho_{\text{cross}} &= \text{diag } (p_{k,\text{ time}}) \\
R &= \begin{bmatrix} \rho_{t2} & \rho_{\text{cross}} \\ \rho_{\text{cross}}^T & \rho_{t4} \end{bmatrix} \\
S &= \text{diag } (\sigma_{k,\text{ time}}) \\
\Sigma &= S \times R \times S
\end{aligned}
\tag{4.3}
$$

We will be able to control the within-time *residual* correlations between measurement types and between-time *residual* correlations within measurement types. The full correlation matrix (R) will set the residual correlation between measurement types at different time points to 0. These will, however, be correlated through within-time-point and pre-intervention correlations.

The pre-intervention values are also conditionally constructed using a separate multivariate normal distribution.

$$
\begin{aligned}
y_{\text{pre }[i]} &\sim \text{MVNormal } (\mu_{k[i]}, \Sigma) \\
\mu_{k[i]} &= \alpha + \beta_k X \\
R &= \begin{bmatrix} 1 & \rho_{12} & \cdots \\ \rho_{21} & 1 & \cdots \\ \vdots & \vdots & \ddots \end{bmatrix} \\
S &= \text{diag } (\sigma_k) \\
\Sigma &= S \times R \times S
\end{aligned}
\tag{4.4}
$$

Using this definition, we can control the correlations among measurement types (at baseline) and their variation. We can also make the means conditional on a set of predictors (design matrix X) with corresponding parameter values (β_k).

The approach detailed above is convenient as we are able to define each pairwise correlation. However, this comes with two caveats. First, for 8 variables we need to set $((8 \times 8 - 8)/2) \times 3 = 84$ correlation coefficients to capture between variable correlations at each time-point. Second, this approach does not guarantee a positive definite covariance matrix. An alternative is to use a factor loading approach[3]. For each variable we can specify a loading onto a factor and a residual variance for each variable (type). This approach is determined by λ, a $K \times F$ matrix where k_i are variables and f_j are latent factors, and ψ, a vector of length K defining residual variation.

$$
\begin{aligned}
\Sigma_{\text{raw}} &= \lambda \times \lambda^T + \text{diag } (\psi) \\
D &= \sqrt{\text{diag } (\Sigma_{\text{raw}})} \\
R &= D^{-1} \times \Sigma_{\text{raw}} \times D^{-1} \\
S &= \text{diag } (\sigma_k) \\
\Sigma &= S \times R \times S
\end{aligned}
\tag{4.5}
$$

This approach is used in `sim_type2` where Σ is built from the factor approach. Lambda is a matrix with K rows, representing variable types and F columns representing factors.

```
build_sigma_factor <- function(Lambda, psi, S) {
  K <- nrow(Lambda)
  R_raw <- Lambda %*% t(Lambda) + diag(psi)
  d <- sqrt(diag(R_raw))
  R <- R_raw / outer(d, d)
```

[3]See here for an introduction on the maths.

```
  diag(S) %*% R %*% diag(S)
}
```

As an example, $K = 5$ variable types are defined using $F = 3$ factors below. Approximate "variance explained" can be used to define the relationship between variables and factors where for k_1 we aim for 50%, 20% and 10% explained by f_1 to f_3 and the remaining 20% is error. The squared loadings plus error sum to 1: $0.5 + 0.2 + 0.1 + 0.2 = 1$. In code:

```
# Loadings as proportions
l1 <- c(0.5,  0.2,  0.1)
# Transforms to loadings
l1 <- sqrt(l1)
error <- 0.2

sum(l1^2) + error
```

```
[1] 1
```

The above example first defines loadings as proportions (summing to 1 with errors) and then adds a step to transform the proportions to loadings. For a full $K \times F$ λ matrix we populate the matrix and use ψ for errors.

```
# Defining a matrix of proportions explained by factors
Lambda <- matrix(
  c(0.5, 0.2, 0.1,
    0.8, 0.1, 0.05,
    0.1, 0.15, 0.1,
    0.05, 0.05, 0.2,
    0.05, 0.1, 0.01
    ),
                ncol = 3,
                byrow = TRUE)
# Defining remaining error
psi <- c(0.2, 0.05, 0.65, 0.7, 0.84)

# Transform from proportions to loadings
Lambda <- sqrt(Lambda)

# Check total proportions
rowSums(Lambda^2) + psi
```

```
[1] 1 1 1 1 1
```

Each row of the λ matrix plus errors now sum to 1. Next, we can use the λ matrix to construct Σ with unit standard deviations. The full covariance matrix (Σ) is used in calling `MASS::mvrnorm` to produce the multivariate normal variable. As we are using standard deviations equal to 1, the covariance matrix equals the correlation matrix.

```
S <- rep(1, 5)
Sigma <- build_sigma_factor(Lambda, psi, S)

out <- MASS::mvrnorm(1000, rep(0, 5), Sigma = Sigma)

colnames(out) <- paste0("k", 1:5)
```

Table 4.4: Correlations used to simulate data built using a factor approach.

	k1	k2	k3	k4	k5
k1	1.000	0.845	0.497	0.400	0.331
k2	0.845	1.000	0.476	0.371	0.322
k3	0.497	0.476	1.000	0.299	0.225
k4	0.400	0.371	0.299	1.000	0.165
k5	0.331	0.322	0.225	0.165	1.000

Table 4.5: Correlations from simulated data (n=1000) built using a factor approach.

term	k1	k2	k3	k4	k5
k1	NA	0.849	0.536	0.378	0.314
k2	0.849	NA	0.515	0.335	0.346
k3	0.536	0.515	NA	0.323	0.249
k4	0.378	0.335	0.323	NA	0.163
k5	0.314	0.346	0.249	0.163	NA

In the loadings we defined k_1 and k_2 to be highly correlated through being influenced to a large degree by factors f_1 and f_2. In contrast, k_5 was given a lot of variation not explained by the factors which resulted in lower correlations with the other variables.

4.0.1.1 Heteroscedasticity and non-normal noise

The synthetic data-generating models described above produces homoscedastic data, i.e., equal residual variation between treatment groups. To create synthetic data under heteroscedastic conditions (unequal variance between treatment groups) an additional data generating model is created to enable setting treatment-specific variation (`sim_type3` function in `R/sim-fun.R`). The `sim_type3` function also includes two arguments to set additional noise in post-treatment measurements `prop_noise` controls the fraction of measurements that gets extra noise added and `sd_noise` controls the zero-centered normal distribution from which additional noise is drawn.

4.0.1.2 Missing data

Two mechanisms for including missing values are used in the simulation functions. First, a proportion of pre- and post-intervention values are set to missing, affecting single observations (e.g., a single measurement within a single participant). Second, the probability of missing observations from a whole participant is simulated using a Bernoulli model. This mechanism can connect baseline predictors to missing values. A Bernoulli simulation is performed, with p_i modeled linearly by α, which controls the average probability of complete missing observations, and the additional parameters β linked to predictors in a predictor data frame.

$$\begin{aligned} y_{\text{miss }[i]} &\sim \text{Bernoulli }(p_i) \\ \text{logit }(p_i) &= \alpha + \beta X \end{aligned} \tag{4.6}$$

Simulating a full (K = 8) data set of synthetic data, with missing values, results in observed patterns similar to the observed data (Figure 4.5, Figure 4.8 and Figure 4.9).

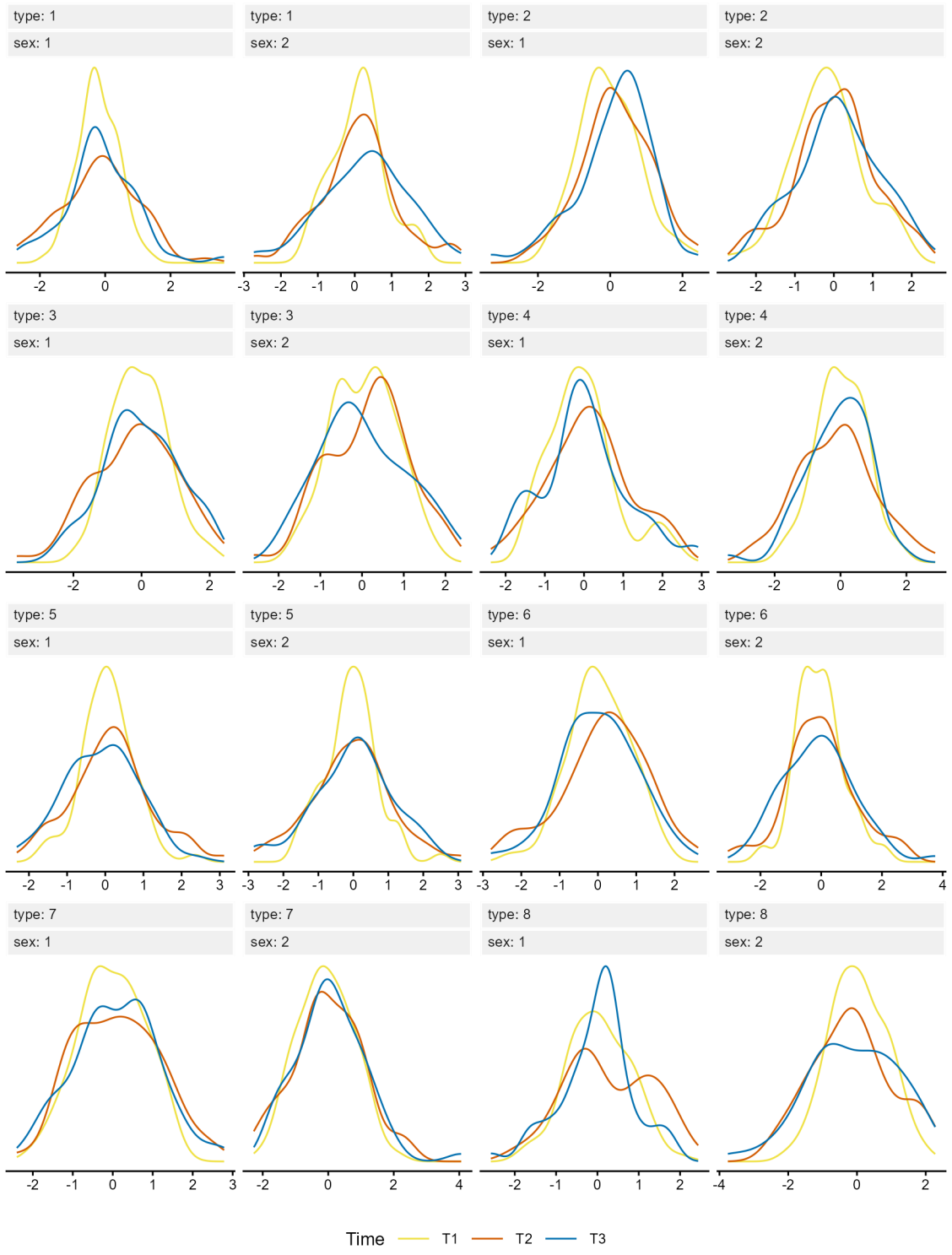

Figure 4.8: Distribution of (standardized) synthetic data across sex and measurement types.

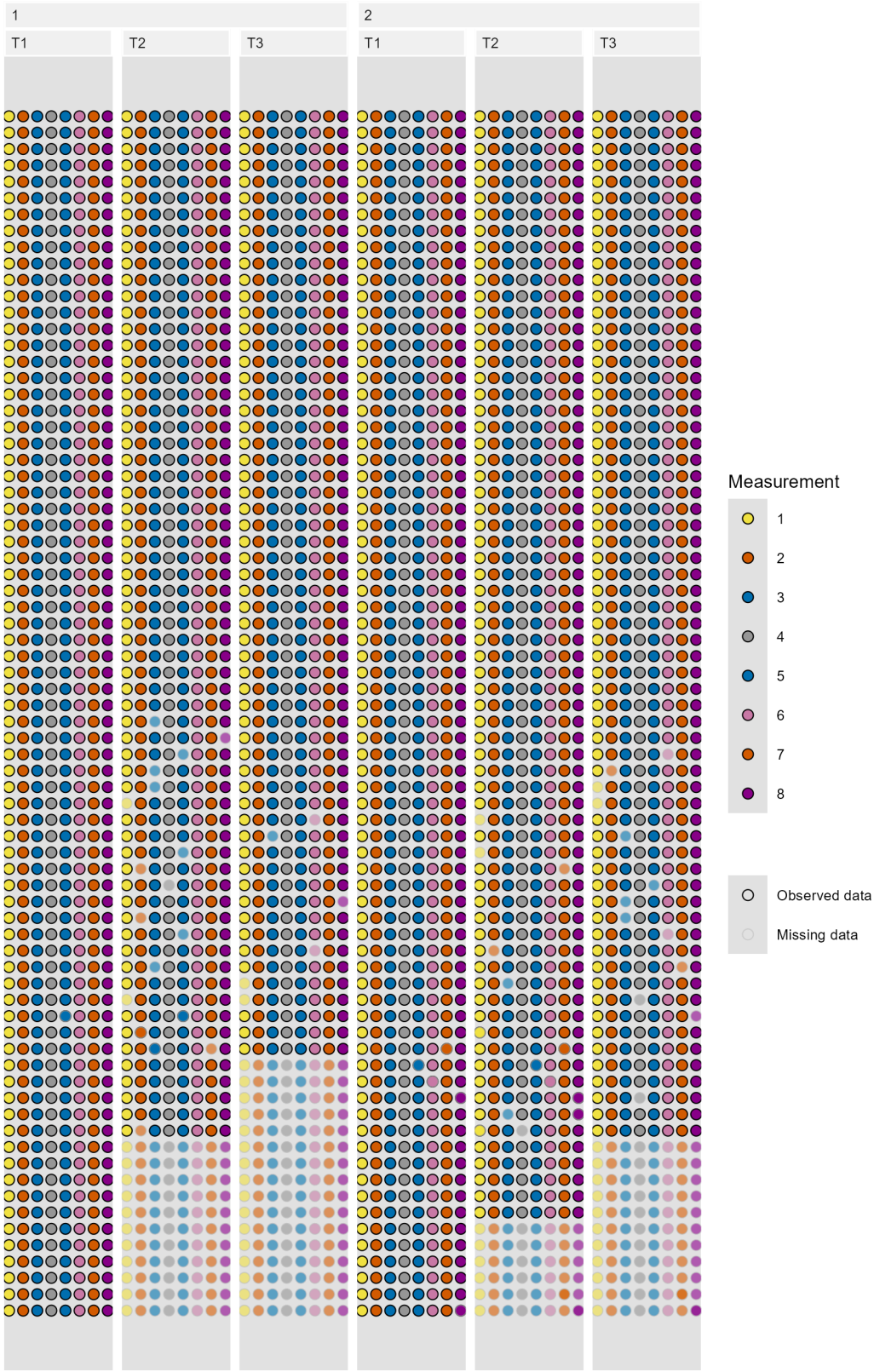

Figure 4.9: Missing data patterns from the data simulation. Missing values are found in specific measurements and in all measurements for a participant at specific time points.

5. Effects of krill oil supplementation on indications of metabolic syndrome - Models

Here models are incrementally developed by incorporating and testing features using prior predictive and posterior predictive checks, using both synthetic and observed data. Model performance (e.g., chain convergence and divergent transitions) is evaluated at relevant steps. The following chapter describes incremental model expansions towards a set of models that will be used to draw inference about the effect of krill oil supplementation on metabolic health.

The first model (model s1, `models/s1.stan`) is an ANCOVA-type model with pre-intervention values as covariates for estimating the post-intervention treatment- and measurement-type-specific means. The model is defined as:

$$
\begin{aligned}
y_i &\sim \text{Normal } (\mu_i, \sigma_i) \\
\mu_i &= \alpha_{\text{type,[TX]}} + \beta_{\text{type}} \times \text{pre}_i \\
\alpha_{\text{type,[TX]}} &\sim \text{Normal } (0, 1) \\
\beta_{\text{type}} &\sim \text{Normal } (1, 0.4) \\
\sigma_{\text{type}} &\sim \text{Exponential } (1)
\end{aligned}
\tag{5.1}
$$

Where $\alpha_{\text{type,TX}}$ is the measurement-type and treatment (TX) -specific intercept, and pre_i is the row-specific pre-treatment value.

Prior to modeling, observed values are placed on a standardized scale (mean-centered and scaled to the standard deviation). Since the model specification gives the same Normal $(0, 1)$ prior for both treatment groups, the implied prior for the differences between groups is Normal $(0, \sqrt{2})$. 95% of the prior probability mass therefore captures differences between groups from about −2.8 to 2.8. This prior is considered weakly regularizing. Similarly, the prior for the type-specific parameter for the pre-value covariates expects a 1-to-1 relationship between pre- and post-intervention values but with a wide range of possible relationships (95% prior probability mass in the range 0.2 to 1.8). The prior for the σ is also considered weakly regularizing allowing for a wide range of possible values but with more prior weight below the average (1).

The second model (model S2, `models/s2.stan`) for the main outcomes uses a multivariate normal likelihood. Here, we model outcomes as sampled from a multivariate normal distribution with a covariance matrix that connects the outcome types. This model extends model S1 by modelling the correlation structure among measurement types, enabling partial pooling across them. In further extensions of this model, the correlation structure will provide precision for imputing missing values, as observed scores will inform them through the structure. The same ANCOVA-type model is used here, in which pre-intervention scores serve as a covariate for the post-intervention group and measurement-type-level averages.

The model is defined as:

$$
\begin{aligned}
y_p &\sim \text{MVNormal}\ (\mu_p, \Sigma) \\
\mu_{p,\ \text{type}} &= \alpha_{\text{type,TX}} + \beta_{\text{type}} \times \text{pre}_{p,\ \text{type}} \\
\alpha_{\text{type,TX}} &\sim \text{Normal}\ (0, 1) \\
\beta_{\text{type}} &\sim \text{Normal}\ (1, 0.4) \\
\Sigma &= \text{S} \times \text{R} \times \text{S} \\
S &= \begin{bmatrix} \sigma_{\text{type}\ [1]} & 0 & .. \\ 0 & \sigma_{\text{type}\ [2]} & .. \\ .. & .. & .. \end{bmatrix} \\
R &= \begin{bmatrix} 1 & \rho_{12} & .. \\ \rho_{21} & 1 & .. \\ .. & .. & .. \end{bmatrix} \\
\sigma_{\text{type}} &\sim \text{Exponential}\ (1) \\
\text{R} &\sim \text{LKJcorr}\ (2)
\end{aligned}
\tag{5.2}
$$

Where y_p is a participant-level vector of outcome scores. Σ is the covariance matrix with standard deviations S and the correlation matrix R. The prior on R is defined using the LKJ distribution which provides regularization away from extreme values (-1, 1).

The third (model S3, `models/s3.stan`) includes all available data for one time-point, and imputes missing data. Imputation is done for both missing pre- and post-intervention observations. The pre-intervention imputation is done using a separate multivariate-normal model, defined as:

$$
\begin{aligned}
y^{\text{all}}_{\text{pre},p} &\sim \text{MVNormal}\ (\mu_{\text{pre},p}, \Sigma_{\text{pre}}) \\
y^{\text{all}}_{\text{pre},\ \text{type},p} &= \begin{cases} y^{\text{obs}}_{\text{pre},\ \text{type},p} \text{ if observed} \\ y^{\text{miss}}_{\text{pre},\ \text{type},p} \text{ if missing} \end{cases} \\
y^{\text{miss}}_{\text{pre},\ \text{type},p} &\sim \text{Normal}\ (0, 2)
\end{aligned}
\tag{5.3}
$$

Where missing values are treated as parameters and given a vague prior (Normal(0,2)), this makes sampling under prior-only more efficient.

The main model uses the $y^{\text{all}}_{\text{pre},p}$ as a covariate in the linear predictor. Again, missing values in the outcome are treated as parameters. The prior for missing observation is only included in prior predictive simulations.

$$
\begin{aligned}
y^{\text{all}}_{p} &\sim \text{MVNormal}\ (\mu_p, \Sigma) \\
y^{\text{all}}_{p,\ \text{type}} &= \begin{cases} y^{\text{obs}}_{p,\ \text{type}} \text{ if observed} \\ y^{\text{mis}}_{p,\ \text{type}} \text{ if missing} \end{cases} \\
y^{\text{miss}}_{\text{type},p} &\sim \text{Normal}\ (0, 2) \\
\text{pre}_{p,\ \text{type}} &= y^{\text{all}}_{\text{pre},p,\ \text{type}} \\
\mu_{p,\ \text{type}} &= \alpha_{\text{type,TX}} + \beta_{\text{type}} \times \text{pre}_{p,\ \text{type}} \\
\alpha_{\text{type,TX}} &\sim \text{Normal}\ (0, 1) \\
\beta_{\text{type}} &\sim \text{Normal}\ (1, 0.4) \\
\Sigma &= \text{S} \times \text{R} \times \text{S} \\
S &= \begin{bmatrix} \sigma_{\text{type}\ [1]} & 0 & .. \\ 0 & \sigma_{\text{type}\ [2]} & .. \\ .. & .. & .. \end{bmatrix} \\
R &= \begin{bmatrix} 1 & \rho_{12} & .. \\ \rho_{21} & 1 & .. \\ .. & .. & .. \end{bmatrix} \\
\sigma_{\text{type}} &\sim \text{Exponential}\ (1) \\
\text{R} &\sim \text{LKJcorr}\ (2)
\end{aligned}
\tag{5.4}
$$

Model S4 (`models/s4.stan`) adds sex as a covariate with the intention to improve predictions. The relevant addition to model s3 is in the linear predictor:

$$\begin{aligned} \mu_{p,\text{ type}} &= \alpha_{\text{type,TX}} + \beta_{\text{type}} \times \text{pre}_{p,\text{ type}} + \beta_{\text{sex}} \times \text{sex}_p \\ \beta_{\text{type,sex}} &\sim \text{Normal }(0, 0.5) \end{aligned} \tag{5.5}$$

The sex_p variable is created as male $= -0.5$ and female $= 0.5$ which gives that the prior for $\beta_{\text{type,sex}}$ expects that most of the difference between males and females in post-intervention scores is within ± 1 SD.

Model S5 (`models/s5.stan`) and S6 (`models/s6.stan`) further extends previous models by including both post-intervention time-points. In model S5 residuals are not correlated between time-points. Instead, separate covariance matrices are built for each time-point. The covariance matrices are therefore

$$\begin{aligned} \Sigma_{\text{time}} &= \text{S}_{\text{time}} \times \text{R}_{\text{time}} \times \text{S}_{\text{time}} \\ S_{\text{time}} &= \begin{bmatrix} \sigma_{1\text{ [time]}} & 0 & .. \\ 0 & \sigma_{2\text{ [time]}} & .. \\ .. & .. & .. \end{bmatrix} \\ R_{\text{time}} &= \begin{bmatrix} 1 & \rho_{12\text{ [time]}} & .. \\ \rho_{21\text{ [time]}} & 1 & .. \\ .. & .. & .. \end{bmatrix} \\ \sigma_{\text{type [time]}} &\sim \text{Exponential }(1) \\ \text{R}_{\text{time}} &\sim \text{LKJcorr }(2) \end{aligned} \tag{5.6}$$

The total number of parameters in each Σ is 36 for 8 measurement types.

Model S6 includes a full covariance matrix for the multivariate normal model. The total number of parameters in the matrix is 136 for 8 measurements with σ indexed 1 to 16. For S:

$$S = \begin{bmatrix} \sigma_1 & 0 & \cdots \\ 0 & \cdots & \cdots \\ \cdots & \cdots & \sigma_{2K} \end{bmatrix} \tag{5.7}$$

Finally, model S7 (`models/s7.stan`) extends model S1 by including both post-intervention time-points and missing data imputation. No correlation structures are included in the model and parameters (σ, β α are time- and type-specific).

5.0.1 Prior predictive checks, preliminary models

Prior predictive checks are performed for each model to confirm expected relationships between the pre-intervention covariate (Figure 5.1 and Figure 5.2), and prior expectations of treatment means (Figure 5.3).

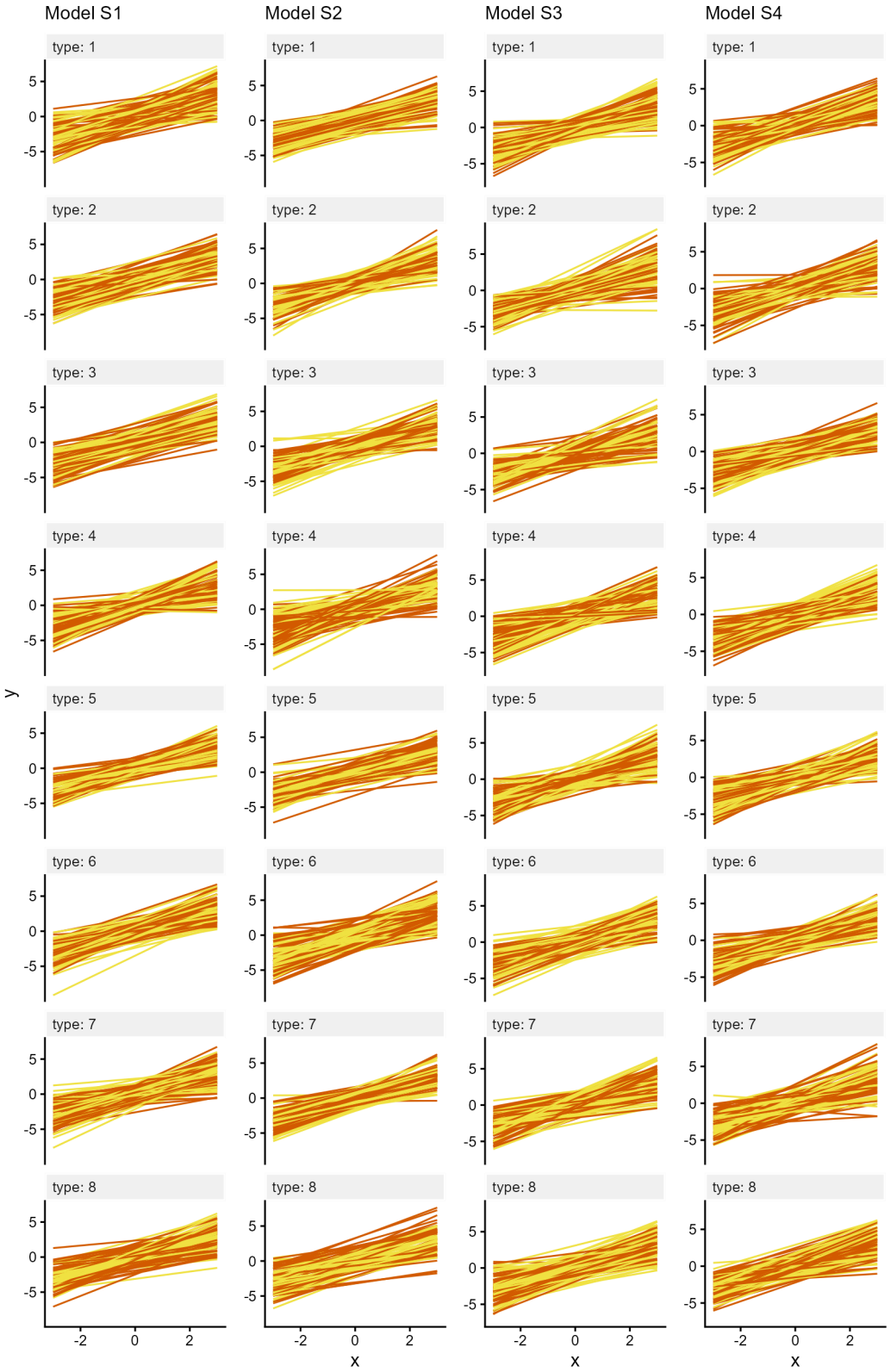

Figure 5.1: Prior predictive check of pre- to post-intervention associations in model S1-S4.

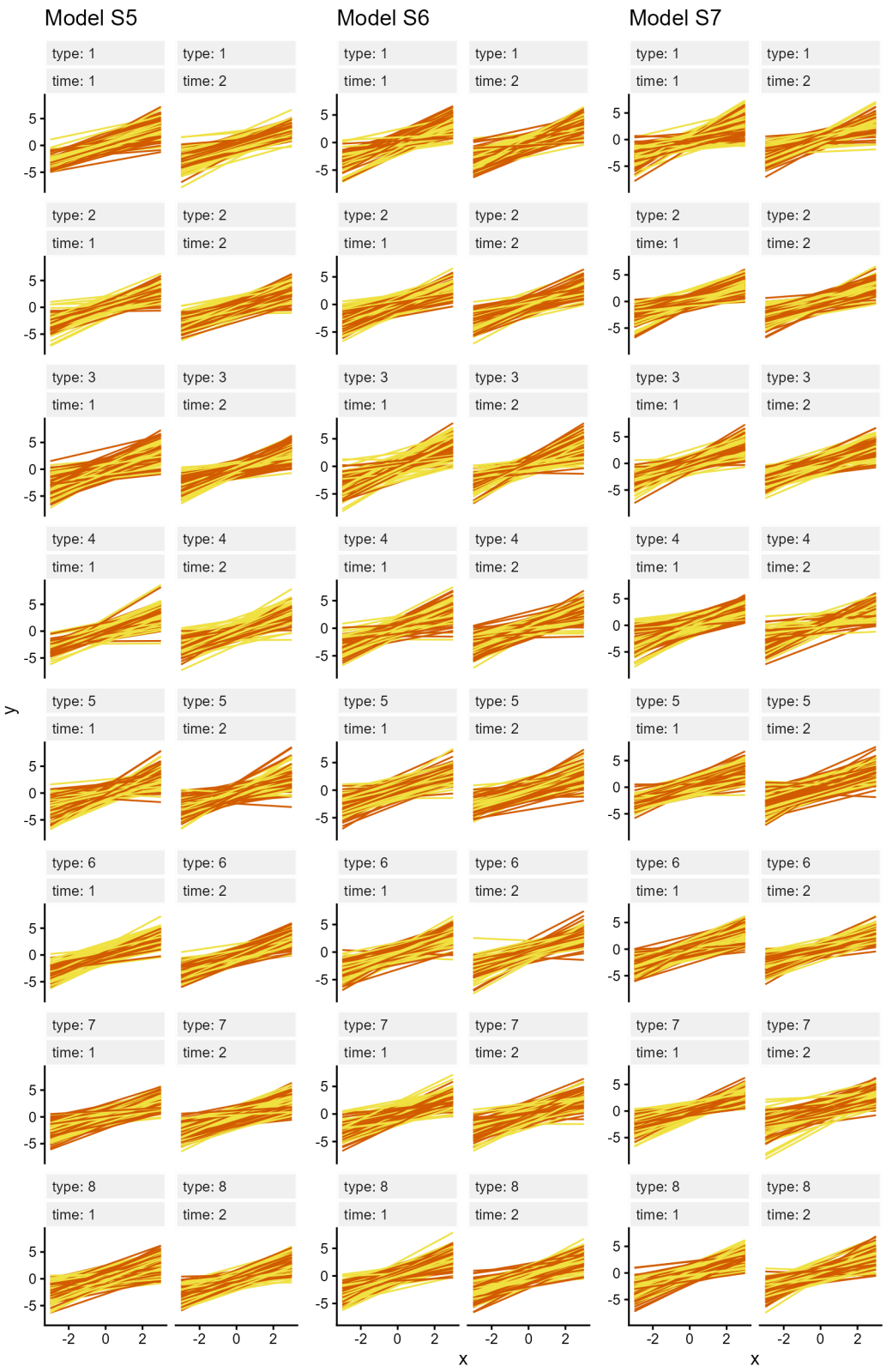

Figure 5.2: Prior predictive check of pre- to post-intervention associations in model S5-S7.

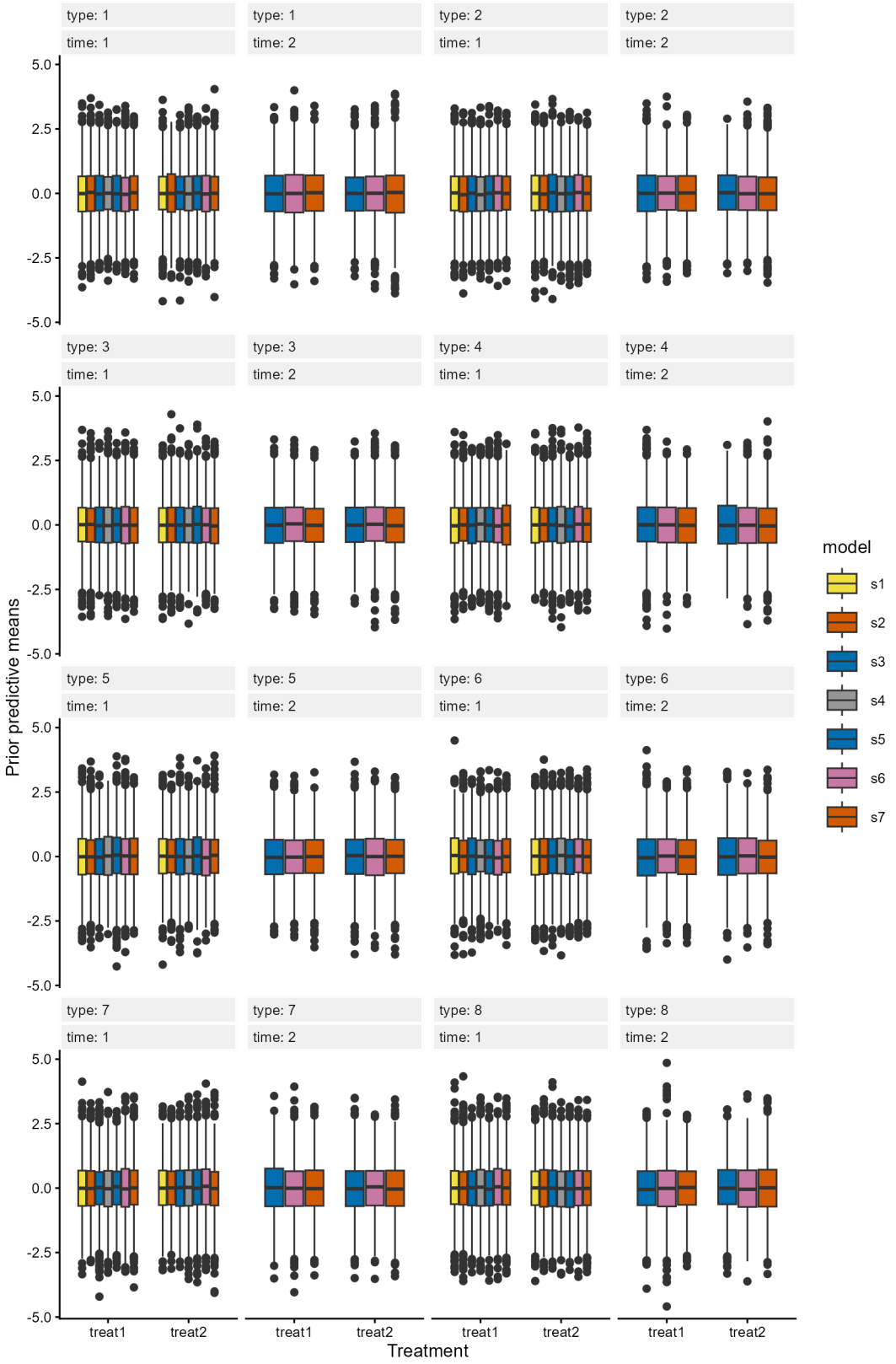

Figure 5.3: Prior predictive distributions of treatment means in models S1-S7.

The prior predictive checks confirm working and comparable models. A further check of model performance is done using synthetic data. We assess if we can recover simulated effects in the model estimates,

we also assess the effects of the models' priors on "reversed" effects, i.e. effect of opposite direction in the two groups.

Here, a simulated data set is constructed where we fix the relationship between pre- and post-intervention values ($\beta_{pre} = 0.95$), residual variation is set to 0.2 and effect sizes of treatment effects are set to 0.5 in type 1 and 7, −0.5 in type 2 and 8, and 0 in all other types. The models capture these effects and this preliminary check does not suggest preference for one treatment over the other (Figure 5.4).

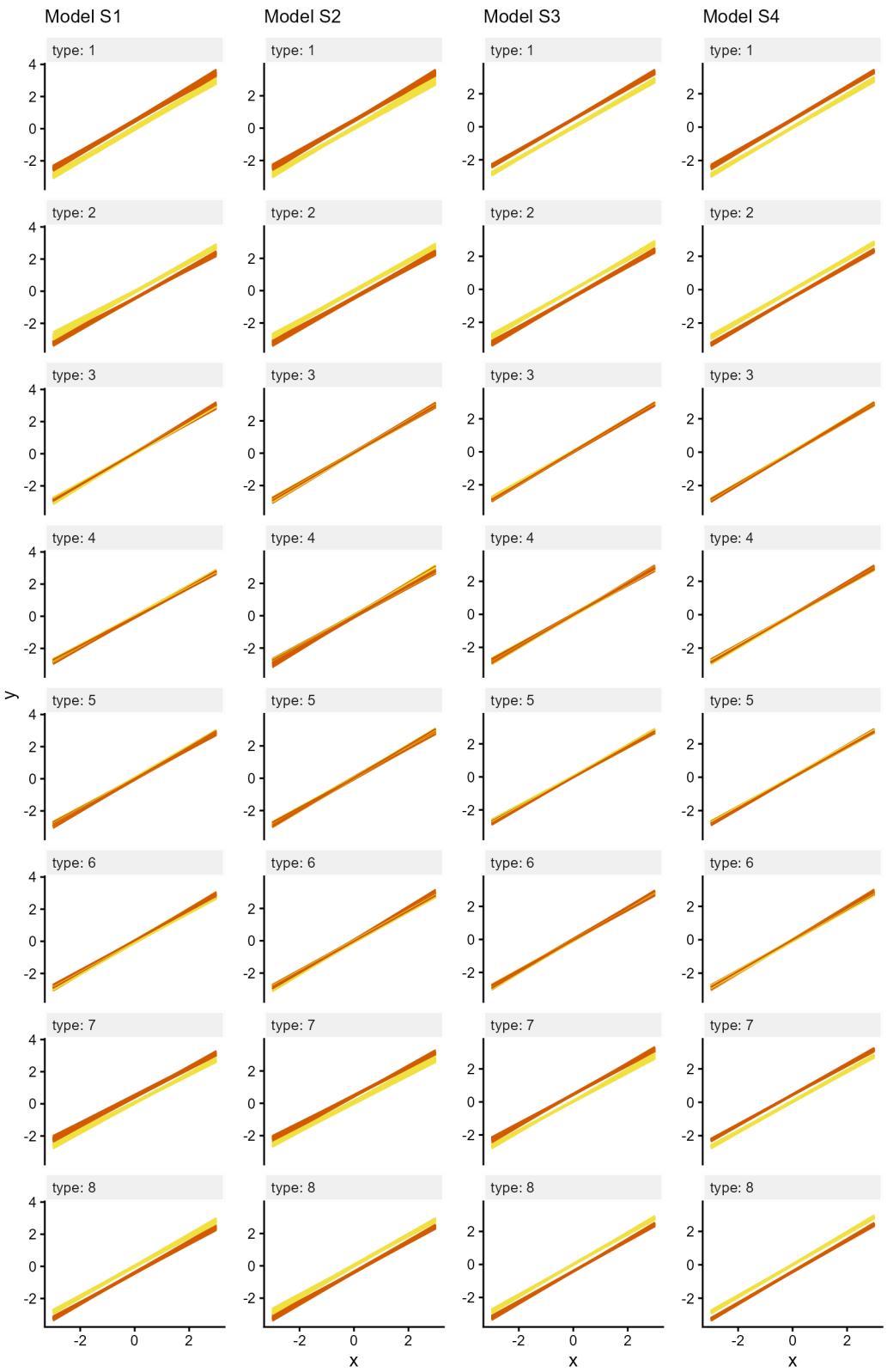

Figure 5.4: Posterior fits of pre- to post-intervention associations in models S1-S4 fitted to synthetic data.

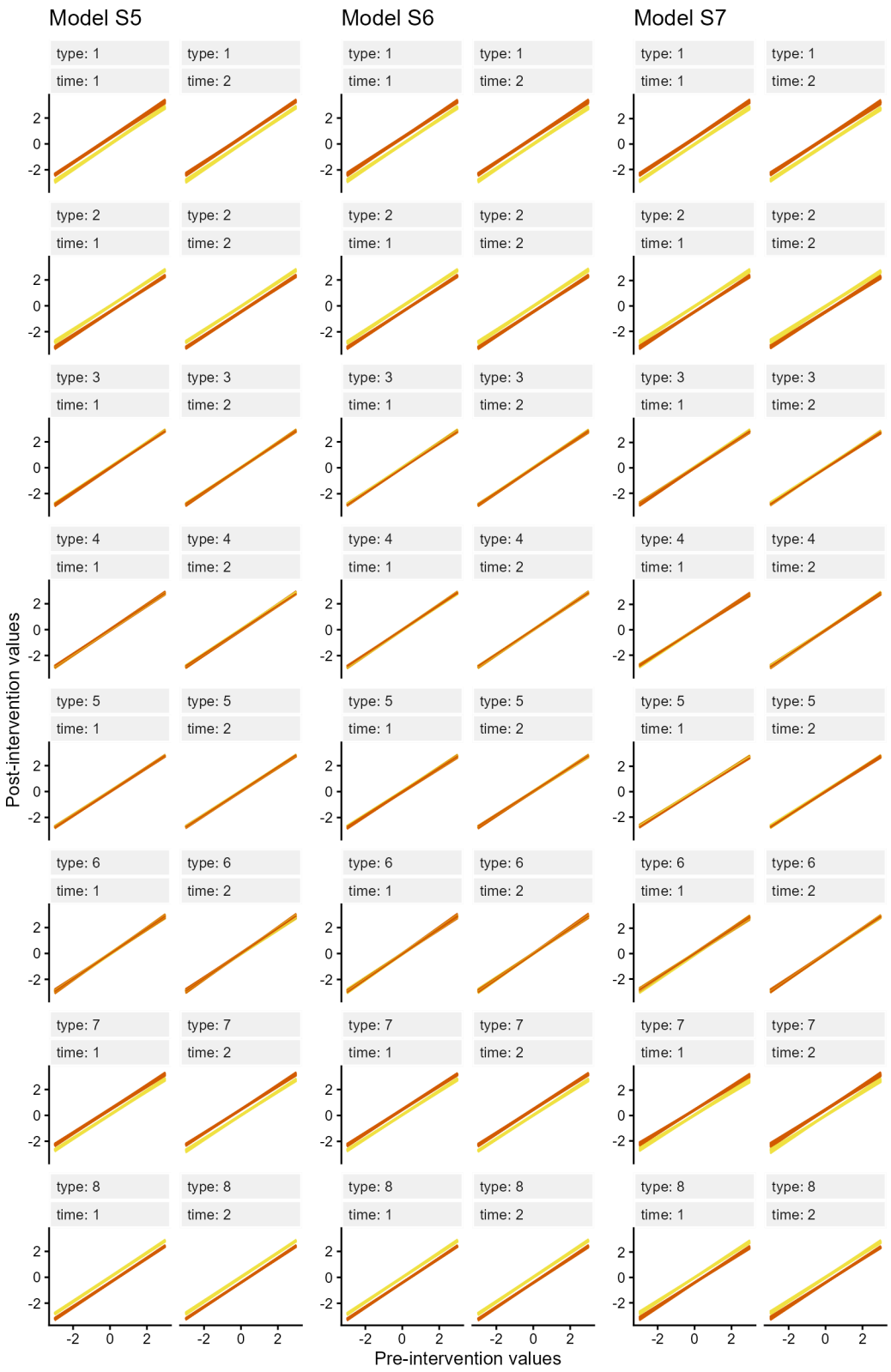

Figure 5.5: Posterior fits of pre- to post-intervention associations in models S5-S7 fitted to synthetic data.

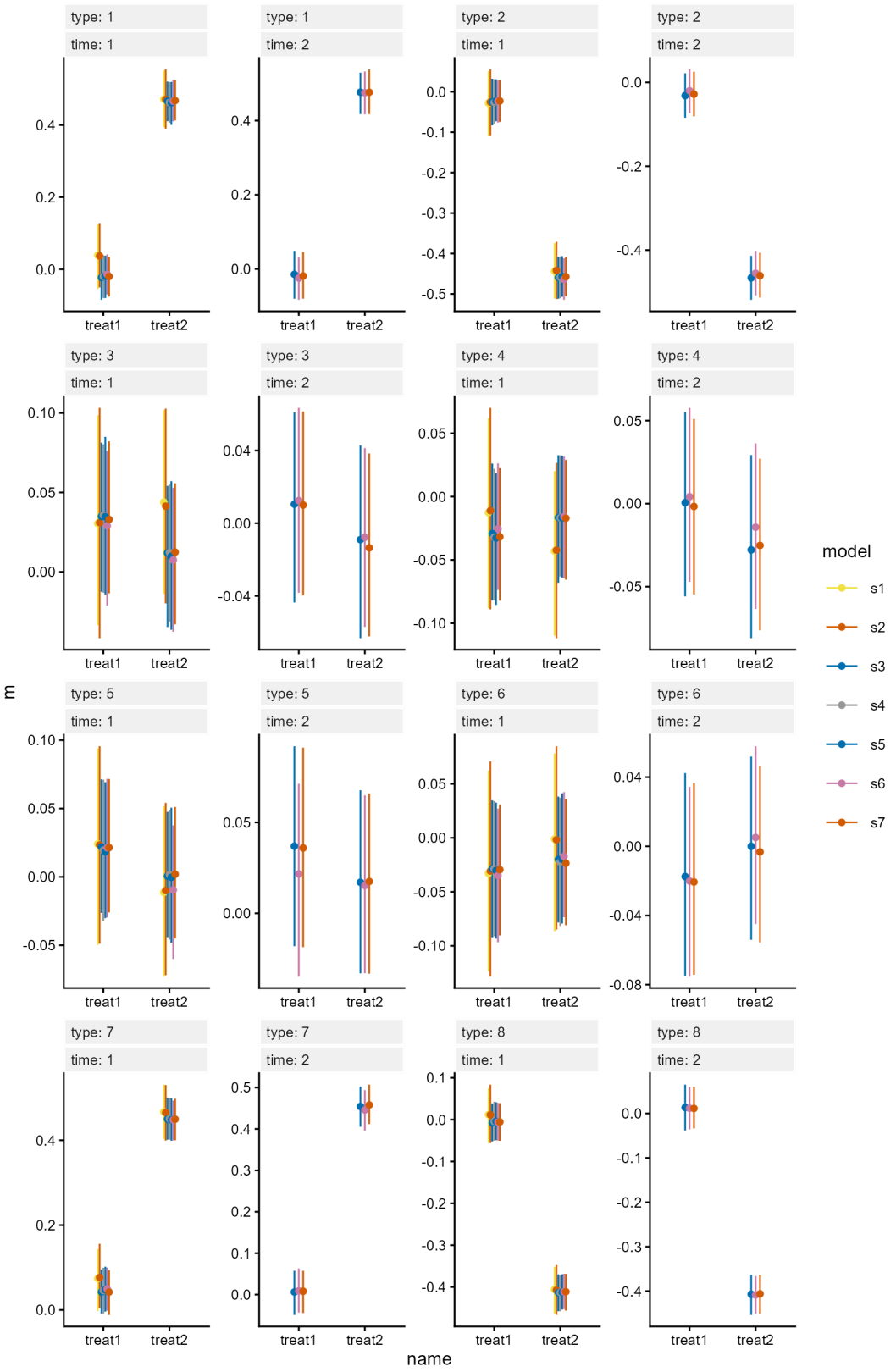

Figure 5.6: Posterior distributions of treatment means in models S1-S7 fitted to synthetic data. Points are posterior medians and error bars are 95% credible intervals.

Using the synthetic data we confirm that the incrementally developed models capture the same patterns when they use the same data. In the above test, synthetic data was generated with large effects and negligible correlation structures across measurement types. We should therefore not expect large differences between models that could potentially capture different between-measurement type correlations patterns.

Based on these preliminary models, model S6 and S7 are kept for further development and evaluation. Model S6 is replaced by S8 that will also generate diagnostic criteria thresholds. Additionally, to allow for treatment-specific variation we include treatment-specific σ by updating model S7 and S8 to S9 and S10, respectively. The relevant update in S7 to S9 is a type-, time- and treatment-specific σ ($\sigma_{\text{type, time, treat}} \sim \text{Exponential}(1)$. Similarly, model s10 adds type-, time- and treatment-specific σ to the model by incorporating it in the multivariate-normal model.

6. Effects of krill oil supplementation on indications of metabolic syndrome - Posterior predictive simulations and model expansions

Models S7 to S10 (Table 6.1) were next fitted with the observed data. Diagnostic statistics indicated that the HMC algorithm converges with satisfactory results (Table 6.2). Figure 6.1 shows per-parameter summaries of $\hat{R}$ and effective sample size indicating that parameters have $\hat{R} < 1.01$ and that bulk effective sample sizes are generally > 1000 per chain (minimum about 680 per chain, in model s9).

Table 6.1: Summary of model configurations

Model	Likelihood	Variance structure
`s7`	Univariate normal	Time- and type-specific σ
`s8`	Multivariate normal	Time- and type-specific σ
`s9`	Univariate normal	Time-, type-, and treat-specific σ
`s10`	Multivariate normal	Time-, type-, and treat-specific σ

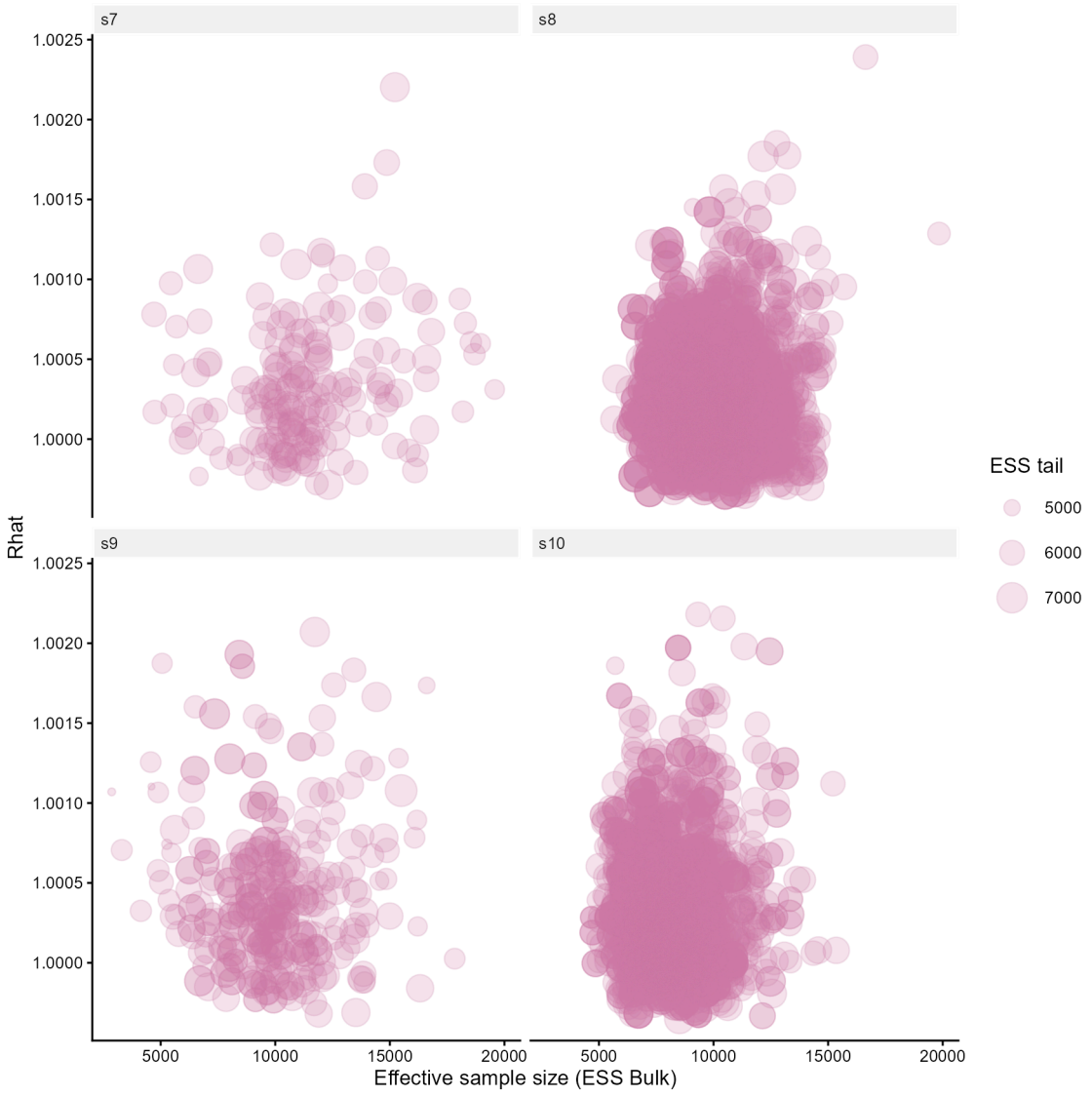

Figure 6.1: Summary chain statistics indicate satisfactory results in models s7-s10.

Table 6.2: Diagnostic summaries of models s7 to s10. BFMI, Bayesian Fraction of Missing Information.

Chain	n divergent trans.	n maximum treedepth	BFMI
		s7	
1	0	0	0.83
2	0	0	0.83
3	0	0	0.88
4	0	0	0.90
		s8	
1	0	0	0.74
2	0	0	0.74
3	0	0	0.85
4	0	0	0.80
		s9	
1	0	0	0.84
2	0	0	0.89
3	0	0	0.84
4	0	0	0.78
		s10	
1	0	0	0.87
2	0	0	0.84
3	0	0	0.77
4	0	0	0.75

Using the observed data set we are next assessing the models tendencies to produce replicated data similar to the observed data. All models (s7-s10) are still considered in these posterior predictive checks. The overall distributions of replicated data (y^{rep}) tend to look very similar to the observed data for each measurement type, treatment, and time-point combinations in all considered models (Figure 6.2, Figure 6.3, Figure 6.4, Figure 6.5).

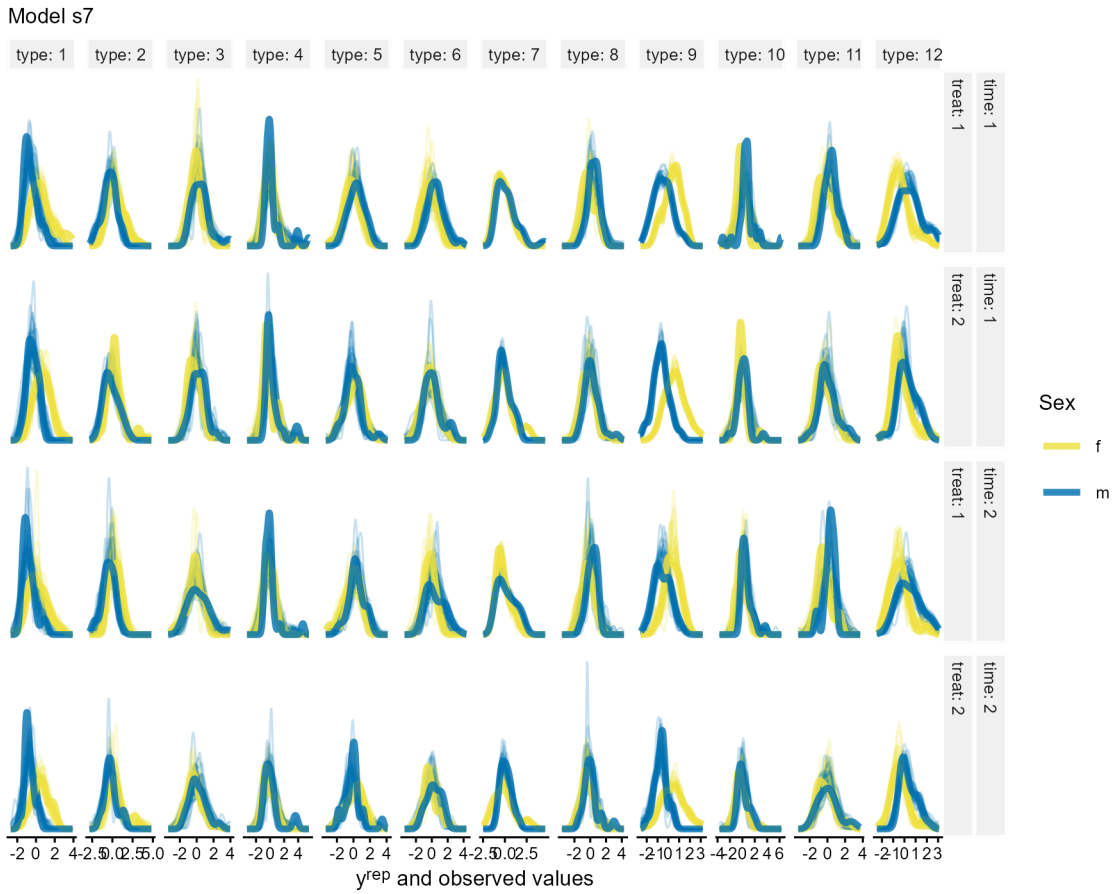

Figure 6.2: Posterior predictive check of model s7. y^{rep} (40 draws from the posterior) and observed data (thick line) indicate that the model replicates the observed data distributions.

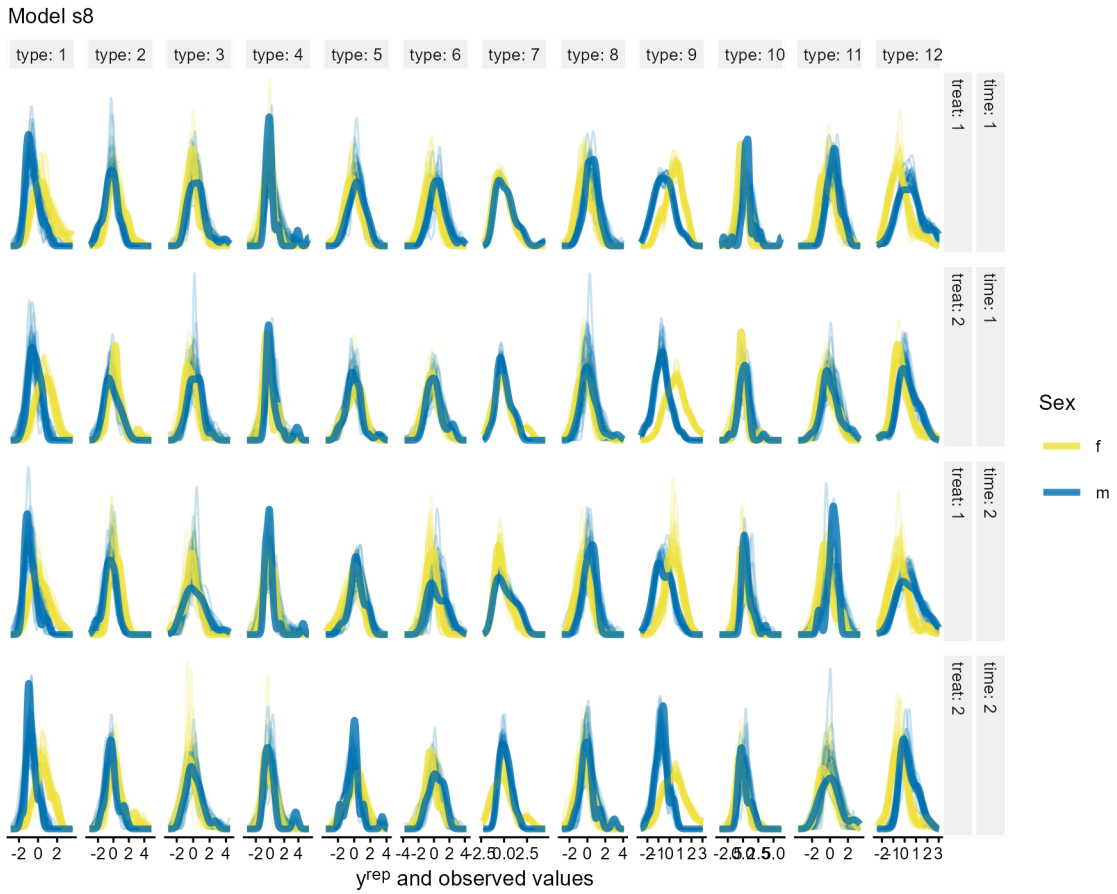

Figure 6.3: Posterior predictive check of model s8. y^{rep} (40 draws from the posterior) and observed data (thick line) indicate that the model replicates the observed data distributions.

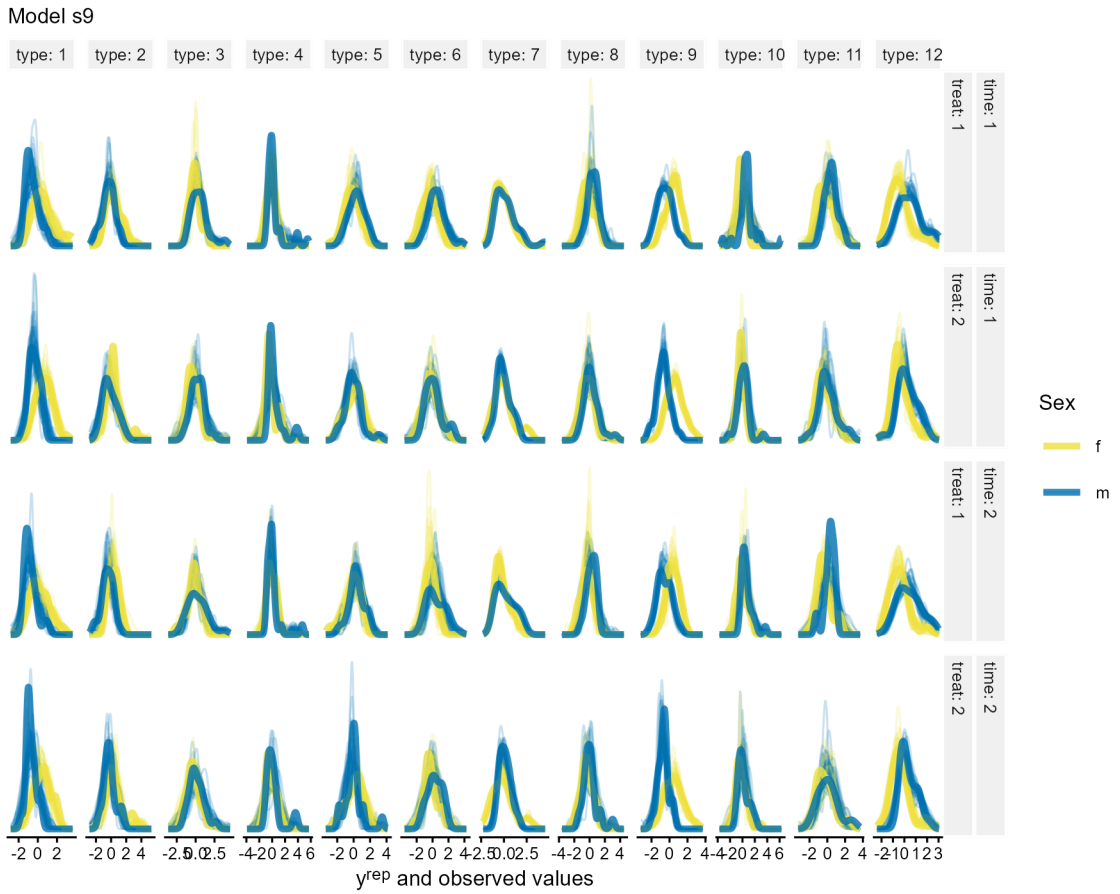

Figure 6.4: Posterior predictive check of model s9. y^{rep} (40 draws from the posterior) and observed data (thick line) indicate that the model replicates the observed data distributions.

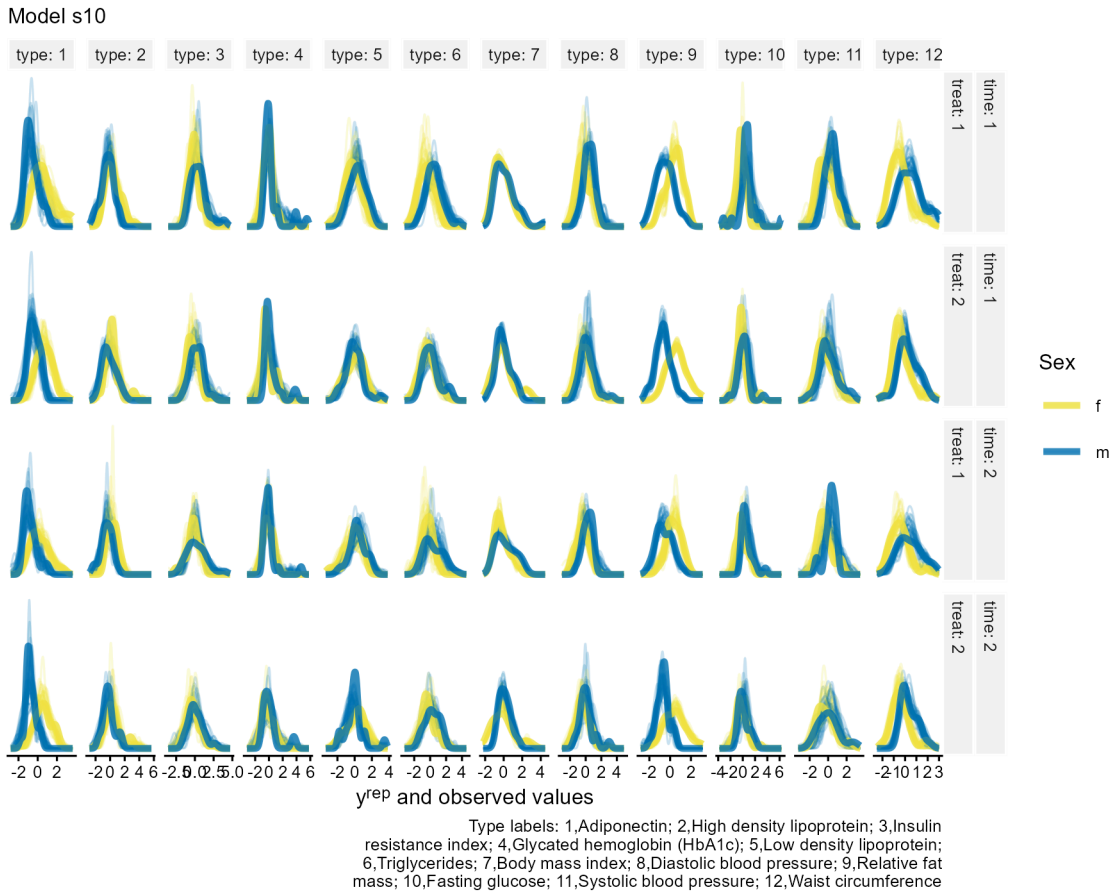

Figure 6.5: Posterior predictive check of model s10. y^{rep} (40 draws from the posterior) and observed data (thick line) indicate that the model replicates the observed data distributions.

To further explore model implications we calculate the observed standard deviation per measurement type and treatment and extract distributions implied by the model through y^{rep}. In Figure 6.6 and Figure 6.7 most distributions of y^{rep} standard deviations contain the empirical SD (shown as contrasts between y^{rep} and y^{obs} where 0 indicate agreement), however, for both model S7 and S8, some of the standard deviations observed in the data do not correspond to those implied by the model.

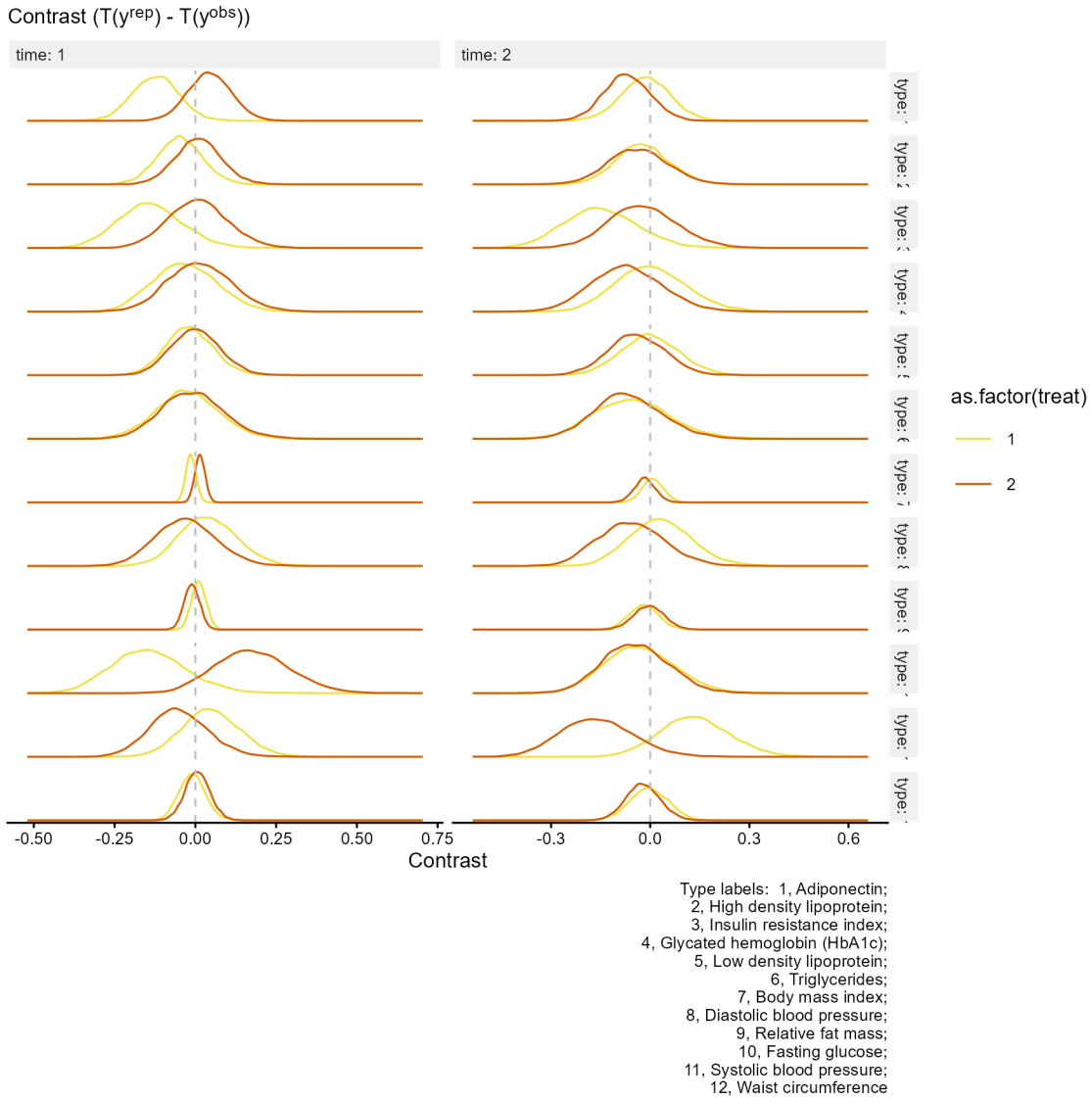

Figure 6.6: Posterior predictive check of model s7. The distributions of standard deviations calculated from y^{rep}

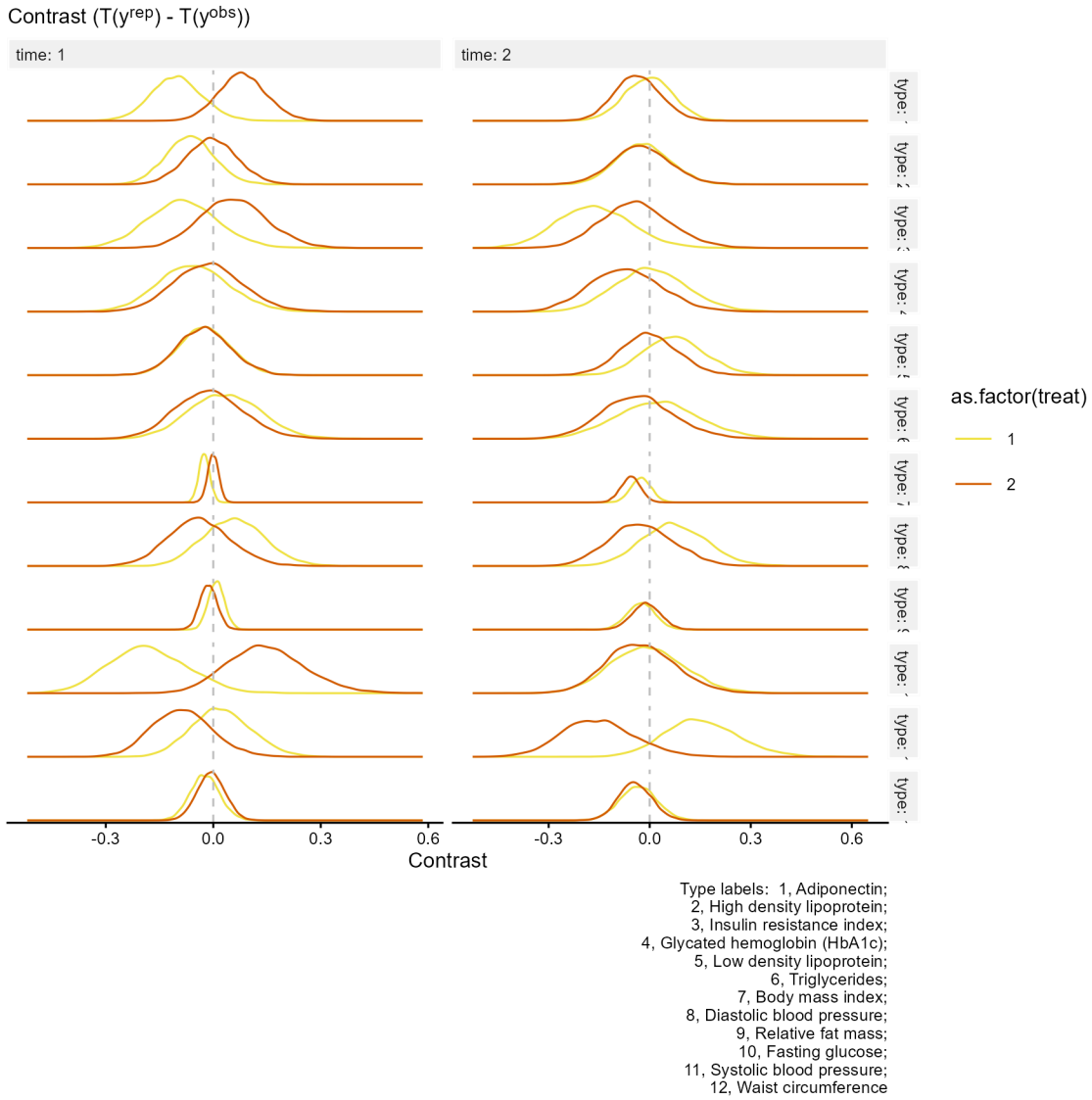

Figure 6.7: Posterior predictive check of model s8. The distributions of standard deviations calculated from y^{rep}

Allowing for the models to capture more heterogeneity in the data could improve the replicate data. Models s9 and s10 are displayed in Figure 6.8 and Figure 6.9

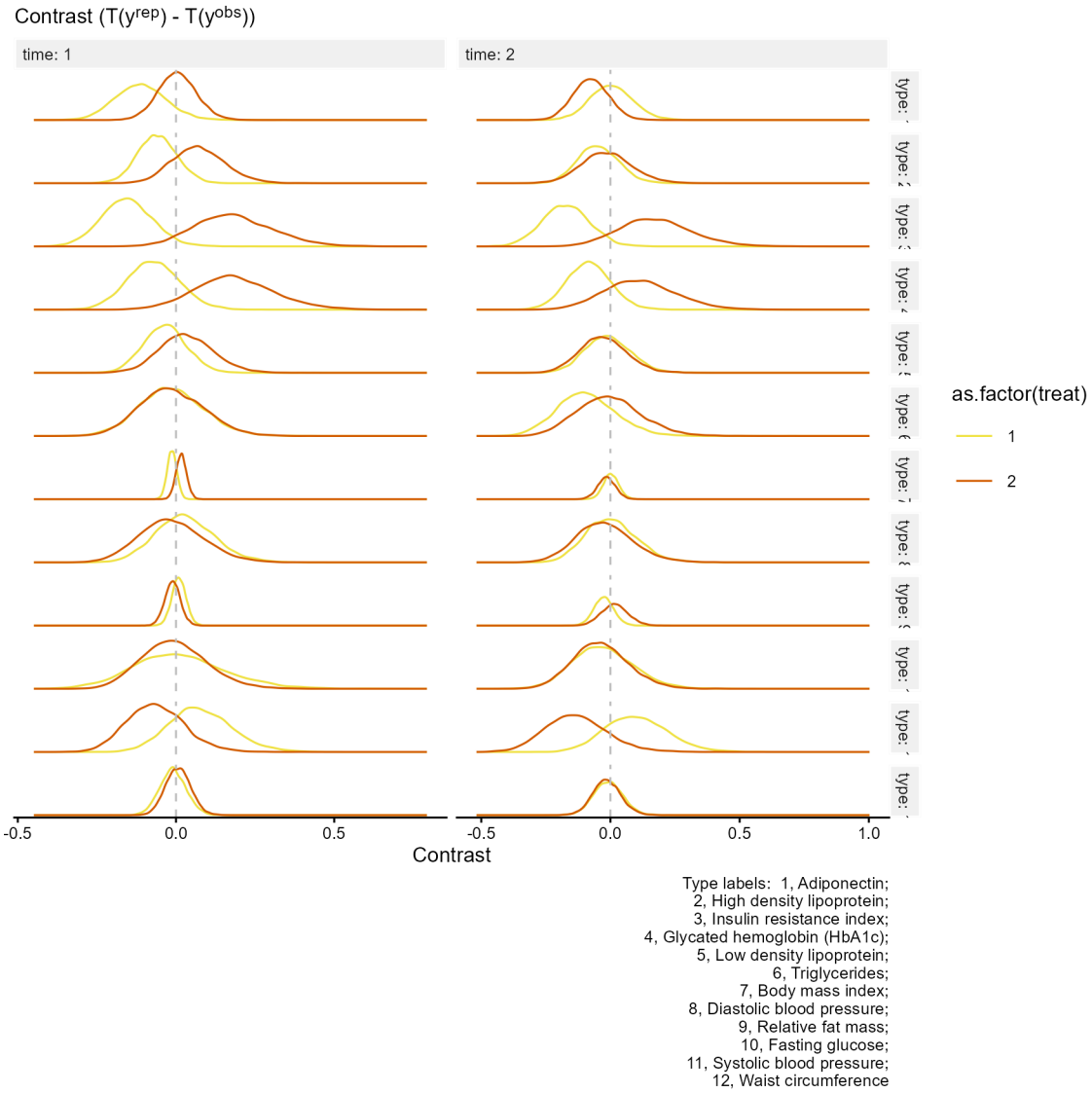

Figure 6.8: Posterior predictive check of model s9. The distributions of standard deviations calculated from y^{rep}

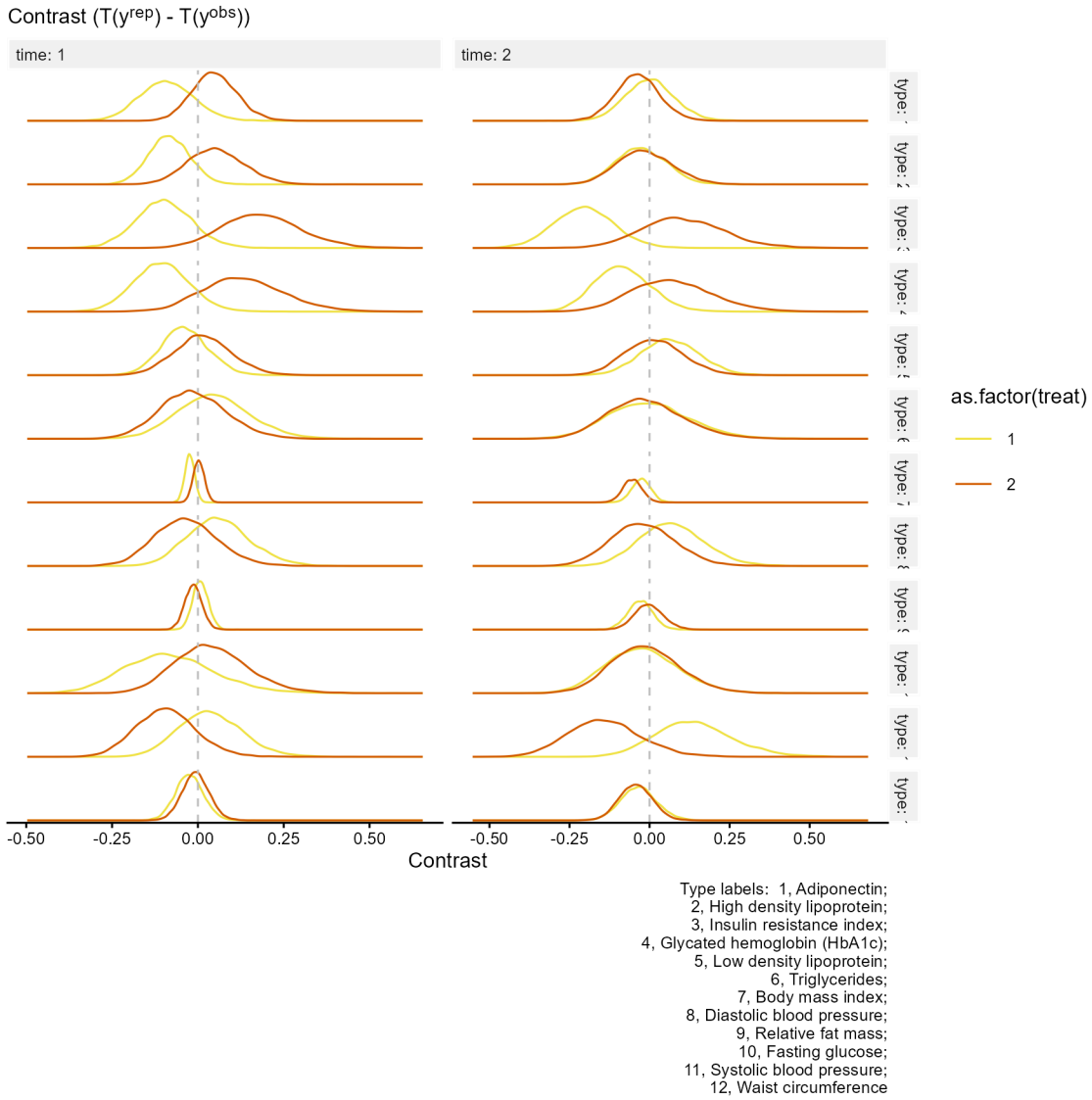

Figure 6.9: Posterior predictive check of model s10. The distributions of standard deviations calculated from y^{rep}

Combining all comparisons between replicated and observed data as p-values indicate that models s8 and s10 tend to have more examples of "extreme" observed data compared to model implications (p-values closer to 0 in Figure 6.10). No systematic under- or over-estimation is evident in any model.

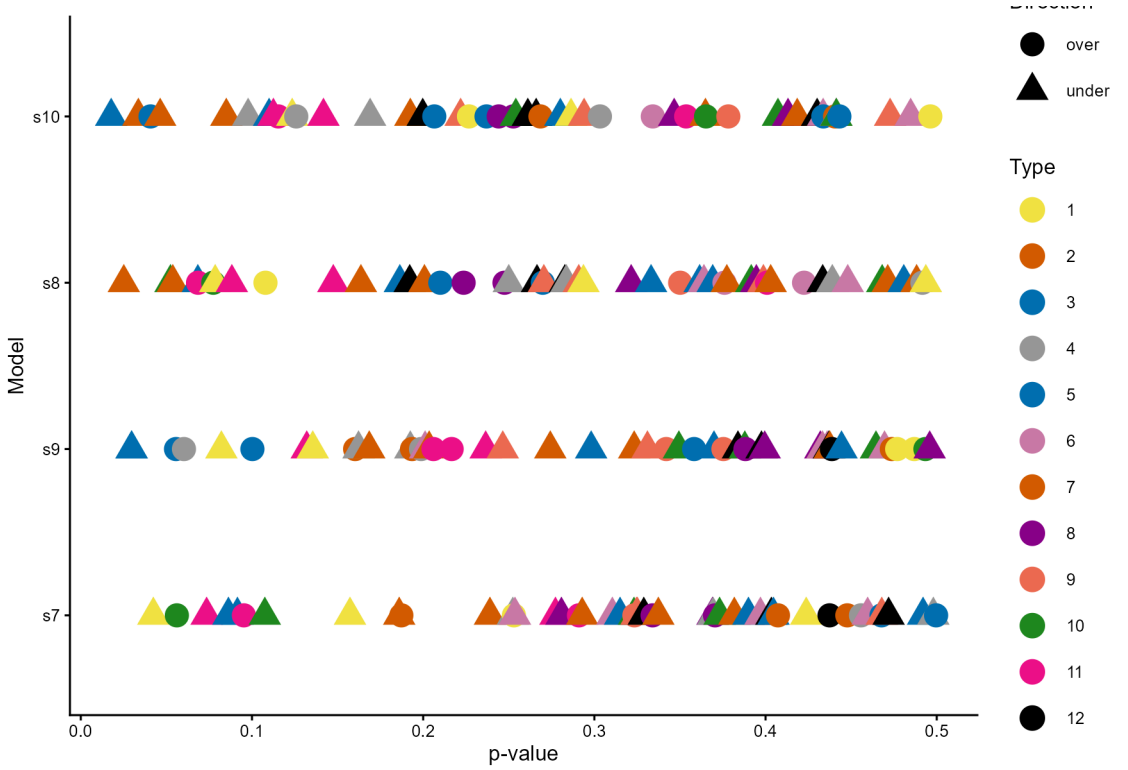

Figure 6.10: Comparing p-values from posterior-predictive checks with SD calculated as test statistics on replicated data compared to observed data.

Next we compute the approximate leave-one-out cross-validation statistics from the models using Pareto smoothed importance sampling (16). As a first step we only consider the Pareto k diagnostic values as these indirectly indicates observations that are influential for the models. Here, one observation corresponds to a participant, i.e., all values corresponding to a participant contribute to the log-likelihood. The multivariate models have more unstable estimates of the Pareto k statistic, indicating that these models are "more surprised" by single observations (Figure 6.11).

Together with the indications from above that suggest that the multivariate models (and more so the heteroscedastic variant, s10) tend to under or overestimate group-specific variation in replicate simulations we will consider a more robust definition of the two model classes (uni- and multivariate models).

Although high Pareto k values indicate unreliable estimates of the expected log-pointwise predictive density we finally compare models for their out-of-sample predictive performance (Table 6.3). These estimates indicate that multivariate models would predict better on new data with no differences between models with different residual variance structures.

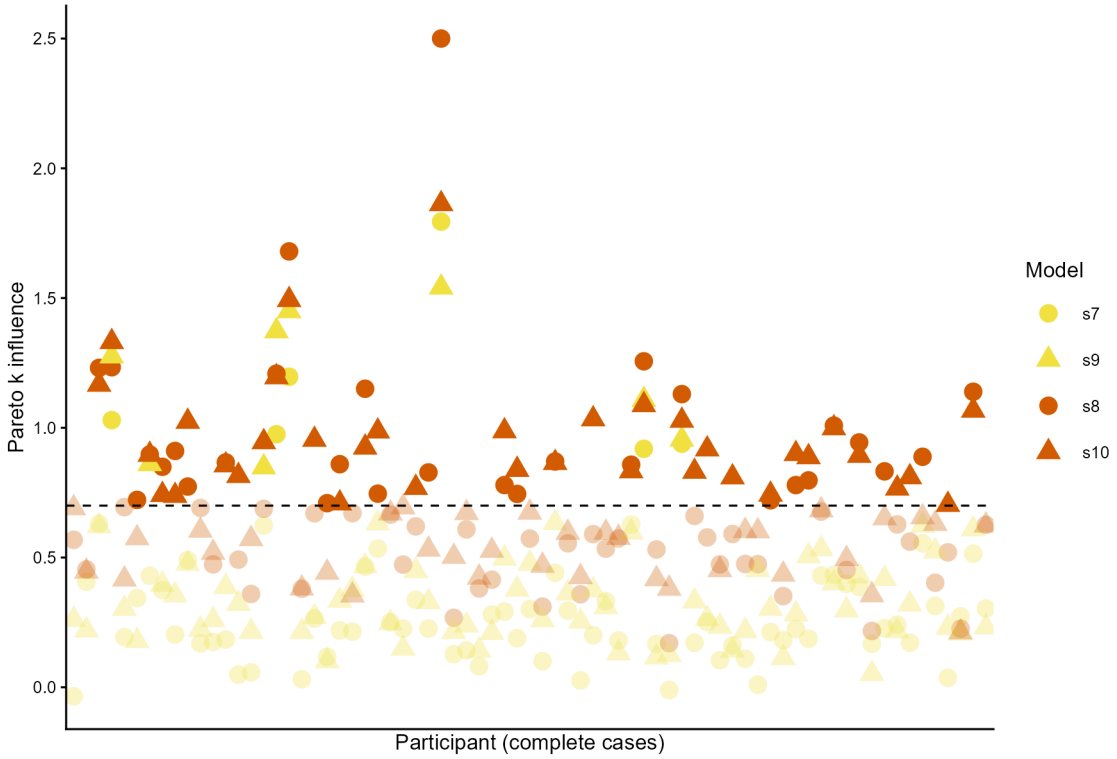

Figure 6.11: Pareto k statistic for participants with complete data in each model.

Table 6.3: ELPD estimate differences from models s7-s10.

Model	ELPD diff	SE
s8	0	0
s10	−6.563	9.177
s9	−284.724	28.589
s7	−298.182	32.969

6.0.1 Model expansion: Robust likelihood

As our initial models might suggest that they are "surprised" by specific data point we will consider a heavy-tailed likelihood to allow for inference robust to extreme observations. The Student's t-distribution is available both as a uni- and multivariate definition. Models s11 and s12 are updated from s9 and s10 to the Student-t versions of these model specifications. In model s11, the relevant change is in the definition of the likelihood ($y_i \sim \text{Student-t}\ (\mu_i, \sigma_i, \nu)$. The ν parameter is set to 4 (as pragmatic starting point, separating it from the normal likelihood (7)). In model s12 the likelihood is updated to a multivariate Student's t-distribution $y_i \sim \text{MVT}\ (\mu_i, \Sigma, \nu)$, also with ν set to 4.

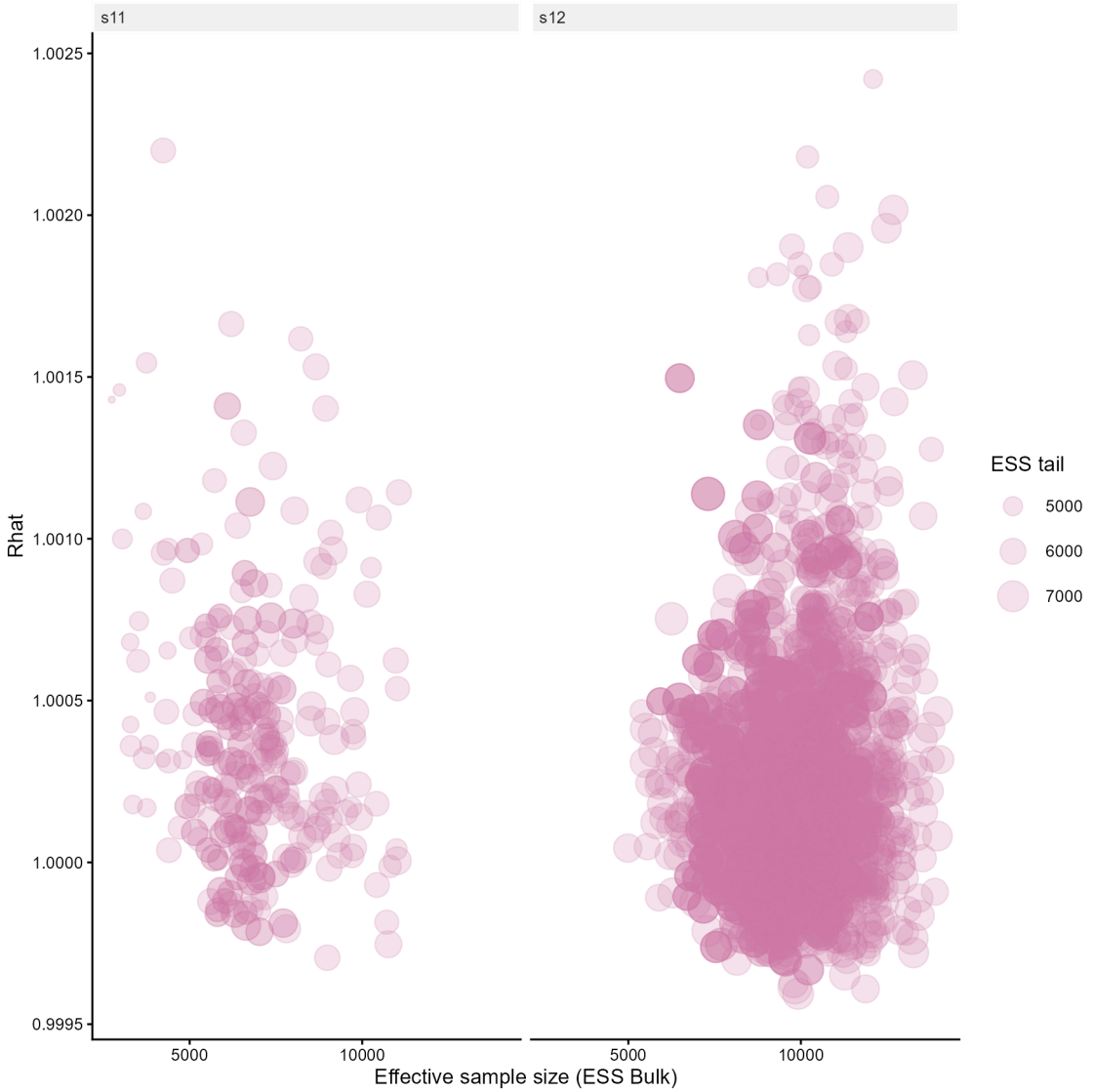

Figure 6.12: Summary chain statistics indicate satisfactory results in models s11 and s12.

Table 6.4: Diagnostic summaries of models s11 and s12. BFMI, Bayesian Fraction of Missing Information.

Chain	n divergent trans.	n maximum treedepth	BFMI
	s11		
1	0	0	0.64
2	0	0	0.60
3	0	0	0.64
4	0	0	0.64
	s12		
1	0	0	0.48
2	0	0	0.45
3	0	0	0.51
4	0	0	0.43

The chain diagnostics indicate that the HMC algorithm converges (Figure 6.12) and that no divergent transitions were evident along with healthy BFMI (Table 6.4; See here for light background on diagnostics). The plots of y^{rep} distributions against observed data distributions indicate that the models replicate the observed values (Figure 6.13, and Figure 6.14).

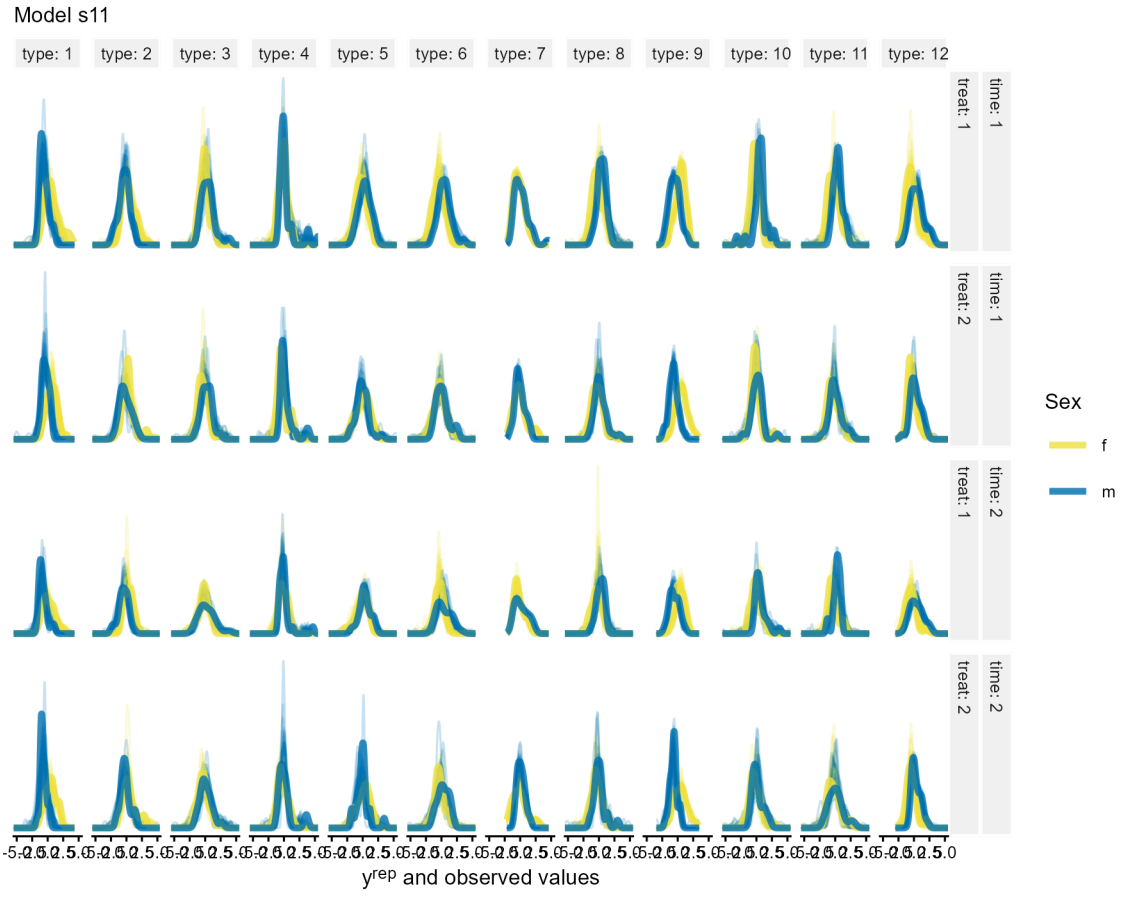

Figure 6.13: Posterior predictive check of model s11. y^{rep} (40 draws from the posterior) and observed data (thick line) indicate that the model replicates the observed data distributions.

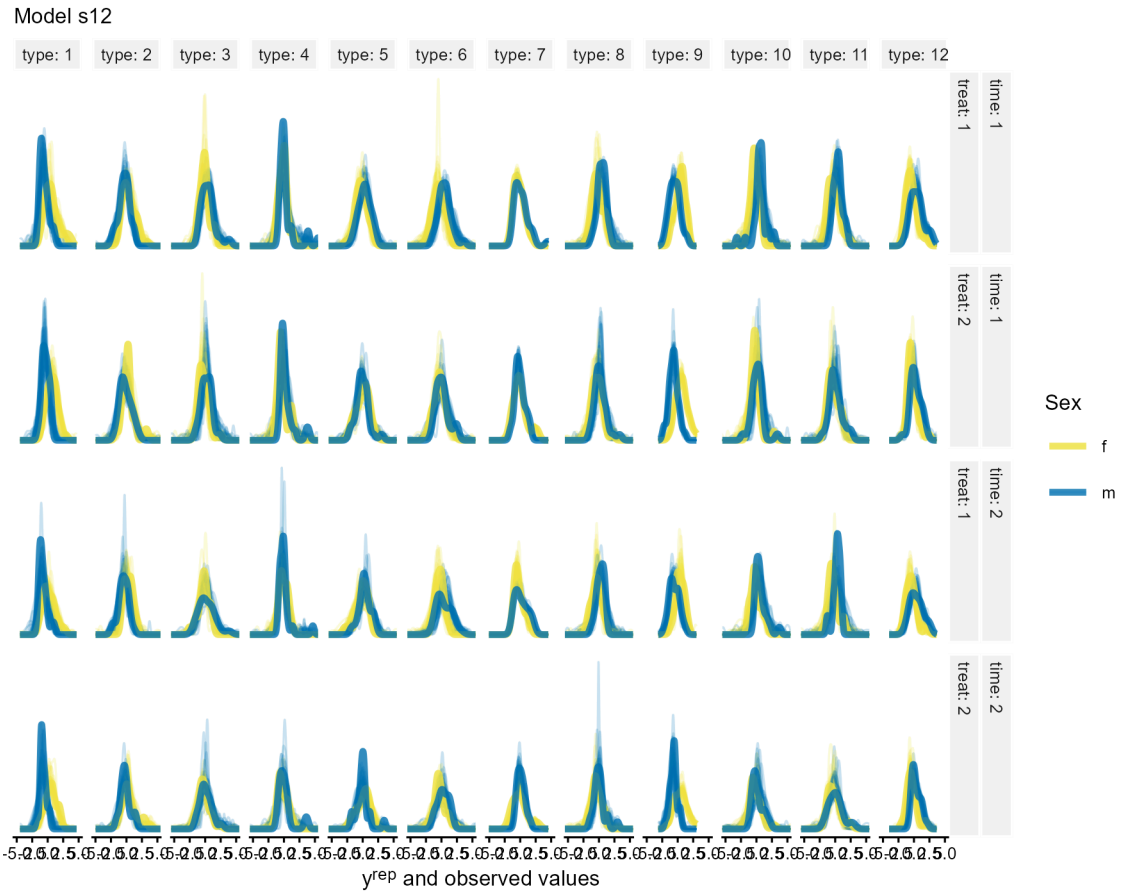

Figure 6.14: Posterior predictive check of model s12. y^{rep} (40 draws from the posterior) and observed data (thick line) indicate that the model replicates the observed data distributions.

No extreme deviations were seen from posterior predictive checks on the SD of replicated and observed data for either model (Figure 6.15 and Figure 6.16)

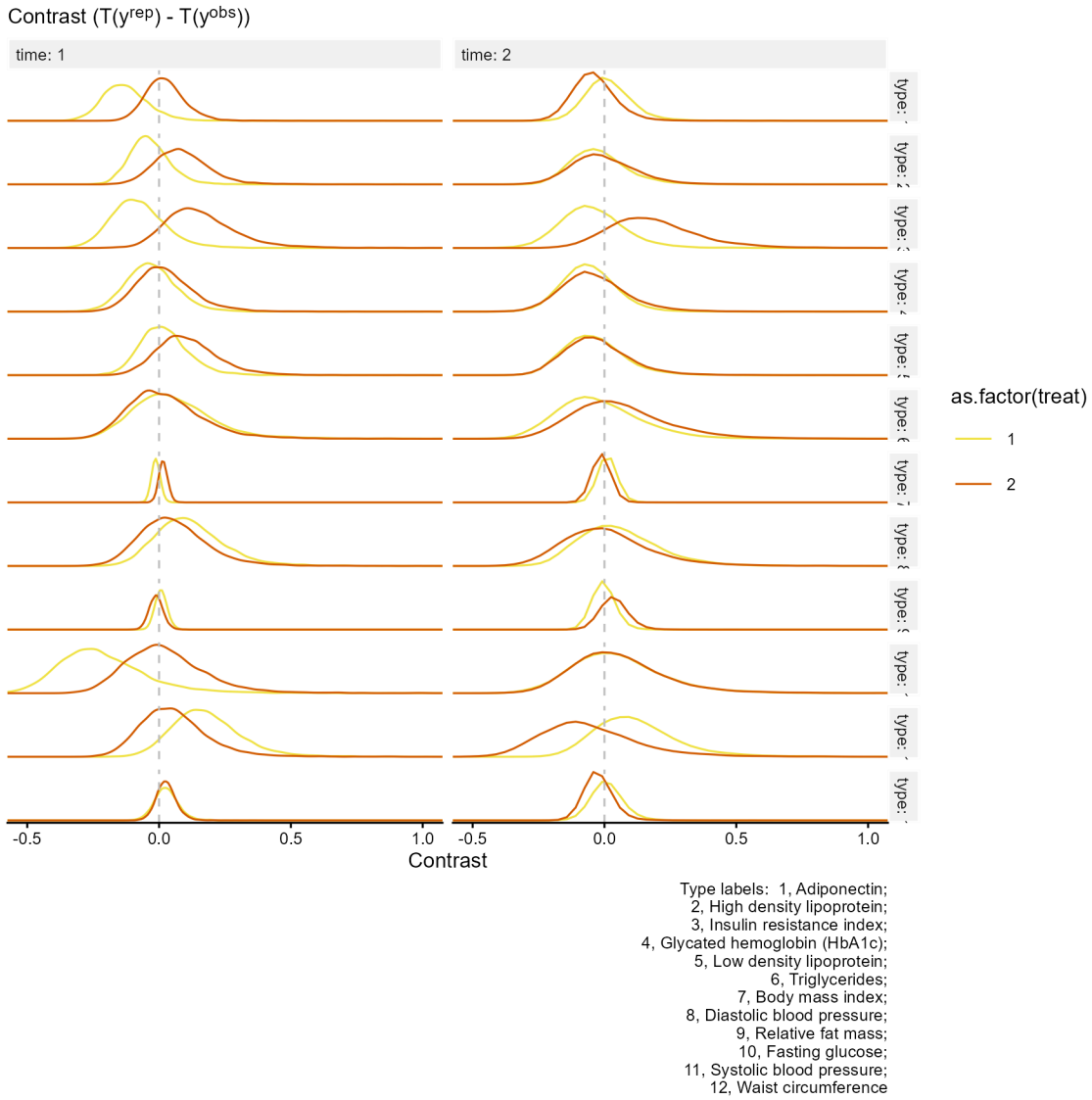

Figure 6.15: Posterior predictive check, SD from replicated data (y^{rep}) compared to observed from SD in the raw data for model s11.

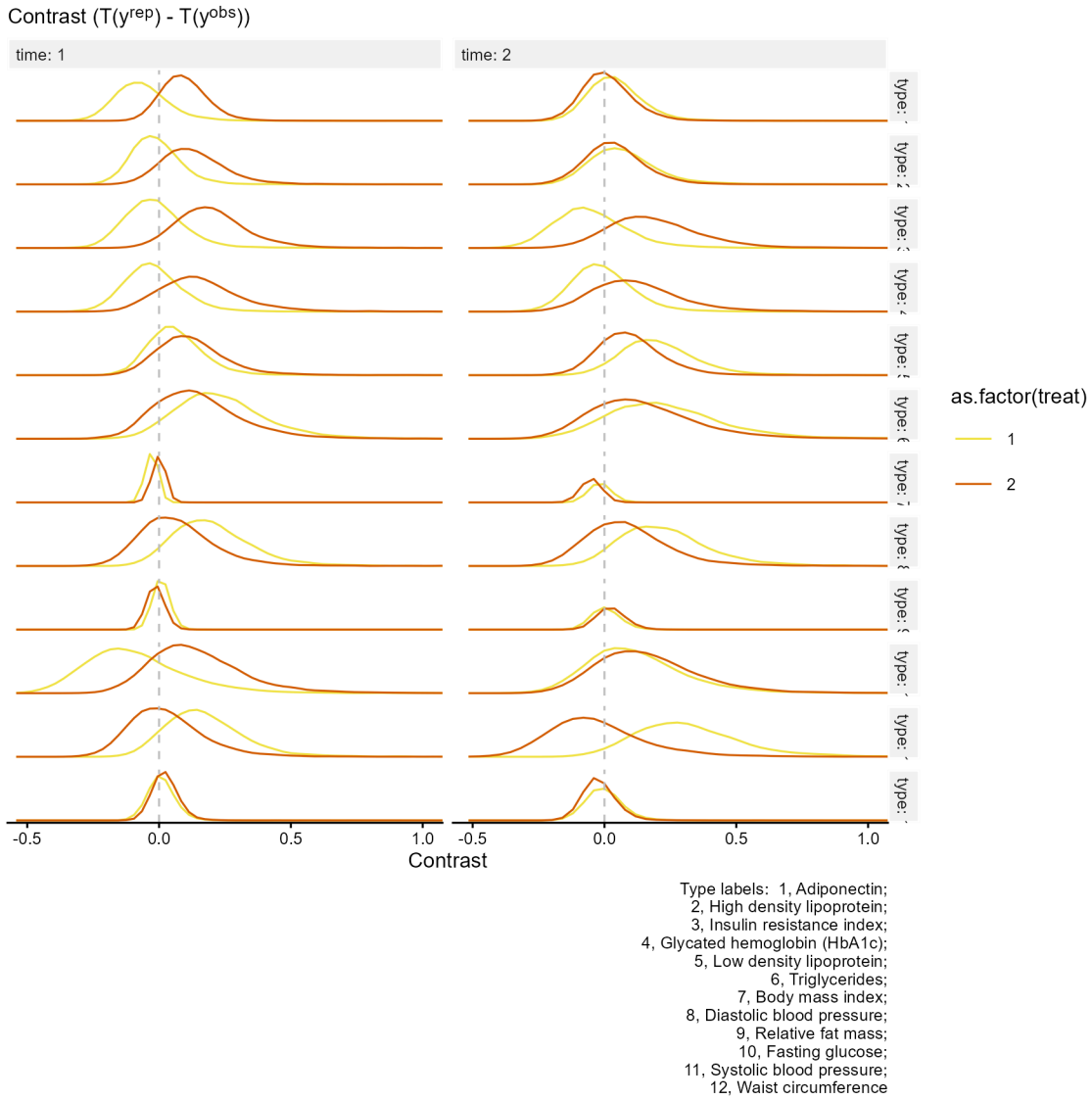

Figure 6.16: Posterior predictive check, SD from replicated data (y^{rep}) compared to observed from SD in the raw data for model s12.

Next we compare the approximate leave-one-out statistics. The univariate robust model had no unreliable Pareto K values but the multivariate variant did. However, although uncertain, the loo-comparison indicate a large advantage for the s12 model over s11.

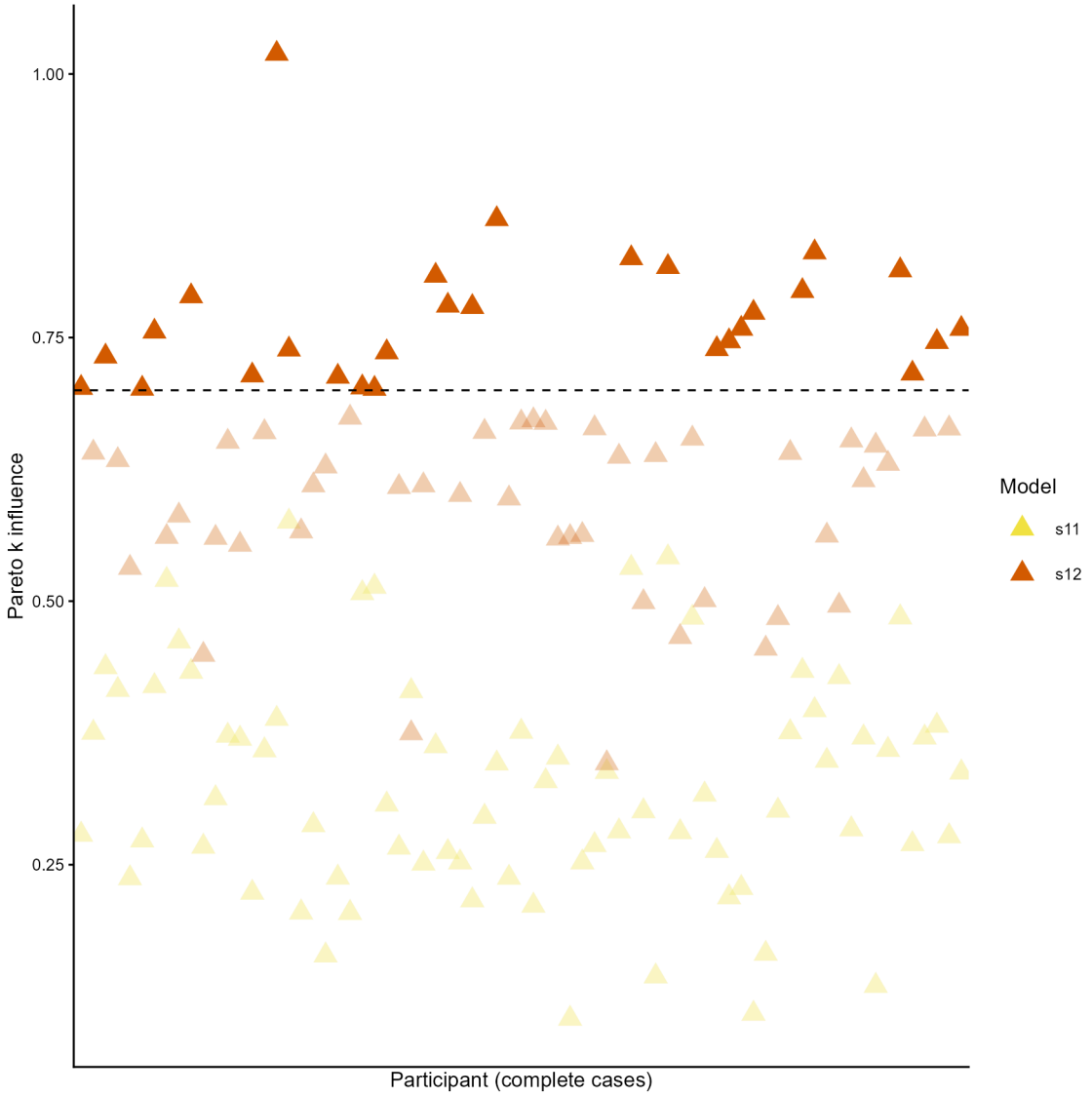

Figure 6.17: Pareto K values for complete-case participants in model s11 and s12.

Table 6.5: Pareto K statistics comparing models s8, s10, s11 and s12.

model	Likelihood	σ structure	ELPD diff	SE	Pareto k > 0.7	Pareto k > 1	Max Pareto K
s12	MVT, full correlation matrix	time × type × treat	0	0	28	1	1.019
s8	MVN, full correlation matrix	time × type	−13.97	16.187	30	10	2.5
s10	MVN, full correlation matrix	time × type × treat	−20.534	13.454	36	10	1.862
s11	Univariate t-distribution	time × type × treat	−170.72	29.131	0	0	0.575

As the simple version of the multivariate normal model (s8) performed better than the more heavily parameterized s10, we will add a simplification of the s12 model. s13 uses the multivariate t-distribution but restricts the structure of σ to time × type. To isolate the contribution of the correlation structure under the t-distributed likelihood, model s18 is the univariate counterpart of s13: a univariate Student-t likelihood with $\nu = 4$ and the same time × type structure of σ, but no correlation among measurement types. s18 thus relates to s13 as s7 relates to s8, and to s11 as s7 relates to s9.

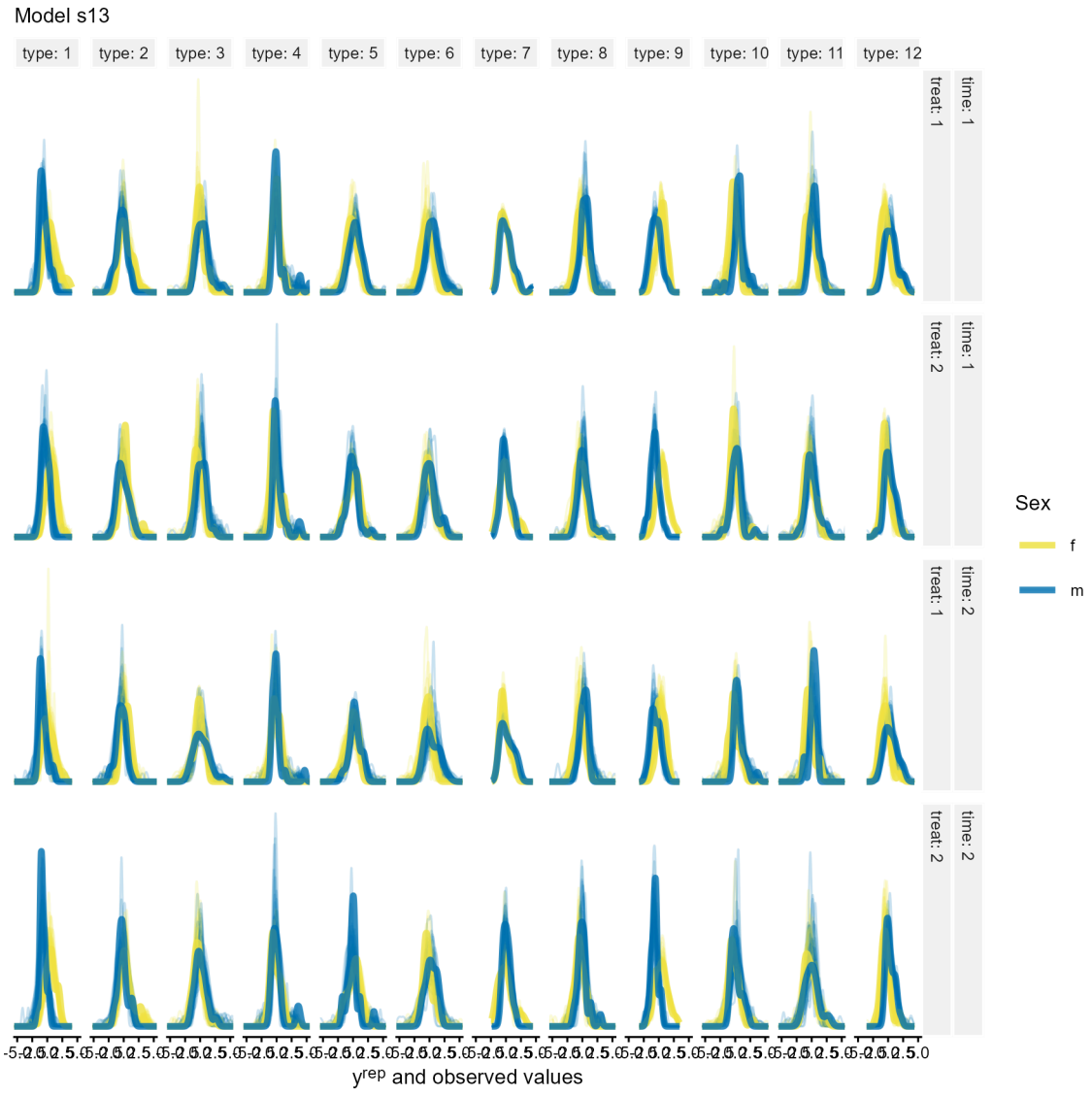

Figure 6.18: Posterior predictive check of model s13. Density of y^{rep} (40 draws from the posterior, thin lines) and observed data (thick lines) per measurement type, treatment, time-point and sex.

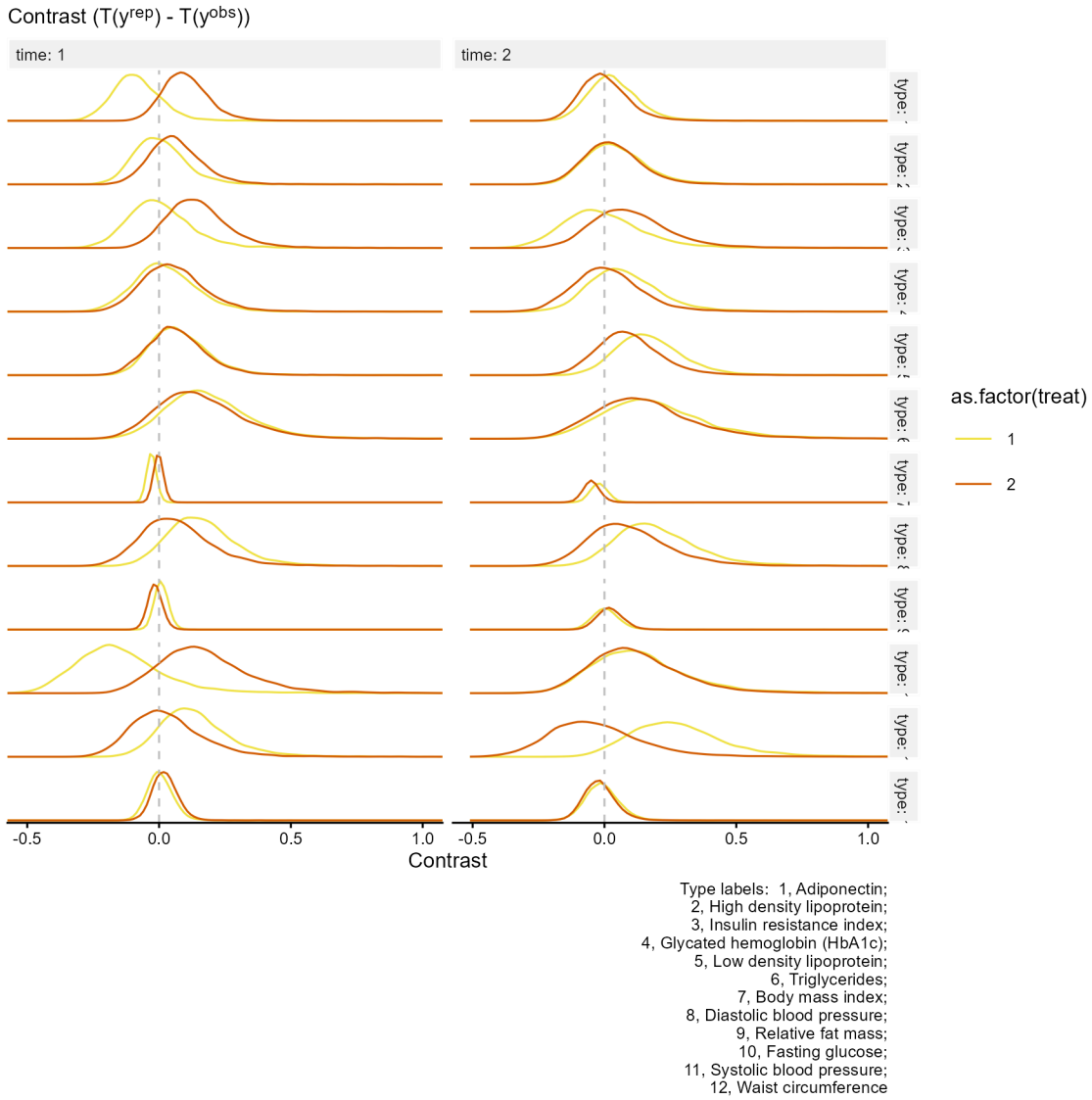

Figure 6.19: Posterior predictive check of model s13 using the SD as test statistic. Distributions of the difference between the SD of replicated data and the SD of observed data ($T(y^{rep})$ - $T(y^{obs})$) per measurement type, time-point and treatment.

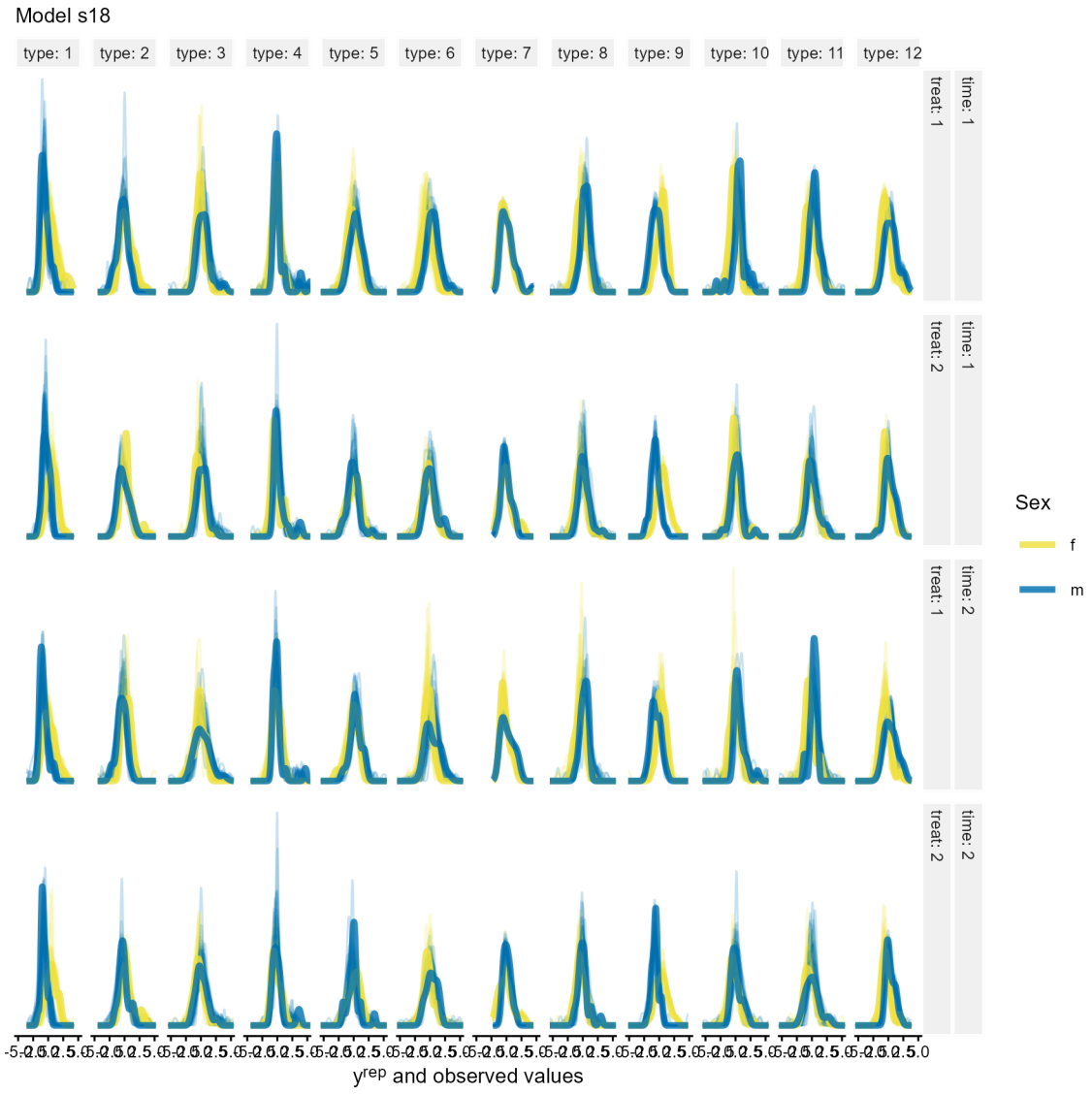

Figure 6.20: Posterior predictive check of model s18. Density of y^{rep} (40 draws from the posterior, thin lines) and observed data (thick lines) per measurement type, treatment, time-point and sex.

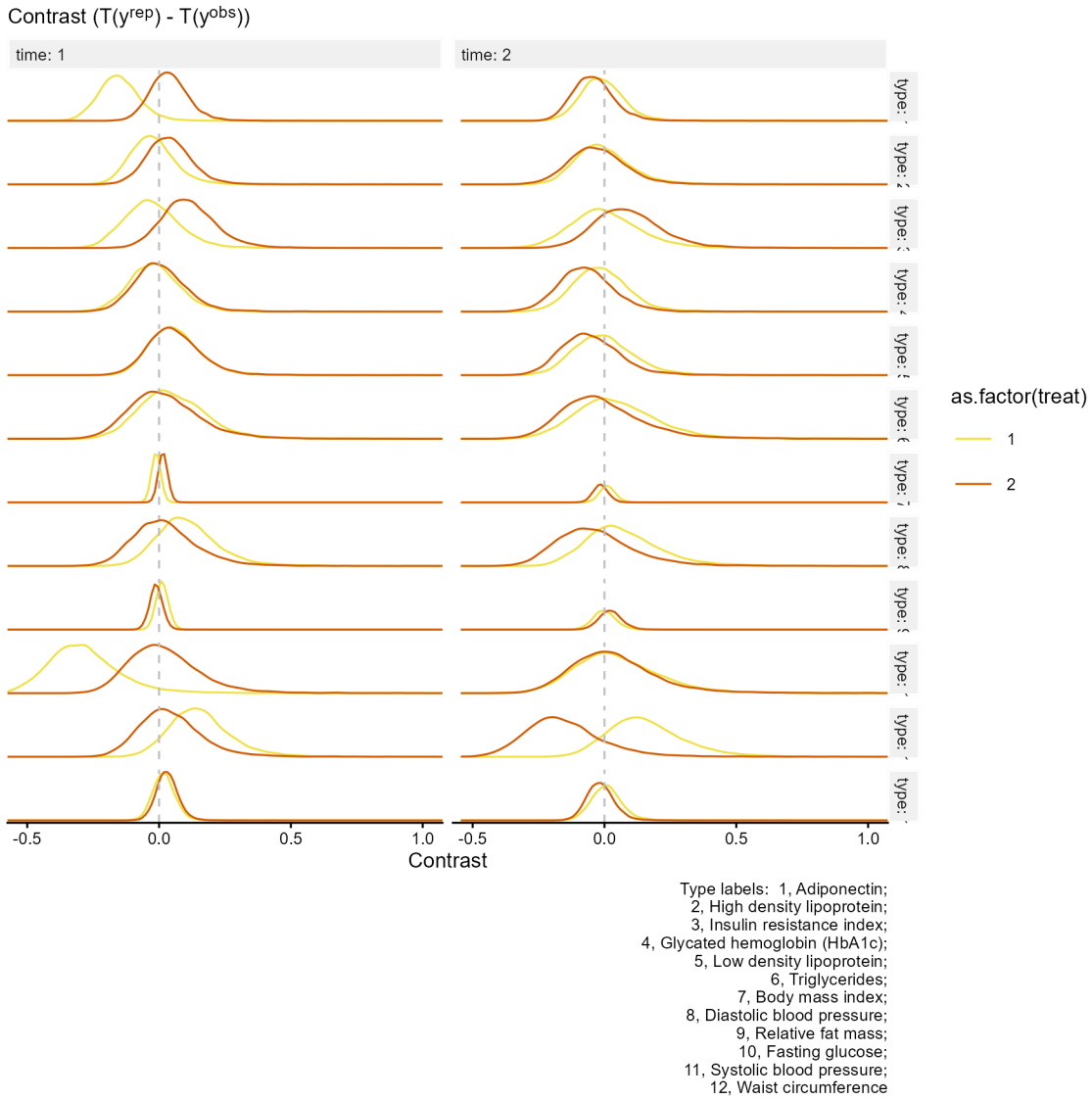

Figure 6.21: Posterior predictive check of model s18 using the SD as test statistic. Distributions of the difference between the SD of replicated data and the SD of observed data ($T(y^{rep})$ - $T(y^{obs})$) per measurement type, time-point and treatment.

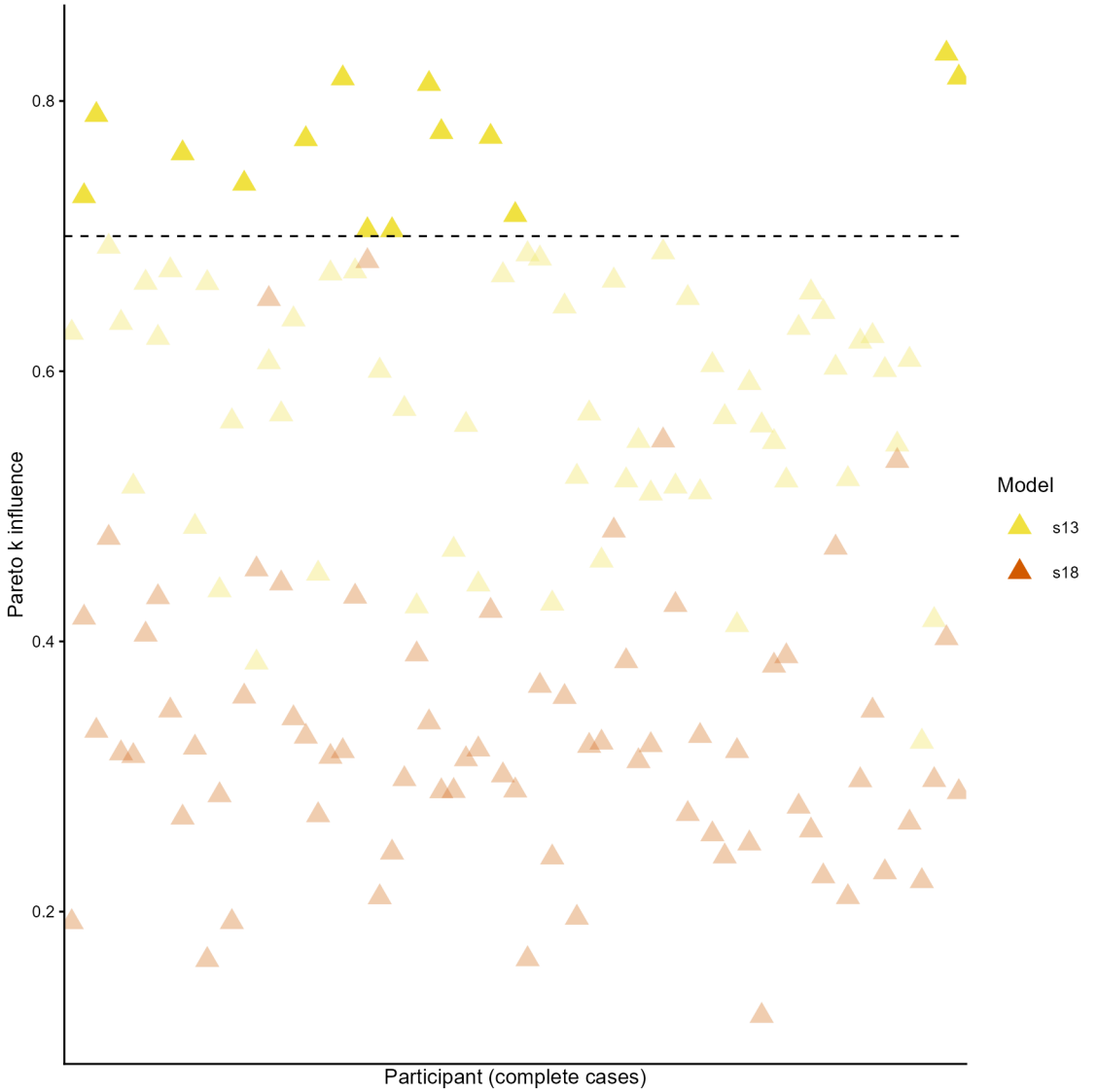

Figure 6.22: Pareto k influence values per participant (complete cases) from PSIS-LOO in models s13 and s18. The dashed line marks k = 0.7; values above are highlighted.

Adding s13 to the model comparison table indicate that this slightly simplified model performs better than s12. The difference is robust with regards to the ELPD difference but still uncertain because of unreliable Pareto k values (Table 6.6).

Table 6.6: Model structures, Pareto K statistics and ELPD for models s8-s13 .

model	Likelihood	σ structure	ELPD diff	SE	Pareto k > 0.7	Pareto k > 1	Max Pareto K
s13	MVT, full correlation matrix	time × type	0	0	14	0	0.835
s12	MVT, full correlation matrix	time × type × treat	−22.386	6.227	28	1	1.019
s8	MVN, full correlation matrix	time × type	−36.356	13.949	30	10	2.5
s10	MVN, full correlation matrix	time × type × treat	−42.92	14.13	36	10	1.862
s18	Univariate t-distribution	time × type	−167.567	26.467	0	0	0.682
s11	Univariate t-distribution	time × type × treat	−193.106	29.588	0	0	0.575

6.0.2 Simplification of the correlation structure

A new set of models simplifies the correlation structure. Instead of estimating the full $M \times M$ correlation matrix with an LKJ prior (as in models s8-s13, where $M = 2K$ is the number of measurement types stacked over the two post-intervention time points), this set of models relates each outcome to a small number of latent variables, following the factor approach used in the simulations (Section 4.0.1; see also the Stan User Guide). Each measurement type at each time point is given a loading onto D latent factors, collected in a $M \times D$ loading matrix Λ, together with a vector ψ of length M containing idiosyncratic (factor-unexplained) standard deviations on the standardised correlation scale, $\psi \sim \text{LogNormal}\ (\log(0.6), 0.4)$. The time × treat × type (s14, s15) or time × type (s16, s17) structure of the residual variation is instead carried by the residual scales σ. Rows 1 to K of Λ hold the loadings for the first post-intervention time point and rows $K + 1$ to M the loadings for the second. Because the two blocks of rows are estimated separately, the within-time correlation structures are free to differ between time points, whereas the between-time correlations are restricted to correlations induced by factor loadings. The implied correlation matrix is obtained by normalising the factor covariance to unit diagonal, and the covariance matrix is then built from the Cholesky factor of this correlation matrix and the residual scales σ:

$$\begin{aligned} \Sigma_{\text{raw}} &= \Lambda\Lambda^T + \text{diag}\ (\psi^2) \\ R &= \text{diag}\ (\Sigma_{\text{raw}})^{-1/2}\, \Sigma_{\text{raw}}\, \text{diag}\ (\Sigma_{\text{raw}})^{-1/2} \\ L_R &= \text{chol}\ (R) \\ L_\Sigma &= \text{diag}\ (\sigma) L_R \\ \Sigma &= L_\Sigma L_\Sigma^T \end{aligned} \tag{6.1}$$

Since R has unit diagonal, σ keeps its interpretation as the marginal residual standard deviation for each time × type (s16 and s17) or time × type × treatment (s14 and s15) combination, exactly as in the models with a full correlation matrix. Only the correlation part is constrained to a low-rank structure (low-rank in the meaning that the correlation matrix is informed by a smaller number of loadings). The factor model is not identified with respect to rotations of Λ, as any orthogonal rotation of the factors leaves $\Lambda\Lambda^T$ unchanged. Following Geweke J, Zhou G. (17), we therefore fix the upper triangle of Λ to zero and

constrain its diagonal to be positive, which enables identification of the loading parameters. With $D = 4$ latent factors and $K = 12$ measurement types this gives $DM - D(D+1)/2 = 86$ free lower-triangular loadings, $D = 4$ diagonal loadings and $M = 24$ idiosyncratic standard deviations (ψ), compared to $M(M-1)/2 = 276$ free correlation coefficients in the full correlation matrix. Loadings were given standard normal priors and ψ a log-normal prior centred at 0.6 to keep the idiosyncratic standard deviations positive and away from zero. The models are otherwise identical to s8 (s16, multivariate normal), s10 (s14, multivariate normal), and their multivariate t-distribution counterparts (s17 and s15, respectively).

Increasing the number of loadings to $D = 6$ gives $DM - D(D+1)/2 = 123$ free parameters and potentially more flexibility. Models s14 to s17 have been run also with $D = 6$ to assess the effect of a larger set of loadings. We include this run in the full comparison table below. Together the full comparison suggests that model s13 (MVT with time × type variance structure) has the best predictive accuracy. A small number of participants have Pareto k values larger than 0.7, but none larger than 1 (Table 6.7). The multivariate models have better estimated predictive accuracy compared to the univariate counterparts and the robust versions (*t*-distributions) perform better than the normal likelihoods. Finally, the simplified variance structure generally performs better than the more complex structure. Together this suggests that the candidate model s13 is preferred for inference. We will use it together with the univariate version in subsequent steps.

Table 6.7: Model definitions, variance structures, ELPD and Pareto K estimates from models s8-s18

model	Likelihood	σ structure	ELPD diff	SE	Pareto k > 0.7	Pareto k > 1	Max Pareto K
s13	MVT, full correlation matrix	time × type	0	0	14	0	0.835
s12	MVT, full correlation matrix	time × type × treat	−22.386	6.227	28	1	1.019
s8	MVN, full correlation matrix	time × type	−36.356	13.949	30	10	2.5
s17-D6	MVT, 6 latent factors	time × type	−42.432	13.134	2	0	0.963
s10	MVN, full correlation matrix	time × type × treat	−42.92	14.13	36	10	1.862
s17	MVT, 4 latent factors	time × type	−58.817	14.925	1	0	0.941
s15-D6	MVT, 6 latent factors	time × type × treat	−61.779	14.167	6	1	1.045
s15	MVT, 4 latent factors	time × type × treat	−80.205	15.934	3	0	0.994
s16-D6	MVN, 6 latent factors	time × type	−81.638	17.36	16	4	2.215
s14-D6	MVN, 6 latent factors	time × type × treat	−86.486	18.106	20	6	1.965
s16	MVN, 4 latent factors	time × type	−136.611	24.297	11	5	1.916
s14	MVN, 4 latent factors	time × type × treat	−167.043	24.952	20	8	2.56
s18	Univariate t-distribution	time × type	−167.567	26.467	0	0	0.682
s11	Univariate t-distribution	time × type × treat	−193.106	29.588	0	0	0.575
s9	Univariate normal	time × type × treat	−321.08	34.08	8	5	1.541

7. Effects of krill oil supplementation on indications of metabolic syndrome - Simulations

To compare the multivariate Student's t models to the univariate counterpart (S13 vs. S18) we will use a more realistic simulation inducing correlations between variables and a larger data set (k = 12). Data are generated as having homoscedastic variance, including equal variance between groups (`sigma_ratio = 1` in `sim_wrap3`, see `R/sim-fun.R`), and added outlier noise. For comparisons, data included participant-level drop-out and values missing at random corresponding to on average 15.6, 32.7 and 43.4% of the follow-up values (low, mid and high missingness, realised values across simulated data sets) with baseline values having about 8% missing values. Correlations were set to weak (realised mean |r| ≈ 0.09 between measurement types and r ≈ 0.16 between time-points) and strong (realised mean |r| ≈ 0.27 between measurement types and r ≈ 0.31 between time-points).

As an example, the following synthetic data set was simulated using the simulation routine used in `R/model-comp3.R` (Figure 7.1).

Simulating the full data set leads to synthetic data patterns presented in Figure 7.1.

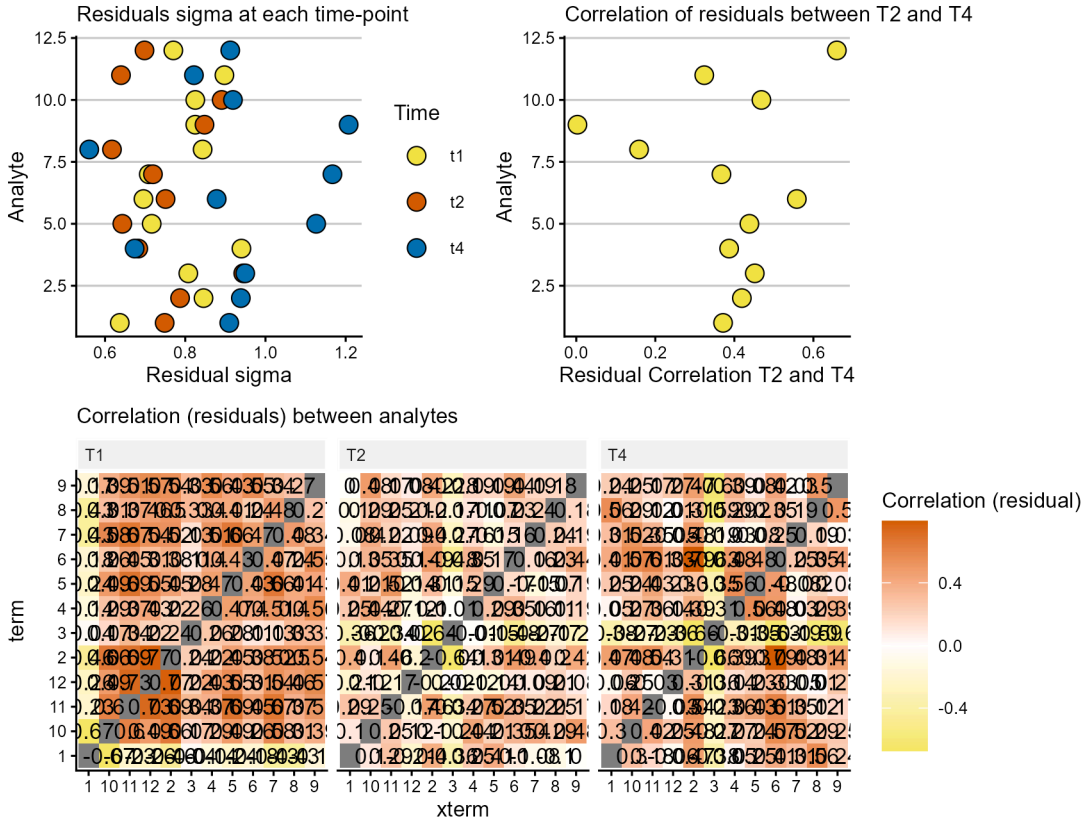

Figure 7.1: Residual variation and correlation pattern within and between variables in the synthetic data example.

The missing data patterns from the medium sized missingness scenario corresponds approximately to the observed data (Figure 7.2, see also Chapter 4.).

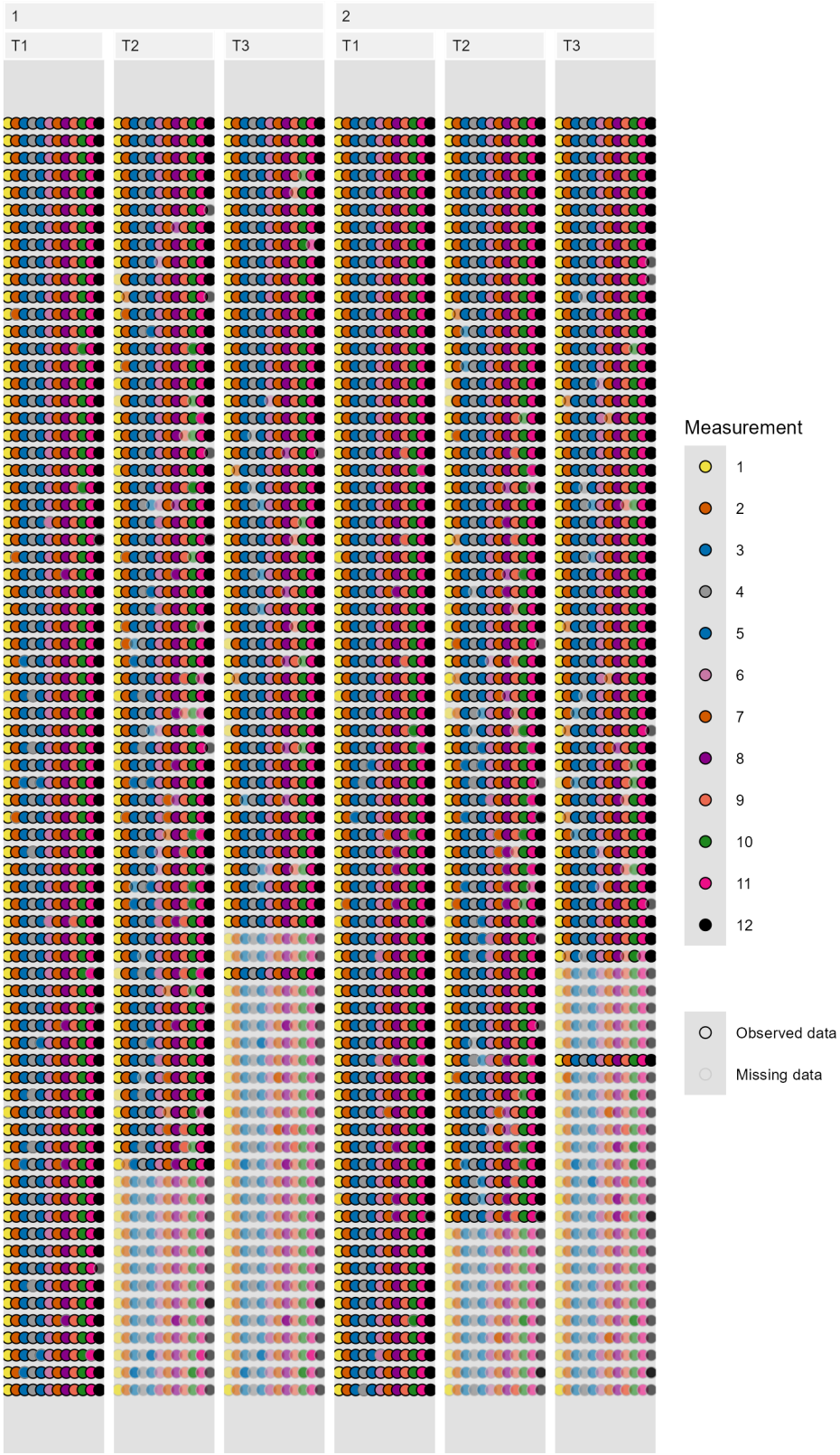

Figure 7.2: Missing data patterns in the example simulated data set.

Using 1200 simulated data sets we are comparing models S13 to s18 for modelling performance using two scenarios of correlations (strong vs. weak) and three levels of missingness (low, mid, high). Figure 7.3

shows chain diagnostics from the fits, no simulated data sets (n = 0) were excluded from comparisons due to unsatisfactory chain convergence or effective sample sizes.

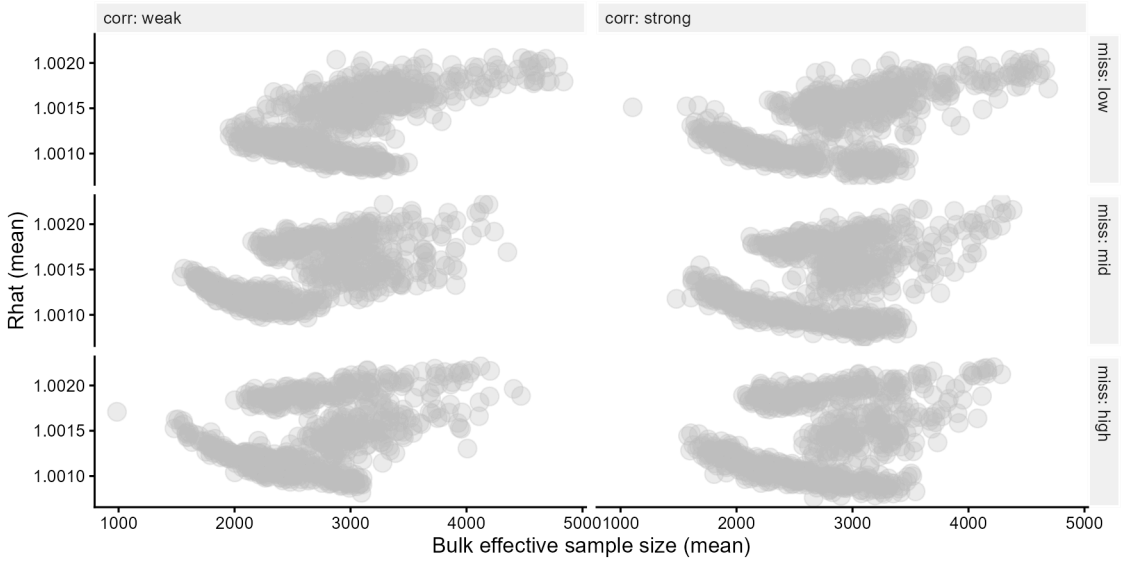

Figure 7.3: Chain diagnostics under the full three by two simulated scenarios.

For each estimate (treatment contrasts, 2 × 12 per data set), coverage of a 90% percentile interval was determined and averaged within each model, variant (full data and missing values) and simulation cell (missingness × correlation strength). With weak correlations, coverage of S13 was on average closer to the nominal level while S18 had slightly lower coverage. With strong correlations, both models had similar coverage, slightly below nominal at low missingness and close to nominal at mid and high missingness (Figure 7.4 A). Credible intervals were wider in the missing data variant and widened with increasing proportions of missing values, while interval widths in the full data variant were similar across conditions (Figure 7.4 B). With weak correlations, S13 had slightly wider intervals than S18 in both variants, consistent with its higher coverage. With strong correlations, interval widths were similar in the full data variant, but S13 tended to have narrower intervals than S18 in the missing data variant, a difference that increased with the proportion of missing values (Figure 7.4 C).

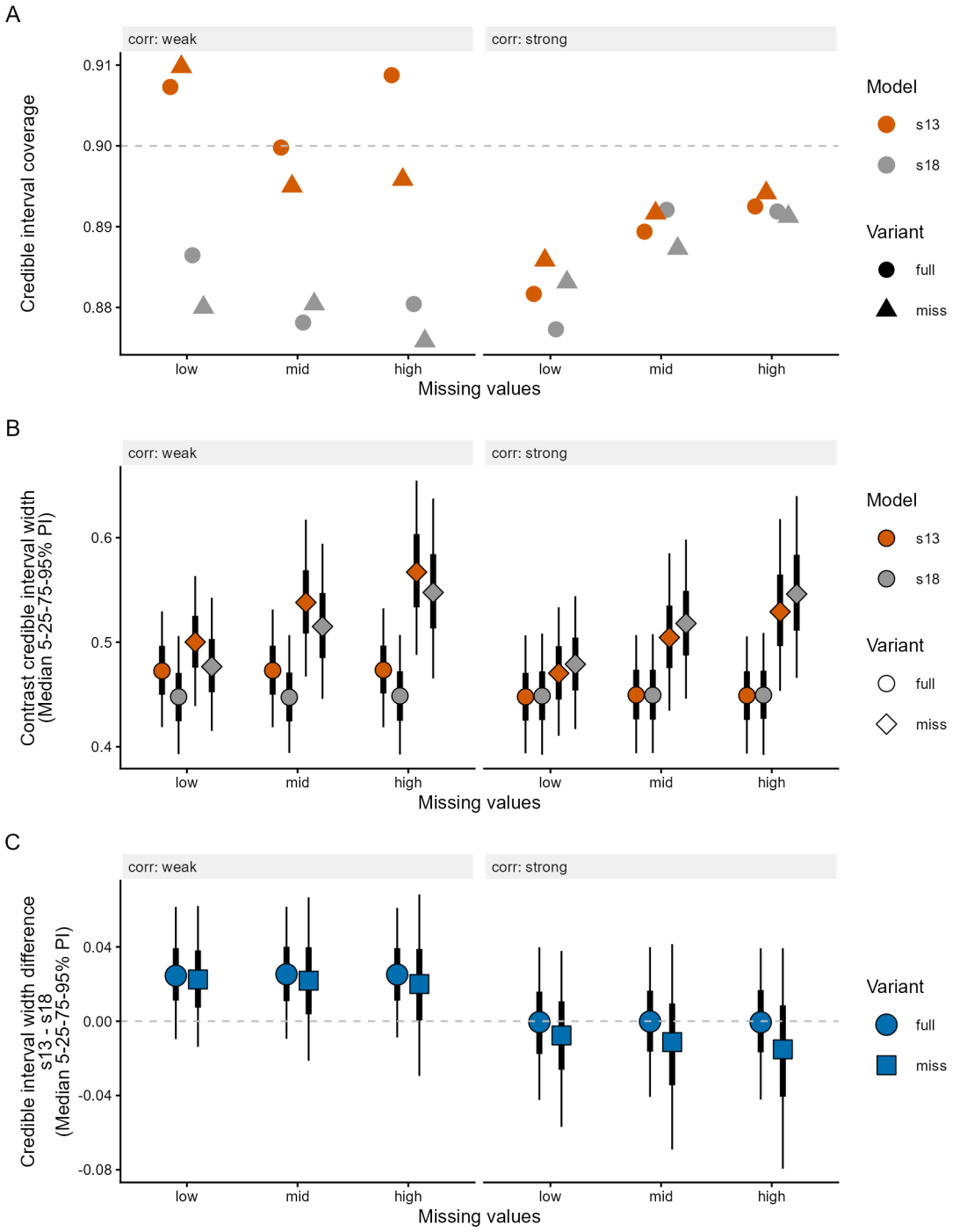

Figure 7.4: (A) Coverage of 90% credible intervals, (B) credible interval widths, and (C) differences in credible interval widths between models.

Accuracy was assessed as the error of the posterior mean of each treatment contrast (Estimate - Simulated). In the full data variant, the root mean squared error (RMSE) was practically identical between S13 and S18. Missing values increased the RMSE for both models, and the increase was smaller for S13 when correlations were strong (Figure 7.5 A). The paired differences in absolute error per estimate were centered close to zero in all comparisons and in both zero and non-zero effects, with wide distributions (Figure 7.5 B). S13 had the lower absolute error in a small majority of estimates in most comparisons (overall about 50-54%), with the largest proportions in the missing data variant with strong correlations (Figure 7.5 C).

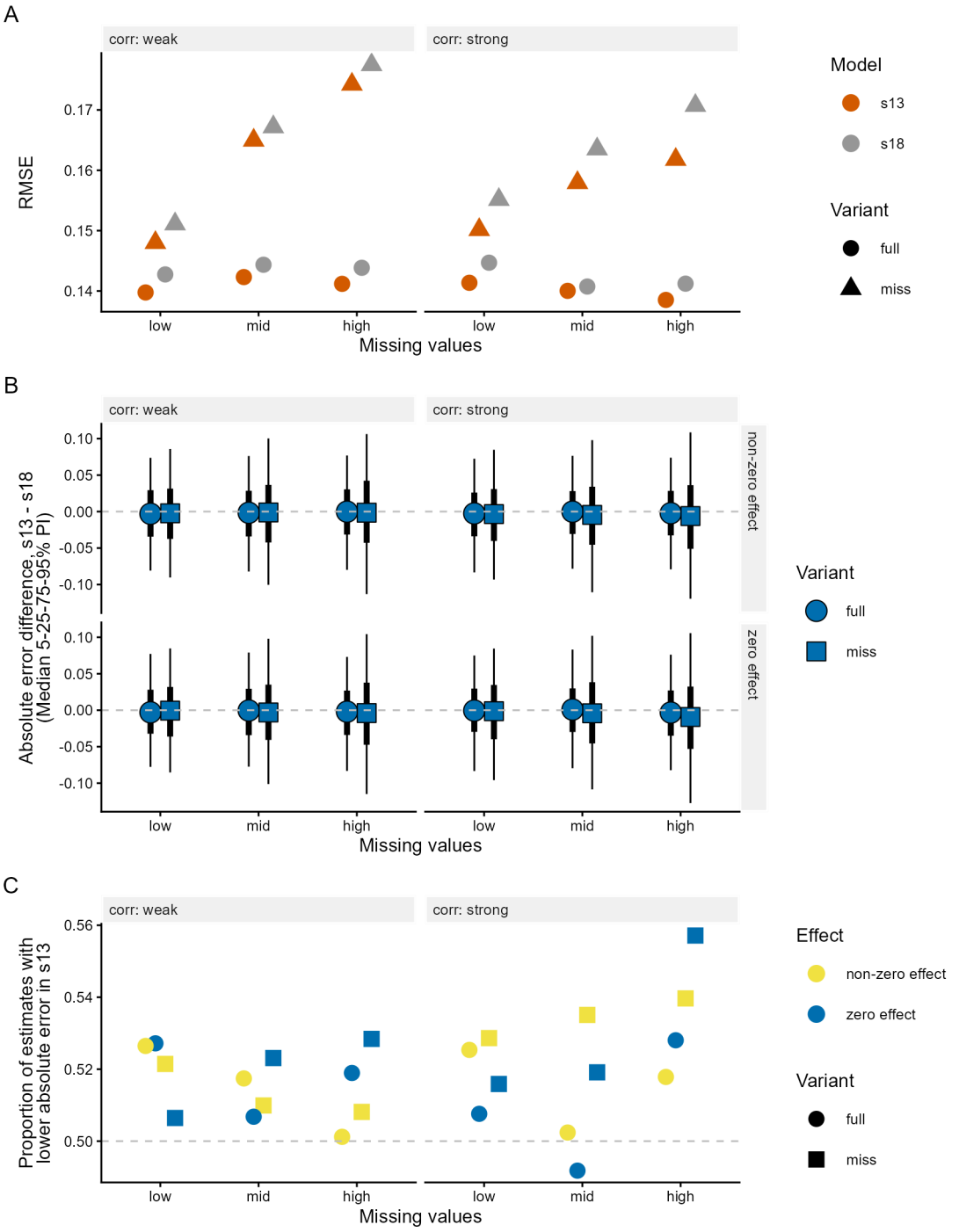

Figure 7.5: (A) Root mean squared error (RMSE) of posterior mean treatment contrasts, (B) paired differences in absolute error between models (S13 - S18) in zero and non-zero effects, and (C) proportion of estimates where S13 had lower absolute error than S18.

Finally, we compared the error associated with data imputation. Overall, the RMSE was lower in s13 compared to s18 with larger benefit of s13 in imputation of pre-values and follow-up values under stronger correlations (Figure 7.6).

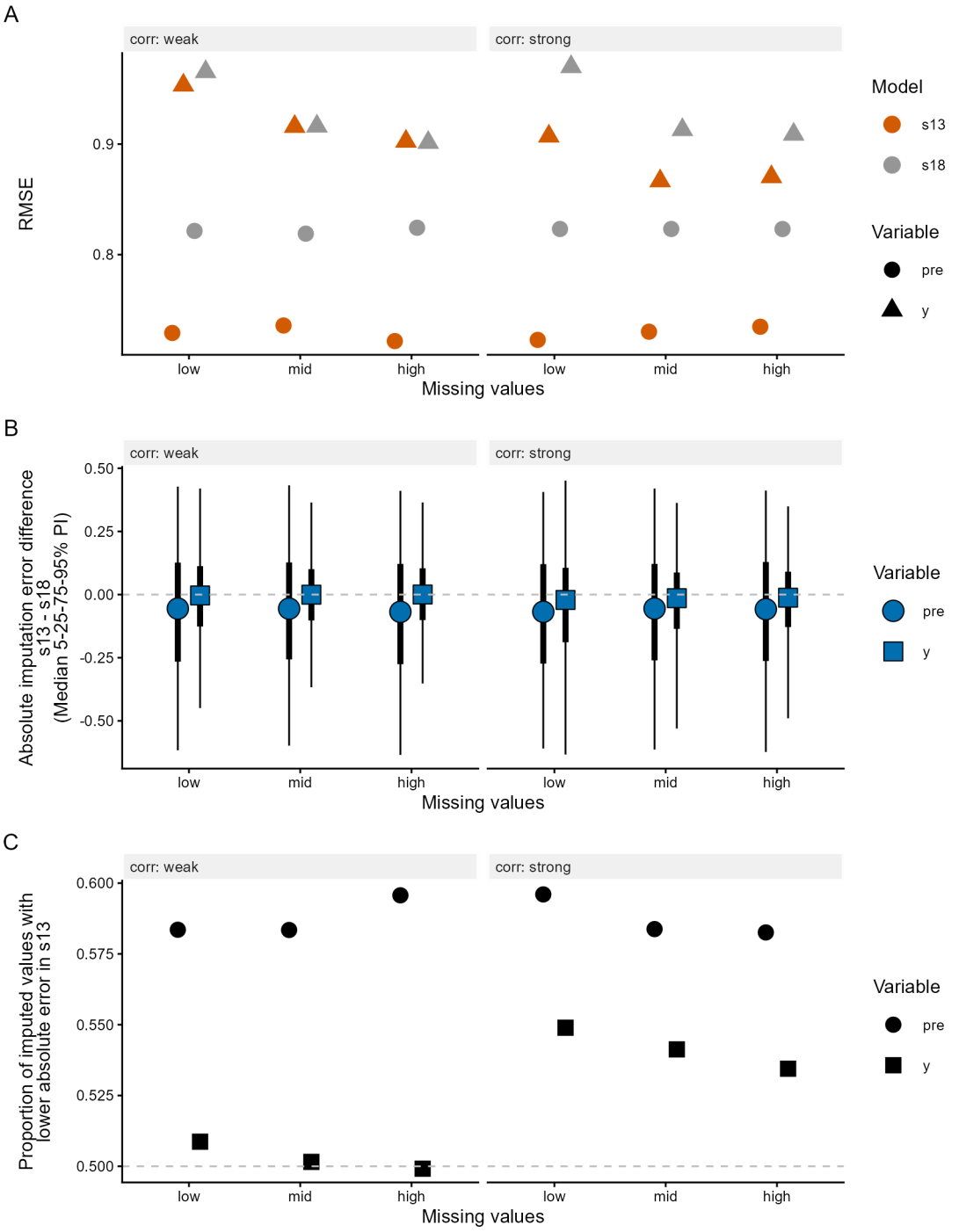

Figure 7.6: (A) Root mean squared error (RMSE) of posterior mean imputed values, (B) paired differences in absolute imputation error between models (S13 - S18) for baseline (pre) and post-intervention (y) values, and (C) proportion of imputed values where S13 had lower absolute error than S18.

Together, although differences are small, these results indicate that S13 is at least as accurate as S18, with a modest gain in accuracy when correlations between variables and time-points are strong and data are missing.

8. Effects of krill oil supplementation on indications of metabolic syndrome - Inference

Here, we draw inferences about the effect of krill oil supplementation on metabolic health markers. First, we examine the differences in outcomes between placebo and krill oil, followed by a combined analysis of the dichotomous diagnostic score (0/1) and a continuous burden score derived as the average of the five diagnostic criteria (HDL with negative sign, blood pressure as the average of systolic and diastolic pressure), with each criterion re-scaled to sex-specific population SD units.

8.0.1 Probability of benefit, negligible effect and harm

For each outcome, we summarise the posterior distribution of the treatment effect (krill oil − placebo) as the probability of benefit, a negligible effect and harm. An effect is negligible when it is smaller in magnitude than the outcome-specific threshold δ, and a benefit or harm when it is larger than δ in the beneficial or harmful direction (higher is beneficial for HDL cholesterol and adiponectin, lower for all other outcomes). Thresholds were derived in the data chapter and are applied on the scale of the contrast, i.e. the log scale for triglycerides, HOMA2-IR and HbA1c. The burden score is the re-weighted score, with each criterion expressed in sex-specific population SD units (own baseline SD for fasting glucose). For the burden score, δ is based on the between-individual SD of this score at baseline rather than the component thresholds (Table 8.1).

Table 8.1: Posterior probabilities (%) of benefit, a negligible effect and harm of krill oil compared with placebo for the metabolic syndrome criteria, the burden score and related outcomes (model s13), after the supplementation period and after the combined period. δ, threshold for a negligible effect, on the scale of the contrast (log scale for triglycerides, HOMA2-IR and HbA1c; population SD units for the burden score).

	δ	Supplementation			Combined		
		Benefit	Negligible	Harm	Benefit	Negligible	Harm
			Metabolic syndrome				
Waist circumference	1.3	13	87	<1	4	94	2
Diastolic blood pressure	1	78	22	<1	56	41	2
Systolic blood pressure	1.9	90	10	<1	63	35	2
Fasting glucose	0.054	66	31	3	54	39	7
High density lipoprotein	0.047	2	92	6	60	40	<1
Triglycerides	0.054	21	65	15	25	56	19
Burden score	0.055	71	29	<1	68	32	<1
			Related outcomes				
Body mass index	0.48	<1	>99	<1	<1	>99	<1
Relative fat mass	0.78	<1	>99	<1	<1	94	6
Low density lipoprotein	0.1	24	72	4	5	66	28
Adiponectin	0.6	6	80	14	5	77	19
HOMA2-IR	0.037	35	61	4	12	57	31
HbA1c	0.017	<1	37	63	3	74	23

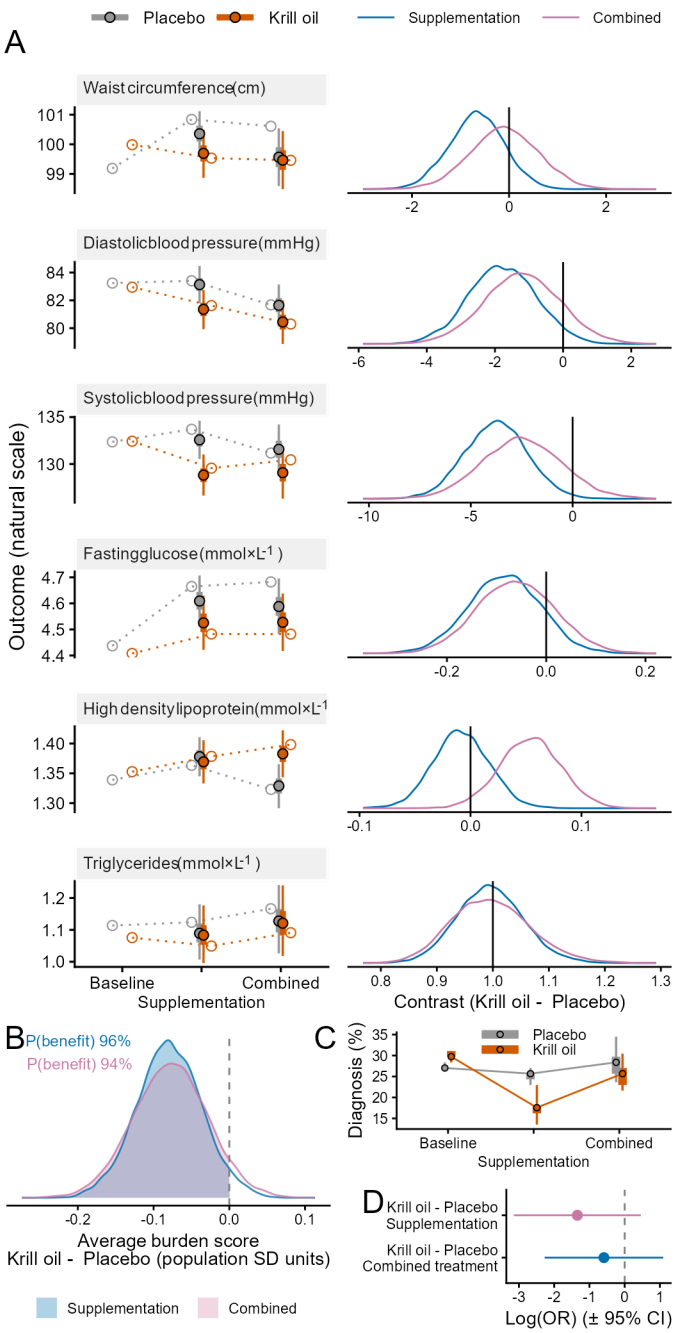

Figure 8.1: A, Observed group means (open symbols, all available observations; geometric means for triglycerides) and model-based adjusted group means (filled symbols, posterior median with 50% and 95% CrI) for each outcome in the metabolic syndrome score on its natural scale. Adjusted means after supplementation and combined periods are model predictions conditional on the overall mean pre-intervention value and averaged over sex. The right panels show posterior distributions of treatment effects (krill oil - placebo) for each measurement. Baseline represents pre-intervention values (observed); Supplementation and Combined show estimates and observed means after the supplementation period and after the Supplementation + resistance-training period, respectively. B, posterior distribution of the burden score contrast (krill oil - placebo, population SD units) at post-supplementation and after the combined period. P(benefit) indicates the percentage of the posterior associated with any benefit for krill oil. C, the percentage of participants meeting the diagnostic criteria in each group; error bars represent the uncertainty associated with missing-value imputation (95th and 50th percentile intervals). D, treatment contrast for the occurrence of metabolic syndrome diagnosis, baseline-adjusted differences between groups (log odds ratio with 95% confidence interval pooled over imputations using Rubin's rules) at post-supplementation and after the combined treatment period.

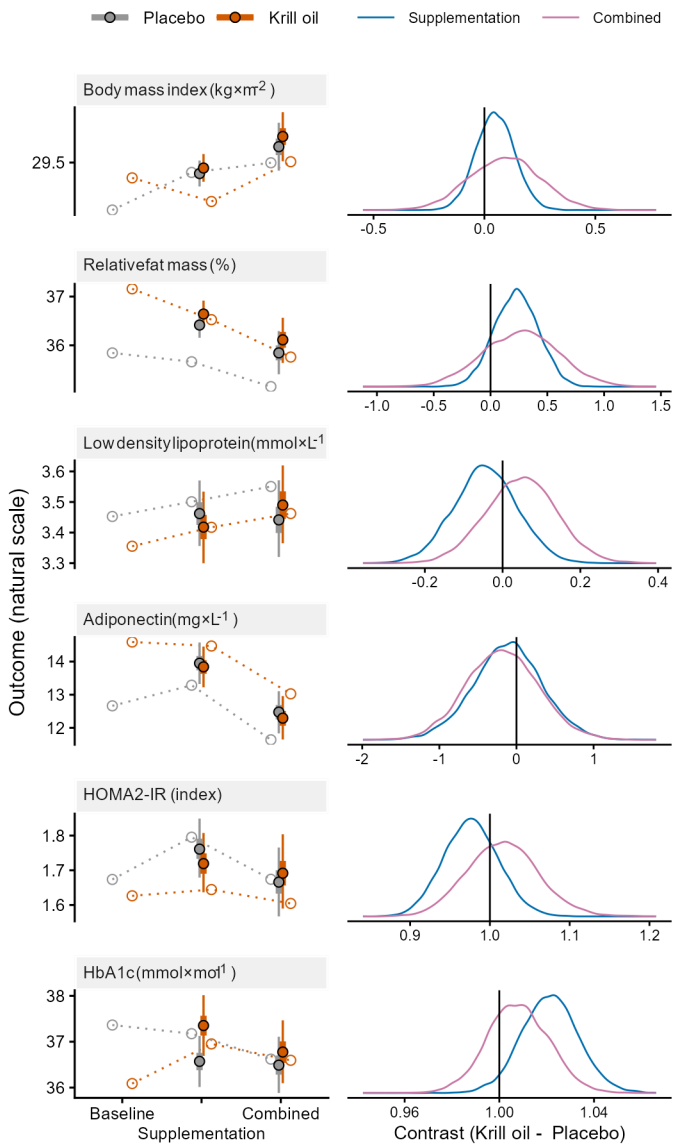

Figure 8.2: Observed group means (open symbols, all available observations; geometric means for HOMA2-IR and HbA1c) and model-based adjusted group means (filled symbols, posterior median with 50% and 95% CrI) for each outcome on its natural scale. Adjusted means after supplementation and combined periods are model predictions conditional on the overall mean pre-intervention value and averaged over sex. The right panels show posterior distributions of treatment effects (krill oil - placebo) for each measurement. Baseline represents pre-intervention values (observed); Supplementation and Combined show estimates and observed means after the supplementation period and after the Supplementation + resistance-training period, respectively.

At post-supplementation (T2), baseline- and sex-adjusted differences between groups indicated a reduced waist circumference in krill oil compared to placebo −0.7 cm (95% CrI: −1.8, 0.4) with the probability of any benefit (proportion of the posterior density indicating benefit for the krill oil group, P(benefit)) being 89%. After the combined period (supplementation and resistance training), the between-group contrast for waist circumference indicated no difference, −0.1 cm (95% CrI: −1.5, 1.3; P(benefit) 56%) (Figure 8.1). Both diastolic and systolic blood pressure indicated beneficial effects in krill oil compared to placebo at T2 (-1.8 mmHg (-3.8, 0.2; P(benefit) 96%) and −3.7 mmHg (-6.7, −0.8; P(benefit) >99%) for diastolic and systolic blood pressure, respectively). Although both the average diastolic and systolic blood pressure were more similar between krill oil and placebo after the second period, the probability distribution indicated a higher

probability of benefit in the krill oil group (-1.2 mmHg (-3.4, 1.0; P(benefit) 86%) and −2.5 mmHg (-6.2, 1.4; P(benefit) 90%) for diastolic and systolic blood pressure, respectively; Figure 8.1).

After the supplementation period, the krill oil group displayed similar levels of high-density lipoproteins (HDL) as compared to the placebo group (-0.01 mmol/L (-0.06, 0.04; P(benefit) 37%)). However, following the combined period, the krill oil group increased HDL levels compared to placebo, indicating a higher probability of beneficial effects of krill oil supplementation (0.05 mmol/L (0.00, 0.11; P(benefit) 98%)). Triglyceride levels were estimated to be similar between intervention protocols, both after the post-supplementation period (-1% (-12, +12%; P(benefit) 54%)) and after the combined period (-1% (-13, +14%; P(benefit) 54%)).

A combined burden score was created as the average of the five metabolic syndrome diagnostic criteria on their continuous scales (HDL entering with a negative sign so that higher scores indicate worse metabolic health, and the average of diastolic and systolic blood pressure counting as one criterion), with each criterion re-weighted to sex-specific population SD units (see `R/pop-ref.R`; fasting glucose scaled by the trial's own sex-specific baseline SD). Comparing the combined scores between treatments indicated a high probability of benefit from krill oil at both post-treatment time points (post-supplementation, −0.08 population SD units (-0.17, 0.01; P(benefit) 96%); post combined treatment, −0.08 population SD units (-0.17, 0.02; P(benefit) 94%); Figure 8.1 B). The differences in the combined score did not yield a significant (95% confidence interval not containing 0) difference in the dichotomous diagnostic criteria (harmonised definition, at least three of the five criteria passing their thresholds (15); Figure 8.1 C and D).

Comparing BMI between treatment groups indicated no clear difference between groups at follow-up time points (0.05 kg/m² (95% CrI: −0.12, 0.22; P(benefit) 29%) and 0.09 kg/m² (-0.22, 0.40; P(benefit) 29%), for after the supplementation and combined periods, respectively). However, relative fat mass indicated a higher probability of an adverse effect of krill oil compared to placebo after the supplementation period (0.2 %-points (-0.2, 0.6; P(benefit) 12%)). After the combined period, differences were smaller (0.3 %-points (-0.4, 0.9; P(benefit) 22%)). Adjusted differences between groups in low density lipoprotein (LDL) and Adiponectin indicated similarity between treatments (LDL post-supplementation, −0.04 mmol/L (-0.20, 0.12; P(benefit) 71%); LDL post-combined treatment, 0.05 mmol/L (-0.13, 0.23; P(benefit) 30%); Adiponectin post-supplementation, −0.11 mg/L (-1.01, 0.78; P(benefit) 40%); Adiponectin post-combined treatment −0.18 mg/L (-1.08, 0.75; P(benefit) 35%)). Insulin resistance, estimated using the HOMA2-IR model score, indicated a greater posterior probability of benefit at post-supplementation after krill oil compared to placebo (-2% (-9, +5%; P(benefit) 75%)), a difference that was not present after the combined time period (+2% (-7, +11%; P(benefit) 36%)). Glycated haemoglobin (HbA1c) was slightly higher in krill oil compared to placebo after the supplementation period, indicating a low probability of benefit (+2% (95% CrI: −0, +5%; P(benefit) 4%)). After the combined period, the difference was smaller and more uncertain (+1% (-2, +3%; P(benefit) 27%)).

8.1 Model expansion: Baseline-dependent treatment effects

To assess whether the effect of krill oil supplementation depends on the participants' baseline status, four expansions of model s13 are considered. All keep the full s13 structure (joint imputation of missing pre- and post-intervention values, the multivariate Student's t-likelihood with $\nu = 4$, the time × type residual scales and the LKJ correlation matrix) and change only the linear predictor for the expected post-intervention value. In s13, the expected value for participant p, measurement type k and post-intervention time-point t is

$$\mu_{p,k,t} = \alpha_{k,t,\,\mathrm{tx}\,[p]} + \beta_{k,t}\, x_{p,k} + \beta^{\mathrm{sex}}_{k,t}\, s_p \tag{8.1}$$

where $x_{p,k}$ is the standardised pre-intervention value (observed or imputed), $s_p \in \{-0.5, 0.5\}$ codes sex, and $\mathrm{tx}\,[p] \in \{1, 2\}$ indexes placebo and krill oil, respectively. The treatment effect is the difference between intercepts, $\alpha_{k,t,2} - \alpha_{k,t,1}$, and does not depend on the baseline value. Let $\omega_p = 1$ for participants in the krill oil group and $\omega_p = 0$ for placebo.

Model s19: treatment × own-baseline interaction. The pre-intervention slope is allowed to differ between treatment groups,

$$\begin{aligned} \mu_{p,k,t} &= \alpha_{k,t,\,\mathrm{tx}\,[p]} + \beta_{k,t}\, x_{p,k} + \delta_{k,t}\, \omega_p\, x_{p,k} + \beta^{\mathrm{sex}}_{k,t}\, s_p \\ \delta_{k,t} &= \tau_\delta\, z_{k,t} \\ z_{k,t} &\sim \mathrm{Normal}\ (0, 1) \\ \tau_\delta &\sim \text{Half-Normal}\ (0, 0.25) \end{aligned} \tag{8.2}$$

so that the treatment effect for a participant with standardised baseline value x is

$$\Delta_{k,t}(x) = \alpha_{k,t,2} - \alpha_{k,t,1} + \delta_{k,t}\, x. \tag{8.3}$$

Since the outcomes are standardised, the baseline mean is close to zero and the intercept difference keeps its interpretation from s13 as the effect at an average baseline value, while $\delta_{k,t}$ is the change in the treatment effect per SD of the baseline value. The $2K = 24$ interaction terms share a common scale τ_δ (non-centred parameterisation), which shrinks individual interactions towards zero unless they are jointly supported by the data. A prior scale of 0.25 places most prior mass on interactions smaller than half an SD of the outcome per SD of baseline.

Model s20: treatment × baseline metabolic burden. Rather than each outcome's own baseline, s20 uses a single moderator shared across outcomes: a continuous baseline metabolic burden score defined as the mean of the sign-aligned standardised baseline values of the five diagnostic criteria,

$$\begin{aligned} b_p &= \frac{1}{5}\Big(-x_{p,\,\mathrm{HDL}} + x_{p,\,\mathrm{TG}} + \frac{1}{2}\big(x_{p,\,\mathrm{dias}} + x_{p,\,\mathrm{sys}}\big) + x_{p,\,\mathrm{glucose}} + x_{p,\,\mathrm{waist}}\Big) \\ \tilde{b}_p &= b_p - \frac{1}{N_P}\sum_{p'} b_{p'} \end{aligned} \tag{8.4}$$

where HDL enters with a negative sign so that higher values of b_p indicate worse metabolic health, and blood pressure contributes as the mean of the diastolic and systolic values. The score is computed inside the model from the partly imputed baseline matrix, so it remains a continuous and differentiable function of the imputed values (in contrast to the number of criteria met, or the baseline diagnosis, which are step functions). The linear predictor becomes

$$\begin{aligned} \mu_{p,k,t} &= \alpha_{k,t,\,\mathrm{tx}\,[p]} + \beta_{k,t}\, x_{p,k} + \gamma_{k,t}\, \omega_p\, \tilde{b}_p + \beta^{\mathrm{sex}}_{k,t}\, s_p \\ \gamma_{k,t} &= \tau_\gamma\, z_{k,t} \\ z_{k,t} &\sim \mathrm{Normal}\ (0, 1) \\ \tau_\gamma &\sim \text{Half-Normal}\ (0, 0.25) \end{aligned} \tag{8.5}$$

with treatment effect $\Delta_{k,t}(\tilde{b}) = \alpha_{k,t,2} - \alpha_{k,t,1} + \gamma_{k,t}\,\tilde{b}$. Centring the burden score on its sample mean keeps the intercept difference interpretable as the effect at average baseline burden.

Model s21: treatment × baseline relative fat mass. s21 has the same structure as s20, but the shared moderator is the participant's baseline relative fat mass. The moderator is taken from the partly imputed baseline matrix and standardised within each posterior draw to mean 0 and SD 1 over participants,

$$\begin{aligned} f_p &= \frac{x_{p,\,\mathrm{fat.rel}} - \bar{x}_{\mathrm{fat.rel}}}{\mathrm{SD}\ (x_{\mathrm{fat.rel}})} \\ \mu_{p,k,t} &= \alpha_{k,t,\,\mathrm{tx}\,[p]} + \beta_{k,t}\, x_{p,k} + \gamma_{k,t}\, \omega_p\, f_p + \beta^{\mathrm{sex}}_{k,t}\, s_p \end{aligned} \tag{8.6}$$

with $\gamma_{k,t}$ given the same partially pooled prior as in s20. The intercept difference is the treatment effect at the average baseline relative fat mass and $\gamma_{k,t}$ is the change in the treatment effect per SD of baseline relative fat mass.

Model s22: treatment × baseline omega-3 index. In s22 the shared moderator is the baseline red blood cell omega-3 index (13), which is not one of the modelled outcomes. The index was standardised to mean 0 and SD 1 over participants with an observed baseline value (o_p). The baseline index was missing for 4 participants; these values were imputed in the model with a standard normal prior, $o_p \sim \mathrm{Normal}\ (0, 1)$, so that all randomised participants are kept. The linear predictor is

$$\mu_{p,k,t} = \alpha_{k,t,\,\mathrm{tx}\,[p]} + \beta_{k,t}\, x_{p,k} + \gamma_{k,t}\, \omega_p\, o_p + \beta^{\mathrm{sex}}_{k,t}\, s_p \tag{8.7}$$

with the same partially pooled prior on $\gamma_{k,t}$ as in s20. Here, $\gamma_{k,t}$ is the change in the treatment effect per SD of the baseline omega-3 index (1 SD = 1.35 %-points).

Treatment effects across the moderator. To show what the interactions imply, treatment effects were predicted from each model at the moderator mean (0) and at ± 1 SD of the moderator,

$$\Delta_{k,t}(m) = \alpha_{k,t,2} - \alpha_{k,t,1} + \gamma_{k,t}\, m, \quad m \in \{-\,\mathrm{SD}, 0, +\,\mathrm{SD}\}, \tag{8.8}$$

with $\delta_{k,t}$ in place of $\gamma_{k,t}$ for s19. For s21 and s22 the moderator is standardised, so SD = 1. For s19, the SD is the observed baseline SD of each standardised outcome, and for s20 it is the SD of the centered burden score over participants, computed per posterior draw. Effects are computed per posterior draw and shown on the standardised scale (outcome SD units), where a negative value indicates a lower value with krill oil compared to placebo (Figure 8.4).

All four models had satisfactory chain convergence statistics and effective sample size enabling their use for subsequent inference (Figure 8.3).

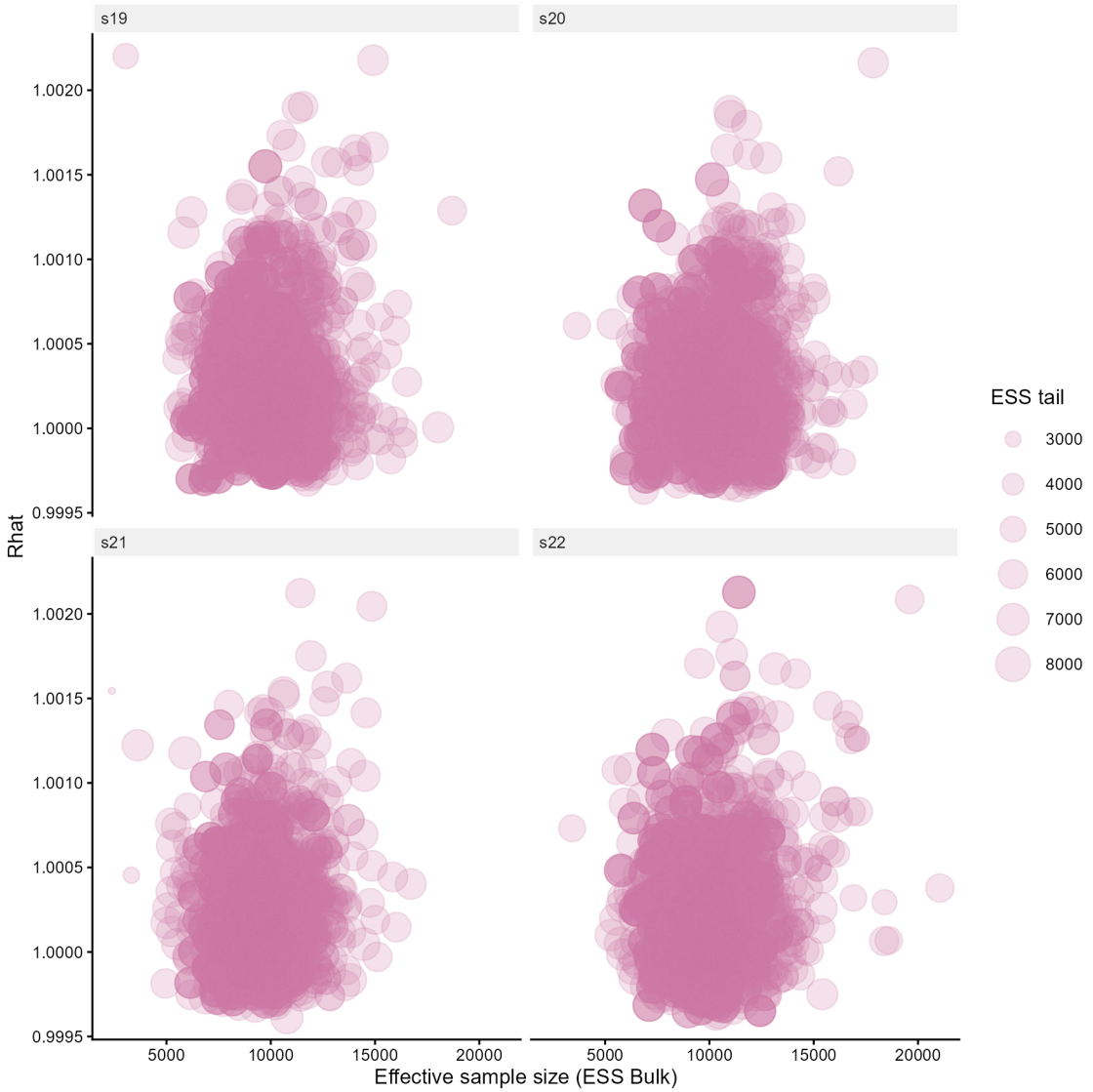

Figure 8.3: Summary chain statistics (Rhat vs. bulk and tail effective sample size) for the moderation models s19 to s22.

The moderation models did not credibly differ from s13, or from each other, in estimated out-of-sample predictive performance (ELPD difference (SE) relative to the best model: s21 0.0 (0.0), s13 −2.7 (2.5), s20 −2.8 (3.0), s22 −3.2 (2.6), s19 −4.7 (2.2); Table 8.2). In each model, 14-18 of 73 participants had Pareto k values > 0.7 (none > 1), so the ELPD estimates should be interpreted with some caution.

Table 8.2: Approximate leave-one-out cross-validation (PSIS-LOO) comparison of model s13 and the moderation models s19 to s22. ELPD diff, difference in expected log pointwise predictive density relative to the model with the highest ELPD; SE, standard error of the difference; Pareto k > 0.7 (> 1), number of participants with Pareto k values above 0.7 (above 1).

Model	ELPD diff	SE	Pareto k > 0.7	Pareto k > 1
s21: Relative fat mass	0.0	0.0	17	0
s13: No moderation	−2.7	2.5	14	0
s20: Metabolic burden	−2.8	3.0	18	0
s22: Omega-3 index	−3.2	2.6	15	0
s19: Own baseline	−4.7	2.2	17	0

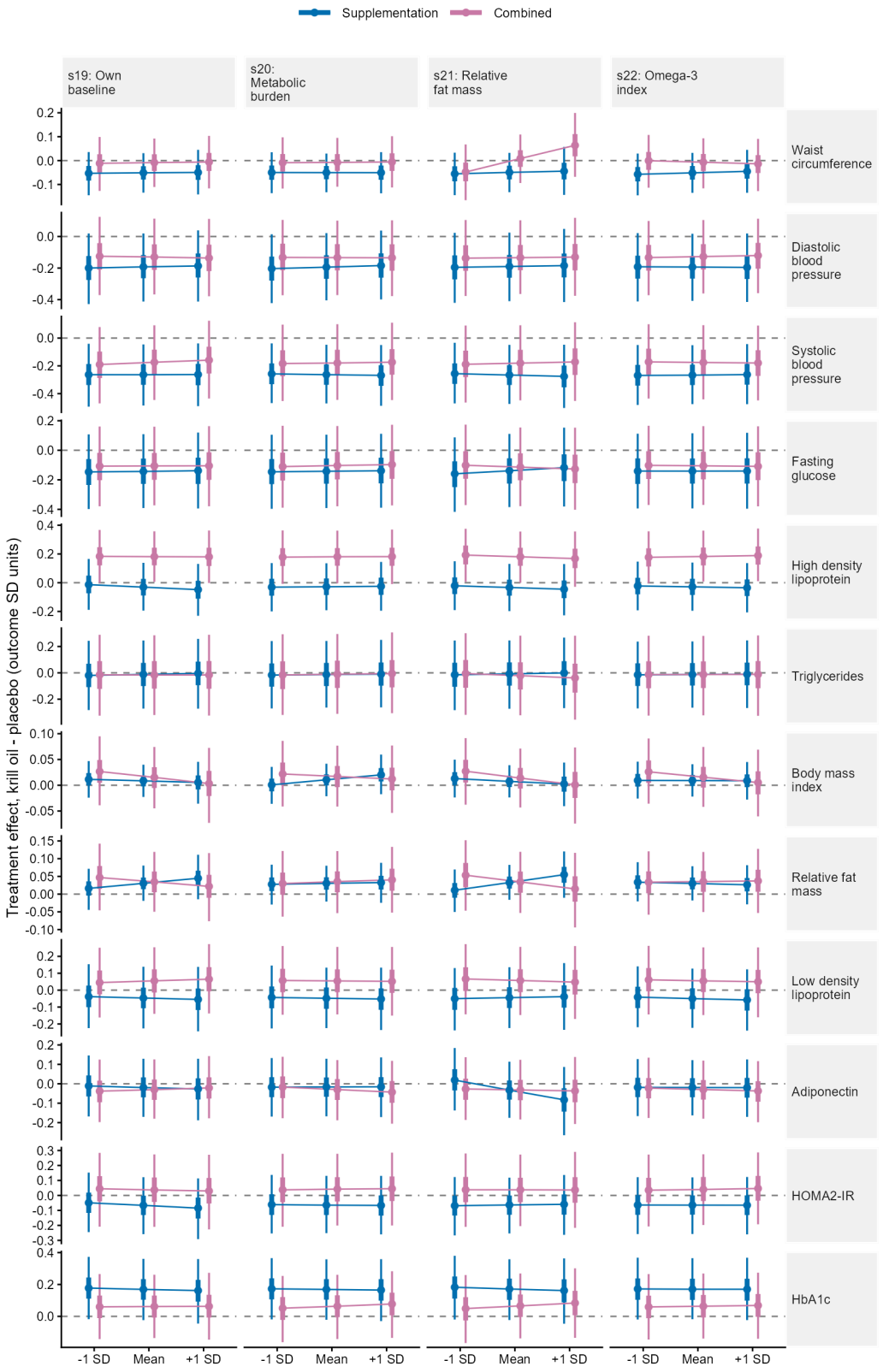

Figure 8.4: Predicted treatment effects (krill oil - placebo) at the moderator mean and at ± 1 SD of the moderator in models s19 (each outcome's own baseline value), s20 (baseline metabolic burden score), s21 (baseline relative fat mass) and s22 (baseline omega-3 index), after the supplementation and combined periods. Points and error bars show posterior medians with 50% and 95% CrI on the standardised scale (outcome SD units).

Combining the predicted effects on the diagnostic criteria per posterior draw, in the same way as for the metabolic burden score in s13 (average of the criteria re-weighted to population SD units), gives the predicted treatment effect on the burden score across each moderator (Figure 8.5).

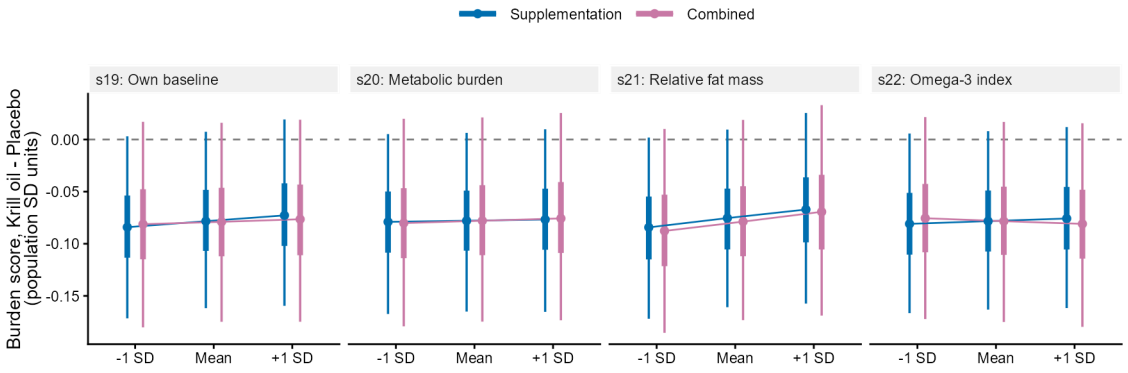

Figure 8.5: Predicted treatment effects (krill oil - placebo) on the metabolic burden score at the moderator mean and at ± 1 SD of the moderator in models s19 (each outcome's own baseline value), s20 (baseline metabolic burden score), s21 (baseline relative fat mass) and s22 (baseline omega-3 index), after the supplementation and combined periods. The burden score effect is combined per posterior draw from the effects on the diagnostic criteria using the same population SD re-weighting as the main burden score. Points and error bars show posterior medians with 50% and 95% CrI in population SD units.

The moderation parameters of all outcomes and the burden score are summarised in Table 8.3.

Table 8.3: Moderation parameters, the change in the treatment effect (krill oil - placebo) per SD of the moderator, in models s19 (each outcome's own baseline value, $\delta_{k,t}$), s20 (baseline metabolic burden score, $\gamma_{k,t}$), s21 (baseline relative fat mass, $\gamma_{k,t}$) and s22 (baseline omega-3 index, $\gamma_{k,t}$), after the supplementation and combined periods. The burden score parameter is combined per posterior draw from the parameters of the diagnostic criteria using the same population SD re-weighting as the main burden score (population SD units). Values are posterior medians with 50% and 95% CrI on the standardised scale (outcome SD units; population SD units for the burden score). P(benefit) is the posterior probability that the treatment effect at +1 SD of the moderator is more beneficial than at the moderator mean (higher values for HDL and adiponectin, lower values otherwise).

	Supplementation				Combined			
	Median	50% CrI	95% CrI	P(benefit)	Median	50% CrI	95% CrI	P(benefit)
s19: Own baseline								
Waist circumference	0.00	−0.01, 0.01	−0.04, 0.05	48%	0.00	−0.01, 0.01	−0.04, 0.05	45%
Diastolic blood pressure	0.00	−0.01, 0.02	−0.05, 0.09	42%	−0.00	−0.02, 0.01	−0.08, 0.05	55%
Systolic blood pressure	0.00	−0.01, 0.01	−0.06, 0.06	50%	0.01	−0.00, 0.03	−0.03, 0.12	33%
Fasting glucose	0.00	−0.01, 0.01	−0.05, 0.07	45%	0.00	−0.01, 0.01	−0.05, 0.06	48%
High density lipoprotein	−0.01	−0.03, 0.00	−0.11, 0.02	29%	−0.00	−0.01, 0.01	−0.06, 0.05	48%
Triglycerides	0.00	−0.00, 0.02	−0.04, 0.08	41%	−0.00	−0.01, 0.01	−0.06, 0.06	50%
Body mass index	−0.00	−0.01, 0.00	−0.03, 0.02	61%	−0.01	−0.02, 0.00	−0.06, 0.02	74%
Relative fat mass	0.01	0.00, 0.03	−0.01, 0.05	19%	−0.01	−0.02, 0.00	−0.07, 0.02	71%
Low density lipoprotein	−0.00	−0.02, 0.00	−0.08, 0.04	62%	0.00	−0.00, 0.02	−0.04, 0.08	38%
Adiponectin	−0.00	−0.02, 0.00	−0.07, 0.04	39%	0.00	−0.00, 0.02	−0.04, 0.07	62%
HOMA2-IR	−0.01	−0.03, 0.00	−0.11, 0.02	71%	−0.00	−0.02, 0.01	−0.08, 0.05	57%
HbA1c	−0.00	−0.02, 0.00	−0.08, 0.04	60%	0.00	−0.01, 0.01	−0.05, 0.06	47%
Metabolic burden score	0.00	−0.00, 0.01	−0.01, 0.04	33%	0.00	−0.00, 0.01	−0.02, 0.03	42%
s20: Metabolic burden								
Waist circumference	−0.00	−0.01, 0.01	−0.03, 0.03	52%	0.00	−0.01, 0.01	−0.03, 0.04	46%

9. Effects of krill oil supplementation on indications of metabolic syndrome - Model sensitivity

9.1 Are conclusions sensitive to the model structure?

The residual correlations modeled in s13 allow us to account for associations between outcome variables when estimating treatment effects. Additionally, correlations between time-points within variables are captured in a set of parameters and baseline values are modeled as separate multivariate distribution. Conceptually this could affect estimates in two ways. First, adding uncertainty associated with the between- and within-variable correlations could increase uncertainty in the treatment effect. However, the correlations will help in estimating missing data, this would reduce uncertainty in estimation of single measurements which propagates into the treatment effects and the combined burden score. A univariate model (s18) is used here to show the sensitivity of the conclusion to this model structure.

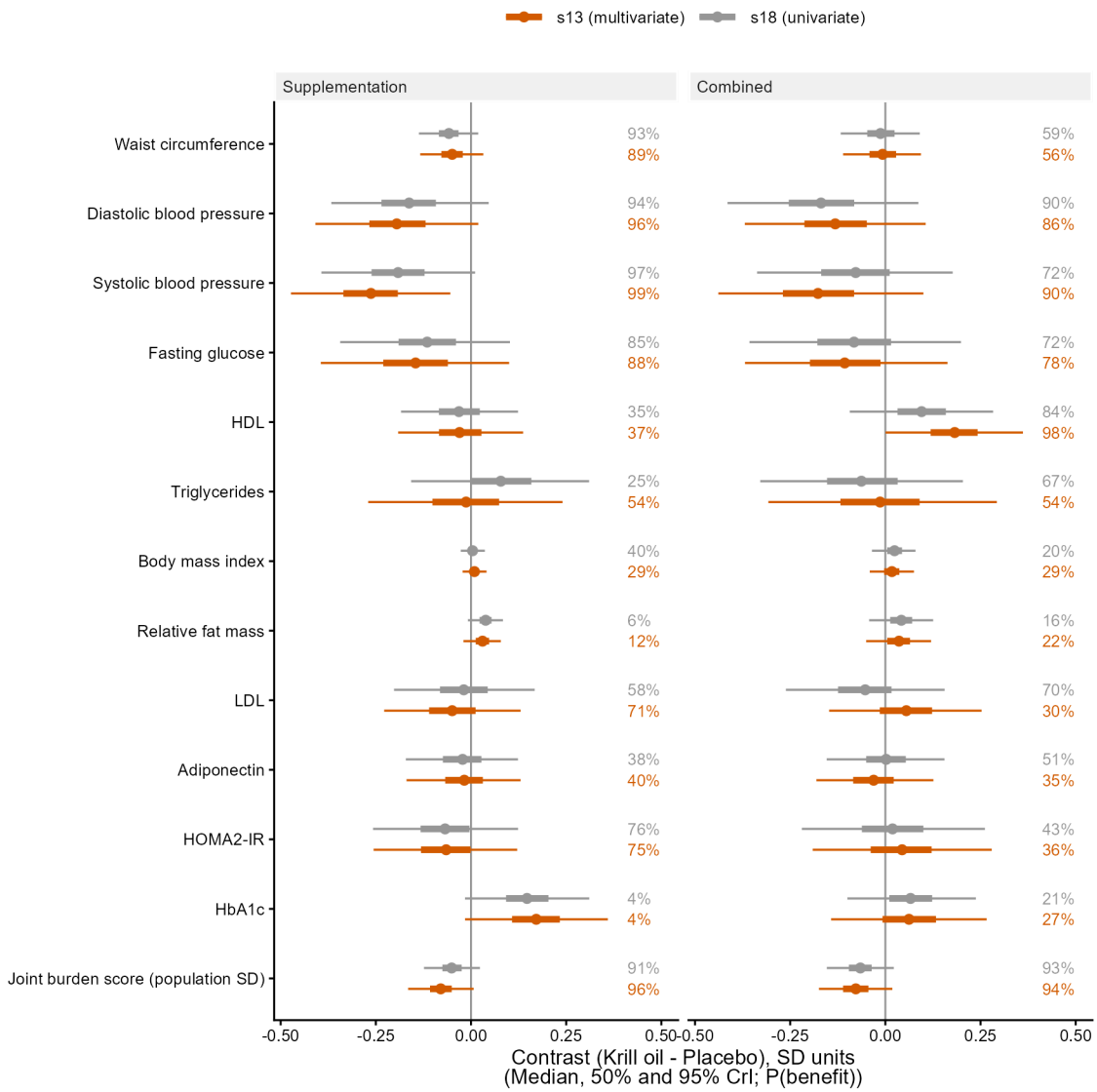

Figure 9.1: Treatment effects (krill oil - placebo) on the standardized scale (SD units) from the multivariate model (s13) and its univariate counterpart (s18). Points are posterior medians with 50% (thick) and 95% (thin) CrI. Numbers indicate P(benefit), the posterior probability of benefit for krill oil (higher values for HDL and adiponectin, lower values otherwise). The joint burden score (main analysis score, population SD units) is calculated per draw from the diagnostic criteria.

We first assess the effects of model structure on conclusions for each variable included in the joint models and the joint burden score. Overall, the models agree that the joint burden score is positively affected by treatment at both follow-up time-points albeit with more uncertainty in the univariate model (91 vs. 96% and 93 vs. 94% probability of benefit in s18 vs. s13 at time *supplementation* and *combined*, respectively; Figure 9.1). For single variables, most agree within 10-15 %-point on the benefit scale. However, triglycerides at the supplementation-only time-point and LDL at the combined time-point changed sign at the median of the posterior probability. Additionally, a somewhat large difference in magnitude between models was seen in HDL at the combined time-point.

The two models differ in how residuals are related between variables and between time-points. In s13, residual correlations are estimated with a regularizing prior, while s18 assumes them to be zero. The correlations are still present in the residuals from s18. Figure 9.2 shows the residual correlation patterns in both models with the between-timepoint within-variable shown on the diagonal, the between-variable correlations at *supplementation-only* and *combined* time-points in the lower and upper triangle, respectively. The correlation patterns are visible in both models but only the s13 model captures them to

carry information forward to the posterior. The estimated correlations are weaker due to regularization in the model.

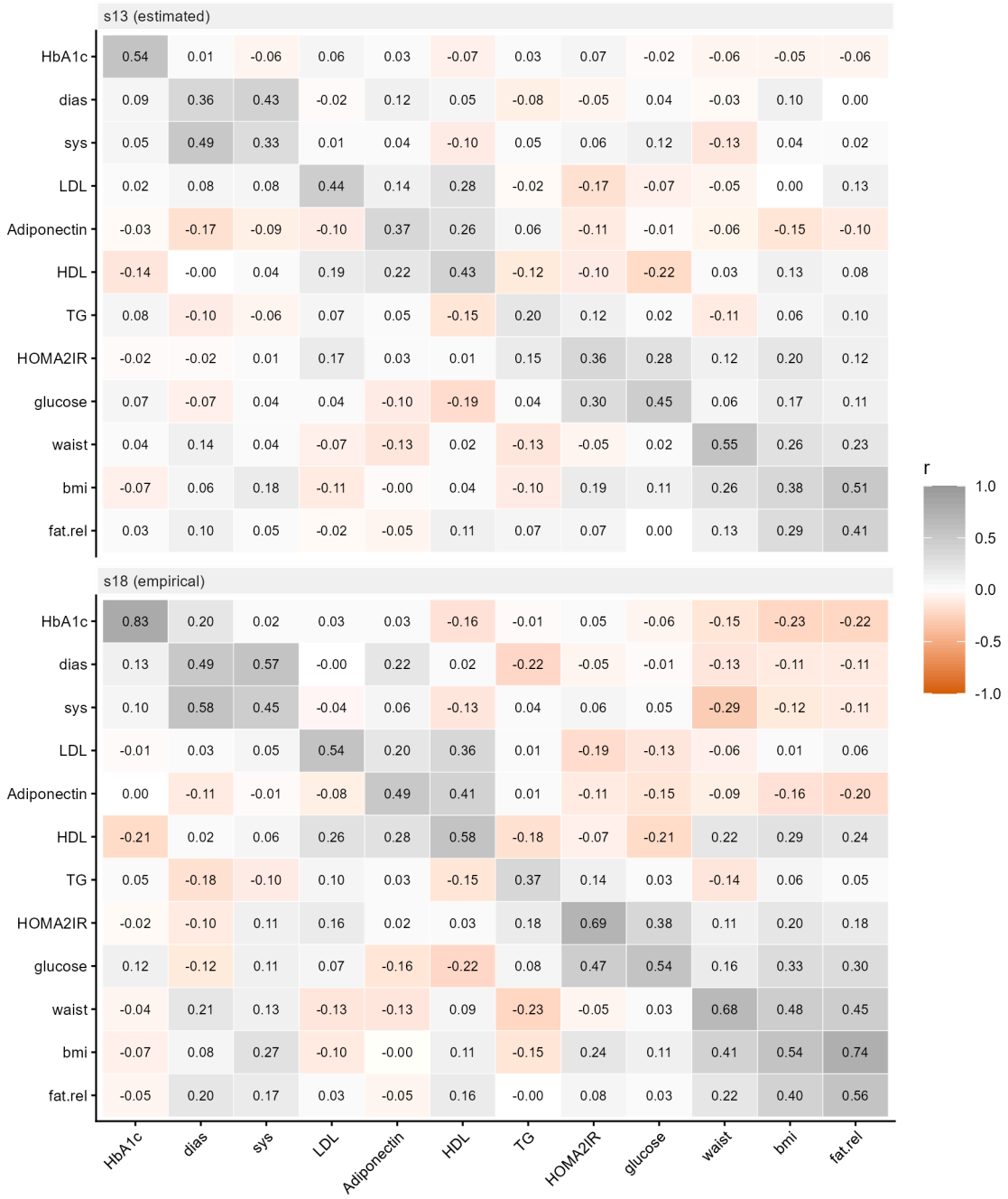

Figure 9.2: Residual correlations from s13 (posterior medians of Ω) and empirical correlations of residuals from s18 (observed values minus the posterior median of μ, pairwise complete observations). Lower triangle: supplementation (T2), upper triangle: combined (T4), diagonal: correlation between time-points within each variable. Variables are ordered by hierarchical clustering of the s13 correlations.

About a third of participants did not attend the combined-treatment (T4) assessment, giving missing values for all variables at T4. When participants have observed data at T2, s13 uses their deviation from the expected value at T2 to impute T4 values through the correlation between time-points, while s18 imputes T4 values from the expected value only (Figure 9.3). This shows up as correlated deviations from expected values at T2 and T4 in the s13 model but not in s18.

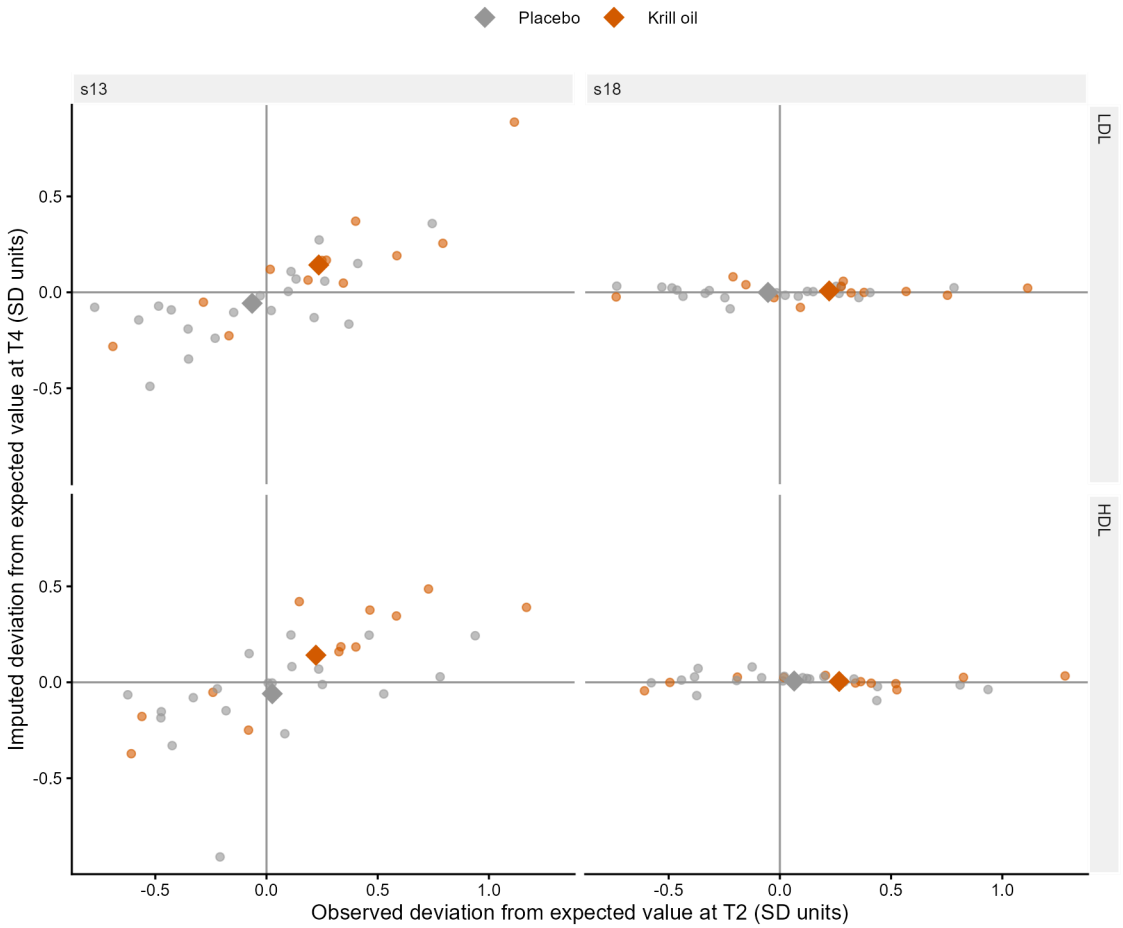

Figure 9.3: Imputed T4 values in participants with missing T4 but observed T2 values for LDL and HDL. The x axis shows the observed deviation from the expected value at T2 and the y axis the imputed deviation from the expected value at T4 (posterior medians, SD units), per model. Large symbols show group means.

Together the above indicate that the multivariate alternative takes advantage of the information available in the data set. However, a simpler model (uninformed) does not change the main conclusions of the analysis.

9.2 Changing the scale of the burden score

The burden score can be calculated from the five criteria measurements for the metabolic syndrome on the model scale, i.e. using the marginal SD (pooled over time-points, treatments and sexes) as the denominator for each criterion. This model-scale (marginal SD) score is a sensitivity analysis; the main analysis uses the score re-weighted to sex-specific population SD units (see below and `R/pop-ref.R`). Scaling by the marginal SD could be problematic as the marginal SD contains sex differences and variation due to potential treatment effects. First we will recalculate the marginal SD score using baseline, sex-adjusted standard deviations.

The data used in the model, for each measurement type k, is mean centered and scaled to the marginal SD:

$$z = \frac{x - m_k}{SD_k^{\text{marg}}} \tag{9.1}$$

Using the observed data we calculate a pooled SD using baseline data only where the variance (v) for each type is pooled within sex (men, m; women, f) and combined into a baseline SD.

$$SD_{\text{pooled}} = \sqrt{\frac{(n_m - 1)\, v_m + (n_f - 1)\, v_f}{n_m + n_f - 2}} \tag{9.2}$$

Because the pooled SD is calculated on the standardized data, it equals the ratio $r_k = SD_k^{\text{base}} / SD_k^{\text{marg}}$. Treatment contrasts estimated in marginal SD units ($\Delta_k / SD_k^{\text{marg}}$) are converted to baseline within-sex SD units by dividing by r_k, which gives $\Delta_k / SD_k^{\text{base}}$. The ratios indicated a slightly higher baseline SD than marginal SD for systolic and diastolic blood pressure (ratio 1.07 and 1.12, respectively). For HDL, triglycerides, glucose and waist circumference the ratios were below 1 (0.96, 0.95, 0.92, and 0.97), indicating a lower baseline within-sex SD than marginal SD.

The transformations can be validated in code as shown below. First we create a two time-point data set (`T1` and `T2`) with slightly different means and standard deviations. The marginal SD is then calculated from the full data set and used as the scale for a Z-score. Additionally, the raw baseline SD is calculated for later reference along with a SD of the Z-score at baseline. The mean difference on the marginal SD scale is calculated (`m_z_marg`) which is then re-scaled using the baseline SD (z-score units). This re-scaled mean difference is compared to the mean difference of a Z-score calculated using the raw baseline SD as the scale (`m_z_base_recalc`). The two numbers are the same.

```
set.seed(1)
T1 <- rnorm(150, 100, 25)
T2 <- rnorm(150, 110, 35)

# Calculate the marginal SD
marg_sd <- sd(c(T1, T2))
# Calculate the baseline SD
baseline_sd <- sd(T1)

# Scale the outcome to marginal SD
z_marg <- (c(T1, T2) - mean(c(T1, T2))) / marg_sd

# Calculate the baseline SD on the standardized scale
baseline_sd_z <- sd(z_marg[1:150])

# Calculate mean in standardized
m_z_marg <- mean(z_marg[151:300] - z_marg[1:150])

# Re-scale the mean
m_z_base <- m_z_marg / baseline_sd_z

# Calculate diff on the baseline_sd scale
z_base <- (c(T1, T2) - mean(c(T1, T2))) / baseline_sd

# Calculate the contrast on the baseline SD-standardized scale.
m_z_base_recalc <- mean(z_base[151:300] - z_base[1:150])

# Check if the re-calculated score matches the
m_z_base_recalc == m_z_base
```

```
[1] TRUE
```

The burden score is recalculated per posterior draw with the same criterion weights as in the main analysis, scaled either by the marginal SD or by the pooled within-sex baseline SD. This gives us a sense of the sensitivity of the conclusion to the scaling procedure (Table 9.1). Scaling the burden score using sex-centered baseline data did not change the conclusion in a meaningful way.

Table 9.1: Comparing inference (Krill oil - Placebo) for the joint burden score between different scales. Marginal uses the full data set to calculate the scale for the Z-score component of each score, baseline uses only baseline data to calculate a pooled standard deviation on sex-centered data.

Scale	Follow-up	Median	PI 2.5%	PI 97.5%	P(Benefit)
marginal	Supplementation-only	−0.082	−0.175	0.011	0.957
baseline	Supplementation-only	−0.081	−0.179	0.017	0.949
marginal	Combined	−0.091	−0.198	0.015	0.954
baseline	Combined	−0.092	−0.204	0.018	0.95

The burden scores presented so far in this section are scaled by SDs from the trial sample (marginal or baseline within-sex SD). **[TODO: incomplete sentence in draft ("The burden score presented so far is scaled ..."); completed here from context, check intended content. Below, an externally weighted score and scores scaled by population reference values are considered.]**

9.3 An externally weighted severity score

The burden score weights each criterion equally on the standardized scale. As an alternative, we used the metabolic syndrome severity score from Gurka MJ, Lilly CL, Oliver MN, DeBoer MD. (18), which weights the criteria according to a confirmatory factor analysis of NHANES data. The score is sex- and race/ethnicity-specific, and we used the equations for non-Hispanic White adults with waist circumference (cm), HDL (mg/dL), systolic blood pressure (mmHg), triglycerides (ln mg/dL) and fasting glucose (mg/dL):

$$\begin{aligned} Z_{\text{men}} &= -5.4559 + 0.0125\,\text{WC} - 0.0251\,\text{HDL} + 0.0047\,\text{SBP} + 0.8244\,\ln(\text{TG}) + 0.0106\,\text{Glu} \\ Z_{\text{women}} &= -7.2591 + 0.0254\,\text{WC} - 0.0120\,\text{HDL} + 0.0075\,\text{SBP} + 0.5800\,\ln(\text{TG}) + 0.0203\,\text{Glu} \end{aligned} \tag{9.3}$$

Diastolic blood pressure is not part of the score. The score is linear in its inputs, and each input is a linear back-transformation of the standardized model scale ($x = m_k + SD_k^{\text{marg}} z$, with TG on the log scale). The expected treatment contrast on the score, for a given sex and baseline profile, is therefore exactly

$$\Delta Z_t = \sum_k b_k\, c_k\, SD_k^{\text{marg}} \left(\alpha_{t,k,\,\text{KO}} - \alpha_{t,k,\,\text{PL}}\right) \tag{9.4}$$

where b_k are the sex-specific coefficients and c_k converts mmol/L to mg/dL ($c_k = 1$ for waist, SBP and ln(TG)). Contrasts are calculated per posterior draw for men and women, and as an average weighted by the proportion of men and women in the sample (`gurka_contrast()` in `R/helpers.R`). Per-participant scores at each time-point are calculated from the observed or imputed values in the model (`gurka_score()`).

As a check on the back-transformation, scores calculated directly from observed complete profiles (n = 340 participant-time combinations) were compared to the model-based scores for the same participants; the largest absolute difference was 3.5^{-8}.

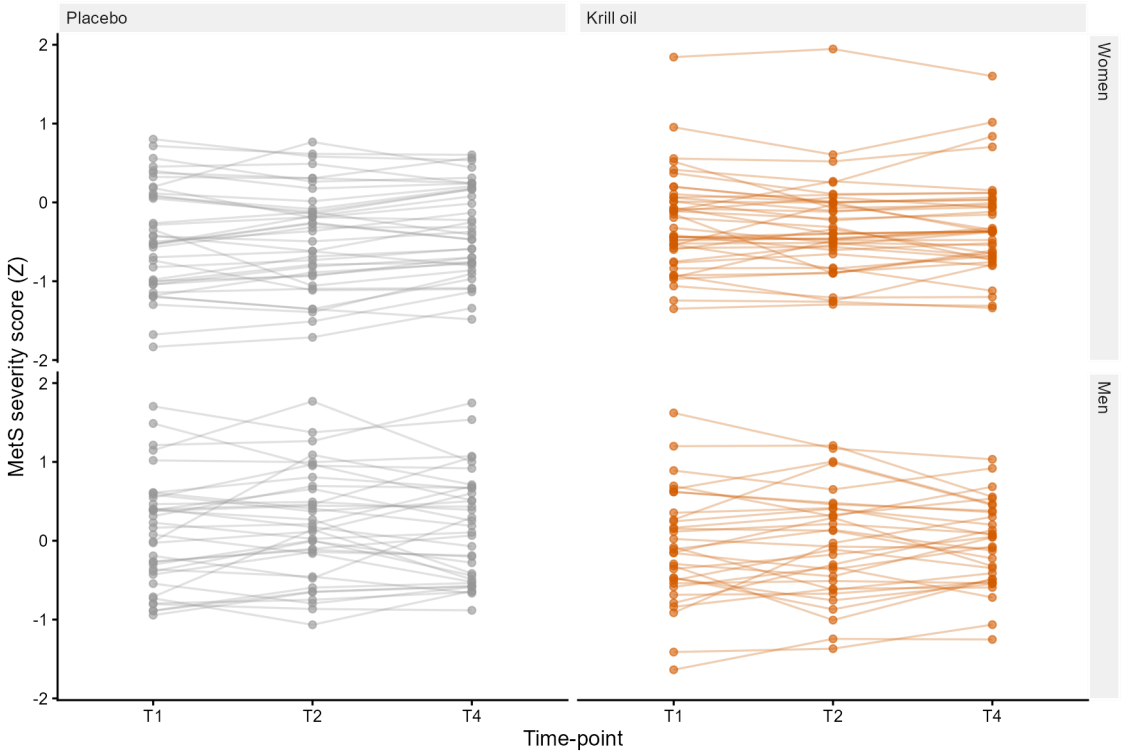

Figure 9.4: Posterior median metabolic syndrome severity score (18) per participant and time-point. Missing measurements are replaced by model-based imputations. Lines connect scores within participants.

Table 9.2: Treatment contrast (Krill oil - Placebo) on the metabolic syndrome severity score (18). The score is a Z-score relative to the NHANES reference population; negative values indicate a less severe metabolic profile in the krill oil group.

Sex	Follow-up	Median	PI 2.5%	PI 97.5%	P(Benefit)
Men	Supplementation-only	−0.039	−0.159	0.079	0.741
Women	Supplementation-only	−0.075	−0.176	0.023	0.935
Average	Supplementation-only	−0.059	−0.166	0.045	0.867
Men	Combined	−0.082	−0.219	0.057	0.879
Women	Combined	−0.071	−0.188	0.042	0.89
Average	Combined	−0.076	−0.2	0.046	0.891

9.4 A continuous score from reference values

Following Guseman EH, Eisenmann JC, Laurson KR, Cook SR, Stratbucker W. (19), a continuous metabolic syndrome score (cMetS) was calculated as the sum of z-scores of waist circumference, HDL (reversed sign), ln(TG), fasting glucose and mean arterial pressure (MAP = diastolic + (systolic - diastolic) / 3), relative to sex-specific reference means and SDs. As for the Gurka score, the score is linear in the model scale and the expected treatment contrast for participant p is

$$\Delta\text{cMetS}_{t,p} = \sum_k w_k \frac{SD_k^{\text{marg}}}{SD_{k,p}^{\text{ref}}} \left(\alpha_{t,k,\,\text{KO}} - \alpha_{t,k,\,\text{PL}}\right) \tag{9.5}$$

with MAP contributing 2/3 of the diastolic and 1/3 of the systolic contrast. Reference means cancel in the contrast, and contrasts are averaged over participants per sex and overall (`cmets_contrast()` in `R/helpers.R`).

Population reference values were retrieved from the literature (10,11) and used to construct population-based scales for each outcome. The standard deviations listed in Table 9.3 formed the basis of set of SD combinations allowing for rescaling the study-specific burden score and extend the analysis to the metabolic syndrome score proposed by Guseman et al. (19). Published fasting glucose estimates were not used, as they included participants with diabetes or were based on non-fasting samples; fasting glucose was instead scaled by the sex-specific baseline mean and SD in the trial sample. The burden score scaled by population SDs is the score used in the main analysis (shared references in `R/pop-ref.R`).

Table 9.3: Published reference values (mean, SD and n) for the components of the continuous metabolic syndrome score from Nordic populations. TG is summarised on the natural log scale (mmol/L); MAP, mean arterial pressure. [a]Derived from the reported arithmetic mean (m) and SD (s) assuming a log-normal distribution: $\sigma^2 = \ln(1 + s^2/m^2)$, $\mu = \ln m - \sigma^2/2$. [b]Derived as DBP + (SBP - DBP)/3; the SD assumes a correlation of 0.8 between SBP and DBP (SD 12.0–12.8 and 12.4–13.2 for men and women, respectively, for correlations of 0.6–0.8). [c]Derived from the reported median (M) and interquartile range (IQR): glucose assuming a normal distribution (mean = M, SD = IQR/1.349) and TG assuming a log-normal distribution ($\mu = \ln M$, $\sigma = \text{asinh}\ (\text{IQR}\ /2M)/0.6745$).

Study	Description	Sex	Age (y)	Analyte	n	Mean	SD
Tromsø 7	Population-based, Tromsø (Norway) 2015–16, ages 40+, non-fasting blood samples (10)	Men	40–69	Waist (cm)			
		Men	40–69	HDL (mmol/L)	7670	1.4	0.4
		Men	40–69	ln TG (mmol/L)			
		Men	40–69	Glucose (mmol/L)			
		Men	40–69	MAP (mmHg)			
		Men	40–69	Systolic BP (mmHg)	7670	130.9	17.1
		Men	40–69	Diastolic BP (mmHg)	7670	78.9	9.6
		Women	40–69	Waist (cm)			
		Women	40–69	HDL (mmol/L)	8896	1.6	0.5
		Women	40–69	ln TG (mmol/L)			
		Women	40–69	Glucose (mmol/L)			
		Women	40–69	MAP (mmHg)			
		Women	40–69	Systolic BP (mmHg)	8896	126.8	18.3
		Women	40–69	Diastolic BP (mmHg)	8896	75.5	10.01 **[TODO: check decimals against the Tromsø 7 source (10.0?); keep in sync with R/pop-ref.R, which uses 10.01]**
FinHealth 2017	Nationally represen-	Men	25–93	Waist (cm)	2621	99.1	12.9

Table 9.4: Treatment contrast (Krill oil - Placebo) on metabolic syndrome scores. Burden score: mean of the z-scores of waist circumference, HDL (reversed sign), ln(TG), glucose and blood pressure (average of systolic and diastolic z-scores), scaled by the marginal SD (model scale), the sex-specific baseline SD in the trial sample, or sex-specific population SDs (Table 9.3; the main analysis score). cMetS: sum of the z-scores of the same criteria with blood pressure as mean arterial pressure (19). MetS severity Z-score: sex-specific equations for non-Hispanic White adults (18). Sex-specific contrasts differ only through sex-specific scaling or coefficients; the average is weighted by the proportion of men and women in the sample. Negative values indicate a less severe metabolic profile in the krill oil group.

Sex	Follow-up	Median	PI 2.5%	PI 97.5%	P(Benefit)
Burden score, marginal SD					
Average	Supplementation-only	−0.082	−0.175	0.011	0.957
Men	Supplementation-only	−0.082	−0.175	0.011	0.957
Women	Supplementation-only	−0.082	−0.175	0.011	0.957
Average	Combined	−0.091	−0.198	0.015	0.954
Men	Combined	−0.091	−0.198	0.015	0.954
Women	Combined	−0.091	−0.198	0.015	0.954
Burden score, trial baseline SD					
Average	Supplementation-only	−0.081	−0.18	0.018	0.947
Men	Supplementation-only	−0.081	−0.179	0.017	0.949
Women	Supplementation-only	−0.081	−0.182	0.019	0.945
Average	Combined	−0.093	−0.207	0.018	0.949
Men	Combined	−0.1	−0.211	0.009	0.961
Women	Combined	−0.088	−0.205	0.025	0.936
Burden score, population SD					
Average	Supplementation-only	−0.08	−0.165	0.007	0.965
Men	Supplementation-only	−0.08	−0.163	0.005	0.968
Women	Supplementation-only	−0.079	−0.167	0.009	0.963
Average	Combined	−0.078	−0.174	0.018	0.941
Men	Combined	−0.081	−0.176	0.014	0.953
Women	Combined	−0.075	−0.174	0.023	0.933
cMetS, trial baseline SD					
Average	Supplementation-only	−0.411	−0.909	0.084	0.949
Men	Supplementation-only	−0.414	−0.903	0.081	0.949
Women	Supplementation-only	−0.408	−0.912	0.092	0.945
Average	Combined	−0.469	−1.033	0.088	0.949
Men	Combined	−0.504	−1.059	0.045	0.963
Women	Combined	−0.44	−1.022	0.123	0.936
cMetS, population SD					
Average	Supplementation-only	−0.407	−0.837	0.029	0.967
Men	Supplementation-only	−0.408	−0.83	0.022	0.97
Women	Supplementation-only	−0.406	−0.847	0.035	0.964
Average	Combined	−0.395	−0.882	0.091	0.943
Men	Combined	−0.41	−0.889	0.067	0.954

9.4.1 Associations of continuous Metabolic syndrome scores to related scores

DeBoer and Gurka (20) examined the correlation of their metabolic syndrome score with HOMA and adiponectin; here we do the same for the burden score scaled by population SD (main analysis), marginal SD and baseline SD, and the two continuous scores (18–20).

Scores, HOMA2-IR and adiponectin were retrieved per participant and time-point from the posterior, using observed values where available and imputed values otherwise. Rank (Spearman) correlations between each score and HOMA2-IR or adiponectin were calculated per posterior draw, so that the uncertainty in imputed values propagates to the correlations (Figure 9.7). The intervals therefore reflect uncertainty from imputation only, not sampling variation, and imputed values follow the residual correlations estimated in the model. Associations are shown on the participant level using posterior medians in Figure 9.5 and Figure 9.6. Together these analyses show general agreement between metabolic syndrome scores in associations with **[TODO: incomplete sentence; complete, e.g. "HOMA2-IR and adiponectin", and check against Figure 9.7]**

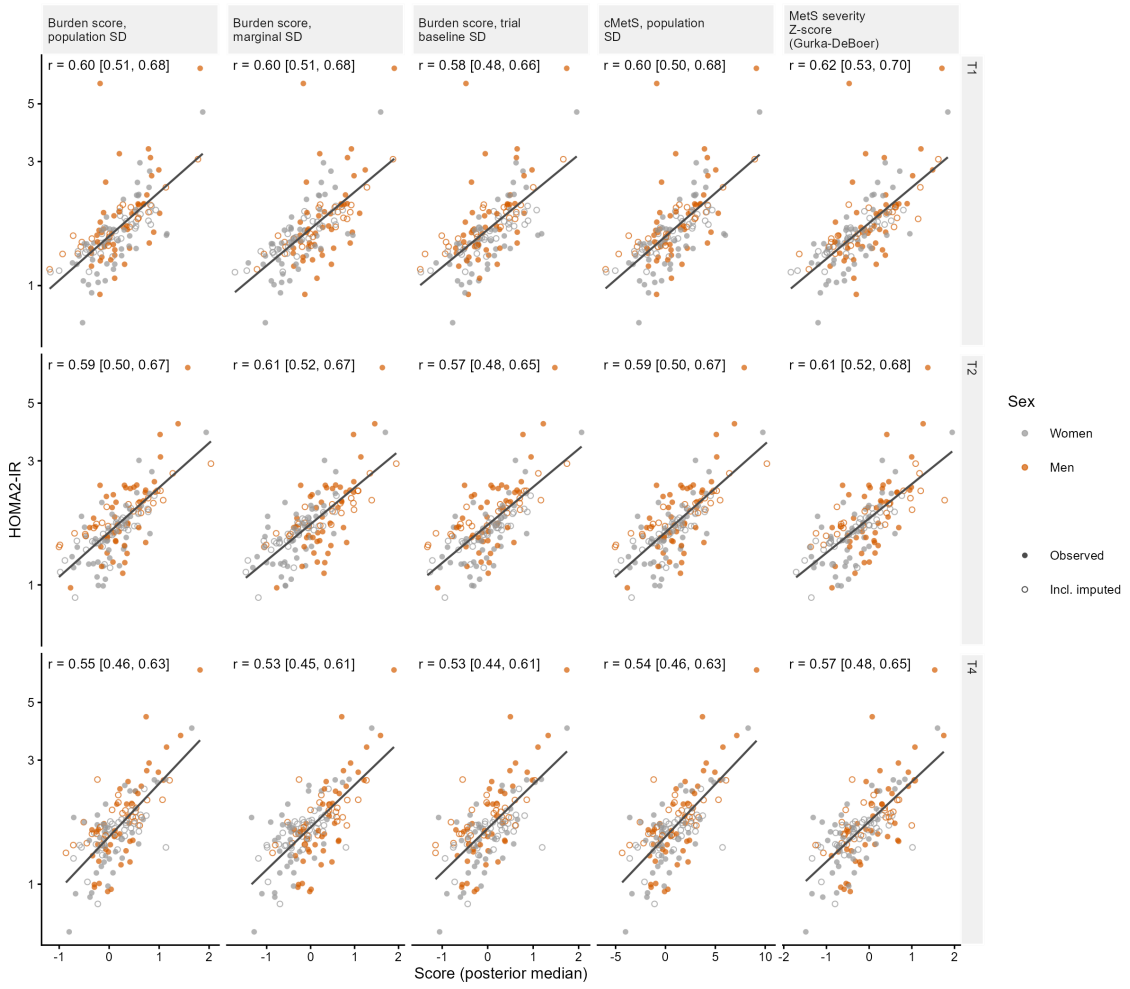

Figure 9.5: Associations between metabolic syndrome scores and HOMA2-IR (log scale) per time-point. Points are participant-level posterior medians; open symbols include imputed values. Spearman correlations (median and 95% interval over posterior draws, reflecting imputation uncertainty) are calculated per posterior draw.

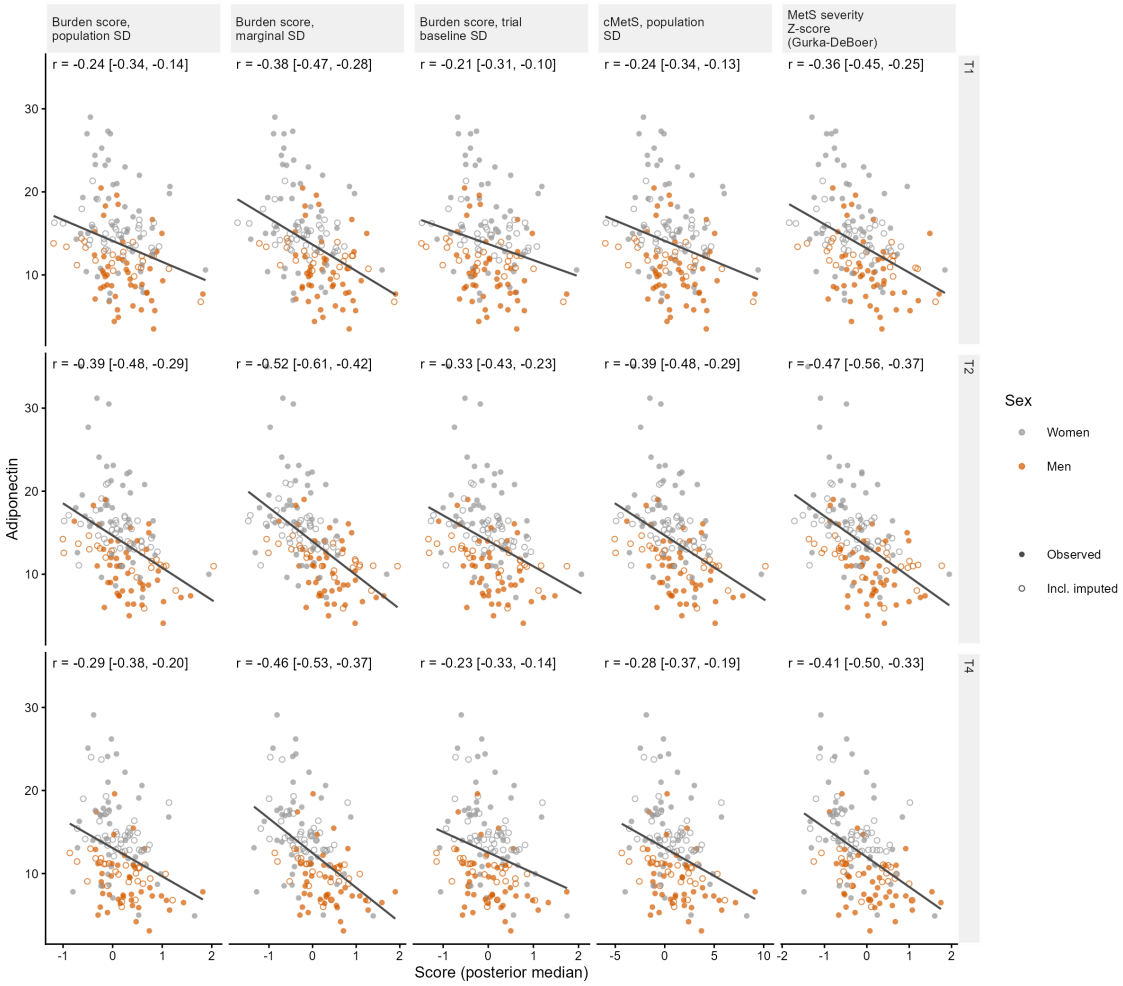

Figure 9.6: Associations between metabolic syndrome scores and adiponectin per time-point. Points are participant-level posterior medians; open symbols include imputed values. Spearman correlations (median and 95% interval over posterior draws, reflecting imputation uncertainty) are calculated per posterior draw.

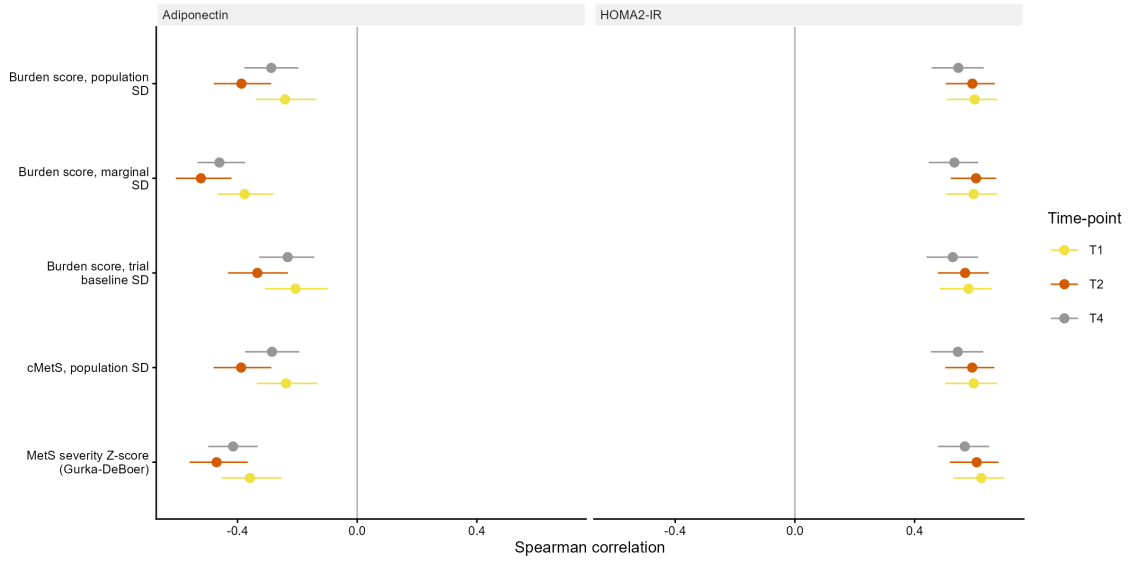

Figure 9.7: Spearman correlations (median and 95% interval over posterior draws, reflecting imputation uncertainty) between metabolic syndrome scores and HOMA2-IR or adiponectin at each time-point.

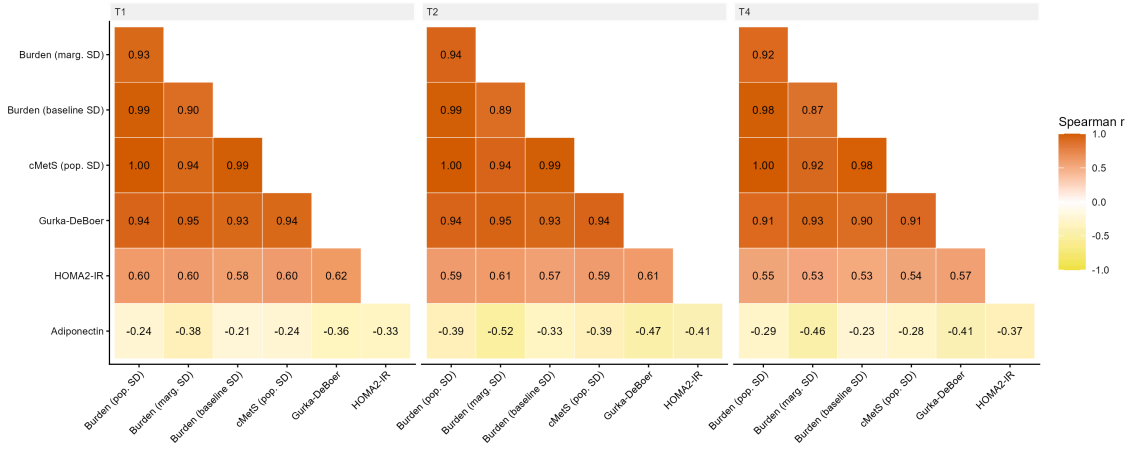

Figure 9.8: Spearman correlations between metabolic syndrome scores, HOMA2-IR and adiponectin at each time-point. Correlations are calculated per posterior draw (observed or imputed values) and summarised as the median over draws.

9.4.2 Contribution of subscores in the joint treatment contrasts

All scores are linear in the standardized model scale, and the treatment contrast on a score can therefore be decomposed exactly into one term per criterion,

$$\Delta\text{Score}_t = \sum_k w_k \frac{SD_k^{\text{marg}}}{SD_k^{\text{ref}}} \left(\alpha_{t,k,\,\text{KO}} - \alpha_{t,k,\,\text{PL}}\right) = \sum_k C_{t,k}, \tag{9.6}$$

where the weight of criterion k is the product of the score weight w_k and the scaling (for the Gurka-DeBoer score, $b_k c_k SD_k^{\text{marg}}$). The contributions $C_{t,k}$ are calculated per posterior draw and sum to the total contrast in each draw. Systolic and diastolic blood pressure are combined into a single blood pressure contribution (0.5 each in the burden score, 1/3 and 2/3 in MAP, systolic only in the Gurka-DeBoer score). Contributions are shown in units of each score (Figure 9.9).

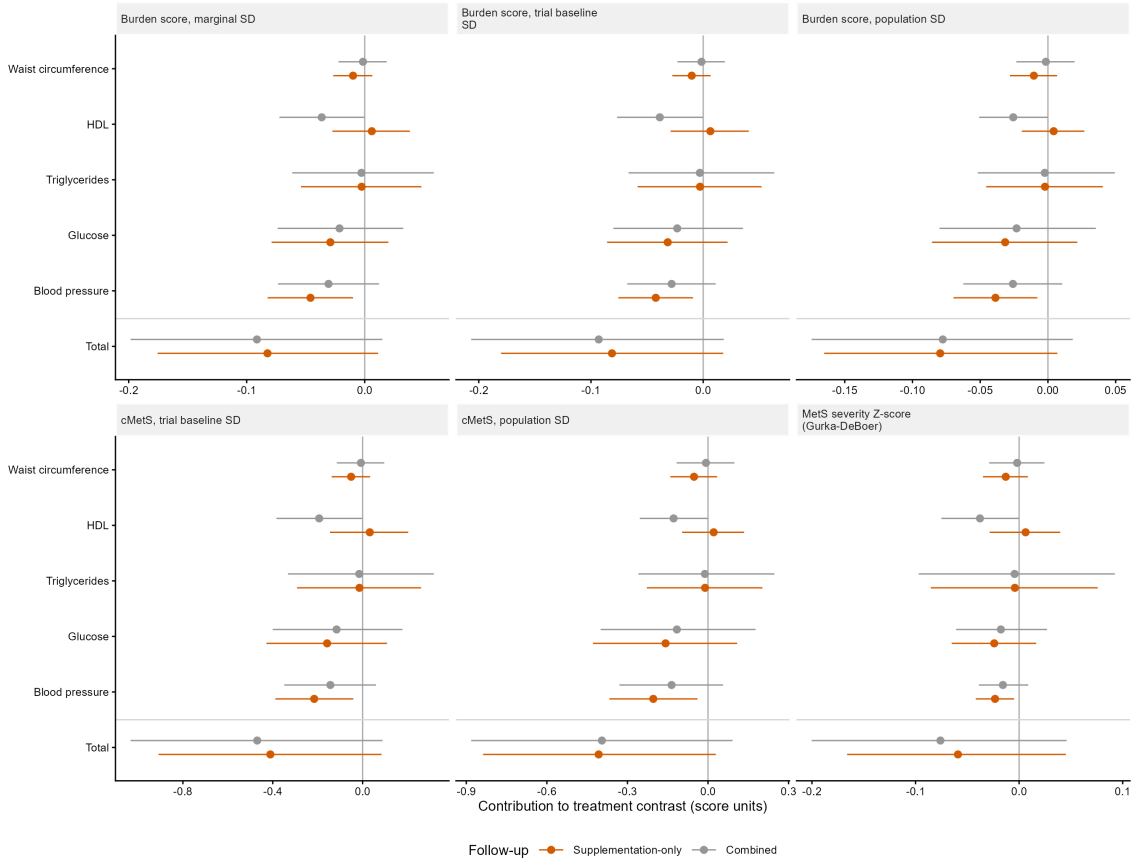

Figure 9.9: Contribution of each criterion to the treatment contrast (Krill oil - Placebo) on the metabolic syndrome scores (posterior median and 95% credible interval), averaged over men and women. Contributions are in units of each score and sum to the total contrast in each posterior draw. Negative values indicate a contribution towards a less severe metabolic profile in the krill oil group.

Because contributions are available per posterior draw, criteria can be compared within each score. A criterion was considered to contribute more than another if its contribution was more negative, i.e. towards a less severe metabolic profile in the krill oil group. Pairwise probabilities, $P(C_{t,j} < C_{t,k})$, are shown in Figure 9.10, and the probability that each criterion is the largest contributor is shown in Table 9.5. As contributions are the product of score weights and treatment effects, comparisons are made within scores; differences between scores reflect the weighting of the criteria.

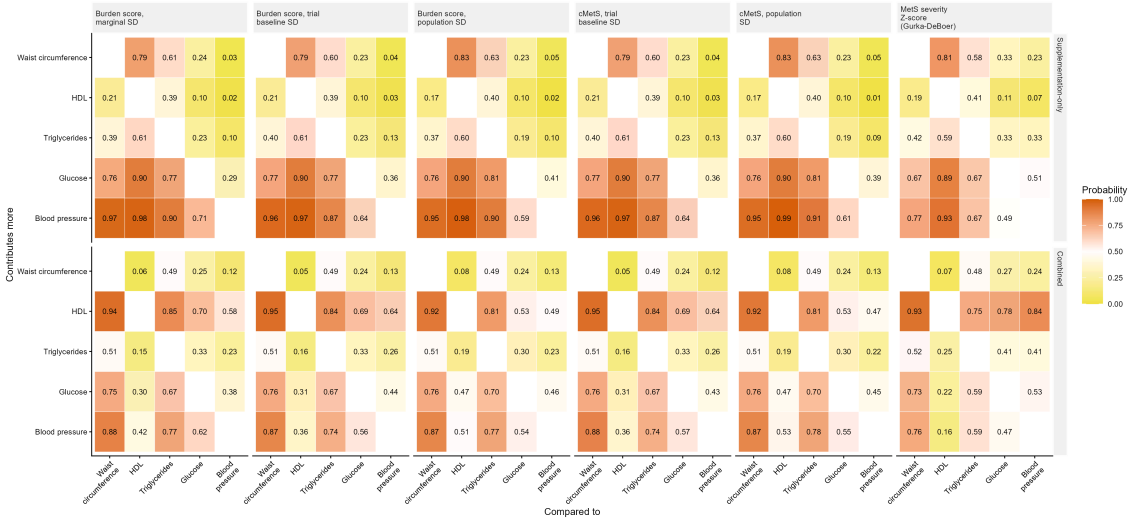

Figure 9.10: Pairwise comparisons of criterion contributions to the treatment contrast (Krill oil - Placebo) on each metabolic syndrome score, averaged over men and women. Each cell shows the posterior probability that the row criterion contributes more (more negative contribution, towards a less severe metabolic profile in the krill oil group) than the column criterion.

Table 9.5: Posterior probability that each criterion is the largest contributor (most negative contribution) to the treatment contrast (Krill oil - Placebo) on each metabolic syndrome score, averaged over men and women. Probabilities sum to one across criteria within each score and follow-up.

Follow-up	Waist circumference	HDL	Triglycerides	Glucose	Blood pressure
Burden score, marginal SD					
Supplementation-only	0.01	0.00	0.06	0.27	0.66
Combined	0.01	0.41	0.08	0.19	0.31
Burden score, trial baseline SD					
Supplementation-only	0.01	0.01	0.08	0.33	0.57
Combined	0.01	0.44	0.09	0.22	0.24
Burden score, population SD					
Supplementation-only	0.01	0.00	0.05	0.39	0.55
Combined	0.01	0.25	0.09	0.33	0.32
cMetS, trial baseline SD					
Supplementation-only	0.01	0.01	0.08	0.33	0.58
Combined	0.01	0.44	0.09	0.21	0.25
cMetS, population SD					
Supplementation-only	0.01	0.00	0.05	0.37	0.57
Combined	0.01	0.24	0.09	0.32	0.34
MetS severity Z-score (Gurka-DeBoer)					
Supplementation-only	0.09	0.02	0.25	0.36	0.28
Combined	0.02	0.54	0.22	0.15	0.07

10. Oral glucose-tolerance test

10.1 Introduction

Here we model the oral glucose-tolerance test data. Blood glucose was measured before (0 min) and 15, 30, 45, 60, 90 and 120 min after a standard 75 g glucose drink. The test was performed before the intervention period (T1), after the supplementation-only period (T2), and after the full intervention (combined period, supplementation and resistance training vs. placebo and resistance training). The goal is to evaluate the effect of the intervention (krill oil supplementation) on the glucose tolerance curve.

We aim to build a model that captures the glucose time-course after ingesting the drink. We will accomplish this by fitting a spline model to participant-level data with treatment-groups and time-points as population-level effects.

10.2 Data preparations

10.3 Preliminary models

10.3.1 Baseline model with group-level effects (participants)

In a preliminary model we fit complete data (seven measurements per participant) from the pre-intervention data (n = 146). The basis splines are visualized in Figure 10.1.

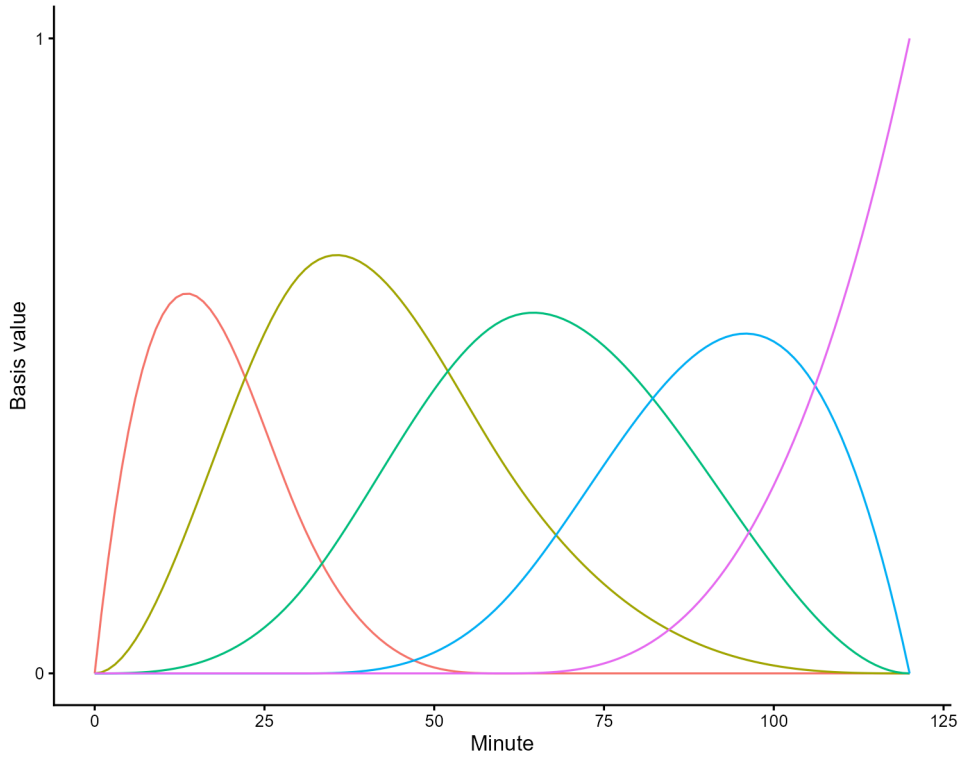

Figure 10.1: Basis splines across the glucose tolerance test.

The pre-intervention multilevel model (`mo1.stan`) for blood glucose (y) gives each participant p their own curve via a random intercept a_p and random spline coefficients $b_{k,p}$, drawn jointly (intercept + spline)

from a correlated multivariate normal (this is equivalent to the brms `m5` parameterization `archive/ogtt-analysis.qmd`):

$$
\begin{aligned}
y_i &\sim \text{Normal}\ (\mu_i, \sigma) \\
\mu_i &= \alpha + a_{p[i]} + \sum_{k=1}^{K} (\beta_k + b_{k,p[i]})\, B_{k,i} \\
(a_p, b_{1,p}, ..., b_{K,p})^\top &\sim \text{MVNormal}\ (0, \mathbf{S}\,\mathbf{R}\,\mathbf{S}) \\
\mathbf{S} &= \text{diag}\ (\sigma_{\text{sd}}) \\
\sigma_{\text{sd}} &\sim \text{Exponential}\ (2) \\
\mathbf{R} &\sim \text{LKJ}\ (1) \\
\alpha &\sim \text{Normal}\ (0, 2) \\
\beta_k &\sim \text{Normal}\ (0, 2) \\
\sigma &\sim \text{Exponential}\ (1)
\end{aligned}
\tag{10.1}
$$

The basis B is `bs(min, df = 5)` (no intercept column), so the random intercept a_p is identified separately from the spline weights. The choice of 5 spline components was done after preliminary prior- and posterior simulations indicating that this sufficiently regularize individual curves. The correlation matrix $\mathbf{R}$ is sampled via a non-centered Cholesky factor.

In preliminary versions of this model (`archive/ogtt-analysis.qmd`), specific priors were used to regularize the population-level spline towards increased glucose in the first half of the test followed by declining values in the second half. Since the data will likely wash away such prior information we will simplify this part of the model to avoid prior-likelihood conflicts.

Below we run a prior predictive check of the pre-intervention model. The Prior-predictive check (Figure 10.2) indicates that the model does not expect systematic changes over the course of the test but allows for variation across participants.

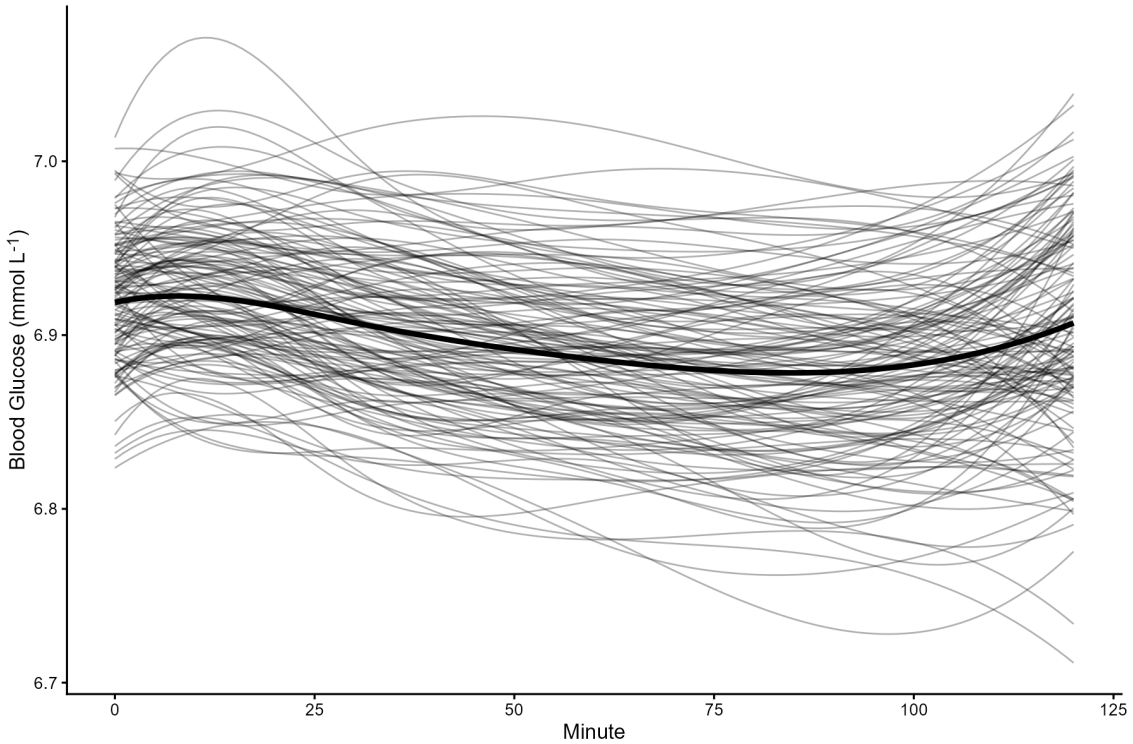

Figure 10.2: Prior predictive simulation of population average and participant-level glucose curves.

Running the model on the actual data lets us explore the model fit to data. The overall model fit gives a population mean curve and allows for large variations between individuals (Figure 10.3 A). Inspecting three individuals indicate that individual curves are shrunken from very wiggly shapes (Figure 10.3 B).

Analysis of residuals indicate that there might be a tendency towards larger positive errors at higher fitted values (Figure 10.3 C), the overall data patterns indicate that the observed data can be replicated using the model. Overall these checks validate the model for further development.

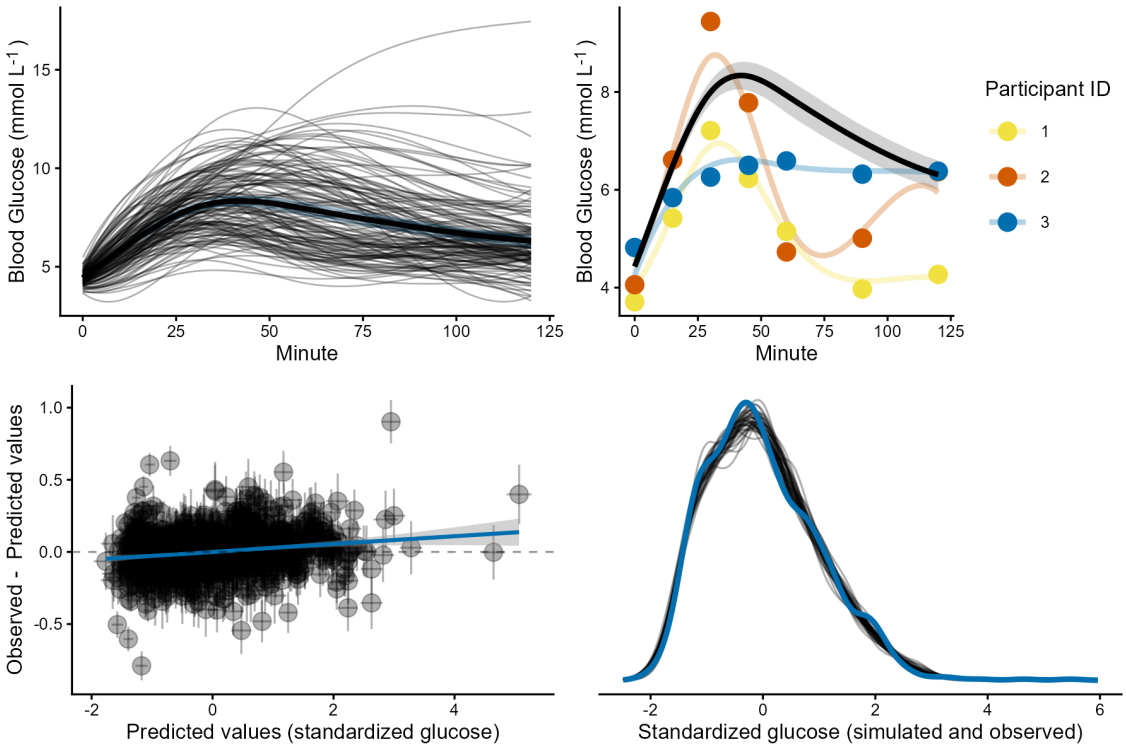

Figure 10.3: Posterior predictive simulation of population average and participant-level glucose curves (A and B). Residuals vs. predicted values (C) and distributions of simulated (y^{rep}) data sets and the observed data.

10.3.2 Three-timepoint multilevel model

The model (`mo2.stan`) extends the pre-intervention model to include three timepoints (T1, T2, T4) and the treatment groups, following the brms `m5` parameterization (`archive/ogtt-analysis.qmd`). The population mean is a cell mean plus a spline curve per cell g (here g = time x treatment). Each participant gets a random intercept and random spline coefficients that may differ by time t, drawn jointly from a correlated multivariate normal:

$$
\begin{aligned}
y_i &\sim \text{Normal}\ (\mu_i, \sigma) \\
\mu_i &= \left(a_{g[i]} + \sum_k b_{k,g[i]}\, B_{k,i} \right) + \left(a_{p[i]} + \sum_k \beta_{k,t[i],p[i]}\, B_{k,i} \right) \\
(a_p, \beta_p)^\top &\sim \text{MVNormal}\ (\mathbf{0}, \mathbf{S}\,\mathbf{R}\,\mathbf{S}) \\
\mathbf{S} &= \text{diag}\ (\sigma_{\text{sd}}) \\
\sigma_{\text{sd}} &\sim \text{Exponential}\ (1) \\
\mathbf{R} &\sim \text{LKJ}\ (1) \\
a_g &\sim \text{Normal}\ (0, 2) \\
b_{k,g} &\sim \text{Normal}\ (0, 2) \\
\sigma &\sim \text{Exponential}\ (1)
\end{aligned}
\tag{10.2}
$$

The participant block has $M = 1 + K\,T$ effects (intercept + a spline per time point), all correlated through $\mathbf{R}$ and sampled with a non-centered Cholesky factor. The prior predictive simulation indicates very flexible curves per time × treatment (Figure 10.4)

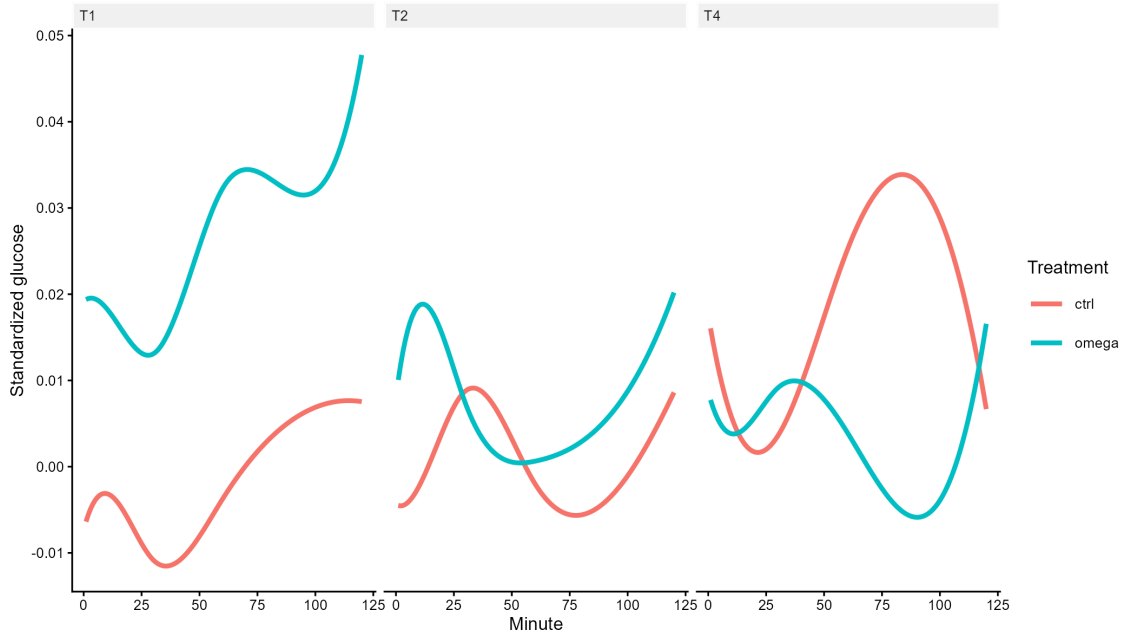

Figure 10.4: Prior predictive simulation using model `mo2` showing average group × time curves.

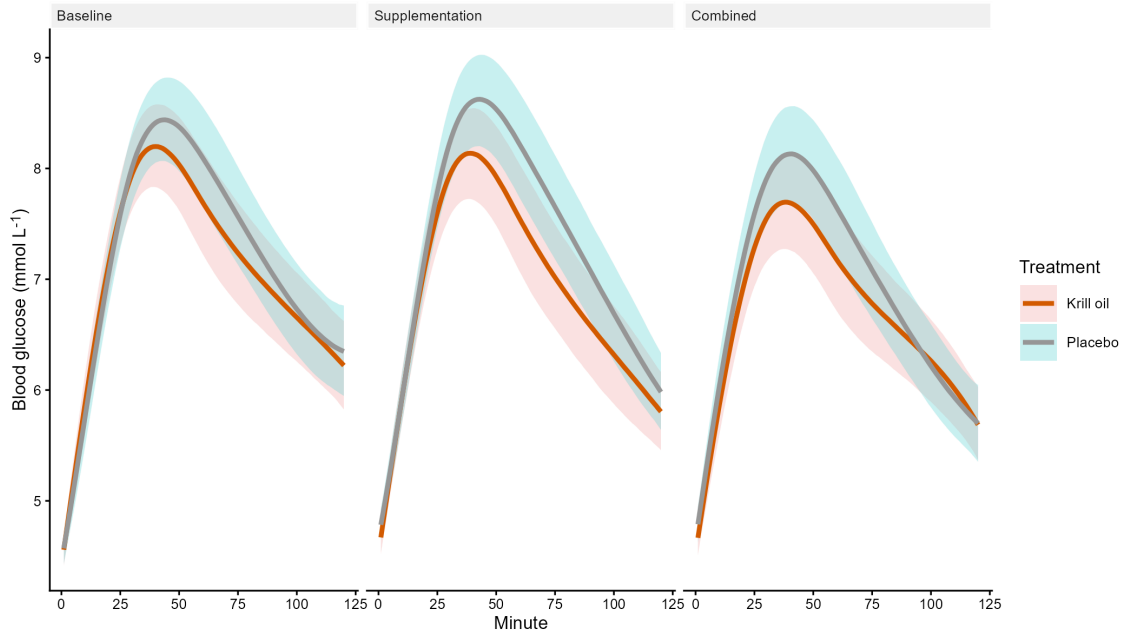

Figure 10.5: Posterior implications of model `mo2` showing average group × time curves with 95% percentile intervals at the three sampling time points, before the intervention (Baseline), after the supplementation-only period (Supplementation) and after the combined supplementation and resistance-training period (Combined).

10.3.3 Missing data model

We further extend the multilevel-model including the time- and treatment effects to a full model including all baseline data and missing data imputation. Similarly to analyses of the metabolic syndrome criteria we want to draw conclusions from estimates that include the uncertainty associated with missing data.

Model `mo3` (`models/mo3.stan`) use the same mechanism for data imputation as used in the models of the metabolic syndrome model sets. Missing data are treated as parameters and are informed by the likelihood. The relevant additions to the model compared to Equation (10.2) therefore include:

$$ $$ {#eq-mo3}

Where the Normal $(0, 2)$ indicate a weak prior (on the standardized scale) for missing values. Again, the prior predictive check indicate that the model allows for very flexible curves per participants and groups (Figure 10.6).

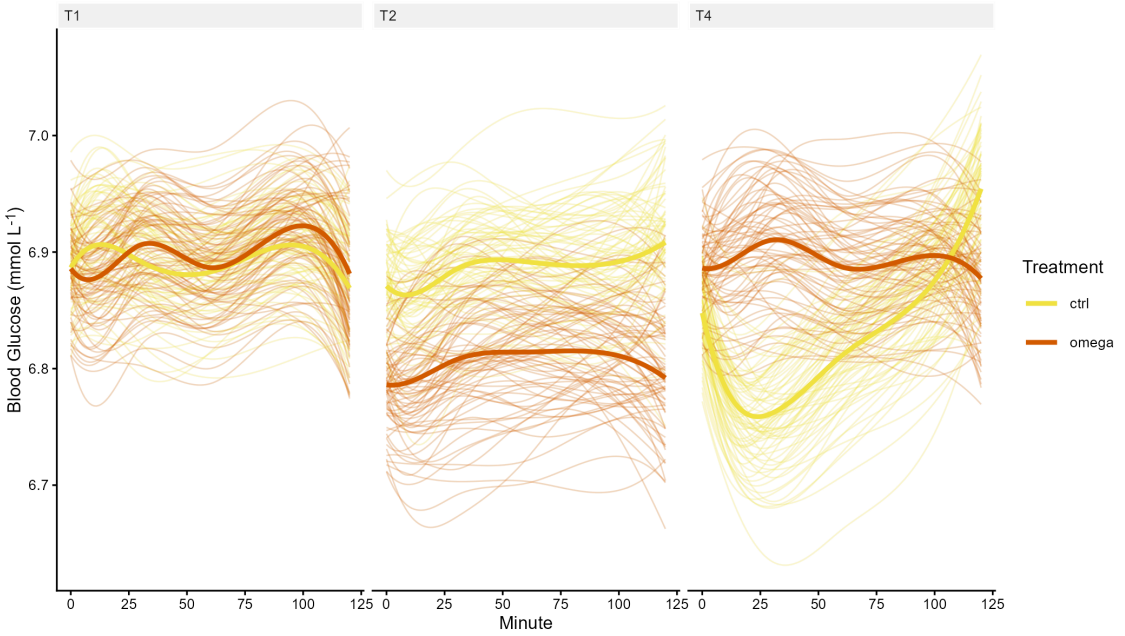

Figure 10.6: Prior predictive simulation of the `mo3` model. The plot shows individual curves and implied group means for each time point.

Next we fit the full data set to the model to get the posterior distribution. Posterior predictive checks indicate that the model capture relevant information from the data (Figure 10.7).

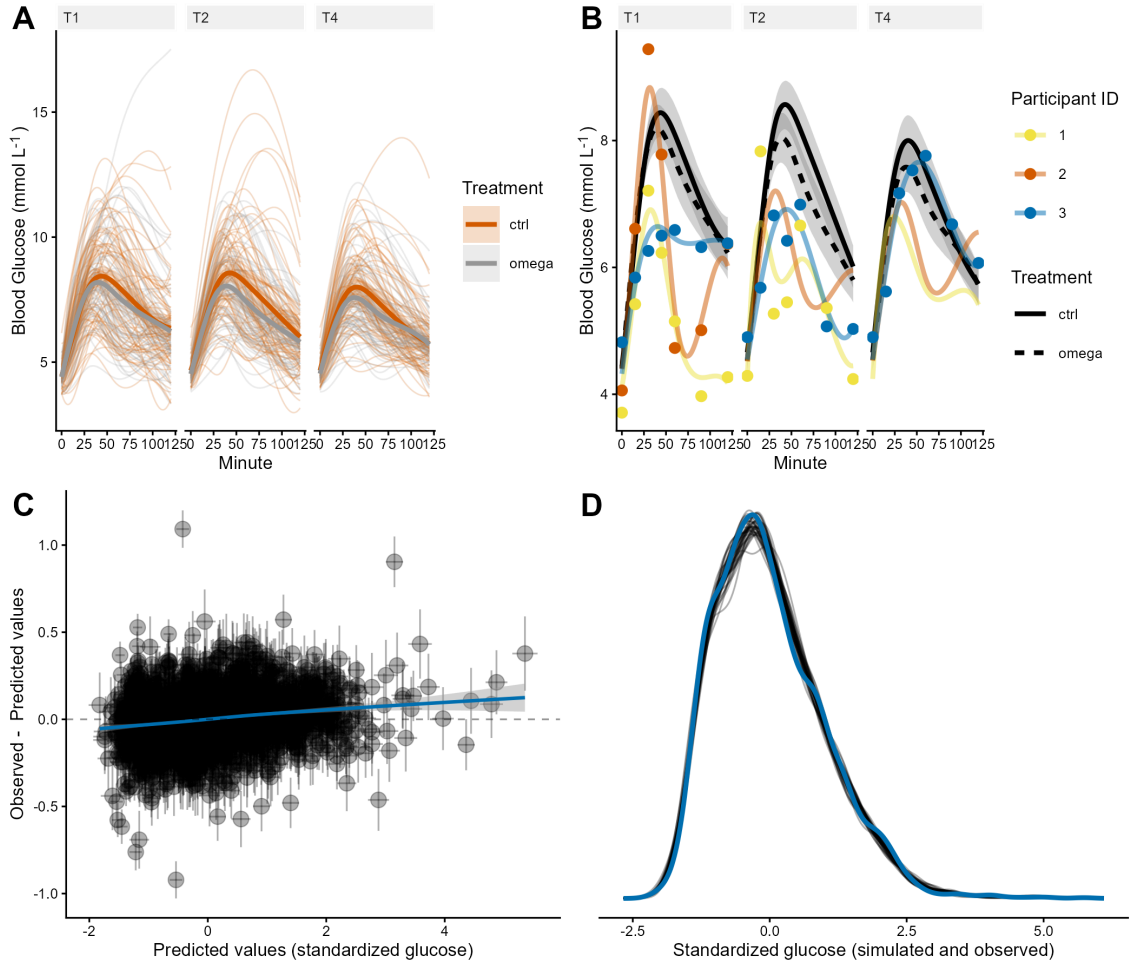

Figure 10.7: Posterior predictive checks of model `mo3`. Population average curves per time × treatment with participant-level curves (A), three example participants with observed data (B), residuals vs. predicted values for observed data points (C) and distributions of simulated (y^{rep}) data sets and the observed data (D).

10.3.4 Inference

We extract two sets of fitted curves from model `mo3`, the population-average glucose curve for every time x treatment combination (`a_g` + cell spline `b_g`), with 95% percentile intervals from the full posterior, and the participant-level curves, which add each participant's random intercept (`r_p[p, 1]`) and time-specific random spline deviations to the population cell they belong to.

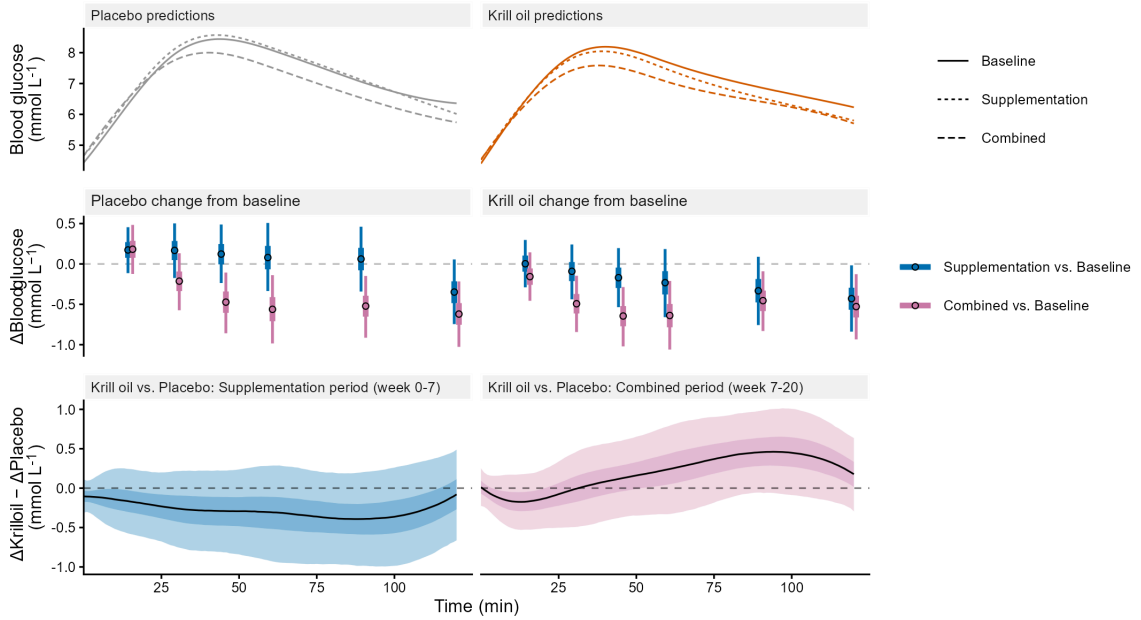

Figure 10.8: Predictions (top panels, posterior medians), within-group changes from baseline (middle panels) and between-group contrasts (krill oil – placebo) of changes (Δ) over the supplementation period (week 0-7) and the combined period (week 7-20; bottom panels). Points and lines are posterior medians; error bars and ribbons represent 50 and 95% CrI in the middle and lower panels.

To evaluate whether the effects of krill oil were beneficial, negligible or harmful, we calculated the posterior probability that the difference in change between krill oil and placebo exceeded the threshold for a negligible effect (δ; see the data chapter), at each minute of the curve (time-point-specific δ, 0.17 mmol/L) and for the average difference across the curve (δ = 0.14 mmol/L). Lower glucose was considered beneficial.

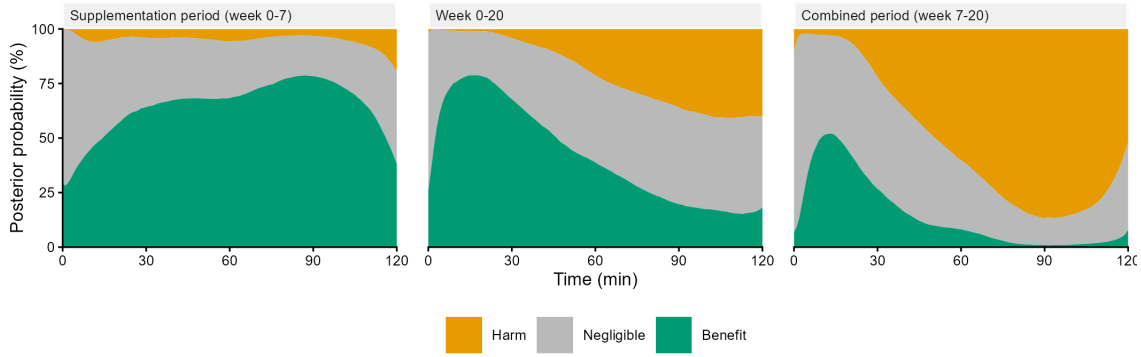

Figure 10.9: Posterior probabilities of benefit (lower glucose with krill oil than placebo), a negligible effect and harm along the OGTT curve, model `mo3`. Effects are differences in change between krill oil and placebo, classified by the time-point-specific threshold for a negligible effect.

Averaged across the curve, the difference in change (krill oil – placebo; posterior median) was −0.28 mmol/L (95% CrI: −0.65, 0.09; P(benefit/negligible/harm) 76/22/1 %) during the supplementation period, 0.19 mmol/L (95% CrI: −0.15, 0.52; P(benefit/negligible/harm) 3/37/60 %) during the combined period and −0.09 mmol/L (95% CrI: −0.45, 0.26; P(benefit/negligible/harm) 39/51/10 %) from baseline to after the combined period (Figure 10.9).

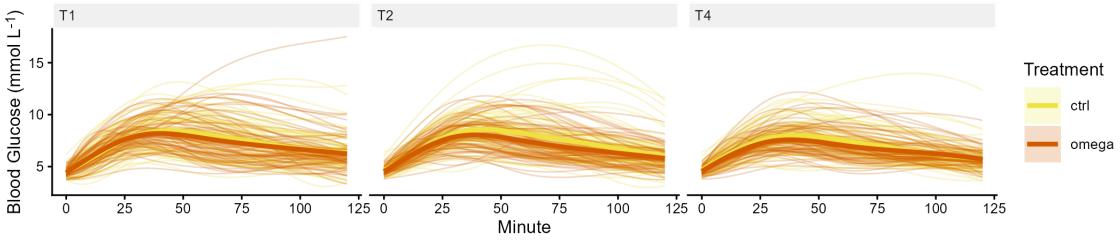

Figure 10.10: Predictions from the OGTT model `mo3` at each time point with participant-level predictions.

10.4 Model expansion: Participant-level time-specific intercepts.

In model `mo3` the participant-level intercept (a_p) is common to all three time points, while the participant-level spline deviations are time-specific. As the basis B has no intercept column, all basis functions are zero at the start of the test (min 0). The participant-level deviation from the population curve in fasting glucose is therefore constrained to be equal at all time points in `mo3`. Model `mo4` (`models/mo4.stan`) relaxes this assumption by decomposing the participant-level intercept at each time point into a shared intercept (a_p) and a time-specific deviation ($\delta_{t,p}$), otherwise the model is identical to `mo3` (including missing data imputation):

$$
\begin{aligned}
\mu_i &= \left(a_{g[i]} + \sum_k b_{k,g[i]}\, B_{k,i} \right) + \left(a_{p[i]} + \delta_{t[i],p[i]} + \sum_k \beta_{k,t[i],p[i]}\, B_{k,i} \right) \\
(a_p, \beta_p)^\top &\sim \text{MVNormal}\ (\mathbf{0}, \mathbf{S}\,\mathbf{R}\,\mathbf{S}) \\
\delta_{t,p} &\sim \text{Normal}\ (0, \tau_\delta) \\
\tau_\delta &\sim \text{Exponential}\ (2)
\end{aligned}
\tag{10.3}
$$

The correlated participant block (a_p and the time-specific spline deviations) is the same as in `mo3`, and the time-specific intercept deviations are independent of this block with a common standard deviation (τ_δ). Model `mo3` is the special case $\tau_\delta = 0$, and the Exponential (2) prior regularizes τ_δ towards this simpler model. The implied correlation between a participant's intercepts at two time points is $\rho = \sigma_a^2/(\sigma_a^2 + \tau_\delta^2)$, where σ_a is the standard deviation of a_p.

We chose this decomposition over a fully correlated block with one intercept per time point, as an LKJ prior on the resulting 18×18 correlation matrix implies a marginal prior on each correlation that strongly favors values close to zero (approximately ± 0.45 95% prior interval for LKJ(1)), regardless of the choice of the LKJ shape parameter. Other priors are the same as in Equation (10.2) and **?@eq-mo3**. The prior predictive simulation is shown in Figure 10.11.

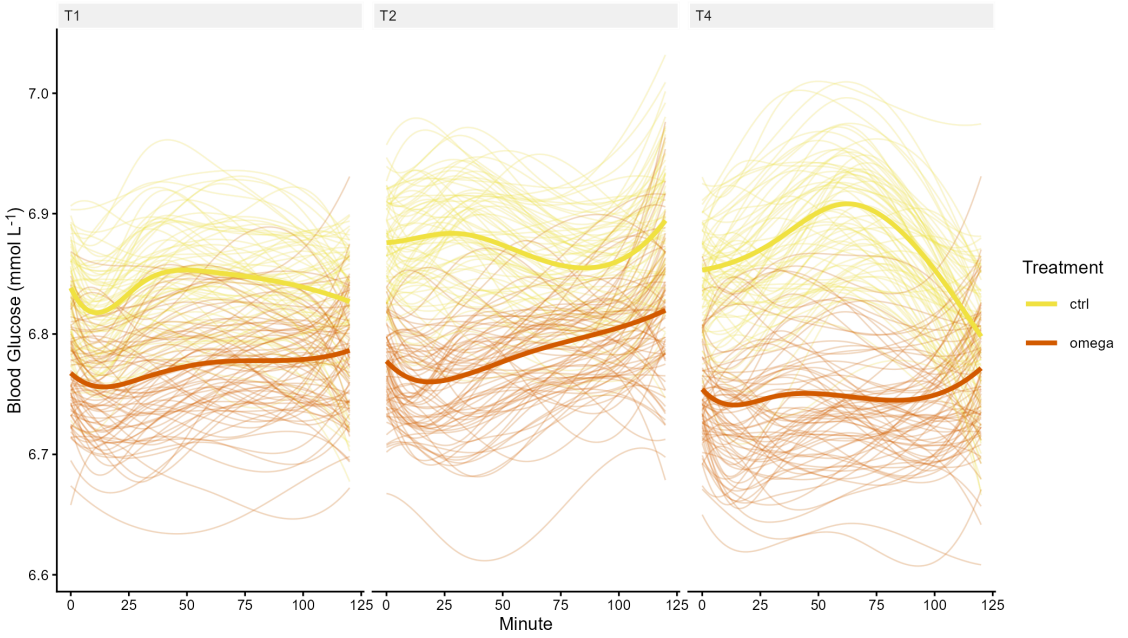

Figure 10.11: Prior predictive simulation of the `mo4` model. The plot shows individual curves and implied group means for each time point.

Next we fit the model to the full data set and perform the same posterior predictive checks as for `mo3` (Figure 10.12).

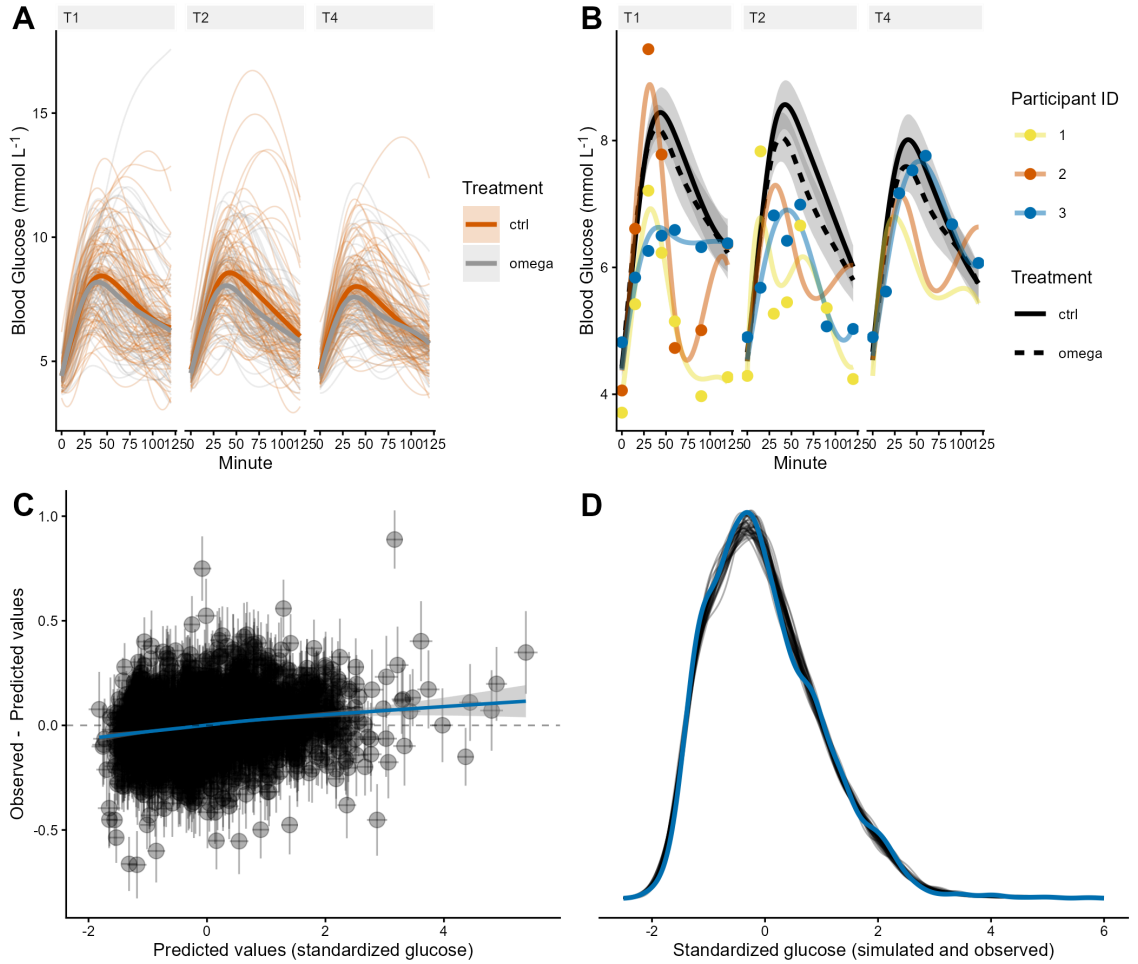

Figure 10.12: Posterior predictive checks of model `mo4`. Population average curves per time × treatment with participant-level curves (A), three example participants with observed data (B), residuals vs. predicted values for observed data points (C) and distributions of simulated (y^{rep}) data sets and the observed data (D).

If time-specific participant intercepts are not needed, the standard deviation of the time-specific deviations (τ_δ) should be close to zero and the implied correlation between a participant's intercepts at two time points (ρ) close to one, i.e. participants keep their relative fasting level across time points. Prior and posterior summaries of these parameters are shown in Table 10.1.

Table 10.1: Prior and posterior summaries (mean and 95% percentile intervals) of the standard deviation of the shared participant-level intercept (σ_a), the standard deviation of time-specific intercept deviations (τ_δ) and the implied correlation between a participant's intercepts at two time points (ρ) in model `mo4`.

	Prior			Posterior		
	mean	2.5%	97.5%	mean	2.5%	97.5%
SD shared intercept	1.00	0.03	3.69	0.20	0.16	0.24
SD time-specific deviations	0.49	0.01	1.82	0.12	0.07	0.16
Implied intercept correlation	0.66	0.00	1.00	0.73	0.56	0.89

10.4.1 Inference (mo4)

We perform the same inference as for mo3 using model mo4. As the population-level part of the models is identical, differences between the models' implied group-level curves and contrasts are due to how participant-level deviations are partitioned across time points.

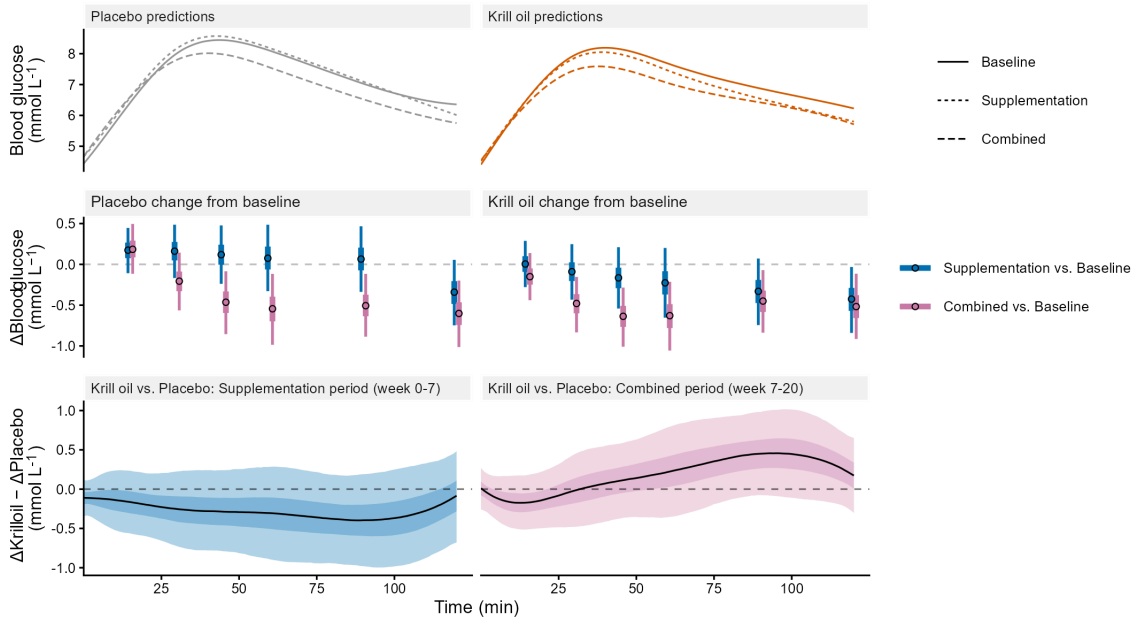

Figure 10.13: Model mo4 predictions (top panels, posterior medians), within-group changes from baseline (middle panels) and between-group contrasts (krill oil – placebo) of changes (Δ) over the supplementation period (week 0-7) and the combined period (week 7-20; bottom panels). Points and lines are posterior medians; error bars and ribbons represent 50 and 95% CrI in the middle and lower panels.

We summarise the mo4 effects as probabilities of benefit, negligible effect and harm, as for mo3.

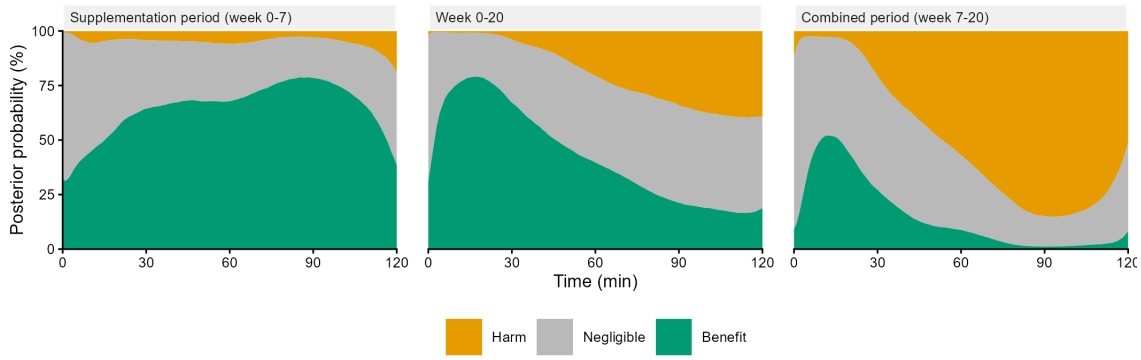

Figure 10.14: Posterior probabilities of benefit (lower glucose with krill oil than placebo), a negligible effect and harm along the OGTT curve, model mo4. Effects are differences in change between krill oil and placebo, classified by the time-point-specific threshold for a negligible effect.

Averaged across the curve, the difference in change (krill oil – placebo; posterior median) was −0.28 mmol/L (95% CrI: −0.65, 0.09; P(benefit/negligible/harm) 77/22/1 %) during the supplementation period, 0.18 mmol/L (95% CrI: −0.15, 0.51; P(benefit/negligible/harm) 3/39/58 %) during the combined period and −0.10 mmol/L (95% CrI: −0.47, 0.26; P(benefit/negligible/harm) 40/50/10 %) from baseline to after the combined period (Figure 10.14).

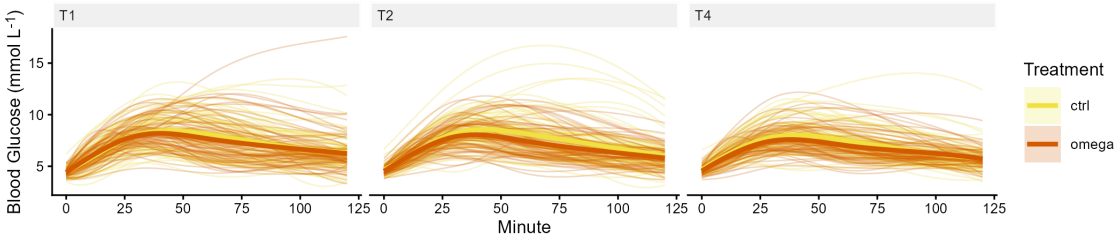

Figure 10.15: Predictions from the OGTT model `mo4` at each time point with participant-level predictions.

11. Methods and results

This chapter reproduces the methods and results sections of the main manuscript.

11.1 Abstract

Background & Aims: Marine omega-3 fatty acids are inversely associated with the metabolic syndrome, and randomized trials have confirmed benefits on triglycerides and blood pressure, but not on glucose regulation. Whether they add to the benefits of resistance training is largely untested, and no trial has raised omega-3 status prior to the initiation of training. We investigated whether krill oil improves glucose tolerance, alone and combined with resistance training, and secondarily whether it affects the metabolic syndrome.

Methods: In this double-blind trial, 148 sedentary adults aged 23–60 years (BMI 18.4–52.1 kg/m²) were randomized to krill oil (4 g/day; 508 mg EPA and 300 mg DHA; n=74) or placebo (n=74) for 20 weeks: 7 weeks of supplementation alone, then 13 weeks combined with supervised resistance training. The primary outcome was the change in glucose tolerance (oral glucose tolerance test, OGTT) from week 7 to 20. Secondary outcomes included glucose tolerance from week 0 to 7, fasting glucose, HOMA2-IR, HbA1c, adiponectin, LDL cholesterol and the metabolic syndrome (five components, a composite burden score and prevalence). Treatment effects were estimated using Bayesian models with integrated missing-data imputation. Models were developed and validated following a simulation-based Bayesian workflow. Treatment effects were classified as beneficial, negligible or harmful using thresholds of 0.1 × the between-participant SD, estimated at baseline or from population-based cohorts.

Results: Ninety-five participants completed the trial, attrition being mainly due to the COVID-19 pandemic. Krill oil raised the Omega-3 Index from 6 % to 8 % (2.1 and 2.3 percentage points above placebo at weeks 7 and 20). For the primary outcome, krill oil did not improve glucose tolerance during the combined period, and the estimates instead favoured placebo (difference in within-group change across the OGTT, weeks 7–20, 0.18 mmol L^{-1} (95% CrI: −0.15, 0.51; P(benefit/negligible/harm) 3/39/58 %)). During supplementation alone, glucose tolerance probably improved with krill oil (weeks 0–7, −0.28 mmol L^{-1} (95% CrI: −0.65, 0.09; P(benefit/negligible/harm) 77/22/1 %)), but this was not maintained (weeks 0–20, −0.10 mmol L^{-1} (95% CrI: −0.47, 0.26; P(benefit/negligible/harm) 40/50/10 %)). Fasting glucose was probably lower with krill oil at week 7 (−0.08 mmol L^{-1} (95% CrI: −0.23, 0.06; P(benefit/negligible/harm) 66/31/3 %)), with a less certain difference at week 20 (−0.06 mmol L^{-1} (95% CrI: −0.21, 0.09; P(benefit/negligible/harm) 54/39/7 %)). HbA1c was probably higher with krill oil at week 7 (+2 % (95% CrI: 0, 5; P(benefit/negligible/harm) <1/37/63 %)) but not at week 20 (+1 % (95% CrI: −2, 3; P(benefit/negligible/harm) 3/74/23 %)). Effects on HOMA2-IR, adiponectin, LDL cholesterol, BMI and relative fat mass were most probably negligible. Krill oil probably lowered the metabolic syndrome burden score during supplementation alone (−0.08 SD; 95 % CrI −0.17, 0.01; P(benefit/negligible/harm) 71/29/<1 %) and during the combined period (−0.08 SD; −0.17, 0.02; 68/32/<1 %), with harm unlikely in both periods. The largest component effects were lower systolic blood pressure at week 7 (−3.7 mmHg; 90/10/<1 %) and higher HDL cholesterol at week 20 (+0.05 mmol/L; 60/40/<1 %). The prevalence of the syndrome was not reliably reduced.

Conclusions: Krill oil did not improve glucose tolerance when combined with resistance training after a supplementation-only period. A probable early improvement during supplementation alone was not maintained. Krill oil probably lowered the combined burden of the metabolic syndrome, with harm being unlikely, through small effects distributed across several risk factors, most clearly systolic blood pressure and HDL cholesterol, without reducing the prevalence of the syndrome. Registered at ClinicalTrials.gov (NCT04279951).

11.2 Data analysis and statistics

Descriptive data are presented as mean (standard deviation, SD) for baseline characteristics and as mean (95 % confidence interval, CI) for dietary intake, and categorical data as n (%). For inferential statistics, a general Bayesian workflow was adopted where models were incrementally built and tested for validity through simulations. First, implications of prior distributions were evaluated using prior-predictive

simulations. Priors were selected to constrain model predictions and parameter values to *a priori* plausible ranges. This approach encourages the use of weakly informative priors on model parameters, which regularize models and shrink estimates and predictions without distorting the influence of the data on inference (5–7). In relevant cases, synthetic data were used to confirm that model inferences were not biased by model misspecification. Samples from the joint posterior distribution were used to create posterior predictive simulations, in which model implications are simulated and compared with characteristics of the observed data (4,7). Selected models were also, in relevant cases, assessed for inferential capabilities using synthetic data simulations where parameter values are known. The sensitivity of model specifications to inference was assessed by comparing predictions across different models.

Missing values were imputed in all models used for inference allowing for the use of all available data while incorporating the uncertainty associated with loss of data due to drop-out (see Fig. 1) into the posterior probability distribution. Data imputation was done by treating missing values as parameters estimated conditionally on available data and priors (7). Missing data parameters were given a weakly informative prior (Normal $(0, 2)$) on the standardized scale (mean centered and scaled by the standard deviation of each variable).

Bayesian models were constructed using the Stan probabilistic programming language (1) and fitted using the `cmdstanr` interface (2) in R (3). All sampling from prior and posterior distributions was done using the Hamiltonian Monte Carlo (HMC) no-U-turn sampler (21). Performance of the HMC algorithm was assessed using chain convergence statistics (R-hat) and by visually inspecting convergence and stationary sampling across multiple HMC chains.

Whole-blood fatty acid composition and the omega-3 index were modeled for each target ($k = 1, ..., K$; $K = 12$; 11 fatty acids and the omega-3 index) using a Gaussian likelihood after log-transformation and standardization of the outcome variables ($y_i \sim \text{Normal}\ (\mu_i, \sigma_{\text{FA}\ [i],\ \text{TX}\ [i]})$, where $\text{FA}\ [i] \in \{1, ..., K\}$, $\text{TI}\ [i]$, and $\text{TX}\ [i]$ index the target, follow-up time-point and treatment of observation i). The linear predictor included a target-, treatment- and time-point-specific intercept and a target-specific coefficient for the association with pre-intervention values ($\mu_i = \alpha_{\text{FA}\ [i],\ \text{TI}\ [i],\ \text{TX}\ [i]} + \beta_{\text{FA}\ [i]} \times \text{Pre}_i$, where Pre_i is the standardized pre-intervention value of the same target and participant). Priors were common to all targets ($\alpha \sim \text{Normal}\ (0, 1)$, $\beta \sim \text{Normal}\ (1, 1)$, and $\sigma \sim \text{Exponential}\ (1)$). Treatment effects were assessed by calculating the contrast between treatments at each follow-up time-point.

Blood glucose measured during the oral glucose tolerance test (OGTT) was modeled using a Gaussian likelihood after standardization ($y_i \sim \text{Normal}\ (\mu_i, \sigma)$). Glucose curves were represented by a cubic B-spline basis with $J = 5$ basis functions (5 degrees of freedom, no intercept column; `bs()` from the `splines` package), where $B_{j,i}$ is the j-th basis function evaluated at the sampling time (min) of observation i. A population-level curve was estimated for each combination of treatment ($\text{TX}\ [i]$) and time-point ($\text{TP}\ [i] \in \{1, 2, 3\}$; baseline, after the supplementation-only period, and after the intervention), with participant-level deviations from the population-level curve: $\mu_i = \alpha_{\text{TX}\ [i],\ \text{TP}\ [i]} + \sum_{j=1}^{J} \beta_{j,\ \text{TX}\ [i],\ \text{TP}\ [i]} B_{j,i} + a_{\text{ID}\ [i]} + \delta_{\text{TP}\ [i],\ \text{ID}\ [i]} + \sum_{j=1}^{J} b_{j,\ \text{TP}\ [i],\ \text{ID}\ [i]} B_{j,i}$, where α and β are treatment- and time-point-specific intercepts and spline parameters, a is a participant-specific intercept, δ are participant- and time-point-specific deviations from that intercept, and b are participant- and time-point-specific spline deviations ($\text{ID}\ [i]$ indexes the participant of observation i). The participant-specific intercept and spline deviations ($1 + 3J$ effects per participant) were drawn from a multivariate normal distribution ($\text{MVNormal}\ (\mathbf{0}, \mathbf{SRS})$, $\mathbf{S} = \text{diag}\ (\sigma_{\text{ID}})$), with an LKJ (1) prior on the correlation matrix $\mathbf{R}$ (non-centered parameterization via its Cholesky factor). Time-point-specific intercept deviations were modeled as independent, $\delta_{t,\ \text{ID}} \sim \text{Normal}\ (0, \tau_\delta)$, with $\tau_\delta \sim \text{Exponential}\ (2)$ regularizing the model towards a common participant intercept across time-points. Remaining priors were $\alpha, \beta \sim \text{Normal}\ (0, 2)$ and $\sigma, \sigma_{\text{ID}} \sim \text{Exponential}\ (1)$, and missing OGTT values were imputed as described above. Treatment effects were assessed as between-group differences in the change from baseline of posterior predicted population-level curves, evaluated across the sampling time. For the main outcome analysis, the first follow-up time-point (week 7) was used as the baseline. We used the observed baseline SD, pooled within sampling minute, as the scale for a threshold for negligible treatment effects at single time-points of the OGTT (δ = 0.1 × SD; SD = 1.66 mmol L^{-1}). For treatment effects averaged across the OGTT (0–120 min), the threshold was based on the between-participant SD of the average blood glucose across the test at baseline (SD = 1.43 mmol L^{-1}). We found our data to indicate a slightly larger variation than reported from a comparable published cohort (9), consistent with this cohort having fewer samples across the OGTT procedure. We therefore concluded that our observed baseline data was more relevant for the purpose of deciding on a threshold for negligible effects.

Metabolic syndrome criteria (15) (waist circumference, systolic and diastolic blood pressure, fasting glucose, high-density lipoprotein cholesterol (HDL) and triglycerides (TG)) and related measures of meta-

bolic health (relative fat mass, body mass index (BMI), HOMA2-IR, adiponectin, glycated haemoglobin (HbA1c) and low-density lipoprotein cholesterol (LDL)) were jointly modeled in a multivariate model ($K = 12$ outcomes). The reason for choosing this model over several univariate models was twofold: first, we wanted to improve the accuracy of imputed outcome values by sharing information across outcomes, allowing for more realistic estimation of the metabolic syndrome diagnostic criteria; second, we wanted to combine information from several outcome variables into a continuous composite score (19,20) while accounting for their correlations. Triglycerides, HOMA2-IR and HbA1c were log-transformed, and all outcomes were standardized. The linear predictor for observation i was $\mu_i = \alpha_{\text{OC }[i],\ \text{TI }[i],\ \text{TX }[i]} + \beta_{\text{OC }[i],\ \text{TI }[i]} \times \text{Pre}_i + \gamma_{\text{OC }[i],\ \text{TI }[i]} \times \text{Sex}_i$, where $\text{OC }[i] \in \{1, ..., K\}$, $\text{TI }[i]$ and $\text{TX }[i]$ index the outcome, follow-up time-point and treatment of observation i, Pre_i is the standardized pre-intervention value of the same outcome and participant, and Sex_i was coded as -0.5 (male) and 0.5 (female). For each participant (p), observations and linear predictors of all outcomes at both follow-up time-points were stacked in vectors $\mathbf{y}_p$ and $\boldsymbol{\mu}_p$ of length $2K$, modeled using a multivariate Student's t-distribution, $\mathbf{y}_p \sim \text{MVT}\ (\nu, \boldsymbol{\mu}_p, \boldsymbol{\Sigma})$, with the degrees of freedom fixed at $\nu = 4$ to reduce the influence of extreme observations. The variance-covariance matrix was parameterized as $\boldsymbol{\Sigma} = \text{diag}\ (\boldsymbol{\sigma})\, \boldsymbol{\Omega}\, \text{diag}\ (\boldsymbol{\sigma})$, with outcome- and time-point-specific scales ($\boldsymbol{\sigma}$) and a $2K \times 2K$ correlation matrix ($\boldsymbol{\Omega}$) capturing correlations between outcomes and between time-points. Missing pre-intervention values were imputed using a multivariate normal model of pre-intervention values ($\text{MVNormal}\ (\boldsymbol{\mu}_{\text{pre}}, \boldsymbol{\Sigma}_{\text{pre}})$). Priors were $\alpha \sim \text{Normal}\ (0, 1)$, $\beta \sim \text{Normal}\ (1, 0.4)$, $\gamma \sim \text{Normal}\ (0, 0.5)$, $\boldsymbol{\sigma}, \boldsymbol{\sigma}_{\text{pre}} \sim \text{Exponential}\ (1)$, $\boldsymbol{\mu}_{\text{pre}} \sim \text{Normal}\ (0, 1)$, $\boldsymbol{\Omega} \sim \text{LKJ}\ (4)$ and $\boldsymbol{\Omega}_{\text{pre}} \sim \text{LKJ}\ (2)$. Comparing the multivariate model to a univariate version of the same model (shared mean and variance structure but without the between-outcomes correlation structure) indicated that the multivariate model had a better out-of-sample predictive performance (difference in expected log pointwise predictive density, ΔELPD, −167.6 (SE 26.5); leave-one-out cross-validation (16) on the level of participant for 73 complete cases). This difference between the two models indicates that the correlation structure carries information that benefits prediction. Assuming that missing values are missing at random, the multivariate model is expected to provide more realistic imputation estimates.

Treatment effects on each outcome (k) were assessed as the contrast between treatments ($\alpha_{k,t,\ \text{Krill-oil}} - \alpha_{k,t,\ \text{Placebo}}$) at each follow-up time-point (t). Thresholds for a potentially meaningful effect were set to δ = 0.1 × SD. For most outcomes, we used SDs pooled from Nordic population-based studies (9–12) (HDL, 0.47 mmol L^{-1}; TG, 0.54 [log scale]; waist circumference, 13.2 cm; systolic and diastolic blood pressure, 18.5 and 10.2 mmHg; BMI, 4.8 kg m^{-2}; LDL, 1.0 mmol L^{-1}; HbA1c, 0.17 [log scale]). When no comparable population-based estimate was available, we used the between-participant SD at baseline in our own data (fasting glucose, 0.54 mmol L^{-1}; relative fat mass, 7.8 %; HOMA2-IR, 0.37 [log scale]; adiponectin, 6.0 mg L^{-1}). For fasting glucose, population-based estimates were available but included participants with diabetes or non-fasting samples, and were therefore not considered comparable.

A composite metabolic syndrome burden score was calculated as the average contrast across the five diagnostic criteria (blood pressure as the average of systolic and diastolic pressure, and HDL with reversed sign), such that positive values indicate a less favorable metabolic profile in the krill-oil group. To avoid the relative weights of the components depending on the variability in our sample, each component contrast was expressed in units of sex-specific SDs from the population-based studies described above (fasting glucose, our own sex-specific baseline SD), averaged over participants. The threshold for a negligible effect on the burden score was set to 0.1 × the between-participant SD of the burden score at baseline, calculated on the same scale (δ = 0.055 SD units). Metabolic syndrome diagnosis was determined for each participant and time-point in every posterior draw (including imputed values), defined according to the harmonized criteria as the presence of at least three of five components (waist circumference ≥ 94 cm in men and ≥ 80 cm in women; triglycerides ≥ 1.7 mmol L^{-1}; HDL cholesterol < 1.0 mmol L^{-1} in men and < 1.3 mmol L^{-1} in women; systolic ≥ 130 and/or diastolic ≥ 85 mmHg; fasting glucose ≥ 5.6 mmol L^{-1}) (15). However, we did not include drug treatment for elevated blood pressure or glucose to count as meeting the respective criterion. Treatment effects on diagnosis were estimated by fitting logistic mixed-effects models (time × treatment interaction, participant-specific intercepts) to 500 posterior draws using `glmmTMB` (22), with weakly regularizing Normal(0, 3) priors on the fixed effects to stabilize estimation, and pooling estimates using Rubin's rules (23). These effects are therefore reported as odds ratios (OR) with 95 % confidence intervals (CI) rather than posterior percentile intervals.

To assess whether treatment effects depended on participants' characteristics at baseline, the multivariate model was extended in four ways. First, the treatment effect on each outcome was allowed to depend on the baseline value of the same outcome by adding $\delta_{\text{OC }[i],\ \text{TI }[i]} \times \text{KO}_i \times \text{Pre}_i$ to the linear predictor, where KO_i is 1 for krill oil and 0 for placebo. Second, the treatment effect on all outcomes was allowed to depend on a baseline metabolic syndrome burden score by adding $\gamma_{\text{OC }[i],\ \text{TI }[i]} \times \text{KO}_i \times$

$\tilde{b}_{\text{ID }[i]}$, where $\tilde{b}$ is the mean-centered average of the standardized baseline values of the five diagnostic criteria (HDL with reversed sign, and blood pressure as the average of systolic and diastolic pressure), calculated from observed and imputed baseline values. Third, the shared moderator was baseline relative fat mass, taken from the partly imputed baseline values and standardized within each posterior draw (mean 0, SD 1). Fourth, the shared moderator was the baseline omega-3 index (13), calculated from whole-blood fatty acids, standardized over participants with an observed value; missing baseline values were imputed with a standard normal prior so that all randomized participants were kept. In the third and fourth models, the interaction ($\gamma_{\text{OC }[i],\ \text{TI }[i]} \times \text{KO}_i \times m_{\text{ID }[i]}$, where m is the standardized moderator) had the same hierarchical prior as in the second model. Interaction parameters were given hierarchical priors that shrink them towards zero unless supported by the data ($\delta = \tau_\delta z$, $z \sim \text{Normal}\ (0, 1)$, $\tau_\delta \sim \text{Half-Normal}\ (0, 0.25)$, and correspondingly for γ). Predictive performance of the extended models was compared with the main model using leave-one-participant-out cross-validation (PSIS-LOO).

Posterior distributions are presented as posterior densities in visualizations, and summarized using the median with 50 and 95 % (visualizations) or 95 % (text) percentile intervals; in the text, 95 % percentile intervals are reported as credible intervals (CrI). Statements about the probability of beneficial, negligible and harmful effects of krill oil supplementation were made using the region of practical equivalence (ROPE; 0 ± δ, defined for each outcome, see above) as the region indicating negligible effects. Beneficial and harmful effects were defined as effects beyond δ (< −δ or > δ) in the beneficial or harmful direction. We also quantified the probability of any benefit, P(any benefit), as the probability of an effect < 0 for outcomes where lower values are favorable and of an effect > 0 for HDL and adiponectin, irrespective of its size. The HDI+ROPE decision rule (8) was used to make inferential decisions about the null hypothesis that the effect is practically equivalent to zero. When the 95 % highest density interval (HDI) was situated entirely inside the ROPE, the null hypothesis was accepted; when the HDI was entirely outside the ROPE, the null hypothesis was rejected; and when the HDI was partly inside the ROPE, the decision was undecided.

11.3 Results

Table 11.1: Treatment effects (krill oil – placebo) after supplementation alone (week 7) and after combined supplementation and resistance training (week 20). Estimates are posterior medians with 95 % highest density intervals (HDI). OGTT, difference in change in the mean blood glucose across the test (0–120 min) from week 0 to week 7 (week 7 column) and from week 7 to week 20 (week 20 column; primary outcome); other outcomes, differences adjusted for the week 0 value, with triglycerides, HOMA2-IR and HbA1c as % differences. P(any benefit), posterior probability of a difference in the beneficial direction (lower values, or higher values for HDL cholesterol and adiponectin), irrespective of size. ROPE, region of practical equivalence (±δ, δ = 0.1 × SD). Decisions: Accept, the 95 % HDI is entirely inside the ROPE; Reject, the 95 % HDI is entirely outside the ROPE (direction of the effect in parentheses); Undecided, otherwise.

	ROPE	Week 7, estimate (95 % HDI)	Week 7, P(any benefit) (%)	Week 7, decision	Week 20, estimate (95 % HDI)	Week 20, P(any benefit) (%)	Week 20, decision
Oral glucose tolerance test							
Mean blood glucose (mmol L^{-1})	±0.143	−0.28 (−0.65, 0.09)	93	Undecided	0.18 (−0.15, 0.51)	15	Undecided
Metabolic syndrome							
Waist circumference (cm)	±1.32	−0.7 (−1.8, 0.4)	89	Undecided	−0.1 (−1.5, 1.3)	56	Undecided
Triglycerides (%)	−5.28, 5.58	−0.6 (−11.6, 11.6)	54	Undecided	−0.6 (−13.6, 13.5)	54	Undecided
HDL cholesterol (mmol L^{-1})	±0.047	−0.01 (−0.06, 0.04)	37	Undecided	0.05 (0.00, 0.11)	98	Undecided
Systolic blood pressure (mmHg)	±1.85	−3.7 (−6.9, −0.9)	>99	Undecided	−2.5 (−6.3, 1.3)	90	Undecided
Diastolic blood pressure (mmHg)	±1.02	−1.8 (−3.8, 0.1)	96	Undecided	−1.2 (−3.4, 1.0)	86	Undecided
Fasting glucose (mmol L^{-1})	±0.054	−0.08 (−0.23, 0.06)	88	Undecided	−0.06 (−0.21, 0.10)	78	Undecided
Burden score (SD units)	±0.055	−0.08 (−0.17, 0.00)	96	Undecided	−0.08 (−0.18, 0.02)	94	Undecided
Related metabolic health markers							
Body mass index (kg m^{-2})	±0.478	0.05 (−0.11, 0.22)	29	Accept	0.09 (−0.24, 0.38)	29	Accept
Relative fat mass (percentage points)	±0.78	0.2 (−0.2, 0.6)	12	Accept	0.3 (−0.4, 0.9)	22	Undecided
HOMA2-IR (%)	−3.66, 3.80	−2.4 (−9.0, 4.5)	75	Undecided	1.6 (−6.9, 10.5)	36	Undecided
HbA1c (%)	−1.71, 1.74	2.1 (−0.1, 4.6)	4	Undecided	0.8 (−1.7, 3.4)	27	Undecided
Adiponectin (mg L^{-1})	±0.596	−0.11 (−1.01, 0.77)	40	Undecided	−0.18 (−1.10, 0.72)	35	Undecided
LDL cholesterol (mmol L^{-1})	±0.100	−0.04 (−0.20, 0.11)	71	Undecided	0.05 (−0.13, 0.23)	30	Undecided

11.3.1 Participant flow and baseline characteristics

Table 11.2: Baseline characteristics of participants. Data are mean (SD) or n (%).

	n	Placebo (n = 74)	Krill oil (n = 74)	Total (n = 148)
Female, n (%)	148	38 (51%)	42 (57%)	80 (54%)
Age (years)	148	47.5 (7.8)	45.7 (8.0)	46.6 (7.9)
Height (cm)	148	174.1 (9.2)	173.9 (9.0)	174.0 (9.1)
Body mass (kg)	148	88.3 (18.2)	88.8 (17.7)	88.5 (17.9)
Body mass index (kg m^{-2})	148	29.1 (5.4)	29.4 (5.5)	29.2 (5.4)
Obesity (BMI ≥ 30), n (%)	148	30 (41%)	28 (38%)	58 (39%)
Relative fat mass (%)	148	35.8 (7.4)	37.2 (8.1)	36.5 (7.8)
Waist circumference (cm)	146	99.2 (15.1)	100.0 (12.9)	99.6 (14.0)
Systolic blood pressure (mmHg)	147	132.4 (16.1)	132.4 (15.0)	132.4 (15.5)
Diastolic blood pressure (mmHg)	147	83.2 (10.1)	82.9 (11.0)	83.1 (10.5)
Fasting glucose (mmol L^{-1})	147	4.4 (0.6)	4.4 (0.5)	4.4 (0.5)
HDL cholesterol (mmol L^{-1})	143	1.3 (0.3)	1.4 (0.3)	1.3 (0.3)
LDL cholesterol (mmol L^{-1})	143	3.5 (0.8)	3.4 (0.9)	3.4 (0.9)
Triglycerides (mmol L^{-1})	143	1.3 (0.7)	1.2 (0.5)	1.2 (0.6)
HOMA2-IR	93	1.8 (1.0)	1.8 (0.9)	1.8 (0.9)
Adiponectin (mg L^{-1})	94	12.7 (6.2)	14.6 (5.7)	13.7 (6.0)
Omega-3 index (%)	144	6.1 (1.5)	6.0 (1.2)	6.0 (1.4)
Metabolic syndrome, n (%)	141	20 (28%)	19 (27%)	39 (28%)

Of the 148 randomized participants (placebo, n = 74; krill oil, n = 74), 95 completed the intervention (placebo, n = 44; krill oil, n = 51). Discontinuation was mainly related to the COVID-19 pandemic (**[TODO: n discontinued due to COVID-19; the text said 45 of 56]** of 53 participants), and all randomized participants were included in the analyses (Fig. 1). At baseline, participant characteristics were similar between arms (Table 1). At baseline, 35 % of participants used medication regularly or as needed (placebo, 28 %; krill oil, 42 %), the most common being analgesics, respiratory and cardiovascular medication (Supplementary Table S1; **[TODO: n, draft 7]** used antihypertensive, **[TODO: n, draft 5]** used lipid-lowering and **[TODO: n, draft 2]** used glucose-lowering medication). BMI ranged from 18.4 to 52.1 kg m^{-2}, and 39 % of participants had obesity (BMI ≥ 30 kg m^{-2}; placebo, 41 %; krill oil, 38 %). Of 141 participants with all diagnostic criteria measured at baseline, 28 % met the harmonized criteria for the metabolic syndrome (placebo, 28 %; krill oil, 27 %). Energy and macronutrient intakes were similar between arms and did not change meaningfully during the intervention (Table 2).

11.3.2 Intervention efficacy

Krill oil substantially raised whole-blood n-3 PUFAs compared with placebo from week 0 to week 7 (supplementation alone), and the increase was sustained to week 20 (combined supplementation and resistance training; Figs. 3 and 4). EPA more than doubled (+105 % (95% CrI: 83, 129) at week 7 and +109 % (95% CrI: 83, 138) at week 20), and DHA increased by +24 % (95% CrI: 18, 32) and +32 % (95% CrI: 24, 41), respectively (Fig. 3). This was accompanied by reductions in the n-6 PUFAs arachidonic acid (−7 % (95% CrI: −9, −4) and −8 % (95% CrI: −11, −5)) and dihomo-ɣ-linolenic acid (−13 % (95% CrI: −17, −8) and −15 % (95% CrI: −20, −9)). Consequently, the Omega-3 Index increased from baseline to 7.9 % and 8.2 % at weeks 7 and 20 in the krill oil group, whereas it remained at a level comparable to baseline values in

the placebo group (5.8 % and 5.9 %; difference, 2.1 (95% CrI: 1.8, 2.4) and 2.3 (95% CrI: 2.0, 2.7) percentage points; Fig. 4). Krill oil supplementation thus effectively increased n-3 PUFA status, with observed changes in line with or above those expected for this dose of EPA and DHA **[TODO: citation, Walker 2019 (EndNote #1476); add to bibliography.bib]** and consistent with a previous trial of the same krill oil product **[TODO: citation, Rundblad 2018 (EndNote #114); add to bibliography.bib]**.

Both groups completed the resistance training as intended. Participants completing the training intervention attended 97 % of the scheduled training sessions (placebo, 98 %; krill oil, 96 %), and training volume increased progressively during the training period, reaching 45 % above session 1 at session 20, Fig. 2. This progression is consistent with that reported for the same protocol in older adults **[TODO: citation, draft "ref 36"; add to bibliography.bib]**.

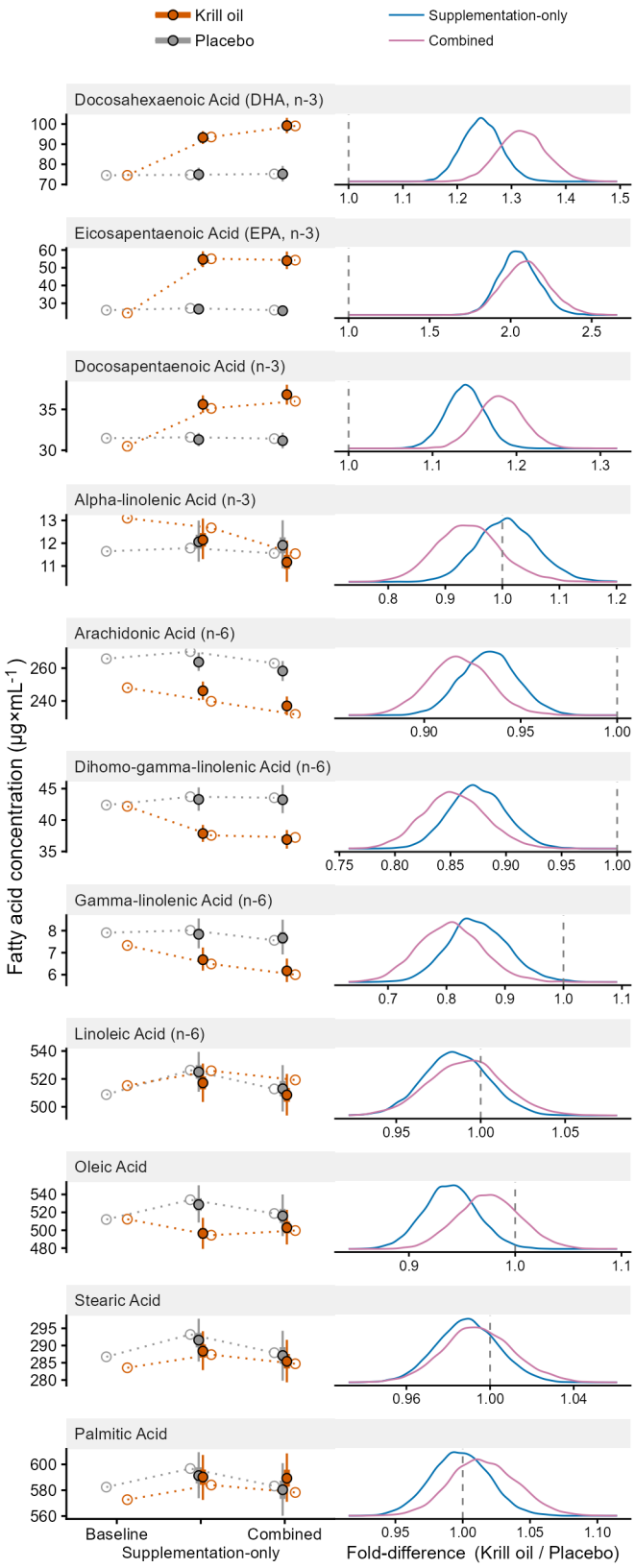

Figure 11.1: Whole-blood fatty acid concentrations in the krill oil and placebo groups at week 0 (baseline), week 7 (after supplementation-only) and week 20 (after combined supplementation and resistance training). Left: observed group means (open circles) and model-estimated medians with 50 and 95 % percentile intervals (filled circles). Right: posterior distributions of the fold difference between groups (krill oil / placebo) at week 7 (blue) and week 20 (pink). The dashed line indicates no difference.

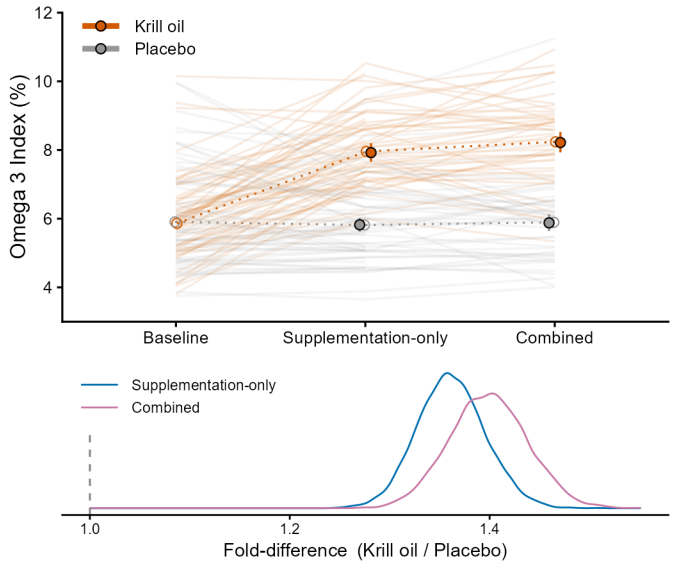

Figure 11.2: Omega-3 Index in the krill oil and placebo groups at week 0 (baseline), week 7 (after supplementation alone) and week 20 (after combined supplementation and resistance training). Upper panel: individual participant values (thin lines), observed group means (open circles) and model-estimated medians (filled circles) with 95 % (thin bars) and 50 % (thick bars) percentile intervals. Lower panel: posterior distributions of the fold difference between groups (krill oil / placebo) at week 7 (blue) and week 20 (pink). The dashed line indicates no difference between treatments.

11.3.3 Glucose regulation

Krill oil did not improve the primary outcome, the change in glucose tolerance during the combined period (from week 7 to week 20; Fig. 5A), compared with placebo. Instead, blood glucose during the OGTT tended to decrease more with placebo than with krill oil from week 7 to week 20 (average difference in change across the OGTT, 0.18 mmol L^{-1} (95% CrI: −0.15, 0.51; P(benefit/negligible/harm) 3/39/58 %); Fig. 5A, right panels; HDI+ROPE decision: undecided; Table 3), most clearly in the later part of the curve (from 80 to 110 min, 0.43 mmol L^{-1} (95% CrI: −0.08, 0.95; P(benefit/negligible/harm) 1/13/86 %)). During supplementation alone (week 0 to week 7), glucose tolerance probably improved with krill oil, mainly during 25 to 100 min (Fig. 5A, left panels; across the OGTT, −0.28 mmol L^{-1} (95% CrI: −0.65, 0.09; P(benefit/negligible/harm) 77/22/1 %); Table 3). Over the whole intervention (week 0 to week 20), blood glucose during the OGTT decreased in both groups (average change across the OGTT; placebo, −0.34 mmol L^{-1} (95% CrI: −0.60, −0.08); krill oil, −0.44 mmol L^{-1} (95% CrI: −0.69, −0.19)), and the difference between groups was most likely negligible (−0.10 mmol L^{-1} (95% CrI: −0.47, 0.26; P(benefit/negligible/harm) 40/50/10 %)), although the HDI+ROPE decision was undecided.

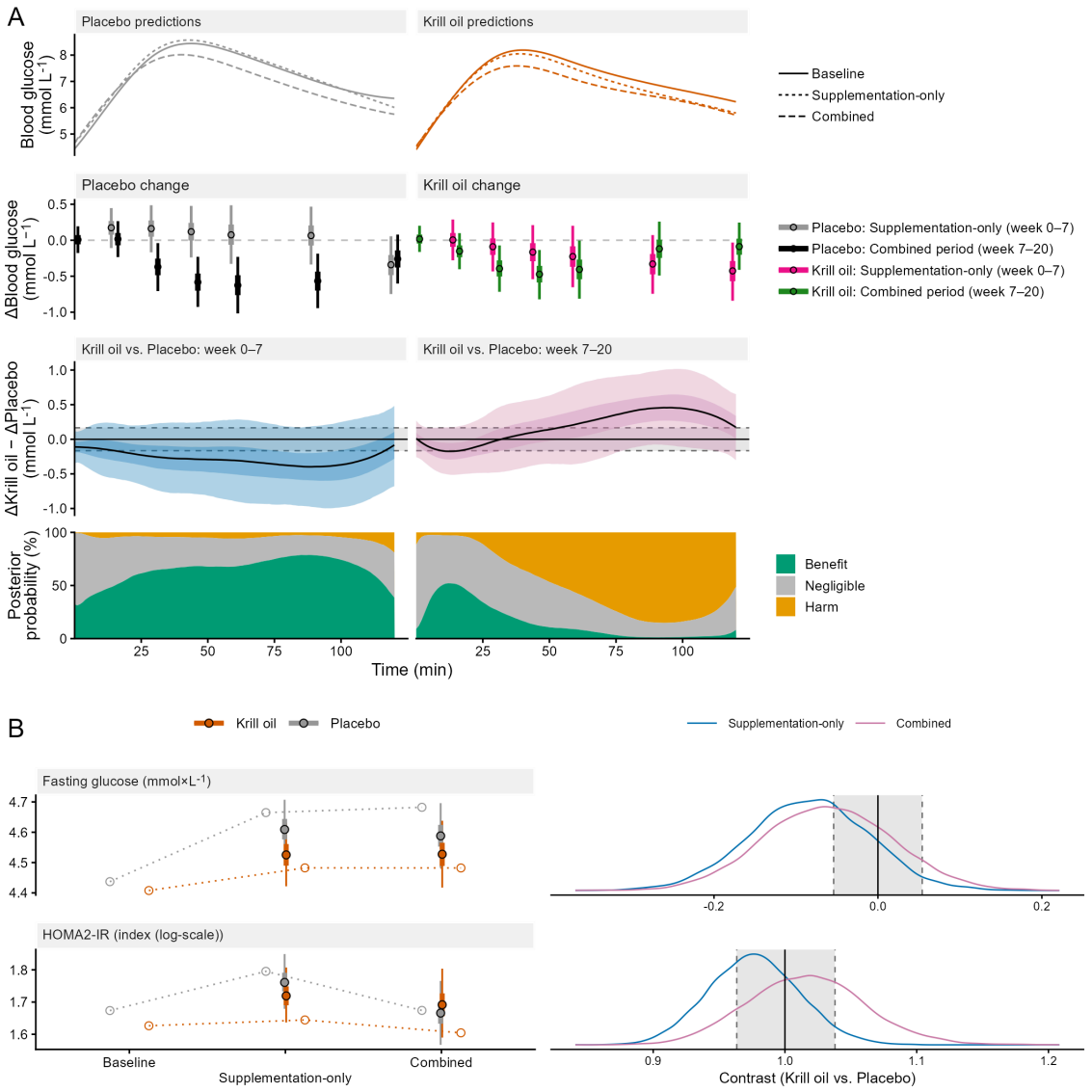

Figure 11.3: Glucose regulation. (A) Blood glucose during the oral glucose tolerance test. First row: model-predicted median glucose curves per group at week 0 (baseline), week 7 (supplementation-only) and week 20 (combined). Second row: within-group change during supplementation alone (week 0 to 7) and during the combined period (week 7 to 20) at the sampling times. Third row: between-group differences in change (krill oil – placebo) for the same periods (left, week 0 to 7; right, week 7 to 20, primary outcome); gray areas indicate ±δ, the region of negligible effects. Fourth row: posterior probabilities of benefit (lower glucose with krill oil), a negligible effect and harm at each minute of the test. Estimates are posterior medians with 50 and 95 % percentile intervals. (B) Fasting glucose and HOMA2-IR at each time point (left panels; observed group means, open circles; model-estimated medians, filled circles with 50 and 95 % percentile intervals), and posterior distributions of the between-group contrasts at week 7 and week 20 (right panels; HOMA2-IR as fold difference). Gray areas in the right panels indicate ±δ, the region of negligible effects.

Krill oil supplementation was associated with, on average, lower fasting glucose at weeks 7 and 20 (week 7, −0.08 mmol L^{-1} (95% CrI: −0.23, 0.06; P(benefit/negligible/harm) 66/31/3 %); week 20, −0.06 mmol L^{-1} (95% CrI: −0.21, 0.09; P(benefit/negligible/harm) 54/39/7 %)), but this difference was uncertain and reflected a smaller increase rather than a decrease, as fasting glucose increased numerically in both groups (Fig. 5B). Krill oil did not affect HOMA2-IR (week 7, −2 % (95% CrI: −9, 5); week 20, +2 % (95% CrI: −7, 11); Fig. 5B), or adiponectin (week 7, −0.11 mg L^{-1} (95% CrI: −1.01, 0.78); week 20, −0.18 mg L^{-1} (95% CrI: −1.08, 0.75); Table 3) but was associated with an increased HbA1c at week 7, resulting in higher probability of harm

(+2 % (95% CrI: 0, 5; P(benefit/negligible/harm) <1/37/63 %)). Differences in HbA1c between treatments were most likely negligible at week 20 (+1 % (95% CrI: −2, 3; P(benefit/negligible/harm) 3/74/23 %)).

11.3.4 Metabolic syndrome

Krill oil lowered the combined burden of metabolic syndrome components compared with placebo, reflecting small and mostly favorable changes across the individual components, most clearly, besides fasting glucose, systolic blood pressure and HDL cholesterol. Among the components, krill oil probably lowered systolic blood pressure during supplementation alone, resulting in a mostly beneficial effect (−3.7 mmHg (95% CrI: −6.7, −0.8; P(benefit/negligible/harm) 90/10/<1 %)), whereas the reduction during the combined period was smaller and less certain (−2.5 mmHg (95% CrI: −6.2, 1.4; P(benefit/negligible/harm) 63/35/2 %)). Krill oil did not affect HDL cholesterol during supplementation alone (−0.01 mmol L^{-1} (95% CrI: −0.06, 0.04; P(benefit/negligible/harm) 2/92/6 %)) but increased it during the combined period (0.05 mmol L^{-1} (95% CrI: 0.00, 0.11; P(benefit/negligible/harm) 60/40/<1 %)). Krill oil resulted in slightly lower diastolic blood pressure in both periods (−1.8 mmHg (95% CrI: −3.8, 0.2; P(benefit/negligible/harm) 78/22/<1 %) and −1.2 mmHg (95% CrI: −3.4, 1.0; P(benefit/negligible/harm) 56/41/2 %)), but these differences were uncertain. Krill oil did not affect waist circumference (week 7, −0.7 cm (95% CrI: −1.8, 0.4; P(benefit/negligible/harm) 13/87/<1 %); week 20, −0.1 cm (95% CrI: −1.5, 1.3; P(benefit/negligible/harm) 4/94/2 %)) or triglycerides (week 7, −1 % (95% CrI: −12, 12; P(benefit/negligible/harm) 21/65/15 %); week 20, −1 % (95% CrI: −13, 14; P(benefit/negligible/harm) 25/56/19 %); Fig. 6A).

When the five components were combined for each participant, krill oil probably reduced the burden score during supplementation alone (−0.08 SD (95% CrI: −0.17, 0.01)) and during the combined period (−0.08 SD (95% CrI: −0.17, 0.02); Fig. 6B; Table 3), with probabilities of any benefit of 96 % and 94 %, respectively. The effect of krill oil was consistent across both periods, however, at week 7, a decreased average blood pressure was most likely the largest contributor to the lower burden score (55 % probability of being the main contributing factor), followed by fasting glucose (39 %), with other factors (waist circumference, HDL and TG) having < 5 % probability of being the main contributor. At week 20, probabilities of being the main contributing factor were spread across fasting glucose (33 %), blood pressure (32 %), and HDL (25 %), with no single dominant contributor (TG, 9 %; Waist circumference, 1 %). The treatment effect did not depend on the burden score at week 0: the reduction was of similar size regardless of baseline metabolic burden (difference in treatment effect when the burden score at baseline increases by 1 SD, week 7, 0.00 SD (95% CrI: −0.01, 0.02); week 20, 0.00 SD (95% CrI: −0.01, 0.02)). Similarly, effects on the individual components did not depend on their own baseline values, the baseline omega-3 index or the baseline relative fat mass (Supplementary Figure 7.4 **[TODO: check the figure number after rendering the book; it is `fig-moderation-pred` in 07-metsyn-inference.qmd]**) and none of the extended models clearly improved predictive performance compared with the main model (ΔELPD, positive values favouring the extended model, −2.0 (SE 1.6), −0.1 (SE 2.1), 2.7 (SE 2.5) and −0.5 (SE 1.2) for interactions with own baseline values, the baseline burden score, baseline relative fat mass and the baseline omega-3 index, respectively).

Despite the lower burden score, krill oil did not credibly reduce the prevalence of the metabolic syndrome. The odds of meeting the criteria were lower with krill oil in both periods, but the uncertainty was large (week 7, OR 0.26 (95% CI: 0.04, 1.58); week 20, OR 0.56 (95% CI: 0.10, 2.99); Fig. 6C–D).

Krill oil did not affect other secondary outcomes, including body mass index (week 7, 0.05 kg m^{-2} (95% CrI: −0.12, 0.22); week 20, 0.09 kg m^{-2} (95% CrI: −0.22, 0.40)) and LDL cholesterol (week 7, −0.04 mmol L^{-1} (95% CrI: −0.20, 0.12); week 20, 0.05 mmol L^{-1} (95% CrI: −0.13, 0.23)) with body mass index and relative fat mass showing high probability of equivalence between groups at week 7 and 20, and week 7, respectively (ROPE: accepted; Table 3).

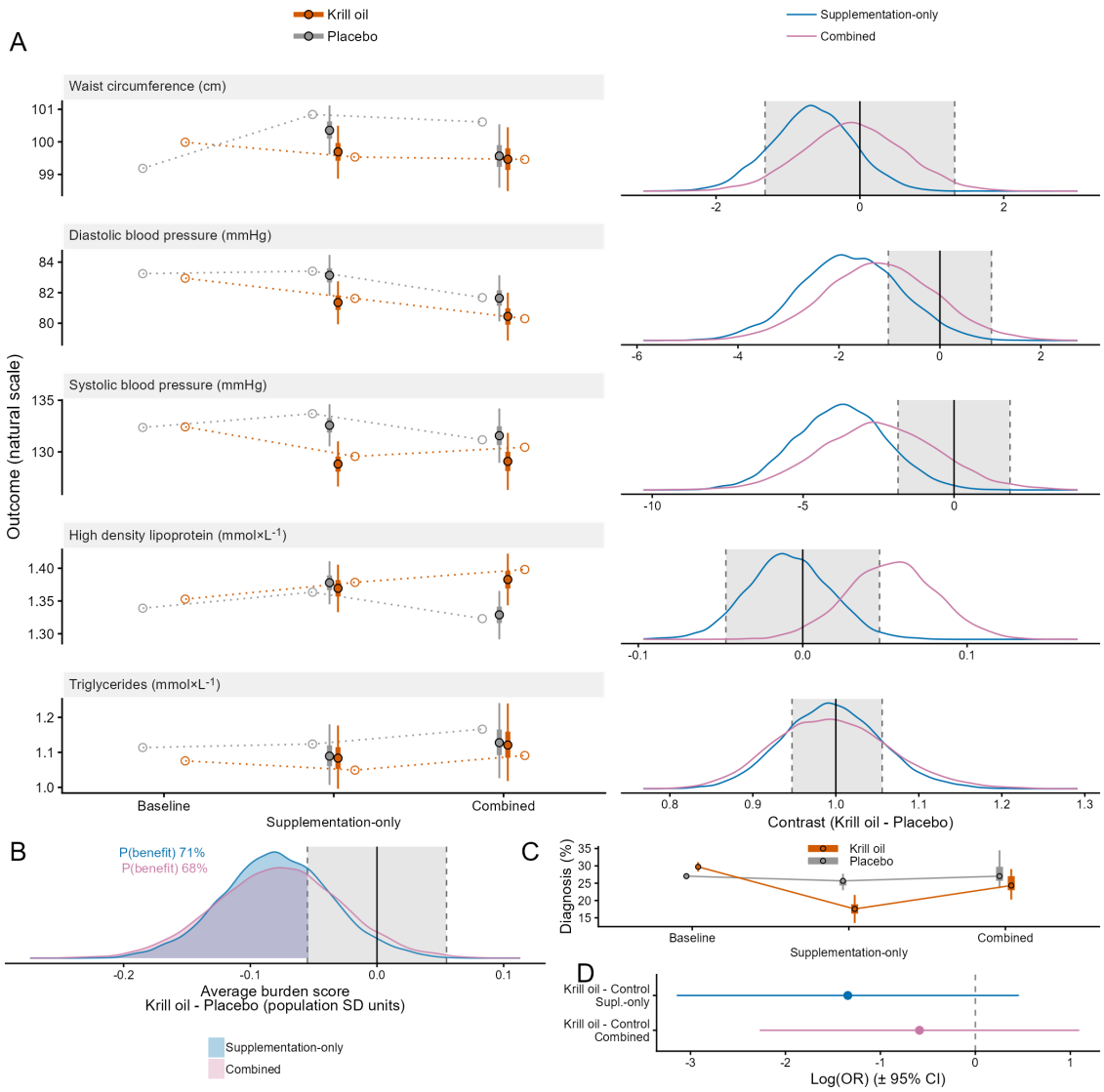

Figure 11.4: Metabolic syndrome outcomes in the krill oil and placebo groups at week 0, week 7 (supplementation-only) and week 20 (combined supplementation and resistance training). (A) Metabolic syndrome components. Left: observed means (open circles) and model estimates (filled points) with 50 and 95 % percentile intervals. Right: posterior distributions of the between-group differences (krill oil – placebo). (B) Difference between krill oil and placebo in burden score (sum of the five standardized components; negative values favor krill oil), shown as posterior distributions with the probability of a benefit. (C) Prevalence of the metabolic syndrome (Albertini 2009). (D) Between-group log odds ratios for the metabolic syndrome, with 95 % confidence intervals. Dark and light shades denote week 7 and week 20. Dashed lines mark no difference.

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